

1 The Health Plan Environment

Lesson Objectives

After completing this lesson, you should be able to:

- Discuss the major factors that shape the environment in which health plans operate and drive change in that environment.
- Identify the four main players in the health plan environment and their interests.
- Identify the other important participants and stakeholders in the environment and give examples of how they influence health plans.
- Briefly state how health plans respond strategically to conditions and changes in the environment.

Introduction: Governance and Regulation

This course in AHIP's series on health insurance plans focuses on governance and regulation.

Governance may be briefly defined as the efforts by the board of directors of an organization (such as a health plan), working with senior management, to create a corporate mission and develop strategies to fulfill that mission. We will use the term **regulation** broadly to refer to laws and regulatory rules, both state and federal, that health plans must comply with or that affect them in some way. Regulation will of course help determine specific actions, decisions, and practices of a health plan. But it will also strongly influence the governance of the plan and its long-term strategy.

In this first lesson of the course, we will focus on the environment in which health plans operate and compete. By "environment" we mean all the external factors that affect a plan's actions and strategy. **External factors** are things that affect a plan that it has little control over, such as the overall economy, technological change, legislation, and others. **Internal factors** are things a plan can control, such as its mission, structure, strategy and tactics, allocation of resources, etc. To be successful, a health plan must contend with its complex environment and keep up with constant change in that environment. In this lesson we will survey the factors and players in this environment.

Factors in the Health Plan Environment

A number of factors shape the environment that health plans operate in and drive change in that environment. They include:

- Legislation and regulation
- Politics
- Litigation and court decisions
- The economy
- The health plan market
- Mergers and acquisitions

- Demographics
- Consumer expectations
- Entrepreneurial and technological innovation

Legislation and Regulation

In the United States, health plans are much more heavily regulated than most industries. For instance, most other types of companies are not required to provide certain minimum services to customers. Few have to maintain a certain level of financial reserves or submit a quality assurance plan with an application to operate in a state.

At both the state and federal level, legislation has been enacted in response to the concerns of consumers and other interested parties and to address practices and changes in the healthcare and health plan fields. This includes narrowly focused bills (such as a mandate that health plans cover a hospital stay of a minimum length for childbirth) and broad legislation addressing numerous problems, such as the federal Health Insurance Portability and Accountability Act (HIPAA) and the Affordable Care Act (ACA). Legislation is implemented by means of rules developed by regulatory agencies.

Laws and regulations restrict and determine the actions and practices of health plans in many areas. To give a few examples, states set minimum amounts of capital a company must have in establishing and maintaining a health plan, and mandates dictate some of the healthcare services a plan must cover. Regulation has costs. Plans must devote substantial resources to ensuring that they are in compliance, and some laws and rules can add to a plan's expenditures on healthcare. This can make it necessary for the plan to raise premiums or modify coverage. However, government policy can also create opportunities for health plans. For instance, legislation and regulatory changes have made it possible for more private-sector plans to serve Medicare and Medicaid beneficiaries. The ACA established health insurance exchanges (Marketplaces) where plans can sell their products.

The Affordable Care Act and its associated regulations have had a major impact on health plans, the products they offer, and the markets they pursue. Some examples:

- Some plans have participated in the Marketplaces.
- Some plans have focused their efforts on non-medical insurance products that are largely not governed by the ACA.¹
- The ACA has also led health plans to streamline their operating models, reshape their distribution strategies, and make significant investments in technology.²

Looking ahead, the impact of legislation and regulation will likely again be felt as lawmakers seek to repeal, replace, and transition from the ACA.

Politics

Of course, legislation and regulation affecting health plans are strongly influenced by the discussions of healthcare that often take place in campaigns and by the results of elections. Some examples:

- The George W. Bush Administration responded to seniors' complaints about the cost of medications by advocating for the creation of Medicare Part D prescription drug plans (offered by private-sector health plans).
- Barack Obama's administration sought to address broader concerns about the cost of health coverage and access to it by supporting the ACA, which has affected health plans in many ways.
- Donald Trump has heard the many people dissatisfied with the ACA and has promised to repeal and replace it.

Litigation and Court Decisions

How legislation and regulations are interpreted by the courts, and the legal precedents that court rulings may establish, also impact health plans. For example, as we will learn in a later lesson, a series of court decisions have shaped to what extent employer self-funded health plans are subject to state insurance laws.

The Economy

The overall state of the economy includes the growth rate, employment, interest rates, spending, production, prices, housing starts, and the money supply. These factors influence various aspects of health plan operations, including:

- The availability of capital
- The demand for healthcare services
- The accessibility of healthcare coverage

A recent example of the impact of the overall economy on healthcare and health coverage: The severe economic downturn that began in December 2007 was accompanied by a marked decrease in the growth rate of healthcare spending (from an annual rate of 6.6 percent in 2000-2007 to 3.3 percent in 2008-2011³). This was not limited to the private sector, but also included reduced Medicare spending.⁴ Later, as the economy recovered and employment rose, access to health coverage increased (since most coverage is still sponsored by employers).⁵

The Health Plan Market

The structure of the market they compete in and changes in the market of course have an impact on health plans. Two examples:

- The health plan market has shifted from mostly local to a mix of local, regional, and national, in part because employers operating in different states or regions often want to work with only one health plan.
- Enrollment has surged since the passage of the ACA, but this has varied by locality and market segment. For example, health plans that have traditionally served poorer populations have generally been more successful in the ACA Marketplaces than those geared toward the more affluent.⁶

Market maturity refers to how much a market has grown and developed. For health plans, this typically means (among other things) how many plans are competing, what products they are

offering, how receptive consumers and employers are, and how willing providers are to participate. For instance, a large metropolitan area might be a mature market, with lively competition among several plans and sophisticated employers, consumers, and providers. In contrast, a rural area might have only one plan and poorly developed provider networks. However, one aspect of a market may be more mature than another. For instance, there may be several competing health plans but provider networks in only the initial stages of development.

Mergers and Acquisitions

Recent years have seen consolidation in the health plan market, as well as in other healthcare sectors such as providers and suppliers.⁷ Health plans have merged with or acquired other plans for two main reasons: to counter provider consolidation and gain greater leverage in negotiating with providers and to reduce administrative expenses in order to meet requirements of the ACA (which may no longer be a factor, depending on what rules may replace the ACA). However, some fear that consolidation will lead to a substantial lessening of competition in the health plan industry, and this raises antitrust issues.⁸

A merger or acquisition can reduce administrative costs, result in operational synergies, and enable a health plan to expand and improve the quality of its provider network.⁹ However, the integration of two formerly separate businesses requires a substantial amount of financial, managerial, and other resources.

Demographics

Characteristics of the U.S. population can create both challenges and opportunities for health plans:

- The population is aging. By 2030 more than one in five Americans will be 65 or older and (under current rules) eligible for Medicare.¹⁰ Private-sector health plans can access this growing market by sponsoring Medicare Advantage plans. (Even with funding cutbacks under the ACA, such plans continue to attract new enrollees.¹¹)
- Concerns about women's healthcare have been addressed by various laws, such as those mandating a minimum hospital stay for childbirth and access to obstetricians or gynecologists without a referral. (Some of these requirements are part of the ACA, but many are likely to be retained in a post-ACA environment.)

Some health plans target certain segments of the population.

- Some seek to make it easier for those who do not speak English (or are not native speakers) to obtain care and service.
- Some ethnic groups have a higher incidence of certain diseases, and health plans have responded with disease management programs.
- Some organizations target more affluent consumers with plans that cost more but offer a greater choice of providers.

Consumer Expectations

Today's consumers have growing and changing expectations for health plans, including the following:

- More health coverage options (including more options for employees in employer-sponsored plans)
- More information and tools to make informed choices among options
- Coverage for more and different types of treatment (experimental treatments, alternative medicine, etc.)
- Direct access (without a referral) to specialists

Consumers also have changing expectations for healthcare:

- Better quality of care and more information about the quality of providers
- Higher value for the money spent on health care
- Convenience in the delivery of healthcare
- Ease in navigating the health care system
- Free and open exchange about healthcare treatment options between physicians and other medical personnel and the consumer

Three trends are driving these expectations:

- Increasing knowledge and awareness of choices (driven in part by technology such as social media)
- Increasing expectations for value and cost transparency
- Increasing desire for a collaborative provider-patient relationship¹²

Innovation

Entrepreneurs can claim credit for many of the innovations in health plans today. By identifying a market need and finding a new way to meet that need, entrepreneurs have established new standard industry practices and developed alternative methods for financing and providing healthcare. For example, entrepreneurs created the concept of physician-hospital organizations, physician practice management companies, and disease management firms, as well as strategic alliances between health plans (all to be discussed in this course).

In recent decades, technological advances have been a strong driver of change in health plans as in other fields. The adoption of electronic medical records (EMRs) has improved efficiency. The capability to analyze large volumes of data has improved care in new ways. On the other hand, technology can, of course, be disruptive of existing markets and companies.

Clinical innovation has also been a factor. Some examples are the application of gene research to treating diseases, breakthroughs in the early detection and treatment of certain forms of cancer, new medical devices and new drugs, and the application of outcomes research and evidenced-based medicine.

The Major Players

Another very important factor in the health plan environment is the participants in the marketplace. These are major players:

- **Consumers** use healthcare and health plan products and services.
- **Purchasers** buy health coverage. They are primarily employers, government programs, and individual consumers.
- **Providers** deliver healthcare services. They include physicians, hospitals, and other healthcare professionals and facilities.
- **Payors** pay for healthcare. They include health plans, insurance companies, government programs, and self-funding (self-insuring) employers (which pay health benefits for employees out of their own funds instead of buying coverage).

These players have diverse and sometimes conflicting interests. A few examples:

- Payors and purchasers want to hold down the cost of care, but consumers are primarily concerned with easy access to high-quality care, regardless of cost.
- Physicians often want unlimited authority to make decisions on care, while health plans seek to ensure that the best decisions (in terms of both quality and cost-effectiveness) are made, through utilization management and quality assurance systems.

Consumers

We have already discussed how consumer expectations help shape the health plan environment. Some examples of how changing consumer ideas about healthcare and health coverage have affected health plans:

- In the past most consumers hesitated to question the diagnosis or treatment prescribed by a physician, but today people are taking a more active role in their healthcare.
- As consumers have faced higher deductibles and cost-sharing, they have pushed for greater transparency in how prices are set.¹³
- Consumers have demanded a greater choice of providers, which has led some health plans to modify their designs (for example, to allow members to go to a non-network provider but pay higher cost-sharing).
- The increased interest in alternative medicine has led some health plans to offer coverage for it.

The preferences of consumers affect employers' decisions in buying group health plans, as the benefit package can be important in attracting qualified employees in an environment of low unemployment. As voters, consumers also affect legislation. Their influence can be seen in the passage the ACA, the reaction against it, and the likelihood that certain features of it will be retained (such as the right to obtain coverage without individual medical underwriting and the ability of young adults to continue coverage under their parents' health plan).

Purchasers

Large purchasers such as employers and government programs have exerted tremendous influence on the products and services offered by health plans. Purchaser initiatives to ensure quality and value have changed the ways plans operate.

- Largely as a result of employers' focus on quality, health plans have sought accreditation.
- To prove that employers' dollars are being well spent, health plans have devoted time and money to outcomes research and other quality-ensuring initiatives.
- Employers' efforts to curb the costs of coverage, such as onsite clinics and wellness programs for employees, have caused health plans to innovate and expand their product and service offerings.

Providers

A health plan's network of providers can contribute to the plan's success in various ways:

- The care providers deliver is a major factor in member satisfaction.
- Providers usually have great influence with members who are their patients.
- Providers may have the clinical expertise and data that health plans need to demonstrate to purchasers their commitment to quality.
- Plans that maintain strong and positive locally based relationships with providers, based on the exchange of mutual expertise, are more likely to prosper.

Many providers are concerned about the continued growth and consolidation of health plans and how this may affect their compensation and professional independence. Because of such concerns, providers often join together in organizations that health plans must negotiate with and contract with in order to obtain healthcare services. Alternatively, providers may establish organizations that contract directly with purchasers, bypassing and competing with health plans.

Payors

Payors have sought to improve both the quality of the care they pay for and the value they get for what they pay. Payors are increasingly seeking provider accountability. In general, this means that, in return for compensation, providers do not just perform certain services but produce positive health outcomes. New compensation methods, such as bundled payments and value-based payment models (to be discussed in this course), are one way to do this.¹⁴

Payors can also contribute to improving quality. While providers have records for their individual patients, relatively few have timely access to the large amount of healthcare data now available. Some payors have this data, as well as extensive experience with risk and population analytics, and they can help providers improve clinical outcomes.

Other Stakeholders

In addition to these four main players, there are other participants and stakeholders in the health plan environment:

- The community in which a health plan operates
- Uninsured or underserved populations
- Vendors
- Academic medical centers
- Patient advocacy groups
- Federal and state governments

The Community

Health plans, even those that are regional or national in scope, serve local communities. Taking an active role in the community, tailoring products and services to the local population, and partnering with community organizations can have many benefits for a plan. It can lead to increased enrollment, facilitate expansion, and improve the health of members. A health plan can work in the community in various ways: teaming up with public health organizations in health promotion activities, joining forces with academic medical centers to perform education and research, and sponsoring community health projects such as childhood immunization initiatives and health fairs.

Some health plans partner with community public health agencies to educate members about certain matters, such as the dangers of substance abuse or obesity. Often the agencies are already engaged in this work, and efforts can be tailored and targeted to the plan's members.

Not-for-profit health plans have special obligations to the community. This is typically mandated by their articles of incorporation and mission statements and often by state law. Community representation on the board of directors may be required. A not-for-profit health plan may be required to make membership available to individuals and small employers; make services available to low-income, high-risk, medically underserved, and elderly populations; or use community rating to set premiums.

The Uninsured and Underserved Populations

Healthcare differs from other industries in an important way: While other products and services are generally available only to those who are able to pay for them, in general, Americans expect healthcare to be available to everyone, whenever it is needed and in whatever quantity is needed. This leads to public policy seeking to ensure the provision of healthcare services to populations that are unable to pay the market price or (sometimes) much at all. We'll look at the uninsured (who have no coverage to pay for healthcare) and the underserved (who may or may not have coverage but have limited access to healthcare services).

Lack of health coverage has a high human cost—individuals may suffer or even die needlessly because they did not receive adequate care. It also has a large monetary cost, because it leads to inefficient use of healthcare services. For instance, treatment that should have been delivered by a

primary care provider is received in an emergency room, or a condition that could have been detected early and easily treated becomes severe and requires extensive and expensive treatment.

Over the years, legislation has been enacted with the aim of reducing the number of uninsureds. At the federal level, this was a key reason for the Children's Health Insurance Program (CHIP) and the Affordable Care Act.¹⁵ State governments have also launched initiatives, with more or less success. As we move into the post-ACA era, this will continue to be an issue. Legislation designed to extend coverage to the uninsured can sometimes increase a health plan's costs of providing coverage to its members, making premium increases necessary. Health plans need to work with policy experts and advocates to devise solutions that benefit everyone.

Residents of rural communities often lack access to healthcare. There may be few physicians and specialists and no hospital. Health plans may not enter the market because there are not enough potential members or because of resistance from local providers. In urban areas often very low-income people have Medicaid coverage, but there is a lack of providers willing to serve this population. Transportation to providers can be a problem for those without cars. Government agencies try to address these problems, but health plans also play a part. A number of preferred provider organizations (PPOs) operate in rural markets. Some plans are able to serve employers that operate in sparsely settled areas but over a large region, providing enough enrollees to be viable.

Vendors

Organizations that provide services to health plans are called vendors. Typically, they have expertise and capabilities that a health plan has not developed, or they can provide services more cost-effectively than the plan could. Functions commonly outsourced to vendors include administrative services (such as claims or billing), call centers for customer service, information technology, pharmacy benefits, and some clinical services (such as behavioral healthcare, radiology, chiropractic, oncology, etc.).

We've mentioned that some employers self-fund (self-insure)—that is, instead of buying coverage for employees from a health plan, they provide healthcare benefits out of their own resources. Employers that self-fund use vendors to perform many functions, and often a vendor is a health plan. That is, a health plan may not provide coverage to an employer but instead (for instance) handle the employer's claims operations. However, as more specialized firms develop, they may increasingly take this function over from health plans.

Academic Medical Centers

Academic medical centers (AMCs) (also called academic health centers) train healthcare professionals and perform clinical and healthcare-related research. They include medical schools and other professional schools such as nursing and dental programs. **Teaching hospitals**, usually part of AMCs, sponsor physician residency programs.

The environmental pressures that have caused dramatic change in the health plan industry are also impacting AMCs.¹⁶ In particular, AMCs have been subsidized by fee-for-service Medicare payments, and as Medicare moves from fee-for-service to other compensation methods, this will be reduced. In response, AMCs are changing. Some have merged with other teaching institutions or contracted with non-teaching hospitals to become participants in integrated delivery systems (IDSs).¹⁷ Some have purchased physician practices to gain access to patients. Some AMCs have contracted with health plans to provide care to members. They can bring unique capabilities and resources to this relationship, including expertise in education and research (especially in medical outcomes and evidence-based medicine). On the other hand, AMCs often lack primary care providers.

Patient Advocacy Groups

Patient advocacy groups seek to represent the interests of patients generally or people with certain illnesses. They often promote legislation designed to further these interests. For instance, an advocacy group might lobby for a law requiring health plans to pay benefits for certain drugs.¹⁸ In the past the relationship between health plans and advocacy groups was often adversarial. They often interacted only when a plan denied coverage of a treatment requested by a person represented by a group. Today, some health plans are collaborating with advocacy groups. For instance, some groups provide psychosocial or holistic care in partnership with a health plan. This can help a plan win the good will of patients and sometimes lower the long-term cost of certain treatments by providing care that more closely monitors and treats a patient's condition and overall health.

Federal and State Governments

As we will describe throughout this course, federal and state governments exert tremendous influence over a health plan's formation, operations, and governance. Governments are stakeholders in two main ways:

- They enact and enforce laws and regulations to protect and promote the interests of consumers and other stakeholders.
- They sponsor health coverage for government employees. They pay for healthcare services for beneficiaries of Medicare, Medicaid, and other healthcare benefit programs, and they sometimes purchase private-sector coverage for these people. As such, they have similar interests as other purchasers and payors.

Strategic Responses

In response to the environmental factors we have discussed in this lesson, health plans make certain specific decisions. But they also set long-term strategy in order to prosper in their environment and keep up with change in it. Later in this course we will look in depth at governance and strategy. But we offer here a brief overview.

Most health plans develop a **mission statement**, which succinctly sums up the organization's reason for existence and overall purpose. Health plan mission statements typically center on defining and meeting (in an ethical and cost-effective manner) the needs of customers and the community. To fulfill their missions, health plans develop strategic plans, which include long-term objectives and strategies for meeting them.

Occasionally, change in a health plan's environment makes it necessary for the plan to restructure or reorganize its operations to remain competitive, become more competitive, or enter new markets. A **corporate reorganization or restructuring** is the process of adjusting the internal structure of an organization by changing reporting relationships; adding, eliminating, or changing the responsibilities of functional departments; moving from a centralized to a decentralized structure (or vice versa); or creating a new subsidiary or holding company.

Other ways that health plans respond to change include leadership changes, converting from a not-for-profit or mutual organization to a for-profit organization, going public, selling portions of the business, acquiring new or existing companies, entering new business ventures, getting out of existing business ventures, or making major changes in plan or product offerings. We will discuss these responses during this course.

Summary

A variety of factors shape the environment that health plans operate and compete in and drive change. These include legislation and regulation (influenced by politics and interpreted by the courts), the overall economy, the health plan market (including market maturity and consolidation), characteristics of the population and consumer expectations, and innovation of various sorts.

The major players in this environment are consumers, purchasers (employers and government programs that buy health coverage), providers of healthcare services, and payors (health plans, government, and others who pay providers). Other participants and stakeholders include local communities, uninsured or underserved populations, vendors that provide services to plans, academic medical centers, patient advocacy groups, and the federal and state governments.

Sources

¹ Marie Carr, Mike Mariani, Christopher Coakley, Jim Quick, and Juneen Belknap “Top Issues: An Annual Report – Group Insurance in Flux,” PWC, Vol. 8, 2016, pp. 52-56,

<http://www.pwc.com/us/en/insurance/publications/assets/pwc-top-issues-the-insurance-industry-2016.pdf>

² Ibid.

³ David Dranove, Craig Garthwaite, and Christopher Ody, “Health Spending Slowdown Is Due Mostly to Economic Factors, Not Structural Change in the Health Care Sector,” *Health Affairs* 33: 8 (2014), pp. 1399-1406, <http://content.healthaffairs.org/content/33/8/1399.full.pdf+html>

⁴ David Dranove, Craig Garthwaite, and Christopher Ody, “The Economic Downturn and Its Lingering Effects Reduced Medicare Spending Growth by \$4 Billion in 2009-2012,” *Health Affairs* 34: 8 (2015), pp. 1368-1375, <http://content.healthaffairs.org/content/34/8/1368.full.pdf+html>

⁵ The Henry J. Kaiser Family Foundation, “2015 Employer Health Benefits Survey,” Sept. 22, 2015, <http://kff.org/health-costs/report/2015-employer-health-benefits-survey/>

⁶ Robert Laszewski, “Why Are Centene and Molina Making Money on the Obamacare Exchanges?” *Forbes*, May 6, 2016, <http://www.forbes.com/sites/robertlaszewski2/2016/05/06/why-are-centene-and-molina-making-money-on-the-obamacare-exchanges/#2310e2d546fb>

⁷ Bara Vaida and Alexander Wess, “Health Care Consolidation,” Alliance for Health Reform, Nov. 2015, http://www.allhealth.org/publications/Consolidation-Toolkit_169.pdf

⁸ Peter Newell and Nikhita Thaper, “The Next Wave: The Impact of Two New Proposed Mergers on New York’s Health Insurance Market” (Special Report), United Hospital Fund, March 2016.

⁹ Paul von Ebers, “Mega Health Insurance Mergers: Is Bigger Really Better?” *Health Affairs Blog*, January 22, 2016, <http://healthaffairs.org/blog/2016/01/22/mega-health-insurance-mergers-is-bigger-really-better/>

¹⁰ Alan R. Weil, “Aging and Health,” *Health Affairs* 34:1 (2015), p. 6. <http://content.healthaffairs.org/content/34/1/6.full.pdf+html>

¹¹ Gretchen A. Jacobson, Patricia Neuman, and Anthony Damico, “At Least Half of New Medicare Advantage Enrollees Had Switched from Traditional Medicare During 2006-11,” *Health Affairs* 34:1 (2015), pp. 48-55, <http://content.healthaffairs.org/content/34/1/48.full.pdf+html>

¹² Alison Kenny Paul, Susan Hogan, “The Tail Wagging the Dog: How Retailers Is Changing Consumer Expectations of the Health Care Patient-Provider Relationship” (Issue Brief), Deloitte Center for Health Solutions, Oct. 2014, <https://www2.deloitte.com/tr/en/pages/life-sciences-and-healthcare/articles/how-retail-is-changing-consumer-expectations-of-the-health-care-patient-provider->

¹³ Brad Hill, “Bundled Pricing Programs Create Opportunities to Deliver Quality Services and Provide Transparency to Patients,” Advance Healthcare Network for Health Information Professionals, Nov. 14, 2016, <http://health-information.advanceweb.com/Features/Articles/Healthcare-Consumerism-and-Payer-Provider-Relationships.aspx>

¹⁴ Larry Schor, "Healthcare's Quest for Greater Payer-Provider Synergy," *Managed Healthcare Executive*, Nov. 13, 2015, <http://managedhealthcareexecutive.modernmedicine.com/managed-healthcare-executive/news/healthcare-s-quest-greater-payer-provider-synergy>

¹⁵ Healthcare.gov, "CHIP," <https://www.healthcare.gov/medicaid-chip/childrens-health-insurance-program/>

¹⁶ Atul Grover, Peter L. Slavin, and Peters Wilson, "The Economics of Academic Medical Centers," *New England Journal of Medicine*, June 19, 2014, <http://www.nejm.org/doi/full/10.1056/NEJMp1403609#t=article>

¹⁷ Mitchell Morris, "Academic Medical Centers: Joining Forces with Community Providers for Broad Benefits and Positive Outcomes," Deloitte, <https://www2.deloitte.com/us/en/pages/life-sciences-and-health-care/articles/academic-medical-centers-consolidation.html>

¹⁸ Katie Thomas, "Furor Over Drug Prices Puts Patient Advocacy Groups in Bind," *New York Times*, Sept. 27, 2016, http://www.nytimes.com/2016/09/28/business/furor-over-drug-prices-puts-patient-advocacy-groups-in-bind.html?_r=0

2 The Legal Organization of Health Plans

Lesson Objectives

After completing this lesson, you should be able to:

- Compare and contrast sole proprietorships, general and limited partnerships, and corporations.
- Identify the special features of limited liability companies and professional corporations.
- Compare and contrast for-profit stock companies, mutual insurers, and not-for-profit health plans.
- Explain the requirements and advantages of tax-exempt status for not-for-profit health plans.
- Identify and compare the main sources of health plan funding.

Determining a health plan's legal form is one of the fundamental issues that must be addressed by a plan's leaders. In this lesson we begin with an overview of the basic forms of legal organization available to health plans, with a focus on corporations, the predominant form. We then examine the for-profit health plan, the not-for-profit health plan, and the mutual insurance company. In this discussion we examine issues such as legal and formal requirements, owners' liability, tax treatment, long-term stability, allocation of profits, and options for raising funds.

Forms of Legal Organization

In the United States today, business entities can be established in a number of legal forms. Many healthcare providers and other vendors with whom health plans do business are sole proprietorships, partnerships, or professional corporations. But health plans are typically established as corporations. While not always required, this legal form affords owners, directors, and executives the greatest protection from individual liability. And particularly in the case of for-profit health plans, the corporation provides the greatest flexibility for raising operating funds.

In some states an entity responsible for financing healthcare, such as an insurance company, is required by law to be organized as a corporation. However, in many states a health plan, such as an HMO, is *not required* to be a corporation and can, for instance, be structured as a limited liability company. We will see later in this lesson that federal law allows certain types of provider-owned organizations, which are not all corporations, to operate as health plans in the Medicare and Medicaid markets.

There are three basic forms of legal organization that are helpful to understand in conducting business in the health care environment:

- Sole proprietorships
- Partnerships
- Corporations

There are also other forms of business that you are likely to encounter, particularly in dealing with providers. These include:

- Limited liability companies
- Professional corporations

Sole Proprietorships and Partnerships

A **sole proprietorship** is a business owned by one person.

- All the debts of the business are the debts of the owner.
- Business profits or losses are treated as individual income for tax purposes.
- Although the sole proprietor can leave the assets and liabilities of the business to someone else, continuing the operation of a sole proprietorship after the owner's death is often difficult.

A **partnership** is a business that has many of the same legal characteristics as a sole proprietorship but is owned by two or more persons. The partners can be individuals or legal entities, such as corporations. A partnership can be established by oral agreement or through a formal partnership agreement. All states have laws that spell out the requirements for establishing a partnership. A partnership can be a general partnership, a limited partnership, or a registered limited liability partnership (LLP).

A **general partnership** is owned by two or more **general partners**.

- The partners share responsibility for business operations.
- General partners, when they are individuals, are responsible for the debts of the whole business.
- Partners declare the appropriate share of their partnership income or losses on their individual tax returns. The partnership itself does not pay federal income tax, although it is required to file a return for information purposes.
- A general partnership ends with the death or withdrawal of any general partner; the remaining partners must form a new partnership if they want to continue the business.

Partners can limit the liability of certain owner-investors by forming a **limited partnership**. A limited partnership consists of **limited partners** and at least one **general partner**.

- Limited partners cannot participate in the day-to-day management of the organization.
- Limited partners are at personal financial risk only for the amount of their investment in the business; the general partners, on the other hand, have full liability, similar to the liability of sole proprietors.

- Typically, limited partners can enter into and opt out of a limited partnership with much greater freedom than can general partners. This is an advantage for raising operating funds because the partnership can more easily attract limited partners if it needs additional investors.
- When the partners in a limited partnership are individuals, the profits and losses of the business are treated as individual income, and the partners each declare the appropriate share of their partnership income on their individual tax returns. The partnership itself does not pay federal or state income tax. However, if the structure of the limited partnership includes more corporate than partnership characteristics, it may be taxed as a corporation.
- Generally, a limited partnership continues upon the death or withdrawal of a limited partner, but it ends with the death or withdrawal of a general partner. However, the partnership agreement can place restrictions on a limited partner's right to withdraw, or can allow for the partnership to continue beyond the withdrawal of a general partner.

A **registered limited liability partnership (LLP)** is permitted by enabling statutes in some states. An LLP agreement eliminates a partner's personal liability for debts and obligations that result from acts committed by another partner or a representative of the partnership while conducting partnership business. A registered limited liability partnership is similar to a general partnership in most other respects.

Corporations

A **corporation**, as defined in 1819 by U.S. Supreme Court Chief Justice John Marshall, is "an artificial being, invisible, intangible, and existing only in contemplation of the law." The owners, directors, and officers of a corporation, unlike the owner of a sole proprietorship or general partners, are not individually liable for the debts of the organization. Moreover, a corporation continues beyond the death of any or all of its owners; the individual investors in a for-profit corporation may change continually, but the entity itself remains intact.

Unlike sole proprietorships and partnerships, many corporations are taxed twice.

- First, the corporation pays a corporate tax before it distributes earnings to its owner-investors.
- Then the owner-investors pay individual income tax on the distributed earnings.

However, not all corporations are subject to this double taxation. For example, some smaller businesses can organize as a **subchapter S corporation**, which is not subject to corporate taxes. To qualify as a subchapter S corporation, an entity must file a formal election with the Internal Revenue Service and must meet other criteria, including requirements limiting the number of stockholders.

Incorporating in a State

When a health plan incorporates in a state, the plan is referred to as a **domestic corporation** of that state, and the state is referred to as the plan's **state of domicile**. A corporation is subject to the applicable laws and regulations of its state of domicile and the jurisdiction of its courts. The corporation must register the name and address of its registered agent for service of process (the act of delivering to a party written notice that a lawsuit or other legal action has been filed against that party).

A corporation can usually conduct business outside its state of domicile, where it is referred to as an out-of-state or **foreign corporation**. Foreign corporations may also be subject to the regulatory requirements of the states in which they do business and to the jurisdiction of their courts. In some states, such as Florida¹ and New York,² all HMOs must be domestic entities. An HMO that is domiciled (incorporated) in another state must incorporate a separate subsidiary in these states. (For instance, an HMO domiciled in Pennsylvania that wants to do business in New York must incorporate a subsidiary in New York.) Some states provide favorable tax treatment to domestic corporations, so a health plan doing business in such a state may wish to incorporate a subsidiary as a domestic corporation in the state.

Liability

A corporation—apart from its owners, directors, or officers—can be considered a “person” under the law and can be a party in a legal action. It can sue and be sued. Any legal actions involving a corporation, however, affect only the assets and liabilities of the corporation. If a corporation goes bankrupt, the organization’s creditors must be satisfied with the assets of the corporation only. The creditors cannot legally require the owners, directors, or officers of the corporation to pay the organization’s debt out of their own individual property. This feature of limited liability is important to potential owners, directors, or officers, especially if a company has to incur large debts in the course of doing business.

However, on rare occasions individuals can be held liable for the debts of a corporation. A court that makes such a determination is said to be **piercing the corporate veil**. Generally, this can happen if:

- The owners, directors, or officers exhibit so much control over the operations of the organization that a court finds it difficult to distinguish between the interests and activities of the company and the interests and activities of the individuals involved.
- These individuals display a blatant disregard for corporate formalities and largely ignore the corporate structure in carrying out the business of the corporation.

For a case in which a plaintiff attempted to pierce the corporate veil and ultimately succeeded, see below. However, keep in mind that courts most often decline to pierce the corporate veil, especially in cases involving regulated industries such as insurance.³

Piercing the Corporate Veil

In *Thrift Drug, Inc. v. Universal Prescription Administrators*, a federal appeals court reversed a trial court's decision and held that a pharmacy chain may not have been entitled to pierce a pharmacy plan administrator's corporate veil in an attempt to recoup unpaid prescription claims. However, in further litigation, the pharmacy chain was able to pierce the veil.

The defendant, Universal Prescription Administrators (UPA), was a pharmacy plan administrator that administered prescription benefit plans, and in doing so entered into agreements with “panel pharmacies” such as Thrift Drug, Inc. (Plaintiff or Thrift Drug). Under these agreements, eligible beneficiaries of ERISA welfare plans would have their prescriptions filled at panel pharmacies, which would then submit the prescription claims to UPA for reimbursement.

As a result of cash flow problems UPA failed to reimburse Thrift Drug for a number of prescriptions.

Thrift Drug sued UPA for breach of contract.

The trial court ruled in favor of Thrift Drug, determining that UPA was liable to the panel pharmacy and awarding a judgment of nearly \$60,000. After a subsequent trial, the court ruled that under state law, Thrift Drug could pierce UPA's corporate veil to recover its judgment from the assets of the administrator's sole director and stockholder. In determining that the panel pharmacy could pierce the corporate veil, the court stated that "where the corporation in fact is run by one person regardless of any corporate form, that person has to be responsible for whatever debt the corporations have."

On appeal, the reviewing court observed that in order to pierce the corporate veil: (1) the owner must have exercised complete domination over the corporation with respect to the transaction at issue; and (2) such domination was used to commit a fraud or wrong that injured the party seeking to pierce the corporate veil. The court concluded that the first prong of the corporate veil test was clearly satisfied. However, the court noted that the lower court made no findings as to whether the executive's domination of the company was used to commit a fraud or wrong against the panel pharmacy that resulted in the pharmacy's injury, as also required under the corporate veil test. For this reason, the appellate court set aside the judgment in favor of Thrift Drug and sent the case back to the trial court with instructions for that court to determine whether the second prong of the two-prong test was satisfied.⁴

On remand (i.e., when the trial court took another look at the case as directed by the appellate court), the trial court specifically found that the executive's domination of UPA was used to commit a fraud against Thrift Drug, resulting in Plaintiff's injury. In so concluding, the trial court relied on the facts that the executive (1) authorized UPA to loan millions of dollars to at least six other corporations that he owned, which loans were never repaid; (2) considered UPA's assets to be available for personal use, readily withdrawing funds for his own use without any indication that he was obligated to repay UPA; and (3) allowed members of his family to live rent free in a house owned by UPA. Accordingly, the district court ruled, "[P]laintiff may pierce UPA's corporate veil and hold [the executive] personally liable for UPA's debt to it"⁵

Long-Term Stability

Another advantage of the corporation: It provide stability that many other legal forms cannot guarantee, and a well-managed corporation can survive the personal misfortunes of any of its owners, directors, or officers. This fact is reassuring to the corporation's customers and, in the case of a for-profit health plan, its investors.

Limited Liability Companies (LLCs)

The **limited liability company (LLC)** combines features of the partnership and the corporation. The owners, who can be either individuals or companies, are called members.

- As in a partnership, when an LLC's members are individuals, the business profits and losses are treated as individual income for federal and state tax purposes. Unlike in most corporations, an LLC's profits are not taxed at the entity level before being distributed to members.

- Similarly to a corporation, the LLC limits the personal liability of its owners for the business's debts and responsibilities.
- Like a corporation, an LLC makes available several options for raising funds. But unlike a corporation, an LLC does not have shareholder investors and does not issue stock.

This combination of limited liability, favorable taxation, and means of raising funds have made LLCs popular.

An LLC is usually established by filing a legal document, typically called **articles of organization**, with the appropriate state agency. Typically, the LLC also draws up a document, called an **operating agreement**, which is similar to a partnership agreement or corporate bylaws. It provides more detailed information about the life of the LLC and governance terms, such as voting rights, economic rights, and the steps to be taken if a member resigns.

LLC statutes provide for a great deal of flexibility in organizational matters, making the LLC an attractive legal entity for attaining certain business objectives. The LLC members may manage the organization themselves (a **member-managed LLC**). The members may appoint one or more managers to run the organization (**manager-managed LLC**).⁶

Some provider groups are formed as LLCs. Although it is not common, a health plan can be formed as an LLC. A plan may also enter a specific business venture by forming an LLC with strategic partners.

***Examples:** An insurer and a healthcare management company, both corporations, decide to launch a commercial health plan venture as an LLC. The LLC may benefit from the financial stability and administrative expertise of the insurer and the medical management know-how of the healthcare management company.*

A health plan decides to expand into the Medicaid market. It seeks a partner with familiarity in this market to jointly establish an LLC.

Professional Corporations

For years, practitioners of the “learned professions,” such as law and medicine, were not permitted to establish corporations. In the early 1960s, however, states began enacting laws to permit and regulate **professional corporations**. As a result, physicians and, in some states, other healthcare practitioners (such as registered nurses, physical therapists, chiropractors, dentists, and pharmacists) began establishing professional corporations and/or professional limited liability companies. Professional corporations are generally formed under and governed by the provisions of a state’s general corporation laws, like other corporations, as well as the provisions of the state’s professional corporation law.

Like other corporations, a professional corporation offers limited liability, centralized management, continuity, and generally a free transferability of interests. However, professional corporations differ from business corporations in three main ways:⁷

- **Ownership.** The shares of stock of a professional corporation can only be owned by individuals (and in some states, partnerships or other professional corporations) licensed to practice that corporation’s profession. Some states allow the owners to be licensed practitioners in different areas of medicine. Any sale or transfer of shares must be to a person or entity meeting this

requirement. Also, if a shareholder dies or is no longer a qualified shareholder, the stock must be transferred to a qualified shareholder or it must be purchased by the corporation within a specified period of time.

- **Directors and officers.** In most states all directors and officers must be licensed to practice the corporation's profession. In states where nonprofessional directors are permitted, laws typically prohibit them from exercising authority over professional matters.
- **Liability.** In a business corporation, the liability of individual stockholders is limited in virtually all circumstances. But in a professional corporation (in most states), stockholders are individually liable for improper acts they perform or that are performed under their supervision if the act is related to their profession. If the act is not related to their profession, they are not liable.

***Example:** A professional corporation is sued about its purchase of office equipment; the physician involved is not personally liable. But later the corporation is sued about a medical service; the physician who performed or supervised the service can be held individually liable.*

Note that a stockholder's liability risk can be insured against.

The Corporate Practice of Medicine Doctrine

Many jurisdictions have in place, either by statute or court decisions, a prohibition against the **corporate practice of medicine (CPOM)**. The CPOM doctrine generally prohibits a business corporation from practicing medicine or employing a physician to provide professional medical services. This doctrine arose largely out of the fear that business organizations would unduly influence the professional judgment of physicians.

The CPOM doctrine manifests itself in a variety of state laws, regulations, and court opinions addressing ownership or control of healthcare providers by individuals or corporations that cannot directly provide healthcare services.

- Some states merely prohibit the practice of medicine without a license or the sharing of fees between licensed and unlicensed individuals.
- Other states flatly prohibit the ownership of medical practices or employment of professionals by nonprofessionals.

But even in the most restrictive CPOM states, there are a number of exceptions to the rule. For example, in New York, where the CPOM doctrine is well-established, a medical school may hire physicians and treat patients as part of its mission to promote medical science and instruction.

The states that follow the CPOM doctrine are: Arizona, Arkansas, California, Colorado, Georgia, Illinois, Indiana, Iowa, Kansas, Kentucky, Maryland, Massachusetts, Michigan, Minnesota, Montana, Nevada, New Jersey, New York, North Carolina, North Dakota, Ohio, Oregon, Pennsylvania, South Carolina, South Dakota, Tennessee, Texas, Washington, West Virginia, and Wisconsin.⁸

Since corporate entities that are neither licensed nor owned by licensed professionals are unable to practice medicine or employ a physician to provide professional medical services, insurers and health plans that contract with providers must ensure that such arrangements are established so as not to violate corporate practice of medicine laws. Granting a business corporation authority over the non-professional business affairs of the practice, while at the same time allowing the physician to treat patients without infringing on his professional judgment, should strike the right balance of control from a regulatory perspective.

Organizational Choices

Those setting up a health plan must decide:

- whether the organization will be a **for-profit or not-for-profit** company and
- (for an insurer) whether it will be a **stock or mutual** company.

Profit can be defined as an organization's total income (the amount of money it brings in) minus its total expenses (the amount of money it spends). In this sense, it is possible for a not-for-profit company or a mutual insurance company to make a "profit." For-profit and not-for-profit health plans and mutual insurers differ in how they use this excess of income over expenses.

- A **for-profit health plan** distributes its profits among its owner-investors.
- A **mutual insurance company** distributes "profits" to its policyowners.
- A **not-for-profit health plan** cannot distribute "profits" to individuals for personal gain but must instead use them to benefit the company and serve its mission.

Any health plan—whether a for-profit company, a not-for-profit organization, or a mutual insurance company—seeks to have its income exceed its operating expenses. The excess can be used to generate additional funds and instills confidence in potential investors and lenders, who provide access to even more funds. If a health plan is unable to convince investors, lenders, or donors of its ability to operate "in the black," the plan is likely to lose access to outside funds that are often critical to continued operation in an increasingly competitive environment.

For-Profit Stock Companies

Most for-profit health plans in the United States today are **stock companies**. A stock company obtains the funds it needs to begin operations from investors who purchase ownership shares of stock in the company. These owner-investors are called **shareholders or stockholders**. Although shareholders may receive a portion of the company's earnings through periodic payments, known as **dividends**, historically health plans have elected either not to pay or to pay relatively low dividends. Shareholders can realize capital gains (or losses) by selling their shares of stock. If shares are offered to the general public, the company is a **publicly traded corporation**. If not, it is a **privately held corporation**.

In a publicly traded corporation, there may be two classes of shareholders.

- **Common stockholders** elect the company's board of directors and vote on company affairs. They ordinarily have one vote per share of stock.
- **Preferred stockholders** generally do not have voting rights regarding the company's directors or company policy. However, they must be paid their dividends before common stockholders are paid theirs.

Typically, the board of directors selects senior management, and the vast majority of shareholders do not participate in the day-to-day operations of the business. But ultimately, the board and management are accountable to the shareholders.

In a privately held corporation, shares of stock are available only to a closed group. The company may be a family business that the owners want to keep in the family. Or the owners may not want to give up control and/or income by sharing ownership with outside investors. Or the company may simply be too small to be publicly traded. The number of owner-investors may be small enough that all of them can sit on the governing board. A privately held corporation always has the option of "going public" if it decides this would be in its best interest. For example, a privately-held corporation may need to generate funds quickly for expansion into new markets or to improve its operations.

When a stock company is established, the articles of incorporation stipulate the number of authorized shares of stock and the **par value** of those shares (the per-share amount or face value for each share). Generally, a company does not issue the total number of authorized shares. Instead, it retains some shares for future investment purposes. The par value of the shares is established for state tax purposes, but the actual investment is invariably greater than the par value.

Governance

A for-profit health plan is run strictly as a business and is focused on growth and generating profits for its shareholders. This business focus and the ability to raise needed funds through owner-investors are two primary reasons that for-profit health plans have become more common in recent years. Compared to not-for-profit plans and mutual insurance companies, for-profit health plans are often better able to attract executives by offering shares of company stock as part of their compensation packages.

Some analysts believe that publicly traded corporations have a governance advantage over other types of companies because their shareholders often take an active interest in company operations. Shareholders, whose money is at stake, hold the board and senior management accountable, and as a result, management tends to be more aggressive, growth-oriented, and adaptable to changes in the economy. However, other analysts consider this to be a disadvantage at times, because the desire for short-term financial results can overshadow the need for long-term investments and strategies.

Regulation

For-profit health plans are generally subject to fewer restrictions than mutual insurers and not-for-profit plans with regard to how profits and surplus funds can be used. However, they are subject to financial solvency regulations including limitations on the types of investments they can make. At one time, many states restricted the formation of for-profit HMOs, but today only Minnesota and Vermont prohibit them. Over the past two decades, a large number of not-for-profit health plans have converted to for-profit status, a trend that has prompted several states, as well as public interest groups, to pay closer attention to the regulation of "for-profit" transactions involving not-for-profit health plans.

Mutual Insurance Companies

Some health plans are **mutual insurance companies**. A mutual insurer is owned and governed not by investors, but by policyowners. Policyowners elect the board of directors, and the board is ultimately accountable to them. Mutual insurers can issue both **participating** and **nonparticipating policies**.

- Only participating policyowners are eligible to elect board members. Each policyowner has one vote, regardless of the amount of insurance or the number of policies she has.
- Only participating policyowners receive policy dividends, which are considered a return of part of the premiums paid.

Most health policies issued by mutual companies are nonparticipating. Additionally, in group insurance, the group policyholder (usually an employer) has voting and dividend rights, not individual employees.

Mutual insurers use the term **surplus** to refer to the excess of income over expenses. The wealth of many mutual insurers lies largely in their accumulated surplus. State regulations typically limit the ways that mutual insurers can use or invest this surplus.

Mutual insurers enjoy certain advantages:

- They are free from the pressures of stockholders who, according to some analysts, may focus primarily on short-term profits at the expense of long-term initiatives.
- They are protected from unwanted takeovers by outside interests because, unlike stock companies, they have no shares of stock that can be acquired by other companies or investors.

However, mutual insurers also face disadvantages:

- They are more difficult to establish and expand because, unlike stock companies, they cannot sell shares of stock to raise start-up funds, nor can they sell additional shares to finance expansion or infrastructure improvements. Mutual insurers must raise capital primarily from company operations—that is, by selling policies.
- The inability to sell stock also makes it more difficult for mutual insurers to take advantage of opportunities for consolidation. Many players in the health plan industry today consider consolidation to be a key strategic advantage, particularly following the passage of the ACA. If two for-profit health plans determine that it is in their best interest to merge, they can accomplish this through an exchange of stock; however, if one is a mutual insurer, this option is not available.

Although some stock companies and not-for-profit health plans are converting to mutual insurance companies (**mutualization**), the trend over the past several years has been for mutual insurers to convert to stock companies (**demutualization**). Also, some mutual insurers have created mutual holding companies to address some of the difficulties associated with being a mutual company.

Not-for-Profit Health Plans

A not-for-profit health plan has no owners or owner-investors and so cannot distribute profits for the benefit of individuals. Like mutual companies, not-for-profit plans typically use the term “surplus” to refer to the excess of income over expenses. Depending on the stated purpose of the health plan, surplus can be used for the benefit of the organization, the community, or a charity, but not to benefit private individuals. Although one might think that not-for-profit health plans would be less “profitable” than for-profit plans, this has not been demonstrated. According to one study, the rates of return (ROI) on assets are not appreciably different between for-profit and not-for-profit HMOs.⁹

If some of the earnings of a not-for-profit organization go to the benefit of private individuals, this is called **private inurement** and is usually unlawful. Salary and benefits paid to employees of a not-for-profit health plan are part of the organization’s operating expenses and are not considered private inurement. But if regulatory authorities determine that compensation is unreasonably high, some of it could be deemed unlawful private inurement.

In a not-for-profit health plan, governance is the responsibility of a board of directors or a board of trustees. This board fulfills a role and functions similarly to the board of a for-profit plan. A board is appointed when the entity is established. Directors may be replaced by board vote or elected by the plan membership. Some states require boards to be at least partly elected by members. In the past, not-for-profit board members were often volunteers; today most are paid, but typically less than in a for-profit company. In a not-for-profit health plan, the directors or trustees are not accountable to voting shareholders or policyowners, but rather, depending on the stated purpose of the plan, to the interests of the members, the community, or the charity the company serves.

When Blue Cross and Blue Shield plans and other prepaid health plans (such as Kaiser Permanente) began to appear in the first half of the 20th century, the federal government determined that they could be established as tax-exempt, not-for-profit organizations. In 1973 the federal HMO Act authorized grants and loans for federally qualified HMOs that, at the time, were required to be “public or nonprofit entities.” These tax advantages and funding incentives accounted for the establishment of a large number of not-for-profit HMOs. But in the 1980s, government funding for HMOs was phased out, and the industry—with encouragement from the federal government—moved in the direction of private investment and for-profit companies.

Taxation

Some not-for-profit health plans obtain tax advantages by qualifying for federal tax-exempt status, but some do not. Tax exemption is largely based on this reasoning: “Charitable exemptions from income taxation constitute a quid pro quo: The public is willing to relieve an organization from paying income taxes because the organization is providing a benefit to the public.”¹⁰ However, qualifying for federal tax exemption is a complex process that involves sections of the Internal Revenue Code (IRC), numerous IRS rulings, and court decisions. The pertinent IRC sections are:

- 501(c)(3)—tax-exempt status as a charitable organization
- 501(c)(4)—tax-exempt status as a social welfare organization

- 501(m)—qualification or non-qualification for tax-exempt status based on extent of commercial marketplace business

A health plan that qualifies as a **501(c)(3) charitable organization** is tax-exempt because it promotes health for the benefit of the community. Two notable court cases have clarified this, although courts continue to face and resolve issues in this area.¹¹

- In *Sound Health Association. v. Commissioner* (1976),¹² the court held that an HMO can benefit the community, rather than solely private interests, if it meets certain criteria. These included providing healthcare services and maintaining healthcare facilities and staff, providing services to nonmembers on a fee-for-service basis, providing care for the indigent at reduced rates, providing care for individuals on public assistance programs, making emergency room facilities available to all members of the community without regard to ability to pay, and appointing a board of directors that is broadly representative of the community.¹³
- However, the *Geisinger Health Plan v. Commissioner* decisions (1993-94)¹⁴ narrowed the interpretation of the community benefit standard so that, in essence, the only types of health plans that could qualify as 501(c)(3) organizations would be staff model HMOs or HMOs owned and operated by providers or healthcare systems.

A health plan that qualifies as a **501(c)(4) social welfare organization** is tax-exempt because it is organized and operated exclusively to promote the general welfare of certain people in the community. A 501(c)(4) exemption is easier to obtain than a 501(c)(3) exemption, and few health plans have qualified for 501(c)(3) status. While 501(c)(4) organizations are allowed to benefit a comparatively small group of individuals, such as the members of an HMO, a 501(c)(3) organization must benefit the community at large.¹⁵

In addition to exemption from federal taxation, a 501(c)(3) charitable organization enjoys other advantages:

- It can raise operating funds through the sale of tax-exempt bonds.
- It can accept tax-deductible donations.
- It can often more easily obtain exemption from state and local property and sales taxes.

A 501(c)(4) social welfare organization does not have these advantages, but it does have more flexibility in using funds for social or political activities.

The Tax Reform Act of 1986 (TRA 86) included **IRC Section 501(m)**, which for the most part eliminated tax exempt status for Blue Cross and Blue Shield plans (which until then were typically established as 501(c)(4) organizations). As a result, “the Blues” were permitted only to take tax deductions for the portion of their earnings placed in reserve funds. IRC Section 501(m) is also used in conjunction with sections 501(c)(3) and 501(c)(4) to determine the tax status of health plans other than the Blues.

- Tax-exempt status is denied to any entity for which a “substantial part of its activities consists of providing commercial-type insurance.”¹⁶

- If an entity is granted tax-exempt status but nonetheless derives a portion of its income from commercial-type insurance, that portion is treated as taxable unrelated trade or business income.¹⁷ IRS rulings suggest that certain indemnity-type features of HMOs (such as non-network emergency care or specialty care) are not considered commercial-type insurance, but others (such as POS options) are.

Some state requirements for tax-exempt status closely resemble federal requirements, while other states are more restrictive.

Examples: *A Massachusetts court ruled that a 501(c)(3) HMO was exempt from local real estate taxes because the HMO's health promotion activities applied to a large enough class of persons to be considered a benefit to the community at large.*¹⁸

*On the other hand, Maryland courts ruled that a 501(c)(3) HMO was not a charitable organization and was subject to state property taxes, finding that the provision of "high-quality medical care to...members for a fee" is not a charitable, benevolent, or educational activity.*¹⁹

Often, even when a health plan is tax-exempt, it is still required to pay a state premium tax, which is levied on premiums collected for certain types of insurance and healthcare coverage. Typically, premiums for indemnity insurance and PPO plans are subject to premium tax, and in some states POS options, exclusive provider only (EPO) plans, and HMOs must also pay this tax.

Health Plan Funding

Generating funds to establish, maintain, and expand operations is critical for all health plans. Access to capital is essential for initiatives such as systems upgrades, development of new products, expansion into new markets, and strategic alliances. There are four main sources of funding for health plans:

- Internal sources of income (the plan's business operations and investments)
- Debt markets (borrowing from creditors, usually through bonds)
- Equity markets (selling shares of ownership to owner-investors)
- Donations

Different types of plans have access to different sources.

All health plan types can obtain funds internally. A health plan's operations (including selling, administering, delivering, and designing health plan products) generate funds.²⁰ Generally, a plan prefers to obtain funds from its operations. In doing so, the plan does not take on investment risk, pay interest on debt, or pay the cost of issuing stock. But it is not always possible to generate sufficient funds in this way.

Investing activities may include purchasing and selling bonds, mortgage loans, stocks, real estate, equipment, certificates of deposit, and other assets, as well as making loans and collecting the principal and interest on those loans.²¹ The kinds of instruments in which a health plan can invest are typically limited by financial solvency regulations. Health plans that pay dividends can increase funds available internally by reducing the amount of dividends.

A health plan can obtain loans (take on debt). Most commonly, it issues bonds, which represent debt the plans owes the bondholders. All health plans can take out loans, although mutual companies do not usually issue conventional bonds. As we have seen, 501(c)(3) charitable organizations can make tax-exempt bond offerings and obtain tax-free loans; for tax purposes, they can deduct both the cost of generating a bond offering and the interest paid. Other types of health plans can deduct only interest payments. Debt is the most expensive method of obtaining funds. Companies must pay interest on debt, whether or not the company is making money. Typically, debt is used by health plans that operate medical facilities (such as staff model HMOs) to build or renovate facilities.

A for-profit stock company can raise funds in the equity markets—that is, it can issue and sell shares of stock. The purchasers of stock buy an ownership interest (equity) in the corporation. A company can make a public offering of stock or raise private equity. Many analysts consider access to equity markets a significant advantage for a health plan, and several not-for-profit plans and mutual companies have converted to stock companies in recent years for this reason. More recently, however, state regulators have begun scrutinizing proposed conversions more closely and rejecting some.²²

All non-profits (and in theory, even for-profit companies) can solicit and receive donations. But in practice, donations are almost never made to health plans other than 501(c)(3) charitable organizations, because only these donations are tax-deductible.

For a summary of the funding sources available to different types of health plans, see the table below.

Type of Plan	Internal Sources (Operations and Investments)	Debt Markets	Equity Markets	Donations
For-Profit	Available	Available	Available	Effectively not available
Not-for-Profit	Available	Available	Not available	Available, but almost never obtained except by 501(c)(3) charitable organizations
Mutual	Available	Available, but mutuals do not usually issue conventional bonds	Not available	Effectively not available

Market Share of For-Profit and Nonprofit Health Plans (2012)

Type of Health Plan	Fully Insured Enrollment		Total Enrollment	
	Number	Percent	Number	Percent
Nonprofit	66,074,819	50%	104,273,365	45%
For-Profit	62,377,369	47%	122,407,737	53%
Government	3,975,493	3%	4,092,843	2%
Totals	132,427,681	100%	230,773,945	100%

Summary

Healthcare providers and other entities that health plans work with may be legally organized as sole proprietorships, partnerships of different kinds, or corporations. A limited liability company (LLC) combines features of a partnership and a corporation. The owners, directors, and officers of a professional corporation must be licensed professionals.

Health plans are usually corporations. Stock companies are owned by and operated for the benefit of their investor-owners. A mutual insurance company is owned by its policyholders. A not-for-profit health plan must be operated for the benefit of the organization, the community, or a charity, but not private individuals. Not-for-profit plans may enjoy tax advantages, but they may also find it more difficult than for-profit plans to raise money for expansion and improvements.

Sources

¹ Fla. Stat. Ann. §641.2015. (HMO must be incorporated or shall be a division of a corporation formed under the provisions of either part I of chapter 607 (Florida Business Corporation Act) or chapter 617 (Florida Not-for-Profit Corporation Act) or shall be a public entity that is organized as a political subdivision.)

² 10 N.Y. Comp. Codes R. & Regs. Tit. 10, §98-1.5(a). ("A foreign corporation or limited liability company shall not be a proper applicant for a certificate of authority [as an HMO].") See also New York Department of Financial Services Office of General Counsel Opinion No. 2005-48, "Licensing of foreign health maintenance organizations (HMOs) in New York," February 24, 2005.

³ Douglas G. Smith, "Piercing the Corporate Veil in Regulated Industries," *Brigham Young University Law Review* Vol. 2008, Feb. 1, 2008; George Mason Law & Economics Research Paper No. 08, <http://ssrn.com/abstract=1090027>.

⁴ *Thrift Drug, Inc. v. Universal Prescription Administrators*, 131 F.3d 95 (2d Cir. 1997).

⁵ *Thrift Drug v. Prescription Plan Service Corp.*, 1 F. Supp.2d 387, 388 (S.D.N.Y. 1998).

⁶ Bruce A. Johnson and Gerald A. Niederman, *Health Plan Legal Issues* (Englewood, CO: Medical Group Management Association, 1996), pp. 200–202.

⁷ *Id.*, pp. 147–50.

⁸ Andrew Roth and Kimberly Gold, "Corporate Practice of Medicine: An Old Doctrine Breathing New Life," *New York Law Journal*, Vol. 251, No. 121, June 25, 2014; *see also* National Hospice and Palliative Care Organization, "Corporate Practice of Medicine Doctrine 50 State Survey Summary," Sept. 2006, <http://www.nhpco.org/sites/default/files/public/palliativecare/corporate-practice-of-medicine-50-state-summary.pdf>.

⁹ Herrington J. Bryce, "Profitability of HMOs: Does Non-Profit Status Make a Difference?" *Health Policy*, Vol. 28, 1994, p. 203.

¹⁰ Mark A. Hall and William S. Brewbaker III, eds., *Health Care Corporate Law: Managed Care* (Gaithersburg, MD: Aspen Publishers, Inc., 1996), pp. 1-23.

¹¹ *See, e.g., IHC Health Plans, Inc. v. Commissioner*, 325 F.3d 1188 (10th Cir. 2003).

¹² *Sound Health Association v. Commissioner*, 71 T.C. 158 (1978), acq. 1981-2 C.B. 2.

¹³ Hall and Brewbaker, pp. 1-23.

¹⁴ *Geisinger Health Plan v. Commissioner*, 985 F. 2d 1210, 1215 (3d Cir. 1993), *aff'd*, 30 F. 3d 494 (3d Cir. 1994).

¹⁵ Hall and Brewbaker, pp. 1-29.

¹⁶ IRC 501(m)(1) provides that an organization described in IRC 501(c)(3) or IRC 501(c)(4) is exempt from tax only if no substantial part of its activities consists of providing commercial-type insurance. *See* Internal Revenue Service, *Internal Revenue Manual*, https://www.irs.gov/irm/part4/irm_04-076-031.html.

¹⁷ *Harvard Community Health Plan, Inc. v. Board of Assessors*, 427 N.E. 2d 1159, 1163 (Mass. 1981).

¹⁸ *Supervisors of Assessments v. Group Health Assn.*, 517 A. 2d 1076 (Md. 1986).

¹⁹ Elizabeth A. Mulligan and Gene Stone, *Accounting and Financial Reporting in Life and Health Insurance Companies* (Atlanta: LOMA, 1997), p. 123.

²⁰ *Id.*

²¹ Susan Conant, Nicholas L. Desoutter, Dani L. Long, and Robert MacGrogan, *Managing for Solvency and Profitability in Life and Health Insurance Companies* (Atlanta: LOMA, 1996), p. 443.

²² "Nonprofit to Profit: Free Money No More," *Forbes*, Jan. 9, 2007 (discussing rejections of conversion applications in Washington, North Carolina, Maryland and Kansas).

3 Corporate Restructuring and Transactions

Lesson Objectives

After completing this lesson, you should be able to:

- Define subsidiaries, spinoffs, and holding companies and explain how health plans can use them to restructure.
- Explain why a health plan would want to mutualize, demutualize, or change from not-for-profit to for-profit status, and discuss the issues that may arise.
- Distinguish between strategic partnerships, joint ventures, acquisitions, and mergers and discuss the advantages and potential problems of each.

Corporate Restructuring and Transactions: Why?

In this lesson we explore various ways that health plans restructure themselves as well as transactions in which they work or combine with other organizations. Why would a health plan do this?¹

- To generate funds needed for various activities.
- To capture a greater market share, enter new markets, or offer new products.
- To reduce administrative and medical expenses by achieving economies of scale. (That is, a plan can perform functions on a larger scale and therefore at a lower unit cost.)
- To integrate or better perform a variety of functions.
- To achieve greater negotiating power in relation to providers. (This is particularly important in today's post-ACA environment, where providers increasingly consolidate.²)

Some examples of how and why health plans change their structure or join with other organizations:

A national health plan consolidates its marketing and administrative support regions to improve efficiency and reduce operating expenses.

Two health plans have geographic strengths that complement each other. By combining operations they become a strong regional health plan with significant market share.

A national health plan has been struggling to obtain market share in a particular city. It joins with a local health plan that is also seeking greater market share. Together they significantly improve their market position.

A health plan determines that it is "behind the curve" in developing a new product critical to its customers. Rather than spend the time and money to develop the product from the ground up, the plan collaborates with another health plan that has already developed such a product.

One health plan is expert at marketing and administration, another at healthcare delivery. By combining resources and drawing on the best of both operations, they develop a more effective operation than either could have built on its own.

Two local health plans decide to join forces because their provider networks effectively complement each other. One has a strong primary care physician panel and the other a strong specialist component.

Subsidiaries, Spinoffs, and Holding Companies

A **parent company** is a company that has a controlling interest in another company, called a **subsidiary**.³ Two subsidiaries of the same parent company are known as **sister corporations**. A not-for-profit health plan cannot be owned, so technically it cannot be a subsidiary. But a parent company may have voting control of a not-for-profit plan, in which case the plan is referred to as an **affiliate**. In this context, an affiliate is a company that is held, but not owned, by a parent company.

A unit of a company that becomes a subsidiary or an independent company is called a **spinoff**. The spinoff may primarily serve the company it was originally part of, or it may have multiple customers.

***Example:** A Green Valley Health Plan creates a pharmacy management unit to serve its members. Later it decides to make this unit an independent company. The new spinoff company serves both Green Valley and other health plans.*

Why would a health plan spin off a unit?

- The unit has expertise or resources that could be marketed more broadly or in response to specific market opportunities.
- It may be believed that the unit, if not part of a larger organization, will be managed more aggressively and competitively, or with lower administrative costs.

If a spinoff is a subsidiary, it is of course owned by the parent company. What if it becomes independent?

- It may be owned by the same shareholders as the original company, who receive shares of the spinoff company's stock.
- It may be sold to another company.
- It may be bought from the original company by its managers (in a **management buy-out**), who then own and operate it.

Holding Companies

A **holding company** is a type of parent company. It is an entity whose primary business is the ownership of (the holding of a controlling interest in) other companies (its affiliates or subsidiaries). A holding company can be used to control a group of companies in the same or related businesses, in order to maximize expertise and market presence. Or it can control companies in widely differing industries. In this way, the holding company can diversify its interests and protect itself from the risks of concentrating on just one business.

A **downstream holding company** is owned or controlled by the company that established it.

***Example:** Red Tower Corporation establishes a holding company. The holding company then creates or acquires subsidiary firms. Red Tower owns or controls the whole system—the holding company and its subsidiaries.*

An **upstream holding company** owns or controls the company that established it.

***Example:** Green Lantern Corporation establishes a holding company, which then creates or acquires additional firms. The holding company owns or controls the whole system—Green Lantern and the other subsidiaries (Green Lantern’s sister corporations).*

A health insurer may form a holding company to avoid restrictions on investments. State regulations often limit the kinds of investments an insurance company and its subsidiaries can make. But a holding company that owns or controls an insurer is not subject to these rules, nor are its other subsidiaries. However, regulators seek to prevent a holding company from separating an insurer’s assets and liabilities from its control. In particular, regulations are designed to keep a holding company from “raiding” the capital and surplus of an insurer for use by other subsidiaries.

A mutual insurer can be owned and controlled only by its policyholders, so it cannot be owned or controlled by an upstream holding company. But mutual insurers can form downstream holding companies, which the mutual company owns or controls.

When a not-for-profit health plan sets up a holding company, it must protect any charitable or community benefit it exists to promote. If a not-for-profit plan creates an upstream holding company, there may be limits on the assets that can be transferred to it. If a not-for-profit plan establishes a for-profit parent, it would likely be deemed to be converting to for-profit status. Conversion are subject to regulatory requirements and restrictions, as we will discuss below.

All states impose registration and reporting requirements on companies that are part of an insurance holding company system. Most state laws are based, in whole or in part, on the *Insurance Holding Company System Regulatory Act (Model Holding Company Act)* of the National Association of Insurance Commissioners (NAIC). Also, most states have adopted regulations based on the NAIC’s *Insurance Holding Company System Model Regulation with Reporting Forms and Instructions (Holding Company Model Regulation)*.⁴

Mutualization and Demutualization

Mutualization is the conversion of a stock company or not-for-profit health plan into a mutual insurance company. A health plan might mutualize in order to merge with another mutual insurer. A not-for-profit plan might do so because mutual insurers are subject to fewer regulatory controls than non-profits. When a health plan pursues mutualization, it typically must obtain state insurance department approval. The mutualization of a not-for-profit plan will undergo special scrutiny to ensure that the plan’s charitable or community benefit purpose is protected.

Demutualization is the conversion of a mutual company into a stock company. Some companies demutualize to obtain funds by selling stock. Demutualization can also allow a company to more easily pursue acquisitions and mergers. (A stock company can use stock to pay for such transactions, and there is usually less regulatory scrutiny than for a mutual insurer.) Demutualization involves substantial legal and administrative costs. Regulation of demutualization varies by state. Some states have no specific regulations, while others prohibit it.

Demutualization requires the distribution of the company’s surplus funds among its policyowners. Several questions arise:

- Which policyowners should share in the surplus? Only current or recent policyowners? All living policyowners? Heirs of deceased policyowners?

- Should policyowners receive cash, shares of stock in the converted company, or both?

The **mutual holding company** is an alternative to demutualization allowed in some states. The mutual insurer can sell up to 49 percent of its ownership to public shareholders. It remains a mutual insurer, because policyowners retain majority ownership, but it gains access to equity markets. However, policyholders no longer have full ownership. And critics of this approach argue that it gives mutual insurers an unfair advantage over stock companies—they can access capital markets and initiate strategic alliances but they remain immune to takeover.

For-Profit Conversion and Transactions

Some not-for-profit health plans may seek to convert to for-profit status. In addition, not-for-profit plans sometimes enter into “for-profit-type” transactions, such as:

- Selling assets to raise funds
- Establishing for-profit subsidiaries
- Transferring assets into existing for-profit subsidiaries
- Entering into joint ventures or mergers with for-profit companies

Why would a not-for-profit plan seek to convert to for-profit status?

- To obtain funds, such as through equity markets.
- To change the overall governance or strategic focus of the organization.
- To avoid the regulatory requirements and restrictions that apply to not-for-profits in many states.

Note also that in some cases tax exemptions are no longer available, removing a reason to keep not-for-profit status.

As we have learned, not-for-profit assets cannot be used for individual gain. There are concerns that a not-for-profit plan’s conversion or its for-profit transactions may result in funds generated for charitable or social welfare purposes going into for-profit ventures that benefit individual owner-investors, or leading to personal financial gain for directors or officers. In some states, the attorney general has the responsibility and authority to ensure that any transactions of a not-for-profit organization are consistent with its stated purpose. In some states, the organization must obtain court approval. Some conversions and transactions have been blocked in the courts, or their terms modified, so health plans taking this approach are advised to proceed cautiously.⁵

When a not-for-profit health plan is converting to for-profit status, regulators in some states seek to ensure that the sale price represents fair market value for the assets sold. The plan may also be required to place assets in a charitable foundation. This raises questions:

- Is the foundation’s mission close enough to the not-for-profit plan’s stated purpose?
- Is the foundation sufficiently independent of the newly established for-profit entity?
- Is the not-for-profit company undervaluing its assets, thereby donating less than fair market value to the foundation?

Example: *Valley Community Health Plan, a not-for-profit organization, converts to a for-profit corporation. The law of the state requires the plan to make a financial contribution to charity equal to the value of its assets. In an agreement with the state, Valley agrees to make contributions to healthcare charities of \$20 million a year for 20 years.*

Reengineering

Some health plans seek to improve their performance through **corporate reengineering**: the fundamental redesign of structures and processes. Reengineering may focus primarily on corporate structure and internal organization and staffing (also called reorganization). Or a plan may focus on processes. Business process reengineering consists of five major steps:

- Refocus company values on customer needs.
- Redesign core processes, often using information technology to enable improvements.
- Reorganize a business into cross-functional teams with end-to-end responsibility for a process.
- Rethink basic organizational and people issues.
- Improve business processes across the organization.⁶

Reengineering differs from more superficial change:

- Companies look past existing models, assumptions, and infrastructures to see the underlying key processes that bring value to the customer.
- The focus is on outcomes and results rather than specific tasks.
- The question is asked: “If we were starting the company today, how would we do it?”

Example: *As part of corporate reengineering, a health plan looks at new group business. The plan does not focus on separate tasks performed for the purchaser by various departments. Instead, it focuses on the items and services delivered to the purchaser (such as information kits, premium notices, the group contract, and member booklets) and seeks to find the most effective ways to provide these deliverables.*

Reengineering can result in restructuring of departments and job responsibilities, consolidation and closing of work sites, or withdrawal from geographic markets or lines of business. In fact, reengineering has been viewed by some critics as a shallow intellectual justification for downsizing, or as something that managers are happy to impose on others but not on themselves. Consequently, reengineering should be approached carefully and with open eyes.⁷

Corporate Transactions

In the first half of this lesson, we have seen how health plans can change their own structure, by forming subsidiaries, affiliates, spinoffs, and holding companies; by mutualizing or demutualizing; or by moving from not-for-profit to for-profit status. In this second part, we’ll see how plans can join forces with other organizations, through strategic partnerships, joint ventures, acquisitions, and mergers.

Working with another plan or forming a larger combined entity can have financial advantages:

- Economies of scale can reduce expenses.
- Increased volume for suppliers and providers can lead to more favorable prices.
- Access to capital, financial strength, and stability can be improved.

Plans can complement one another's strengths:

- If they sell different products, they can create a more diversified product mix, making them less vulnerable to market downturns.
- If they serve different regions, they can cover more territory and increase their business with national employers.
- A combined provider network can provide more services and offer greater choice to members.
- One plan may perform well in functional areas (such as marketing, claims, or contracting) where the other plan is weak.

Strategic Partnerships

In a **strategic partnership**, two or more entities work together on a particular function or project but are not directly involved in each other's operations, in accordance with a contract or agreement.

***Example:** A health plan enters into a strategic partnership with a management consulting company. The company provides the expertise needed to help bring a Medicare Advantage plan to market but is not otherwise involved in the plan's operations.*

Joint Ventures

In a **joint venture**, a company is formed, owned, and operated by two or more entities as a separate business or project. Entities typically have resources and expertise that complement one another, and the venture benefits both. Often, a joint venture is formed to share the risks and rewards of developing a new product or entering a new market.

***Example:** A national health plan and a local hospital system, which operates its own health plan network, enter into a joint venture in which they share ownership and control of a newly created health plan operation. The objective of the hospital system is to shift its focus back to hospital operations while obtaining patients from the health plan. The objective of the health plan is to gain market share in a region where in the past it has been unable to compete effectively.*

***Example:** A provider group seeks access to a health plan's care management and analytic capabilities. The plan wants access to a well-trained and respected physician network and numerous hospitals and outpatient facilities in a region. They enter into a joint venture.⁸*

A contract generally specifies the rights and responsibilities of each party in a joint venture, such as the percentage of the venture that each owns, the organizational structure, and the decision-making process. One partner may have a greater economic stake than the others, but as long as no single partner actually controls the venture, it is considered a joint venture. Often, the board of directors of a joint venture consists of an equal number of representatives from each of the partners.

In today's post-ACA environment, with its emphasis on value-based payment, joint ventures have become a popular way for health plans and providers to provide higher-value care. And in today's climate of constant change, strategic partnerships and joint ventures allow a health plan to move quickly into a new and potentially profitable endeavor without altering its legal form.

Acquisitions

An **acquisition** occurs when one company purchases a controlling interest in another company. Ownership of more than 50 percent of the stock generally gives controlling interest, but in practice, if the remaining stock is widely distributed, a much smaller percentage can give effective control.

There are two main types of acquisitions:

- In a **stock purchase** a company acquires 100 percent of the voting shares of another company's stock, making it a wholly owned subsidiary. The purchaser takes on all of both the assets and the liabilities of the purchased company.
- In an **asset purchase** a company acquires all or specified assets of another company; it may take on some specified liabilities.

An asset purchase may be the best way to sell a company that would otherwise bring substantial liabilities to the buyer. It may be necessary when a not-for-profit or mutual company, which has no stock, is involved.

Mergers

A **merger** occurs when two or more companies become a single legal entity. This can happen in different ways:

- The existing companies are dissolved, and an entirely new company is formed (a **consolidation**).
- One company becomes part of the other.
- One company forms a new subsidiary, and the other company becomes part of it.
- One company forms a new subsidiary, and this subsidiary becomes part of the other company.

In some cases the objective is to merge into the company that has licenses and other government approvals, so that a new company does not have to acquire them.

Sometimes the operations of the existing organizations are merged. The management of one company may take over, or a new management team may be named. Sometimes operations are not merged, and the management of each organization continues to run it.

Implementing Mergers and Acquisitions

A health plan's leadership should make sure that a merger or acquisition opportunity actually meets a strategic goal of the plan and creates economic value. And when pursuing such opportunities, leaders should guard against "getting caught up in the chase" and allowing the purchase price to rise to a level beyond real value, benefiting only the sellers.

Acquisitions and mergers are complex transactions with substantial potential financial and legal risks. **Due diligence** must be performed—that is, a health plan must carefully investigate potential transferred liabilities and tax and accounting implications. The results of due diligence may determine the type of transaction and how it is structured.⁹

All states require some form of regulatory notification and/or approval for an acquisition, merger, or change of control of a health plan. The degree of scrutiny and the requirements vary from state to state. State holding company acts often require the insurance commissioner to approve any proposed transaction in which one entity will either acquire control of a domestic insurer or merge with it. When an acquisition or merger involves a not-for-profit health plan, regulators are concerned that the plan's assets are used to advance the plan's purpose. If a health plan is in financial difficulties, a merger or acquisition involving it will attract closer regulatory scrutiny.

When a merger or acquisition involves an HMO, transferring the HMO license to a new entity may be a concern.

- In some states prior regulatory approval of this transfer is needed; in others only notification is required.
- In some states an HMO license cannot be transferred, and the new entity must obtain a new license. (However, it may be possible to structure the transaction so that the existing HMO is the surviving entity and its license is retained.)

In recent years the U.S. Department of Justice has taken an increased interest in health plan merger activity. As of this writing, it has challenged two proposed mergers of large national plans on antitrust grounds, arguing that market concentration will decrease competition, resulting in higher premiums and less consumer choice.¹⁰

For a merger or acquisition to benefit a health plan, the transition to the new structure and the integration of operations must be carefully planned and effectively managed. On one hand, management should move quickly to make the most of the immediate advantages of the transaction (for instance, eliminating overlap and redundancy, realizing administrative savings, and leveraging increased volume). But at the same time, leadership must not lose sight of the big picture—the longer-term synergies and strategic strengths that were the main reasons for the transaction.¹¹

Summary

Health plans can improve their performance and meet strategic goals by restructuring. They can create subsidiaries, spin off units into subsidiaries or independent companies, set up holding companies to own and control several related companies, change from a mutual insurer to a stock company or vice versa, convert from a not-for-profit organization to for-profit corporation, and reengineer the structure and processes of the organization.

Health plans can also collaborate or combine with other health plans or organizations. They join in strategic partnerships in particular areas or form joint ventures that they both own and control. They can acquire or merge with other organizations.

Sources

¹ Joy M. Grossman and Bradley C. Strunk, "For-Profit Conversions and Merger Trends Among Blue Cross Blue Shield Plans," Center for Studying Health System Change, Issue Brief No. 76, January 2004, <http://www.hschange.com/CONTENT/644/?PRINT=1>.

² Paul von Ebers, “Mega Health Insurance Mergers: Is Bigger Really Better?” *Health Affairs* blog, January 22, 2016, <http://healthaffairs.org/blog/2016/01/22/mega-health-insurance-mergers-is-bigger-really-better/>.

³ Some content in this section was originally adapted from Kenneth Huggins and Robert D. Land, *Operations of Life and Health Insurance Companies* (2nd ed.) (Atlanta: LOMA, ©1992), pp. 49–53. Used with permission. All rights reserved.

⁴ National Association of Insurance Commissioners, “Insurance Holding Company System Regulatory Act,” <http://www.naic.org/store/free/MDL-440.pdf>]

⁵ James C. Robinson, “For-Profit Non-Conversion and Regulatory Firestorm at CareFirst BlueCross BlueShield,” *Health Affairs*, 23:4 (2004), pp. 68-83, <http://content.healthaffairs.org/content/23/4/68>.

⁶ Darrell K. Rigby, *Management Tools 2015: A Executive’s Guide*, Bain & Company, Inc. http://www.bain.com/Images/BAIN_GUIDE_Management_Tools_2015_executives_guide.pdf.

⁷ “Business Process Re-Engineering,” *The Economist*, Online Extra, Feb. 16, 2009, <http://www.economist.com/node/13130298/print>.

⁸ Rachel Zimlich, “Trend to Watch: Payer-Provider Joint Ventures,” *Managed Healthcare Executive Health Plans*, December 3, 2016, <http://managedhealthcareexecutive.modernmedicine.com/managed-healthcare-executive/news/trend-watch-payer-provider-joint-ventures>.

⁹ Sandra M. DiVarco, “The Top Five Traps in Health Care M&A Transactions,” *The National Law Review*, Oct. 14, 2011, <http://www.natlawreview.com/article/top-five-traps-health-care-ma-transactions>.

¹⁰ Leemore Dafny, “The Risks of Health Insurance Company Mergers,” *Harvard Business Review*, Sept 24, 2015, <https://hbr.org/2015/09/the-risks-of-health-insurance-company-mergers>; “Justice Department to Challenge Two Health-Insurance Mergers,” *Wall Street Journal*, July 19, 2016, <http://www.wsj.com/articles/justice-department-to-challenge-two-health-insurance-mergers-1468947479>; Robert Brook, “After the ACA, How Much Do Health Insurance Mergers Matter?” *Forbes*, March 31, 2016, <http://www.forbes.com/sites/theapothecary/2016/03/31/after-the-aca-how-much-do-health-insurance-mergers-matter/#18125f62542e>; Richard M. Scheffler, Daniel R. Arnold, Brent D. Fulton, and Sherry A. Giled, “Differing Impacts of Market Concentration on Affordable Care Act Marketplace Premiums,” *Health Affairs*, 35: 5 (2016), pp. 880-888. <http://content.healthaffairs.org/content/35/5/880>.

¹¹ For more on this topic, see: Roberta Hill and Sandy Weiner, “Seven Steps to Merger Excellence,” *Ivy Business Journal*, Sept./Oct. 2008, <http://iveybusinessjournal.com/publication/seven-steps-to-merger-excellence/>

4A Health Plan Structures and Arrangements, Part I

Lesson Objectives

After completing this lesson, you should be able to:

- Identify the major types of health plans and healthcare organizations.
- Describe the main sponsors of health plans and discuss the roles they currently play.
- Discuss consolidation and alliances among healthcare providers.
- Compare and contrast the principal methods of compensating healthcare providers.

In this lesson we will survey the ways health plans are structured and the arrangements they enter into with providers and others.

- In Part I, we will look at the types of health plans and healthcare organizations, the sponsors of plans, consolidation and alliances among providers, and health plan compensation arrangements.
- In Part II, we will discuss regulatory issues, health plan integration, and employer-sponsored plans (including self-funded plans).

Types of Health Plans and Healthcare Organizations

Health plans, various types of healthcare organizations, and other entities together form a greater healthcare system. In the past, many elements of this system were only loosely coordinated. For instance, providers delivered care and insurers paid benefits for it, with relatively little communication and collaboration between them. Today, the goal of health plans is to organize operations so that the various participants (including hospitals, physicians, other providers, and payors) can effectively share information, produce economies of scale, and provide continuity of care—and in doing so, improve care quality and reduce costs.

The following might be thought of as the three most basic types of health plans:

- A **health maintenance organization (HMO)** is a system designed to deliver healthcare to a voluntarily enrolled population in a particular geographic area, usually in exchange for a fixed, prepaid fee. An HMO brings together the delivery, financing, and administration of healthcare into a single integrated system.
- A **preferred provider organization (PPO)** is an organization that offers a healthcare benefit arrangement designed to supply services at a discounted cost by providing incentives for members to use network providers, while also providing more limited coverage for services rendered by providers who are not part of the PPO network. Sometimes the product is called a preferred provider arrangement (PPA) to differentiate it from the organization that offers it (the PPO).
- An **exclusive provider organization (EPO)**, as its name suggests, only covers healthcare rendered by participating providers; it does not cover healthcare rendered by nonparticipating providers (except in an emergency).¹

Some organizations use an HMO or PPO model for a subset or single specialty of healthcare. These are **specialty health plans** (for instance, a dental HMO). Some states do not allow specialty HMOs; they require a licensed HMO to provide a comprehensive set of services and supplies. In these states specialty health plans must be PPOs or some other type.

An **integrated delivery system (IDS)** is a combination of two or more legally affiliated health plans, group practices, clinics, or hospitals. The entities combine their assets, efforts, risks, and rewards to deliver comprehensive healthcare and, in certain instances, to arrange for the financing and administration of that care.

Some organizations provide services to health plans.

- A **third-party administrator (TPA)** contracts with insurers, health plans, or employers to provide services to help administer healthcare benefits. Because TPAs often perform insurance-like functions, such as claims administration, they are regulated by the insurance department in many states.
- A **utilization review organization (URO)** conducts utilization review activities for health plans and purchasers. A URO may offer medical cost management services in any number of areas, such as inpatient review, outpatient review, and case management of high-risk conditions. Some UROs have expanded their services to include additional functions, such as claims administration, provider contracting, provider credentialing, medical outcomes measurement, member satisfaction surveys, and plan design consulting.

Some healthcare organizations are sponsored by healthcare providers.

- A **provider-sponsored organization (PSO)**—also called an at-risk provider organization, a physician-owned organization, or a provider-owned organization—is, broadly speaking, a health plan entered into or established by providers to arrange for the delivery, financing, and administration of healthcare.
- A **Medicare PSO** is entered into or established by providers who deliver a substantial proportion of services under a Medicare Advantage contract. These providers share substantial financial risk and have at least a majority financial interest in the entity.²

Typically, PSOs are subject to less rigorous solvency and licensing requirements than HMOs (although regulators continue to consider changing these requirements).

Some other provider organizations:

- An **independent practice association (IPA)** is an association of individual physicians (or physicians in small group practices) that contracts with a health plan to provide healthcare services.
- A **physician-hospital organization (PHO)** is structurally much the same as an IPA, except that it includes a hospital. The primary purpose of a PHO is to contract with other health plans, payors, and purchasers on behalf of its participating hospitals and medical practices.
- An **accountable care organization (ACO)** is a network of doctors and hospitals that share financial and medical responsibility for providing coordinated care to patients. A primary care physician is at the heart of each patient's care. The aim is to enhance quality and limit unnecessary spending.³ (ACOs are discussed later in this lesson.)

Some companies support physicians.

- A **management service organization (MSO)** provides a variety of management and administrative services for physicians' practices, freeing physicians to focus on the clinical aspects of healthcare. Services may include centralized purchasing, administrative support, marketing, and practice management.
- A **physician practice management (PPM) company** provides the same services as an MSO, plus sometimes contract negotiations. But a PPM typically purchases the assets of the practices and enters into long-term contracts with the physicians, and often supplies capital for improving and expanding existing assets. Some PPMs perform payer, utilization review, and quality management functions; others have acquired HMOs.

Finally, some other organizations:

- A **medical foundation** owns and manages all purchased assets of physicians' practices. However, unlike an MSO or PPM, it is organized as a not-for-profit entity.
- A **medical foundation integrated delivery system** integrates a hospital and a tax-exempt physician practice into a tax-exempt organization. Typically, a medical foundation IDS consists of a holding company and two not-for-profit subsidiaries: a hospital and a medical foundation. The medical foundation (not the holding company or hospital) enters into contracts with payors and receives the negotiated fees.
- An **academic medical center (AMC)**—also known as an academic health center (AHC)—is an institution that trains healthcare professionals and performs basic and clinical research. Because AMCs receive government funding, they are required to provide medical care for the poor.
- A **community health center (CHC)** is a private, not-for-profit organization that, directly or indirectly (through contracts and cooperative agreements), provides primary health services and related services to residents of a defined geographic area that is medically underserved. CHCs receive federal, state, and often private grant funding. They are typically located in an inner city or rural area, and they often serve Medicare beneficiaries, Medicaid recipients, or individuals without health coverage.
- A **healthcare cooperative** (sometimes a rural cooperative) is a consumer-sponsored, physician-operated medical facility that provides care to members on a prepaid basis.
- **Consumer operated and oriented plans (CO-OPs)** were created by the Affordable Care Act. They are not-for-profit, member-controlled health plans that offer ACA-qualified coverage in both the individual and business markets.⁴ Their financial experience has been mixed so far. A large number have been forced to close, but others remain viable, although most have had to seek outside sources of funding.⁵

Health Plan Sponsors

Many kinds of organizations may establish or fund a health plan. These health plan **sponsors** include:

- Insurance companies
- Investor groups
- Other health plans

- Providers
- Employers
- Government
- Medical schools and universities
- Consumer groups
- Labor unions

Insurance companies began setting up or acquiring health plans (managed care plans) in addition to their traditional indemnity insurance products decades ago. There were many reasons, but a key one was to help hold down healthcare costs. Companies also saw advantages in diversifying their product portfolios. Today, the majority of those covered by private-sector health insurance are members of managed health plans.⁶

Investor groups can be involved in establishing and funding health plans and healthcare organizations—except not-for-profit organizations (such as medical foundations or community health centers) and mutual insurers (which are owned by their policyholders). Venture capitalists from the technology world have entered this field, seeking to marry digital resources, insurance, and clinical information.⁷

Health plans are often sponsored by other health plans, as part of a strategy of expansion or diversification. For example, a large plan operating successfully in one region of the country might establish, merge with, or acquire a plan another region.

Historically, when an HMO or other type of health plan was established in an area, hospitals and physician groups often responded by sponsoring a similar plan. Providers have continued to form various types of provider-sponsored organizations (PSOs). This has included physician practice management (PPM) companies, some of which have gone on to sponsor health plans.⁸ In addition, some non-provider sponsors of health plans offer physicians or hospitals an ownership interest, making them co-sponsors of the plan.

Of course, many employers “sponsor” health plans for their employees in the sense of purchasing coverage and paying a portion of the premiums. But some employers go further, taking on financial risk for providing health benefits. They may fully or partially self-fund a health benefit plan. In doing so, they may enter into contractual arrangements with insurers, TPAs, or health plans for use of networks and administrative services. They may also, either on their own or through coalitions, contract directly with providers.⁹ A historic example: In 1937 the Kaiser company, a large construction firm, set up a prepaid health plan for its employees, which evolved into the Kaiser Permanente health plans.

Over the years, municipal, state, and federal government entities have sponsored health plans. Some examples:

- The City of New York established the Health Insurance Plan of Greater New York as a prepaid plan for city employees. This evolved into Emblem Health, which serves not only city workers but other residents of the greater New York metropolitan area.¹⁰
- Many of the HMOs serving Medicaid recipients came about largely as a result of state funding and initiatives.

Medical schools (usually connected with universities and often university hospitals)—a type of academic medical center (AMC) mentioned above—must maintain a sufficient patient base for their teaching activities. One way to do this is by sponsoring a health plan. Some examples: QualChoice of Arkansas, Inc. was originally sponsored by the University of Arkansas for Medical Sciences. UPMC is closely affiliated with the University of Pittsburgh.¹¹

Healthcare consumers can pool resources to form a health plan to serve them. Several early HMOs were founded in this way. More recently, the ACA has promoted CO-OPs, as discussed above. Such plans may require that the board of directors include a specified number or percentage of consumer members.

Historically, labor unions were a driving force behind the establishment of prepaid health plans in several cities. They no longer play such a large role, but there are exceptions. When the automobile industry was bailed out by the federal government in 2009, unions created a trust to manage healthcare benefits for retirees, and this endeavor has been a success.¹²

Consolidation and Alliances of Providers

An ongoing trend is the consolidation of healthcare providers, alliances between provider organizations, and relationships between providers and health plans. Simply put, the days of the free-standing hospital and solo medical practitioner are passing. This process is accelerating, in part because of systematic changes brought about by the ACA.¹³

Why do providers and provider organizations form relationships with health plans?

- **Patient volume.** Today, providers that do not participate in health plan networks find it increasingly difficult to obtain an adequate volume of patients.
- **Nonmedical activities.** Health plans can take on many administrative, management, and technological activities; they can supply expertise, personnel, and resources in these areas. This allows providers to focus on care.
- **Risk.** Health plans have great experience and expertise in managing risk and can offer agreements and arrangements that offer providers protection from financial risks. However, providers are also being asked to take on some risk, as we will see in the following section.

Hospitals, physicians, and other providers seek competitive advantage by joining forces in consolidations and strategic alliances.¹⁴

- Greater size makes it easier to obtain health plan contracts and gives greater negotiating leverage. A larger organization can relieve smaller providers of the burden of evaluating and entering into contractual relationships.
- Greater size brings economies of scale.
- Providers can share skills, resources, and expertise.
- Consolidation can provide easier access to capital, needed for operating costs as well as investments in construction, medical and information technology, and other improvements.
- Physicians and practices can be relieved of many administrative, management, and technological activities by the larger organization.

- Many younger physicians prefer the security of working for a large organization.¹⁵

Hospital systems or other provider groups may form health plans. This means assuming considerable risk, and such endeavors have not always been successful. But some provider-led plans offer certain advantages in the competition with other plans.

- They often have reputations for high quality.
- They can form dense local networks. These can create local economies of scale and be attractive to employers and other consumers and to national companies interested in forming alliances in order to expand into the area.¹⁶

Provider Compensation

Health plans have long sought to develop and implement methods of compensating providers that promote high-quality care while holding down costs. But plans recognize that there is unlikely to be a “one size fits all” approach that works well in all situations. A number of approaches have been designed and tried; some build on the traditional fee-for-service system, while others are intended to replace it. Although, under the Affordable Care Act, the federal government has taken steps to promote new compensation methods, private-sector health plans have the flexibility and experience that allow them to take the lead in developing and refining methods to respond to changing needs.

Health plans can compensate healthcare providers in a number of ways:

- Fee-for-service
- Pay-for-performance
- Care coordination
- Shared saving (including gainsharing)
- Bundled payments
- Global payments

In this section we will also look at a healthcare structure strongly promoted by the Affordable Care Act—the accountable care organization (ACO).

Fee-for-Service

Fee-for-service is still the predominant payment model for medical services in the United States.¹⁷ Under the FFS model:

- The insurer pays the provider a fee for any services she renders that are covered by the insurance (after any deductibles or other cost-sharing are satisfied by the consumer, and according to the terms of the policy).
- The consumer can go to any doctor or other healthcare provider she chooses. She does not have to use a network provider or pay more out-of-pocket for a non-network provider. She does not need a referral to see a specialist.

- There may be little coordination between providers, as providers are not compensated for any time spent on this.

FFS gives consumers great freedom to choose providers and obtain services. Many providers like being paid for each service they perform. There is little guesswork in calculating how much they will receive. But there are also disadvantages to this approach.

- **Incentives.** Under FFS providers have a financial incentive to increase the scope and number of services—the more services they provide, the more they earn, whether the services improve the health of the patient or not. In other words, the incentives in the system favor volume over value.¹⁸ Patients, too, have little reason not to seek more services, since they pay little of the cost.
- **Lack of coordination.** Because there is little coordination among providers, a patient may receive many services but not see improvement. The quality of care can be poor.
- **Little accountability.** Providers are paid for services, whether those services are high-quality and whether they result in positive outcomes. In other words, the provider bears no financial risk for her decisions.

***Example:** James suffers from a lingering sore throat. He goes to Dr. Sanchez, who orders a number of tests, all of which are negative. Not satisfied, James goes to another doctor his sister recommends. This doctor orders some of the same tests, but does not identify the problem. Money has been wasted on duplicate tests, and James is no better. Under another system, Dr. Sanchez would have recommended a specialist and coordinated efforts with her, so that James' illness could be resolved quickly and cost-effectively.*

Pay-for-Performance

The **pay-for-performance** approach rewards providers for delivering high-quality care and so motivates quality improvement.¹⁹

- The typical P4P program provides a bonus to providers if they meet or exceed agreed-upon measures of quality or performance. (For a description of such measures, see below. A program may also reward improvement in performance over time, such as year-to-year decreases in the rate of avoidable hospital readmissions.
- P-4-P programs can also impose financial penalties on providers that fail to achieve specified goals or cost savings. For example, a plan might not pay for a patient that contracts a bed sore during a hospitalization because this is preventable.²⁰

P4P programs are typically added to a base FFS structure. In other words, providers are paid mostly on a FFS basis, but may also receive bonuses or be charged penalties based on P4P.

Quality Measures Used in Pay-for-Performance Programs

- **Process measures** assess the performance of activities that have been demonstrated to contribute to positive health outcomes for patients. Examples: whether aspirin was given to heart attack patients; whether patients were counseled to quit smoking.
- **Outcome measures** refer to the effects that care had on patients. Example: whether a patient's diabetes is under control based on laboratory tests.

- **Patient experience measures** assess patients' perception of the quality of care and their satisfaction with the care experience. Examples: how patients perceived communication with their doctors; whether they found their hospital rooms clean and quiet.
- **Structure measures** relate to the facilities, personnel, and equipment used in treatment. Example: whether certain health information technology has been adopted.

The advantages of pay-for-performance:

- Because a P4P program is generally added on top of a fee-for-service system, it may be a good first, partial step away from pure FFS for some plans.
- P-4-P can supplement efforts that seek to promote quality by encouraging consumers to choose higher-quality providers. Such efforts are generally not sufficient, especially where those providers perceived to be the best cannot take any more patients.²¹

The disadvantages:

- Providers who are not prepared to meet goals may be put at risk of financial loss.
- P4P programs often focus on certain populations (such as diabetics) and neglect others.
- P4P may encourage condition-specific treatment when integrated care management might be most efficient because of high rates of comorbidity.²²
- Measures may not be under the control of the provider. Outcome measures often depend in large part on patient behavior or other factors, not provider performance. However, many structure and process measures are under the provider's control, and according to one study, most programs focus on these.²³
- Some programs, by paying bonuses for the attainment of certain measures, in effect reward past performance, not current improvement efforts.²⁴

Care Coordination and Medical Homes

Under the **care coordination payment model**, in addition to regular fee-for-service compensation, health plans pay providers monthly care coordination payments. These payments are to be applied to enhancing care coordination in some way, such as investing in technology to facilitate the use of electronic medical records or developing disease registries to help track and manage patient care. Health plans may also hire care coordinators to work with providers and make available to them a variety of care coordination services.

A health plan may provide care coordination payment and services to providers organized in a **patient-centered medical home (PCMH)**. This is a model of healthcare designed to improve the quality and continuity of care delivered by primary care physicians and specialists.²⁵ For more on PCMHs, see below.

The Patient-Centered Medical Home (PCMH)

A **patient-centered medical home** is not a place but rather a model of healthcare. Patients are served by a team of providers who may be housed together or may simply be virtually linked. A PCMH has five main attributes:

- **Patient-centered.** Care is relationship-based, oriented to the whole person, and respectful of each patient's unique needs.
- **Comprehensive care.** The PCMH is accountable for meeting the majority of each patient's healthcare needs including wellness, acute care, chronic care, and mental healthcare.
- **Coordinated care.** Primary care, specialty care, hospitals, home healthcare, and community services and supports are all coordinated.
- **Excellent access to care.** Services are easily accessible, with shorter waiting times for urgent needs, enhanced in-person hours, round-the-clock telephone or electronic access to a member of the care team, and alternative methods of communication such as email and telephonic care.
- **Systems-based approach to quality and safety.** A variety of tools are used to enhance and improve quality: evidence-based medicine, clinical decision-support tools, shared decision-making with patients and families, response to patient experiences and patient satisfaction, performance measurement and improvement, and population health management.²⁶

Like pay-for-performance, care coordination is added on top of a FFS structure. This is good in that it can serve as a first step away from pure fee-for-service. But will plans in fact make that move? And if so, how long will it take? A small but growing number of studies suggest that care coordination payments in the context of a PCMH can improve patient care and lower healthcare costs.²⁷ An example of this model resulting in reduced hospital admissions and fewer emergency room visits is Geisinger Health Systems of Pennsylvania.²⁸ For more on another example, Horizon Healthcare Services, see below.

A Care Coordination/PCMH Success Story: Horizon Healthcare Services of New Jersey

Horizon Healthcare Services of New Jersey offers care coordination financial incentives and support tools to participating medical homes. This program began in 2009, serving only diabetes patients. The plan paid practices a quarterly care coordination fee in addition to fee-for-service reimbursement. Practices were also eligible for bonus payments if they achieved certain measures of diabetes management and smoking cessation.

Initially, there were problems. Most medical homes did not meet goals. And although the program reduced overall patient costs, the reduction was not enough to cover all program costs. In response, Horizon decided to expand the program beyond diabetes patients to all plan members served by eight medical homes. Horizon also provided care coordinators to these medical homes. As a result, early analyses indicate that patients are benefiting compared to patients not in the program.

This is an example of how private health plans, with their ability to adjust and improve programs as they are implemented and operated, can lead the way in this area.²⁹

Shared Savings

Under the **shared savings model**, if providers achieve savings in providing care to a specified population—that is, if the cost of care turns out to be less than a stipulated benchmark level—they

receive a portion of that savings. That is, they receive payments in addition to their FFS compensation. Shared savings agreements between providers and health plans or other payors vary. They may include upside risk only or both upside and downside risk.

- In **upside risk** providers are rewarded if costs are under the benchmark.
- In **downside risk** providers pay a financial penalty if costs exceed the benchmark.³⁰

To be able to achieve the goals of a shared savings agreement, many providers must make considerable changes and investments (in care management, technology, etc.). Providers often lack the resources to do so. Some health plans provide support in the form of technology and personnel (plan employees working for providers on care management and patient outreach). Often a program will begin with only upside risk. This enables providers to assess and improve their capabilities so that they can achieve benchmarks before they must pay penalties for failing to do so.

A number of concerns have been raised about shared savings agreements:

- Once again, a shared savings agreement does not replace a fee-for-service system, it only supplements it.
- Shared savings payments are often insufficient to cover providers' cost of making changes, so if the health plan does not supply other support, such an agreement may not be in providers' interest.
- When a medical practice includes multiple providers, some providers may reap the rewards without making investments and changes. However, programs can be structured to address this problem.
- The achievement of benchmarks may depend on factors outside the control of providers, such as patient behavior. However, there are actions providers can take to help patients stick to treatment plans and learn how to self-manage their care.
- While health plans may wish to focus on chronically ill patients because of the potential for savings, providers may be reluctant to enter in shared savings agreements for these populations, because of the time and resources they require. In response to this problem, some plans have sought to make such agreements more attractive by making them risk-adjusted and offering more support.
- Shared savings programs often realize savings in the first few years, but then their impact diminishes. But programs can be adjusted or a transition made to other payment models.

Gainsharing agreements are a limited form of shared savings. They focus on a narrow set of services or costs. For example, a gainsharing arrangement might share savings with providers who are able to move patients from a high-cost medication to an equally effective but less expensive drug.

Bundled Payment

The **bundled payment model** provides a single payment of a set amount to a provider (or group of providers) for all the healthcare services associated with a defined episode of care. This is sometimes called the episode or episode-based payment model. A bundled payment may be made for certain procedures, such as a knee replacement; for care for a medical event, such as a heart attack; or for care related to a chronic condition (such as diabetes) for a certain period of time.

Bundled payments are for care related to these specific conditions; compensation for other services is by fee-for-service.

A bundled payment model can include various adjustments:

- Allowances to cover increased costs if complications occur
- Risk adjustments for older or sicker populations
- Payment amounts based on the cost of adhering to clinical standards of care (rather than historical experience)
- Performance incentives

Bundled payments offer several advantages:

- Because providers receive the same amount regardless of the number of services they deliver, they have no financial incentive to deliver more than necessary. They also have an incentive to eliminate redundant services and avoid unnecessary costs such as hospital readmissions.
- Where payments are based on clinical standards of care (rather than current patterns of care, which often reflect misuse, underuse, or overuse of care), adherence to such standards is encouraged.
- Quality is improved by better coordination of care among providers and a better focus on patient outcomes (to avoid readmissions and other costs). There is greater flexibility in how and where some care is delivered across a group of providers.
- Compensation is simpler, as there is one bill (rather than many) for the health plan to pay.

There are also concerns about the bundled payment model.

- As with the other models we have discussed so far, the bundled payment approach is added onto a fee-for-service system and does not replace it.
- While for some conditions it is easy to define the boundaries of an episode for care, for others it is more complicated. Some types of care may not be suited for this model.
- If a catastrophic event is considered part of an episode of care, providers could incur enormous costs. However, health plans have sought to make adjustments in agreements to protect providers from such risks.
- Some fear that providers may seek to cut costs by limiting patient access to specialists. But quality and patient satisfaction measures can be used to make this less likely.

Global Payment

Under the **global payment model**, providers receive, in advance, a fixed payment for all or most of the services that they provide to a group of patients over a contract period. This model is sometimes called population-based payments, comprehensive care payments, and risk-based capitation.

Unlike the other models we have discussed so far, global payment is not an add-on to fee-for-service—it replaces it. Global payment is a type of capitation, although it differs in some ways from traditional capitation. For instance, today's global payment agreements usually offer bonuses based on meeting quality, utilization, and patient satisfaction measures. This helps to ensure that providers do not withhold needed care to hold down costs.

The global payment model is generally used for large organizations that can provide a wide array of healthcare services to a large population. Smaller organizations and medical practices are not able to deliver all the services needed, and they are not able to take on the financial risks typically involved in a global payment agreement.

A global payment agreement can have two levels of risk for providers:

- **Full risk model.** If an organization's cost of providing care is less than the global payment it receives, it keeps the entire difference; but if costs exceed the payment, the organization must bear the entire loss.
- **Shared risk model.** The healthcare organization and the health plan (or other payor) share both the savings (if costs are lower than projected) and the losses (if they are higher).

The global payment model has a major advantage: It is in the healthcare organization's interest to avoid unnecessary services and costs, as the organization will bear those costs (at least in part). But there are also concerns about this model:

- A healthcare organization can underestimate costs and agree to a payment amount that does not cover the cost of providing care, leaving it in financial distress.
- Because this model is suitable only for large organizations, it tends to encourage provider consolidation, which may decrease competition.
- Organizations compensated by global payment tend to encourage patients to seek services within the organization; some consumers may want greater freedom to choose providers.

Accountable Care Organizations

An **accountable care organization (ACO)** is a group of doctors, hospitals, and other providers that come together to give coordinated, high-quality care. An ACO agrees to be accountable to payors (health plans, the government, and others) for quality, health outcomes, and cost. In other words, the ACO is compensated based on these factors, typically through global payments or shared savings agreements.

ACOs seek to ensure and improve quality and outcomes while holding down costs in a variety of ways:

- An emphasis on coordinated care across a spectrum of providers
- A greater awareness by plan members of their health and how to take charge of it
- A strong emphasis on primary care, preventive care, and evidence-based medicine
- Decision-making shared by providers, members, and sometimes members' families

- Substantial use of electronic medical records (EMRs) to facilitate care and communication among providers

The ACO model offers health plans opportunities to leverage their expertise to both improve the quality of care and hold down costs. These opportunities include:

- The deployment of medical management techniques beyond preventive care
- The reduction of unnecessary spending and waste
- Increased member satisfaction
- Increased administrative cooperation and clinical integration

More information on ACOs can be found in the AHIP courses *Accountable Care Organizations (ACOs)* and *Understanding Private Accountable Care Organizations (ACOs)*

Summary

There are many types of health plans and healthcare organizations that work with health plans. Health plans can be structured in different ways and combined into an integrated delivery system (IDS). Third-party administrators (TPAs) and utilization review organizations (UROs) provide services to health plans. Healthcare providers form organizations of various sorts to offer care to consumers and contract with health plans. Some companies provide support services to providers and provider organizations.

Health plans are commonly sponsored by insurance companies, investor groups, other health plans, providers, and employers. Government, medical schools and universities, consumer groups, and labor unions may also play a role.

There are several methods by which healthcare providers are compensated by health plans and other payors. The tradition system is fee-for-service, and this remains predominant. Pay-for-performance, care coordination, shared saving, and bundled payments can be added to and supplement an FFS system. Global payments actually replaces fee-for-service.

Sources

¹ Healthcare.gov, “Glossary,” <https://www.healthcare.gov/glossary/exclusive-provider-organization-EPO-plan/>.

² Joseph F. Stenken, “19 Things to Know About Medicare Advantage,” *Life Health Pro*, August 21, 2014, http://www.lifehealthpro.com/2014/08/21/19-things-to-know-about-medicare-advantage?page_all=1.

³ Jenny Gold, “Accountable Care Organizations, Explained,” *Kaiser Health News*, September 14, 2015, <http://khn.org/news/aco-accountable-care-organization-faq/>.

⁴ Health Insurance.org, “Glossary,” <https://www.healthinsurance.org/glossary/consumer-operated-and-oriented-plan/>.

⁵ Louise Norris, “CO-OP Health Plans: Patients’ Interests First,” *Health Insurance.org*, October 21, 2016, <https://www.healthinsurance.org/obamacare/co-op-health-plans-put-patients-interests-first/>.

⁶ D. Andrew Austin and Thomas L. Hungerford, “The Market Structure of the Health Insurance Industry,” Congressional Research Service, April 8, 2010, p. 22, <http://www.ncsl.org/documents/health/MrktStrOfHlthIns.pdf>.

-
- ⁷ Bob Herman, “Why Venture Capital Firms Are Pouring Money into Health Insurance,” *Modern Healthcare*, March 19, 2016, <http://www.modernhealthcare.com/article/20160319/MAGAZINE/303199964>
- ⁸ David B. Muhlestein and Nathan J. Smith, “Physician Consolidation: Rapid Movement from Small to Large Group Practices, 2013-15,” *Health Affairs*, 35: 9 (2016), 1638-42, <http://content.healthaffairs.org/content/35/9/1638>; See also Kevin N. Fine, “A Look at Vertical Integration Between Payers and Providers,” *Physician’s Money Digest*, May 25, 2016, <http://www.hcplive.com/physicians-money-digest/practice-management/a-look-at-vertical-integration-between-payers-and-providers>.
- ⁹ Austin and Hungerford, pp. 23-24.
- ¹⁰ Emblem Health, “About Us,” <http://www.emblemhealth.com/About-Us/Press-Room/Press-Releases/EmblemHealth-City-Partnership.aspx>.
- ¹¹ UPMC, “About,” <http://www.upmc.com/about/why-upmc/Pages/story.aspx>
- ¹² Brent Snively, “How UAW’s Risky Health Care Experiment Became a Success,” *Detroit Free Press*, Sept. 9, 2015, <http://www.freep.com/story/money/business/2015/09/08/how-uaws-risky-health-care-experiment-became-success/71633988/>.
- ¹³ Muhlestein and Smith.
- ¹⁴ James Ellis and Aaron Razavi, “Three Reasons Why Hospital Mergers Are Advantageous,” *Healthcare Finance News*, February 8, 2012, <http://www.healthcarefinancenews.com/blog/3-reasons-why-hospital-mergers-are-advantageous>.
- ¹⁵ Gregory Curfman, “Everywhere Hospitals Are Merging: But Why Should You Care? *Harvard Health Publications*, April 1, 2015, <http://www.health.harvard.edu/blog/everywhere-hospitals-are-merging-but-why-should-you-care-201504017844>.
- ¹⁶ Marsha R. Gold, Robert Hurley, and Timothy Lake, “Provider Organizations at Risk: A Profile of Major Risk-Bearing Intermediaries, 1999,” *Health Affairs*, March/April 2001, pp. 175-185, <http://content.healthaffairs.org/content/20/2/175>; Gunjan Khanna, Ebben Smith, and Saum Sutaria, “Provider-led Health Plans: The Next Frontier – or the 1990s All Over Again,” McKinsey & Company Healthcare Systems and Services Practice, 2015. <http://healthcare.mckinsey.com/provider-led-health-plans-next-frontier%E2%80%94or-1990s-all-over-again>; Paul Keckley, “Four Things Providers Should Consider Before Offering a Health Plan,” *Hospitals & Health Networks (H&HN)*, January 4, 2016, <http://www.hhnmag.com/articles/6829-four-things-providers-should-consider-before-offering-a-health-plan>
- ¹⁷ Robert A. Berenson and Eugene C. Rich, “U.S. Approaches to Physician Payment: The Deconstruction of Primary Care,” *Journal of General Internal Medicine* 24:6 (June 2010), pp. 613-618, <http://www.ncbi.nlm.nih.gov/pmc/articles/PMC2869428>
- ¹⁸ Nicole C. Lallemand, “Reducing Waste in Health Care” (Health Policy Brief), *Health Affairs*, Dec. 13, 2012, p.4.
- ¹⁹ M.B. Rosenthal, R. Fernandopulle, H.R. Song, and B. Landon, “Paying for Quality: Providers’ Incentives for Quality Improvement,” *Health Affairs* 23:2 (March/April 2004).
- ²⁰ “Pay-for-Performance” (Health Policy Brief), *Health Affairs*, Oct. 11, 2012, pp 1-2.
- ²¹ Rosenthal et al.
- ²² Ibid.
- ²³ Ibid.
- ²⁴ Ibid.
- ²⁵ K. Grumbach, T. Bodenheimer, and P. Grundy, “The Outcomes of Implementing Patient-Centered Medical Home Interventions: A Review of the Evidence on Quality, Access, and Costs from Recent Prospective Evaluation Studies,” Patient-Centered Primary Care Collaborative, August 2009, www.pcpc.net
- ²⁶ U.S. Department of Health and Human Services, Agency for Healthcare Research and Quality, “What Is a PCMH?” http://pcmh.ahrq.gov/portal/server.pt/community/pcmh_home/1483/what_is-Pcmh
- ²⁷ Grumbach et al.

²⁸ R. Paulus, K. Davis, and G. Steele, “Continuous Innovation in Health Care: Implications of the Geisinger Experience,” *Health Affairs* 27:5, (Sept./Oct. 2008), pp. 1235-1245.

²⁹ U.B. Patel, C. Rathjen, and E. Rubin, “Horizon’s Patient-Centered Medical Home Program Shows Practices Need Much More Than Payment Changes to Transform,” *Health Affairs* 31: 9 (Sept. 2012), pp 2018-2027.

³⁰ Harriet Walsh, “Affordable, Quality Care: Improving Health and Health Spending,” presentation Dec. 5, 2012 at 3rd Annual Healthcare Web Summit.

4B Health Plan Structures and Arrangements, Part II

Lesson Objectives

After completing this lesson, you should be able to:

- Discuss the concerns of regulators about health plan structures and arrangements.
- Compare and contrast the different types of health plan integration.
- Describe how employers self-fund and self-administer health benefits.
- Describe other employer arrangements: direct contracting, purchaser alliances, and private exchanges.

Regulatory Issues

Federal and state regulators seek to ensure that, as health plans create structures and enter into various arrangements, the interests of consumers are protected and promoted.

- They examine how these structures and arrangements affect solvency, access to care, healthcare quality, benefits, networks, and provider contracting.
- They are also concerned about structures and arrangements that might reduce competition.

As health plan structures and arrangements evolve, regulators constantly adapt. They monitor developments and make changes as necessary, and they have created more sophisticated regulatory tools. For instance, regulators have developed a risk-based capital formula for health plans based on the level of risk assumed, as well as a uniform licensing law for health organizations assuming insurance risk. Such innovations have been created both by individual states and the National Association of Insurance Commissioners (NAIC).

At the same time, the evolution of health plans is shaped by the regulatory framework. For instance, in states where provider-sponsored organizations are not subject to HMO licensing and solvency requirements, providers may be more inclined to establish a PSO. In states where PSOs are subject to such requirements, providers may choose instead to enter into strategic partnerships with licensed HMOs.

In recent years, factors have pushed both health plans and providers to consolidate:

- ACA incentives to provide efficient, high-quality care (which are likely to be carried over if the ACA is repealed and replaced)
- Tighter reimbursement for hospitals and tighter margins for plans
- Implementation of technology (including electronic health record systems and telemedicine), requiring capital

But federal and state regulators are concerned that mergers, acquisitions, and alliances could result in a higher concentration of both plans and providers, leading to less competition and ultimately higher premium rates and fewer choices for consumers.¹ They question whether some consolidations will benefit consumers through improved quality, integrated care, or decreased

costs.² At the federal level, the U.S. Department of Justice (DOJ) has tended to concern itself with mergers, acquisitions, and alliances involving health plans, while the Federal Trade Commission (FTC) has focused on providers. State regulators and attorneys general are also involved. An example: The DOJ recently blocked proposed mergers of four of the nation's top five health plans.

Integration

Integration is the combination of two or more endeavors into a single endeavor, which results in previously separate entities sharing the risks and rewards of the combined endeavor. Health plans may pursue integration in an effort to better meet customer needs and compete in the marketplace, by joining with organizations that have strengths matching their weaknesses or simply by becoming larger.

Under antitrust law, the three basic types of integration are:

- Horizontal integration
- Vertical integration
- Conglomerate integration

Horizontal integration is a combination of two or more entities that are in the same business segment and that may directly compete with one another.

***Examples:** One health plan acquires another. Two medical groups operating in the same market engage in a joint venture.*

This type of integration has received the most regulatory scrutiny. The DOJ and FTC have jointly issued and rely on a series of guidelines last updated in 2010. Actions are taken based not on one single factor, such as market share, but on a fact-specific process.³

Vertical integration is a combination of two or more entities in different segments of the same industry. The entities may be economically related, but they are dissimilar in nature and do not substantially compete with one another.

***Examples:** A joint venture between a health plan, a hospital, and a medical group; the merger of a hospital and a medical group.*

The ACA and market changes (such as the continued movement toward value-based payments) have increased vertical integration. There have been mergers between physician groups and hospitals and between insurers and providers. Vertical integration is seen as way of controlling costs while improving quality.

Vertical integration, like horizontal integration, is subject to review by the both the DOJ and the FTC as well as state attorneys general under state antitrust laws. As with horizontal integration, the actions of the federal government are governed by guidelines. Regulators consider whether a vertical integration has the potential to eliminate specific potential entrants into the market, create barriers to market entry, or harm downstream rivals of the merged firm.⁴

The DOJ has challenged the use of **most favored nation (MFN)** (or most favored customer) clauses in health plan contracts with providers. In such clauses, a seller (in this case the provider) promises a buyer (the health plan) that it will not offer another buyer better terms before offering those terms or better terms to the first buyer. In other words, a provider contracting with a health plan agrees

not to give another health plan better terms. Such clauses can make it more difficult for other health plans to enter into a market.⁵

Conglomerate integration is a combination of entities that produce unrelated products or services. Such integration can create economies and efficiencies and make it easier to access capital.

***Example:** A holding company that owns various types of businesses in different industries acquires a health plan.*

So far, conglomerate mergers involving health plans have attracted less antitrust attention. But antitrust guidelines in this area continue to evolve.⁶

Whether integration is horizontal, vertical, and conglomerate depends on *what entities* are integrating. Now let's look at *how* they integrate.

- **Structural integration** consists of acquisitions and mergers of entities or formal partnerships or joint ventures between them. Separate businesses create a legal entity that is either a single organization or a combination of two or more organizations under common ownership and control.
- **Virtual integration** does not involve creating a single entity or affiliated organizations under common ownership and control; it requires only that organizations work closely together.⁷ The organizations enter into contractual arrangements but remain separate. Virtual integration includes strategic partnerships and outsourcing.

Whether integration is horizontal, vertical, or conglomerate, structural or virtual, it is unlikely to succeed without **operational integration**. This occurs when activities are consolidated to realize efficiencies and coordinated to achieve common goals. For instance, healthcare organizations that are integrating seek to coordinate services to provide the most efficient and effective care.⁸ In merged organizations, operational integration may include placing all lines of business under a consolidated budget, unifying governance and management, and coordinating and making compatible all business functions (such as information systems, accounting, billing, utilization management, and quality management).

An issue that arises in integration is **span of control**—the number of people or activities being managed.

- If a leadership team is charged with managing a widely disparate set of functions, it may not be very effective.
- On the other hand, if there is no central control over all parts of an organization, they will not work together to achieve the organization's strategic goals.

Employer Self-Funded Plans

Employers wanting to help meet employees' healthcare needs have four main options:

- Purchase coverage from a health plan
- Self-fund (self-insure) their own benefit program
- Contract with providers to deliver healthcare services for employees

- Hire providers to deliver healthcare services to employees

In this section we examine self-funded plans; in the next, we will look at contracting with providers.

In a **self-funded (self-insured) plan**, an employer pays benefits to help cover employees' healthcare expenses out of its own funds. It takes on the financial risk of having to pay very large amounts. Most large employers that provide healthcare benefits to employees self-fund (82 percent of those with 500 or more employees according to a study by the U.S. Department of Health and Human Services).⁹

When an employer self-funds, the usual administrative functions of a health plan—eligibility, enrollment, claims, etc.—must be performed.

- An employer can handle these functions itself, in a **self-administered plan**.¹⁰
- Or an employer can contract with a health plan or third-party administrator to perform some or all of them under an **administrative services only (ASO) contract** (so called because the health plan or TPA only provides services, it is not liable for claims as under an insurance contract).¹¹

Self-administration is relatively simple for traditional indemnity-type benefits. But for a modern health plan—which handles healthcare management functions such as provider contracting, utilization review, and medical management—it is much more complicated.

An employer can pay a health plan or TPA administering its self-funded plan in various ways. There might be a flat monthly fee for all services, or a main monthly fee plus additional fees for certain services (such as network administration and utilization management). Since the employer pays benefit costs, the plan or TPA bills the employer for the actual amount it paid out for claims (or other provider compensation, such as capitation).

Most employers that self-fund do so for financial reasons or to gain more control over benefit design.¹²

- A self-funding employer has at its disposal funds that would otherwise be tied up in claim reserves held by the health plan.
- The self-funding employer does not pay a health plan's risk charge (the portion of the premium that is collected to cover the risk that actual medical costs might be higher than anticipated)
- Premium taxes that apply to health plans in some states do not apply to self-funded plans.
- Many state regulations, including benefit mandates and rules for premium rates, do not apply to self-funded plans, giving employers more freedom in designing benefit packages.

Why do some state regulations not apply to self-funded plans? The federal Employee Retirement Income Security Act of 1974 (ERISA) regulates most employer-sponsored benefit plans, including health plans. Under ERISA, a self-funded plan is not considered an insurer and so not subject to state insurance laws, as long as the employer is providing benefits only to its own employees. In this case state laws are said to be *preempted* by ERISA. However, ERISA has its own requirements for employer health plans (both insured and self-funded), which we will study in a later lesson.

An employer sponsoring a self-funded plan can pay benefits and expenses out of:

- a trust that holds reserves for the plan or
- its general resources and assets (a **nontrusteed plan**).

Employers can address the risk of having to pay unexpectedly large amounts in benefits by purchasing **stop-loss insurance**. Under stop-loss insurance, the employer covers all costs up to a certain level and the insurer covers any costs above that level. (However, some states have maintained that in certain situations a self-funded plan with stop-loss insurance constitutes insurance and is therefore subject to state insurance laws.)

Employers choose not to self-fund for two main reasons:

- The risk of having to pay for very large amounts in healthcare costs
- The time, effort, and financial resources needed to establish and operate a self-funded plan

Also, smaller employers cannot generally handle administrative functions on their own. They can contract with a health plan or TPA to perform them, but the cost of doing so can outweigh the financial gains of self-funding. (However, in recent years the administrative requirements of the ACA have made self-funding more attractive to smaller employers. It remains unclear whether this will remain the case if the ACA is repealed and replaced.)

Other Employer Arrangements

Employer Direct Contracting

In **employer direct contracting**, an employer—instead of going through a health plan—contracts directly with providers to deliver healthcare services to its employees. The employer generally contracts with an existing network of providers that also has contracts with other employers or health plans. However, some very large employers establish their own exclusive networks that serve only their employees.¹³ (Boeing is an example.¹⁴) In a variation, an employer sets up its own clinics that provide employees with primary care, and it contracts with a network for specialty care. However, this is viable only where there are large concentrations of employees in one locality.

Purchasing Alliances

In a **purchasing alliance**, multiple employers join together to purchase health coverage. Because the group offers health plans a large number of potential members, it has more power to negotiate plan features and premiums with health plans. Such alliances are also known as employer coalitions, employer purchasing coalitions, or health insurance purchasing cooperatives (HIPCs). A less common alternative is for employers to form alliances in contracting directly with providers. Alliances are often used by a large group of small employers to gain the negotiating strength of a large employer.¹⁵ But they are also used by medium and large employers in an effort to hold down costs and obtain other concessions from health plans.

The history of purchasing alliances has been mixed.

- The Pac Advantage Health Plan, a California-based alliance of small employers, was very successful in its early years, but it was forced to cease operations in 2006 as health plans withdrew because of losses.¹⁶

- On the other hand, 20 large employers (including American Express, Macy's, and Verizon) with about 4 million potential members have recently banded together to form the Health Transformation Alliance. If this initiative is successful in curbing costs, other large employers may seek to take similar actions.¹⁷

The Small Business Health Options Program (SHOP)—the ACA exchanges (marketplaces) for small employers—traces its lineage, in part, to earlier purchasing alliances.¹⁸ Whether SHOP will continue in a post-ACA environment remains unclear.

Private Exchanges

Private health insurance exchanges work much like the ACA marketplaces—consumers visit a website to learn about health plan options and enroll in one. The main difference is that private exchanges are sponsored and operated by non-government entities, such as insurers and health plans, employee benefit specialists, brokers, and technology companies.

Employers can use a private exchange to provide coverage to employees. They typically work like this.

- The employer contracts with the private exchange to provide coverage to its employees.
- The employer selects the products that the exchange will offer its employees. This might include a selection of health plans, such as an HMO, a PPO, and a high-deductible plan. Additional products—such as dental and vision care benefits and life and disability insurance—may also be offered.
- The exchange may be sponsored by a health plan organization and offer only that company's plans and products. Or it may be sponsored by a broker or benefit specialist organization and have offerings from several companies.

Once the employer has contracted with the exchange, employees can obtain coverage.

- Employees visit the website to review, select, and enroll in plans and products. There is also typically a call center to provide support.
- Employees can also use the exchange website and call-center as a year-round benefits resource.
- The employer makes a contribution to each employee's premiums. This is typically a defined contribution—that is, a fixed amount such as a flat dollar figure or a percentage of the cost of a standard plan.
- Employees pay any cost above the employer contribution; this gives them an incentive to enroll in lower-cost plans but the freedom to choose a more generous and expensive plan if they wish.
- The employer receives a list of employees and the products they have selected and a single bill for all employee enrollments.¹⁹

What is the role of health plans in a private exchange? A health plan organization may:

- Offer its plans and products on an exchange operated by another entity (such as a benefit specialist), where it will compete with other companies for consumer/employee enrollments.
- Establish its own exchange offering only its own plans and products and offer this exchange to employers.
- Do both.

There are other differences between public exchanges (primarily ACA marketplaces) and private exchanges, as shown in the table below.

Private vs. Public Exchanges²⁰

	Public	Private
Sponsorship	Government entity	Insurers and health plans, employee benefit specialists, brokers, technology companies
Offerings	Generally only health plans	In addition to health plans, sometimes dental and vision care benefits, life and disability insurance
Funding/subsidies	In ACA individual marketplaces, lower-income consumers may be eligible for subsidies	Employees receive employer contribution (usually a defined contribution)
Shopping and support experience	Exchange is primarily for enrollment	Exchange is year-round benefits resource

Private exchanges are likely to expand for several reasons:

- They ease many administrative burdens of employers (many of which may continue if the ACA is repealed and replaced).
- They give employees choice and flexibility in using their employer's contribution.
- Evidence has shown that participating employers have saved healthcare dollars.²¹

Factors in Health Plan Structures and Arrangements

We conclude this lesson by noting the most important broad factors that determine the structure and arrangements of a health plan:

- The **local environment and market**. In one area, a hospital or hospital system may be powerful; in others, physicians. In one, a particular health plan may be dominant; in another, a large employer. Or there may be no single dominant entity.
- The **strategic objectives** of an organization.

***Example:** A hospital sponsors a management services organization as a way to encourage formation of a physician medical group and to strengthen the relationship between the hospital and physicians. The hospital also expects that at some point in the future the medical group will integrate with the hospital to form an integrated delivery system. Consequently, the hospital is willing to take a loss on the MSO venture so it can achieve a longer-term strategic goal.*

- **Provider involvement**. Some health plans offer ownership interests to participating providers. Ownership creates a strong financial incentive for providers to effectively manage care and can enhance physician loyalty to the plan. (However, while this strategy can be advantageous to both the plan and providers, it is subject to regulatory scrutiny, in part because of concerns that financial incentives might influence the type and amount of healthcare services provided.)
- **Responses to market condition**.

***Example:** A provider group decides that it needs to enter into a contract-based arrangement to respond to competition from other types of health plans. Once it establishes a foothold, it may later decide to enter into a more structurally integrated approach, such as a physician-hospital organization or an integrated delivery system.*

Summary

Regulators are concerned about how health plan structures and arrangements may affect the interests of consumers and the competitiveness of the market. Business entities in the same market segment may join together in horizontal integration, those in different segments of the same industry in vertical integration, and those in unrelated industries in conglomerate integration. Structural integration involves mergers, acquisitions, and common ownership and control; virtual integration does not, simply two entities working closely together.

An employer may sponsor a self-funded health plan for financial reasons and to gain more flexibility in plan design. An employer may contract directly with providers to serve its employees. Multiple employers may join forces in a purchasing alliance to negotiate with health plans. An employer may provide coverage to employees through a private exchange.

Sources

¹ Arthur N. Lerner, "Mergers: Antitrust Issues for Hospitals and Health Plans," American Health Lawyers Annual Meeting 2008, https://www.crowell.com/documents/Mergers_Antitrust-Issues-for-Hospitals-and-Health-Plans_Lerner.pdf; Douglas Hervey, David Muhlestein, and Austin Bordelon, "How Might Proposed Payer Mergers Impact State Insurance Markets?" *Health Affairs Blog*, December 1, 2015,

<http://healthaffairs.org/blog/2015/12/01/how-might-proposed-payer-mergers-impact-state-insurance-markets/>

² Richard J. Zall, "Managing Risk in Today's Healthcare M&A Transaction," Healthcare Financial Management Association, April 2016, pp. 1-7, http://www.proskauer.com/files/News/4f7e2e2d-51f7-4e46-8af3-2115e57f2a52/Presentation/NewsAttachment/f4b13c59-f81c-41ad-8e37-dae53a31d5f3/0416_HFM_Zall.pdf

³ Carl Shapiro, "The 2010 Horizontal Merger Guidelines: From Hedgehog to Fox in Forty Years," *Antitrust Law Journal*, 77 (2010), p. 701 et seq., <https://www.law.berkeley.edu/wp-content/uploads/2015/04/Shapiro-The-2010-Horizontal-Merger-Guidelines-From-Hedgehog-to-Fox-in-40-Years-2010.pdf>

⁴ United States Department of Justice, "Non-Horizontal Merger Guidelines," <https://www.justice.gov/atr/non-horizontal-merger-guidelines> ; See also, Steven C. Salop and Daniel P. Culey, "Revising the U.S. Vertical Merger Guidelines: Policy Issues and an Interim Guide for Practitioners," *Journal of Antitrust Enforcement* 4:1 (Dec. 2015), pp. 1-41, <http://antitrust.oxfordjournals.org/content/4/1/1.short?rss=1>

⁵ Jaime Stilson and Roger Feldman, "Vertical Mergers in Healthcare: Evaluating (and Avoiding) Potential Antitrust Risks," Minnesota State Bar Association: Antitrust and Health Law Sections, February 18, 2015, <https://www.mnbar.org/docs/default-source/sections/vertical-mergers-cle---final.pdf?sfvrsn=0>

⁶ John Vanderstar, "Conglomerate Mergers: The Developing Antitrust Guidelines," *St. John's Law Review*, 44:5 (Dec. 2012), p. 595 et seq., <http://scholarship.law.stjohns.edu/cgi/viewcontent.cgi?article=3498&context=lawreview>

⁷ Jenny Grant, "What Does It Take to Make Integrated Care Work?" McKinsey & Company, January 2010.

⁸ Thomas Peck, "Operational Integration: Creating A High-Performing Healthcare Organization," White Paper presented by Em+are, <https://www.emcare.com/resources/white-papers/pdf/operational-integration-whitepaper.pdf>

⁹ U.S. Department of Health and Human Services in collaboration with the U.S. Department of Labor, "Report to Congress on a Study of the Large Group Market," <https://aspe.hhs.gov/sites/default/files/pdf/76181/index.pdf>

¹⁰ Albert Risk Management Consultants, "Is Self-Administration Ever the Right Answer?" IRMI, <https://www.irmi.com/articles/expert-commentary/is-self-administration-ever-the-right-answer>

¹¹ Steve Benson, "What Is Administrative Services Only (ASO) – The Facts," *Self-Funding Magazine*, Oct. 25, 2010, <http://www.selffundingmagazine.com/article/what-is-administrative.html>

¹² Alan Goforth, "The Rise of Self-Funding," *Benefits Pro*, March 9, 2015, <http://www.benefitspro.com/2015/03/09/the-rise-of-self-funding>

¹³ Alan Lyles, Jonathan P. Weiner, Andrew D. Shore, et al, "Cost and Quality Trends in Direct Contracting Arrangements," *Health Affairs* 21:1 (2002), pp. 89-102, <http://content.healthaffairs.org/content/21/1/89.full.pdf+html>

¹⁴ Marty Stempniak, "Will Boeing Change Health Care?" *Trustee Magazine*, Dec. 15, 2015, <http://www.trusteemag.com/articles/1020-will-boeing-change-health-care> ;

¹⁵ James M. Pool and Lauren C. DeMoss, "Are Purchasing Pools the Answer to Small Employers' Health Insurance Needs?" *ABA Health eSource* 5:7 (March 2009), http://www.americanbar.org/content/newsletter/publications/aba_health_esource_home/Pool.html

¹⁶ Ibid.

¹⁷ Louise Radnofsky, "Companies Form New Alliance to Target Health-Care Costs," *The Wall Street Journal*, Feb. 4, 2016, <http://www.wsj.com/articles/companies-form-health-insurance-alliance-1454633281>

¹⁸ Centers for Medicare and Medicaid Services, "Small Business Options Program," <https://www.cms.gov/CCIIO/Programs-and-Initiatives/Health-Insurance-Marketplaces/SHOP.html>

¹⁹ Akshay Kapur, Ashish Kaura, Minoo Javanmardian, and Paolo Borromeo, "The Emergence of Private Exchanges: Fueling the Consumerization of Employer-Sponsored Health Insurance," Booz & Co, 2012, <http://www.strategyand.pwc.com/media/uploads/Strategyand-Emergence-Private-Health-Insurance-Exchanges.pdf>

²⁰ Alan Cohen, “Private Exchanges:101,” The Institute for Healthcare Consumerism (IHC), 2014, http://www.theihcc.com/en/communities/health_care_exchange_solutions/private-exchanges-101_isui6nsu.html

²¹ Lola Butcher, “Four Reasons Why Private Exchanges Will Grow,” *Hospitals and Health Networks (H&HN) Magazine*, Feb. 16, 2016, <http://www.hhnmag.com/articles/6947-four-reasons-why-private-exchanges-will-grow>

5 Overview of Laws and Regulations

Lesson Objectives

After completing this lesson, you should be able to:

- Identify the sources of law in the United States.
- Name and briefly describe some of the laws that form the foundation of health insurance plan regulation.
- Name and briefly describe some of the recent laws governing health insurance plans.
- State the most important provisions of the Affordable Care Act of 2010.
- Identify the main areas of health insurance plan operations that are the focus of regulation.

Health insurance plans are heavily regulated, at both the state and federal level, and in this lesson we will take a broad look at the regulatory environment, examining the main laws and the principal areas of regulation.

Sources of Law in the United States

The term “law” is used broadly to refer to “the regime that orders human activities and relations through systematic application of the force of politically organized society.”¹ In the United States, law has a number of sources, including federal and state constitutions, statutes, administrative rules and regulations, case law, and model laws. In this section we will describe each of these sources of law, and throughout this course we will use the term “law” to include these various sources.

Separation of Powers

Both federal and state governments operate under a doctrine called **separation of powers**. Separation of powers means that a government is divided into branches, each of which primarily performs a distinct function. Constitutions in the United States provide for three branches of government—legislative, executive, and judicial—and establish a system of checks and balances to guard against any branch becoming too powerful. A branch of government performs a primarily legislative function when it enacts, or makes, laws; a primarily executive function when it enforces, or carries out, laws; and a primarily judicial function when it interprets laws and adjudicates disputes under the law.

Constitutions

A **constitution** sets forth in general terms the principles that form the legal foundation of a government. It describes the structure of the government, defines the extent of its powers, and outlines the principles on which it is to operate. In addition, a constitution usually guarantees citizens certain basic human rights. In the United States, there are two kinds of constitutions: the federal Constitution and the state constitutions.

Because these principles are intended to remain relatively stable and consistent over time, most constitutions include safeguards designed to prevent frequent or ill-considered changes. Necessary changes to a constitution that must be made to meet the needs of a changing society are generally accomplished through interpretation of its terms by the courts, rather than by amendment.

Statutes

A **statute**, or **act**, is “an act of the legislature declaring, commanding, or prohibiting something; a particular law enacted and established by the will of the legislative department of government.”² Statutes enacted by Congress are called either **federal statutes** or **acts of Congress**. Statutes enacted by state legislatures are called **state statutes**.

Administrative Rules and Regulations

Administrative rules, or **administrative regulations**, are prescribed guides for conduct or action created by an administrative agency that carry the force of law. Legislators cannot have the necessary technical knowledge about every policy or issue that must be regulated; consequently, legislatures have adopted the practice of enacting regulatory statutes in general terms. They then authorize the administrative officer having the duty of executing the law to fill in the details by enacting administrative rules and regulations that have the force and effect of law. Courts have held that this delegation of rulemaking power to administrative officers does not violate the separation of powers doctrine, as long as the rules so made are within the scope of the delegated power, are made following specified procedure, and are subject to judicial review. In addition to rulemaking, administrative officers also perform judicial functions. They hold hearings, make decisions, and impose penalties. Thus, administrative officers function in all three spheres: executive, legislative, and judicial.

In the context of health insurance plans, the commissioner, director, or superintendent of insurance is a state administrative officer. Today, state legislatures have authorized such officers to adopt appropriate rules and regulations to implement the provisions of the state insurance and health plan statutes. Often such administrative rules and regulations constitute a major part of the law governing the health insurance plan industry. Similarly, Congress authorizes various federal administrative officers to adopt appropriate rules and regulations implementing the provisions of certain federal statutes that affect health insurance plans. For example, the Centers for Medicare and Medicaid Services creates and adopts administrative rules and regulations to implement Medicare.

Sometimes the rules and regulations issued by administrative agencies do not provide enough detail for health insurance plans. Often, administrative agencies make informal policy documents available and provide guidance to health insurance plans trying to comply with such rules and regulations.

Case Law

A court applies laws to resolve the controversies brought before it. In so doing, a court interprets those laws and thus creates more law. In the United States, federal and state court decisions form a vast and growing body of **case law** known as **common law**. Federal case law is made up of federal trial court decisions, federal intermediate court decisions, and U.S. Supreme Court decisions. State case law consists of state intermediate and supreme court decisions, because most state trial court decisions are not published.

The term **common law** is sometimes used to distinguish **case law** (court decisions) from **statutory or regulatory law**. In that sense, the common law of a jurisdiction is made up of the decisions of the courts, but not the enactments of the legislature or administrative bodies.

Case law governing health insurance plans has evolved from the decisions that state and federal courts have made involving the laws and regulations that impact plans. For example, case law has been used to interpret how the provisions of the Employee Retirement Income Security Act of 1974 (ERISA) should be applied to employee benefit plans that are governed under ERISA.

Model Laws

A **model law**, also known as a **uniform law**, is an act proposed by experts in a particular field for consideration by the states; this model may (or may not) be adopted and enacted, in whole or in part, by a state or may serve as the basis for state legislation, particularly in a technical area. A model law or regulation itself is not a law and does not have the force or effect of law.

In the field of insurance, the **National Association of Insurance Commissioners (NAIC)**, a nongovernmental association of state insurance regulators, develops and adopts model laws and regulations for consideration by legislators and regulatory agencies in developing particular state laws and regulations. The model laws and regulations address subjects relating to health insurance plan financial requirements, quality assurance, utilization review, and statutory accounting standards. While model laws do not have the force of law, many of the NAIC's model laws and regulations serve as the basis for state or federal legislation or regulations and therefore can have a direct impact on health insurance plans.

Laws That Affect Health Insurance Plans

In this lesson we will identify and briefly summarize major laws that have a substantial impact on health insurance plans. The most important of these are discussed in more depth in later lessons.

Note that few of these laws were written specifically to address health insurance plans. Some focus on health insurance or health care more generally; some are even broader but include provisions impacting health benefits. Some have to do with the Medicare and Medicaid programs and so affect health plans that contract to serve the beneficiaries of these programs.

Federal Laws Affecting Health Insurance Plans

Part I: The Foundations

Antitrust Laws: The Sherman Act (1890), the Clayton Act (1914), and the Federal Trade Commission Act (1914)

To counter anti-competitive practices by some businesses, Congress enacted the Sherman Act in 1890, and it passed the Clayton Act and the Federal Trade Commission Act in 1914 to supplement and reinforce the Sherman Act. Historically, healthcare providers and provider groups have used antitrust laws against health insurance plans—they alleged antitrust violations to combat the development of organizations that they believed threaten their livelihood. As health insurance plans have evolved, entrepreneurs have created partnerships and affiliations among providers, payers, and employers, and these, along with mergers and acquisitions, have been subjected to antitrust scrutiny.

Health Maintenance Organization Act of 1973 (HMO Act)

The **Health Maintenance Organization Act of 1973** was intended to help contain rising healthcare costs by encouraging the development of HMOs. It was also the first major attempt by the federal government to influence the development of managed care plans (now known as health insurance plans), which previously had been governed largely by state laws not designed for such entities.

The HMO Act provides for optional federal qualification of HMOs and applies only to plans that choose to become federally qualified. To be qualified, a plan must meet certain requirements, and qualified plans are regulated by the Department of Health and Human Services to ensure quality of care. But federal qualification offers certain advantages:

- Federally qualified plans were (in the early years of the law) eligible for federal funds intended to promote the formation and development HMOs.
- Federally qualified plans are exempted from state laws with more stringent standards for HMO development.
- Employers with 25 or more workers sponsoring health coverage are required to offer a federally qualified HMO as a choice if the HMO requests it.³

Initially, many HMOs became federally qualified, but today the majority are not. Amendments to the HMO Act were passed from 1976 through 1995.

Employee Retirement Income Security Act of 1974 (ERISA)

Many larger employers, instead of purchasing a group health insurance policy for their employees from an insurer, provide employee health benefits out of their own funds. These self-funded or self-insured plans are not generally subject to regulation as insurance. The **Employee Retirement Income Security Act of 1974 (ERISA)** was enacted primarily to prevent abuse of employer-sponsored pension plans, but the law also sets standards for most employment-based health benefit plans, including self-insured plans. ERISA standards address documentation explaining benefits available under the plan and any modifications to those benefits, reporting requirements, and procedures for appeals of denial of claims. ERISA also sets standards that must be upheld by anyone who acts as a plan fiduciary. The protections provided by ERISA have been expanded by COBRA, HIPAA, and other legislation.

Omnibus Budget Reconciliation Act of 1981 (OBRA-81)

Medicaid is a federal-state program that pays for healthcare for certain low-income persons. From the program's establishment in 1966, state Medicaid programs have contracted with health insurance plans to provide healthcare to some recipients. But the **Omnibus Budget Reconciliation Act of 1981 (OBRA-81)** increased health plan involvement substantially, by making these changes:

- States were now allowed to set their own qualification standards for contracting HMOs.
- Previously, no more than 50 percent of a contracting HMO's membership could be Medicare or Medicaid beneficiaries; this was raised to 75 percent (and was later eliminated altogether).⁴

- The Centers for Medicare and Medicaid Services (CMS) was given greater leeway in granting waivers from the rules of the Social Security Act. CMS could allow states to set up demonstration projects using new financing and delivery options, including managed care strategies and health insurance plans, to serve Medicaid recipients.

Tax Equity and Fiscal Responsibility Act of 1982 (TEFRA)

The **Tax Equity and Fiscal Responsibility Act of 1982 (TEFRA)** made it more attractive for many HMOs to serve the Medicare population. Previously, only federally qualified HMOs were allowed to enter into risk contracts with Medicare (in which the plan took on financial risk but could earn a profit). Other HMOs could participate only through cost contracts (in which the HMO was reimbursed for expenses and assumed no risk but could not earn a profit). Now more HMOs could potentially earn a profit by contracting with Medicare.

Consolidated Omnibus Budget Reconciliation Act of 1986 (COBRA-86)

A provision of the **Consolidated Omnibus Budget Reconciliation Act of 1986 (COBRA-86)** addresses the problem of individuals losing their employer-sponsored health coverage when they quit their jobs or are laid off. COBRA requires employers with 20 or more employees to allow continuation of group health coverage for up to 18 months, at the employee's expense, for employees (and their dependents) who leave the company for any reason except gross misconduct.

Americans with Disabilities Act of 1990 (ADA)

The **Americans with Disabilities Act (ADA)** was passed to ensure the rights of people with disabilities. Of course, the activities and facilities of health insurance plans, like those of other businesses, are subject to this law. For example, the ADA requires that a health plan facility must be accessible to wheelchairs. Also, certain plan benefit exclusions have been challenged in court as ADA violations.

Health Insurance Portability and Accountability Act of 1996 (HIPAA)

The **Health Insurance Portability and Accountability Act of 1996 (HIPAA)** and the regulations that implement it addressed many aspects of health coverage. A few of the most important provisions:

- Health insurers are required to provide coverage to small businesses and certain eligible individuals who have lost their group coverage.
- The use of preexisting condition exclusions by health insurers is limited in certain ways.
- Insurers must renew group and individual health coverage except in a few circumstances.
- Health insurance plans and other entities must take steps to protect the privacy and security of health information.

Balanced Budget Act of 1997 (BBA)

The **Balanced Budget Act of 1997 (BBA)** made important changes in the participation of health insurance plans in Medicare and Medicaid.

- Medicare’s managed care program—called “Medicare+Choice” or Medicare Part C—offering beneficiaries an alternative to Original Medicare (Part A and Part B) now made available options beyond HMOs, including preferred provider organizations (PPOs), point of service (POS) plans, private fee-for-service plans, and others.
- Payment was reformed to make the Medicare market more financially attractive to health insurance plans.
- The requirement that the membership of participating HMOs be no more than 75 percent Medicare and Medicaid recipients was eliminated.⁵
- States could now require most Medicaid recipients to enroll in a health insurance plan without obtaining a waiver from the federal government.⁶

In addition, the BBA established the Children’s Health Insurance Program (CHIP), which provides states with financial support to expand publicly funded coverage to uninsured children who cannot afford private health insurance but are not eligible for Medicaid.

Federal Laws Affecting Health Insurance Plans

Part II: More Recent Legislation

Medicare Prescription Drug, Improvement, and Modernization Act of 2003 (MMA)

The **Medicare Prescription Drug, Improvement, and Modernization Act of 2003** (for short, the **Medicare Modernization Act**, or **MMA**) made several substantial changes to Medicare.

- Medicare+Choice was replaced with the Medicare Advantage program. Medicare Advantage further broadened the options available to beneficiaries and encouraged health insurance plans to participate through a competitive bidding process and more favorable risk selection. Medicare Advantage establishes payments based on a system of bids and benchmarks and provides a stabilization fund to create incentives for plans to enter into and remain in the program.⁷
- A prescription drug benefit for Medicare beneficiaries (Medicare Part D) became available in 2006. Part D is voluntary, and beneficiaries who choose to enroll pay a monthly premium. Coverage is provided through private-sector plans, either a standalone Prescription Drug Plan (PDP) or a Medicare Advantage plan.⁸
- New tax-advantaged accounts for medical expenses, called health savings accounts (HSAs), were authorized.
- Fee-for-service Medicare payments were generally increased (especially for rural healthcare providers), and numerous regulatory and administrative practices were modified.⁹

Deficit Reduction Act of 2005 (DRA)

The **Deficit Reduction Act of 2005 (DRA)** made a number of changes in the rules for health insurance plans participating in Medicaid and Medicare. Two of the most important:

- It was made easier for state Medicaid programs to charge premiums and cost-sharing and to modify the benefit package for certain Medicaid recipients. The goal was to reduce Medicaid spending and enable states to design benefit packages that are better aligned with the specific needs and resources of different populations.¹⁰
- The rules were adjusted allowing states to implement Medicaid managed care organization (MCO) provider taxes, and the Medicaid MCO provider class was expanded to include all MCOs.¹¹

Medicare Improvements for Patients and Providers Act of 2008 (MIPPA)

The **Medicare Improvements for Patients and Providers Act of 2008 (MIPPA)** made several changes in Medicare, including the following:

- Rules were modified to improve access and affordability for beneficiaries with limited resources.¹²
- Restrictions were imposed on the marketing of private-sector products serving Medicare beneficiaries—Medicare Advantage (MA) plans and Part D prescription drug plans (PDPs). MA plan sponsors were made responsible for the actions of agents, brokers, and other third parties marketing and selling their products.¹³
- The standardized plans for Medicare supplement (Medigap) insurance were revised, and as of June 1, 2010, all new plans had to meet the new rules.¹⁴

Genetic Information Nondiscrimination Act of 2008 (GINA)

The **Genetic Information Nondiscrimination Act of 2008 (GINA)** prohibits insurers and employers from discriminating against individuals based on genetic information. Insurers may not require or request genetic testing, and they may not use genetic information for underwriting purposes, or to deny coverage, adjust premiums, or impose a preexisting condition exclusion. The privacy of genetic information must be protected.¹⁵

Mental Health Parity and Addiction Equity Act of 2008 (MHPAEA)

The **Mental Health Parity and Addiction Equity Act of 2008 (MHPAEA)** generally states that group health plans and health insurance issuers that provide mental health or substance use disorder (MH/SUD) benefits may not impose less favorable benefit limitations on those benefits than on medical/surgical benefits. The Affordable Care Act of 2010 expanded this rule from group plans to individual coverage,¹⁶ and it also mandated that qualified health plans (QHPs) offered in ACA marketplaces cover many behavioral health treatments and services.¹⁷

American Recovery and Reinvestment Act of 2009 (ARRA)

The **American Recovery and Reinvestment Act of 2009 (ARRA)** was a response to the economic crisis of 2008. Among many other measures, this law directed billions of federal dollars to the healthcare industry, for Medicaid, to subsidize private health insurance for people losing their jobs, for health information technology, and for other purposes.¹⁸ A component of ARRA was the **Health Information Technology for Economic and Clinical Health (HITECH) Act**, which supports the adoption and use of certified electronic health records (EHR) by providing financial incentives under the Medicare and Medicaid programs to eligible professionals and hospitals.¹⁹ ARRA also expanded the HIPAA health data privacy protections in various ways.²⁰

Medicare Access and CHIP Reauthorization Act of 2015 (MACRA)

The **Medicare Access and CHIP Reauthorization Act of 2015 (MACRA)** makes many changes in how Medicare compensates providers. It repeals the sustainable growth rate (SGR) formula for calculating updates to Medicare physician payment rates and establishes an alternative set of annual updates. It creates a new merit-based incentive payment system (MIPS), which will consolidate and replace several existing incentive programs and apply to payments for items and services beginning in 2019. It establishes processes for developing, evaluating, and adopting alternative payment models (APMs).²¹

MACRA also extends for two years federal funding for the Children’s Health Insurance Program (CHIP). (Legislative action will be required to extend CHIP funding past September 2017.)²²

Federal Laws Against Fraud and Abuse

We should also note that over the years Congress has enacted federal laws that can be used to combat fraud and abuse in health coverage, including the False Claims Act, the federal Anti-Kickback Statute, and the Stark legislation. Other federal laws, such as those against mail and wire fraud, while not focused on health insurance, can also be used to fight health care fraud.

Affordable Care Act of 2010 (ACA)

The **Patient Protection and Affordable Care Act of 2010** (commonly called simply the **Affordable Care Act**, or **ACA**) represents a comprehensive reform of health insurance. Under the ACA’s **individual mandate**, all persons (with some exceptions) are required to have health coverage or pay a tax penalty. So that everyone can access and afford coverage, the ACA includes these provisions:

- Medicaid is expanded to cover many low-income people who did not previously qualify. (Under a Supreme Court decision, states have the option of expanding their Medicaid program or not.)
- Large employers (50 or more “full-time equivalent” workers) must provide coverage to employees or pay penalties.
- Smaller employers are encouraged to provide coverage by means of a tax credit.
- Individuals and small businesses can buy private-sector coverage through state-based health insurance marketplaces (exchanges).

- For those who purchase coverage through an exchange and have family income up to 400 percent of the Federal Poverty Level, premium subsidies are available. Lower-income people also qualify for cost-sharing reductions.
- People up to age 26 have the right to coverage under their parents' health plan.

Other provisions of the ACA are intended to ensure that health coverage meets the needs of consumers.

- To be sold through a marketplace, a health insurance plan must be a **qualified health plan (QHP)**—it must provide **essential health benefits (EHBs)** and meet other requirements and standards.
- Premiums cannot vary based on an individual's health; variations based on age and tobacco use are allowed but limited.
- Preexisting condition exclusions are prohibited.
- Lifetime or annual limits on essential health benefits are prohibited.
- There are limits on cost-sharing and waiting periods; cost-sharing for certain preventive services is prohibited.
- Minimum loss ratios apply: 85 percent for large groups and 80 percent for small groups and individual policies. (That is, insurers must spend at least this percentage of premium dollars received on healthcare, not administration and profit.)

The ACA is funded through various means, including the following:

- An annual fee on health insurers
- For high-income taxpayers, an increase in the Medicare payroll tax and its application to unearned income
- Taxes on medical devices and brand-name drugs
- Beginning in 2018, a tax on generous health insurance plans (the "Cadillac Tax")

Note that these are just a few of the most important of the ACA's many provisions. For instance, there are numerous programs and rules designed to enhance the quality and cost-effectiveness of healthcare. And since the law's enactment, numerous regulations and regulatory guidance have been published by various federal departments, setting specific rules and clarifying certain matters.

Other Federal Programs Involving Health Insurance Plans

We've seen that private-sector health insurance plans provide coverage to some beneficiaries of both Medicaid and Medicare. Two other federal health benefit programs involve private health plans.

- **FEHBP.** The **Federal Employees Health Benefits Program (FEHBP)** makes health insurance available to federal employees, retirees, and their dependents and survivors, offering them a

choice of fee-for-service and managed care plans. Many health insurance plans participate in FEHBP and must comply with rules and requirements for this program set by the Office of Personnel Management (OPM).

- **TRICARE.** Most of the healthcare received by members of the military and other uniformed services of the U.S. government is provided by the government itself in military hospitals and clinics. However, these people sometimes receive care from civilian facilities and providers, as do retirees and dependents. When this occurs, most costs are covered by **TRICARE**, a health benefits program of the U.S. Department of Defense. TRICARE offers an HMO-type option, a PPO option, or the traditional fee-for-service option. Private-sector health insurance plans bid to provide TRICARE services in different regions.

State Laws and Regulation for Health Insurance Plans

States have had insurance laws since the 1800s, but these laws were geared toward traditional indemnity insurance. As managed care plans emerged, states have developed and enacted laws to regulate them. In 1972 the National Association of Insurance Commissioners (NAIC) and the National Association of HMO Regulators developed the Health Maintenance Organization Model Act, which governs major aspects of the formation and operation of HMOs. The HMO Model Act has been adopted, in whole or in part, by many states, and it has been modified subsequently as necessary. State insurance law also often governs risk-bearing preferred provider organizations, and some states have laws regulating non-risk-bearing PPOs. Many states have enacted laws to regulate independent functions (such as utilization review and third-party administration) performed by a health insurance plan under contract to another organization. A few states have enacted legislation or issued administrative rules that either create a special licensure category for risk-bearing entities or require such organizations to obtain a license as an HMO. A number of states have specific laws or regulations that apply to at-risk provider organizations.

The federal HMO Act of 1973 specified the services that a federally qualified HMO must provide or cover, and many states have benefit requirements for HMOs based on the federal Act. In addition, some states have passed laws mandating specific benefits to be covered by health insurance plans or mandating the providers who may deliver healthcare services to plan members. For example, some states require health plans to cover certain emergency room services, a minimum hospital stay for mastectomy patients, alcoholism/substance abuse, mammograms, and Pap smears.

States have also enacted legislation to protect the rights of healthcare providers. For example, **“any willing provider laws”** generally allow any provider who meets a health insurance plan’s terms and conditions of participation to become part of its network or receive payment from the plan for covered services provided to plan enrollees, whether the provider is a member of the plan’s network or not.

Areas of Regulation of Health Insurance Plans

Health insurance plan regulators, consisting largely of state departments of insurance, state departments of health, and (in some instances) federal agencies, have addressed a number of aspects of plan operations, discussed below. Since regulatory authority is dispersed among various agencies, organizations, and jurisdictions, health plans must often comply with multiple standards in each of these areas. And in addition to this formal regulation, health plans may be subject to accrediting bodies such as the National Committee for Quality Assurance (NCQA) and/or URAC.

- Quality assurance (including utilization review).** Although it is difficult to draw a line between quality assurance activities and other aspects of health insurance plan operations, examples of quality assurance might include, for example: quality assurance programs, utilization review, and external review for quality of care. Typically, plans must have quality assurance programs that reflect their activities in monitoring quality, assessing any quality problems, imposing corrective actions, and analyzing patterns of care. Also, plans are typically required to involve physicians or other qualified medical professionals in establishing the program and reviewing the care process. Utilization review would fall under this broad category and involves a prospective, concurrent, or retrospective determination of whether the covered service or benefit is or was “medically necessary.” When a health plan denies a claim or request for authorization on the basis of medical necessity, the member and the provider have the right to appeal that decision, first internally to the health plan and then externally to the regulator or another third party. Specific requirements are based on state and federal law.
- Solvency/licensure.** Solvency standards are intended to ensure that health insurance plans have sufficient resources to provide promised benefits. Although solvency standards differ from one regulatory program to another, state regulators require insurance carriers and HMOs to meet minimum capital and surplus requirements for licensure, and to file periodic reports, including annual and quarterly financial statements and risk-based capital (RBC) reports, among others. In addition, other entities that bear risk may be required to obtain certifications or licenses from state regulators. These would include a license as a risk-bearing entity, an organized delivery system, or a PPO, depending on the jurisdiction.
- Benefits.** There are a number of state and federal requirements that health insurance plans provide certain benefits. States have traditionally mandated some benefits, and health plans participating in ACA marketplaces must include “essential health benefits.” Some states are fairly prescriptive, requiring the health plan to use set model language.
- Enrollment.** In most health plans, prospective members are permitted to enroll during open enrollment and special enrollment periods prescribed by law. Plans may choose to offer coverage outside of these periods, but most do not (typically a business consideration). Plans may be offered directly to groups and individuals or through a public or private insurance marketplace (exchange).
- Enrollee information/marketing.** State and federal regulators require a number of disclosures to be made to prospective and current members. In addition, advertising and marketing materials are subject to numerous state laws, including a requirement to retain such materials for review by the applicable regulator. Marketing and advertising rules pertain to the form and content of the material, the use of the health plan’s proper name, and information about where the health plan is licensed, among other things. Some jurisdictions have advertising and marketing laws specific to health plans, while others regulate the materials under the state’s Unfair Trade Practices Act.
- Access to care.** Because many health insurance plans provide benefits primarily through networks of providers, it has become increasingly common for regulators to set standards to measure the adequacy of these networks (that is, whether there are enough providers to furnish covered services to an enrolled population without undue delays). There also have been

efforts to regulate other aspects of access, such as member choice of providers and “direct access” to providers.

- **Provider contracting.** State regulators generally regulate health insurance plan contracts with providers to varying degrees. For example, regulators may require health plans to file and/or obtain prior approval of such agreements and/or to include certain standard provisions. Regulators may impose requirements or prohibitions on the providers with which the health plan is permitted to contract. In some cases, the health plan may be responsible for monitoring the financial viability of the providers with which it contracts.
- **Premiums and rating practices.** Health insurance plan premiums affect the costs incurred by purchasers, the pool of funds available for covered benefits, and the out-of-pocket costs of enrollees. Because of this, regulatory oversight of plan premiums is substantial.
- **Management and organizational structure.** Regulatory requirements regarding a health insurance plan’s management and organizational structure fall into three main categories. Some are intended to ensure clear accountability for plan policies by easily identifiable individuals. Others are designed to ensure the involvement of individuals with appropriate expertise and experience in specific decision-making areas (for instance, physician involvement in the development of practice guidelines). Still others aim at excluding from health plan management individuals with a conflict of interest or prior convictions for healthcare-related offenses.
- **Reporting and filing.** Health insurance plans often must report extensive information to regulators (particularly in the areas of solvency and quality assurance) and provide reasonable access to actual plan records to permit verification of reported information. Sometimes this information is used to monitor continuing compliance with applicable standards; in other instances, it is used as the basis for consumer information. Health insurance plans are also required to file certain documents with regulators (form filing, rate filing, and data calls, among others.)
- **Confidentiality.** Health insurance plans are required to comply with applicable state and federal laws (such as HIPAA) intended to protect the confidentiality of protected health information, personal financial information, medical records, and other similar information. Some also establish standards for assuring prompt access by patients to their own medical records and the accuracy of information contained in such records.

Health insurance plans are highly regulated. The foregoing list provides an extremely brief summary of only certain aspect of this regulation. There are a number of other requirements such as state and local health and safety standards, zoning laws, certificate of need requirements, tax requirements, and civil rights or antidiscrimination laws.

Summary

In the United States, law has a number of sources, including federal and state constitutions, statutes, administrative rules and regulations, case law (common law), and model laws. The most important federal laws affecting health insurance plans include the following:

- Antitrust laws seek to maintain a competitive marketplace.
- The **Health Maintenance Organization Act of 1973** sets requirements and grants advantages to HMOs that choose to become federally qualified.
- The **Employee Retirement Income Security Act of 1974 (ERISA)** sets standards for most employment-based health benefit plans, including self-insured plans.
- The **Omnibus Budget Reconciliation Act of 1981 (OBRA-81)** increased health plan involvement in Medicaid.
- The **Tax Equity and Fiscal Responsibility Act of 1982 (TEFRA)** made it more attractive for many HMOs to serve the Medicare population.
- The **Consolidated Omnibus Budget Reconciliation Act of 1986 (COBRA-86)** allows people to keep their employer-sponsored health coverage for a time after employment ends.
- The **Americans with Disabilities Act (ADA)** seeks to ensure the rights of people with disabilities.
- The **Health Insurance Portability and Accountability Act of 1996 (HIPAA)** addresses many aspects of health coverage, including access to coverage and the privacy and security of health information.
- The **Balanced Budget Act of 1997 (BBA)** made changes in health insurance plan participation in Medicare and Medicaid and created the Children's Health Insurance Program (CHIP).
- The **Medicare Prescription Drug, Improvement, and Modernization Act of 2003 (MMA)** further expanded health plan participation in Medicare and established Medicare Part D (prescription drug benefits).
- The **Deficit Reduction Act of 2005 (DRA)** made a number of changes in the rules for health insurance plans participating in Medicaid and Medicare.
- The **Medicare Improvements for Patients and Providers Act of 2008 (MIPPA)** made several changes in Medicare.
- The **Genetic Information Nondiscrimination Act of 2008 (GINA)** prohibits insurers and employers from discriminating against individuals based on genetic information.
- The **Mental Health Parity and Addiction Equity Act of 2008 (MHPAEA)** supports benefits for the treatment of mental health or substance use disorders.

- The **American Recovery and Reinvestment Act of 2009 (ARRA)** directed billions of federal dollars to the healthcare industry for various purposes.
- The **Medicare Access and CHIP Reauthorization Act of 2015 (MACRA)** made many changes in how Medicare compensates providers.
- The **Patient Protection and Affordable Care Act of 2010 (Affordable Care Act, or ACA)** contains a great many provisions that seek to ensure that everyone can access and afford health coverage and that coverage meets people's needs.

Health insurance plan regulators, consisting largely of state departments of insurance, state departments of health, and (in some instances) federal agencies, have addressed a number of aspects of plan operations: quality assurance (including utilization review), solvency/licensure, benefits, enrollment, enrollee information/marketing, access to care, provider contracting, premiums and rating practices, management and organizational structure, reporting and filing, and confidentiality.

Sources

¹ *Black's Law Dictionary*, 10th ed., 2014.

² *Ibid.*

³ 42 U.S.C. §300e *et seq.*

⁴ Medicaid and CHIP Payment and Access Commission (MACPAC), "Report to the Congress: The Evolution of Managed Care in Medicaid," June 2011. https://www.macpac.gov/wp-content/uploads/2015/01/Context_and_Overview_of_Medicaid_Managed_Care.pdf.

⁵ *Ibid.*

⁶ *Ibid.*

⁷ John C. Garner, *Health Insurance Answer Book*, 12th ed., 2014.

⁸ Congressional Research Service, "Medicare Prescription Drug, Improvement, and Modernization Act of 2003 Summary." Accessed online on December 8, 2015 at: <https://www.congress.gov/bill/108th-congress/house-bill/1>.

⁹ *Ibid.*

¹⁰ Commonwealth Fund, "Deficit Reduction Act: Changes to Medicaid, States in Action Archive," March 29, 2006. <http://www.commonwealthfund.org/publications/newsletters/states-in-action/2006/mar/march-2006/federal-activity-affecting-performance-of-state-health-care-systems/deficit-reduction-act--changes-to-medicaid>.

¹¹ National Conference of State Legislatures, "Deficit Reduction Act of 2005: Summary of Medicaid/Medicare/Health Provisions," updated April 2007. <http://www.ncsl.org/print/health/DRAsum1106.pdf>.

¹² National Association of States United for Aging and Disabilities, "MIPPA." Accessed online on December 8, 2015 at: <http://www.nasuad.org/policy/key-issues/mippa#sthash.Oq3NJS6L.dpuf>.

¹³ *Ibid.*

¹⁴ *Ibid.*

¹⁵ Congressional Research Service, "The Genetic Information Nondiscrimination Act of 2008 (GINA)," August 2015. <https://www.fas.org/sgp/crs/misc/RL34584.pdf>.

¹⁶ Centers for Medicare & Medicaid Services, “The Mental Health Parity and Addiction Equity Act.” Accessed on November 30, 2015 at: https://www.cms.gov/CCIIO/Programs-and-Initiatives/Other-Insurance-Protections/mhpaea_factsheet.html.

¹⁷ Substance Abuse and Mental Health Services Administration, “Implementation of the Mental Health Parity and Addiction Equity Act (MHPAEA).” Accessed on November 30, 2015 at: <http://www.samhsa.gov/health-financing/implementation-mental-health-parity-addiction-equity-act>.

¹⁸ *Healthcare IT News*, “American Recovery and Reinvestment Act (ARRA).” Accessed on December 8, 2015 at: <http://www.healthcareitnews.com/directory/american-recovery-and-reinvestment-act-arra>.

¹⁹ Ann M. Bildsten and Michael J. Vanselow (Oppenheimer Wolff & Donnelly LLP), “ARRA Impacts on Health Care,” *Law360*, October 15, 2009. Accessed on December 8, 2015 at: <http://www.oppenheimer.com/uploadedFiles/News/ARRA%20Impacts%20On%20Health%20Care.pdf>.

²⁰ *Ibid.*

²¹ Congressional Research Service, “The Medicare Access and CHIP Reauthorization Act of 2015 (MACRA; P.L. 114-10),” November 2015. <https://www.fas.org/sgp/crs/misc/R43962.pdf>.

²² Georgetown University Health Policy Institute Center for Children and Families, “About CHIP,” <http://ccf.georgetown.edu/chip/about-chip/>.

6 Regulatory Agencies

Lesson Objectives

After completing this lesson, you should be able to:

- Identify the main state government departments that govern or affect health plans.
- Name the federal agencies that regulate and oversee health plans that provide coverage or services to federal health benefit programs such as Medicare, Medicaid, and FEHBP.
- Identify the other main federal agencies that regulate aspects of health plans.

In the last lesson we saw that Congress and the state legislatures have enacted a great number and variety of laws that regulate health plans or affect them in some way. To put these laws into practice—to implement and enforce them, and to develop rules and regulations stipulating what they require—the states and the federal government have also created a wide variety of regulatory agencies.

State Regulatory Agencies

The most important state government entity regulating health plans is the **state insurance department**. The insurance department regulates and oversees many aspects of health insurers and health plans. It seeks to ensure that a plan has the financial resources to remain solvent and stable and to meet its obligations to policyholders and others. It also promotes and protects the interests of consumers. Some of the activities of the state insurance department:

- Licensing insurance companies and agents
- Reviewing policies for compliance with state requirements and fairness to the consumer
- Reviewing premium rates
- Requiring reporting on finances
- Dealing with insurers that become financially unstable
- Examining insurer operations for compliance with laws and regulations
- Examining insurers' sales and marketing (market conduct examinations)

Other state departments and agencies can have an impact on health plans:

- The department of health regulates and oversees the delivery of healthcare, including access to and quality of care.
- The secretary of state (or a similar agency) is responsible for incorporation of companies and related issues.
- The attorney general is involved in various legal issues, such as not-for-profit status and conversion to for-profit status.

- The department of revenue handles taxation of health plans.

In addition, if a health plan provides coverage under a state program, it must work with the state agency operating the program. These include:

- The state Medicaid agency (often connected to the department of social services or human services).
- The state department of labor, for workers' compensation products.
- The state personnel or human resources department, for health coverage for state employees.

The number of state agencies and their overlapping areas of responsibility can complicate the efforts of health plans to comply with state laws and regulations. Not only do different state entities regulate different aspects of a health plan's operations, in some states different entities may regulate the same aspect of a plan, applying differing standards. For instance, a plan may have to meet different quality standards set by the insurance department, the health department, and the Medicaid agency.

Health Plan Participation in Federal Programs

Many private-sector health plans provide coverage under federal programs:

- Medicare Advantage
- Medicare Part D prescription drug plans (PDPs)
- Various Medicaid programs
- The Federal Employees Health Benefits Program (FEHBP)

Some companies offer health plans to individuals and small businesses on the federal and state marketplaces (exchanges) established under the Affordable Care Act.

For each of these programs, a federal agency oversees participating private-sector health plans. The agency must:

- Select the plans that will participate (or set requirements that must be met for participation)
- Form contracts with plans
- Set requirements, rules, and regulations for plans
- Supervise plans to ensure they are complying with requirements
- Calculate and pay compensation to plans

In this section we'll look at the main agencies that perform this role.

The **Centers for Medicare and Medicaid Services (CMS)** (part of the U.S. Department of Health and Human Services) administers the Medicare and Medicaid programs. CMS mainly acts as a purchaser of healthcare services for beneficiaries. But it also:

- Seeks to ensure that programs are properly administered by contractors, including health plans
- Establishes policies for the compensation of health plans that provide services or coverage
- Combats waste, fraud, and abuse

In addition, CMS is responsible for regulating federally qualified HMOs.

A division of CMS, the **Center for Medicare**, is responsible for Medicare program policies and operations. As regards health plans that provide coverage to beneficiaries through Medicare Advantage plans and Medicare Part D PDPs, the center:

- Takes the lead in management, oversight, budget, and performance issues
- Oversees CMS interactions, collaboration, and communication with plans
- Identifies vulnerabilities and implements strategies to eliminate fraud, waste, and abuse¹

Another entity within CMS, the **Consortium for Medicaid and Children's Health Operations (CMCHO)**, coordinates the efforts of CMS's regional offices to oversee Medicaid (coverage for low-income people) and the Children's Health Insurance Program (CHIP) (coverage for children in families with incomes too high to qualify for Medicaid but unable to afford private coverage).² Each state administers its own Medicaid and CHIP program, including overseeing the involvement of private-sector health plans. But states must operate within broad federal guidelines. Each regional office has an associate regional administrator for Medicaid who works closely with state Medicaid and CHIP agencies to ensure consistent implementation of CMS programs, policy, and guidance.³

CMS includes the **Center for Medicare and Medicaid Innovation**. This center seeks to improve the quality of healthcare received by beneficiaries while holding down costs. To do so, it identifies, validates, and disseminates information about new care models and payment approaches. These may include new ways to compensate health plans for providing coverage, and new standards and requirements designed to enhance quality that plans will have to meet.⁴

Many health plans offer coverage to federal employees, retirees, and their families through the Federal Employees Health Benefits Program (FEHBP). FEHBP is run by the **Office of Personnel Management (OPM)**, the federal government's human resources agency. OPM sets standards for plans that wish to participate in FEHBP. These standards typically relate to the plan's management experience, enrollment rates, network of providers, and ability to meet proposed budget projections. OPM also conducts audits of health plans participating in FEHBP.

The **Department of Defense (DOD)** provides healthcare directly to members of the military. But DOD also sponsors TRICARE, a program that pays benefits when service members and their families receive care from civilian providers and facilities. TRICARE contracts with private plans to provide various administrative services and in some cases healthcare services. Such plans of course are regulated and overseen by DOD and subject to its requirements.

Other Federal Agencies

The **Center for Consumer Information and Insurance Oversight (CCIIO)** has responsibility for implementing many of the provisions of the Affordable Care Act related to private health insurance.

- CCIIO establishes and enforces standards for health insurance.
- It is involved in the ACA health insurance exchanges (marketplaces), through which private health plans can sell their products.⁵
- It provides consumers with information on their health plan options.⁶

The **Health Resources and Services Administration (HRSA)** (a division of HHS) works to make healthcare available to underserved populations. This includes people who are geographically isolated or economically or medically vulnerable. HRSA also supports the training of healthcare professionals, the distribution of providers to areas where they are needed most, and improvements in healthcare delivery.⁷ HRSA does not regulate health plans. But it educates the populations it serves about health plans and encourage participation in them. It also assists in the building of health plan systems that are responsive to the needs of underserved populations.

The **Office of Inspector General (OIG) of HHS** is the largest inspector general's office in the federal government. Its primary function is to prevent fraud, waste, and abuse in HHS programs. The HHS OIG maintains a nationwide network of audits, investigations, and evaluations, including audits of health plans, to ensure compliance with HHS rules and regulations. It imposes sanctions for such violations, and develops cases for criminal, civil, and administrative enforcement. The HHS OIG also develops and distributes resources to assist the health care industry in its efforts to comply with fraud, waste, and abuse laws and to educate the public about fraudulent schemes so they can protect themselves and report suspicious activities.⁸

The **Employee Benefits Security Administration (EBSA)** (an agency of the Department of Labor) is responsible for administering, regulating, and enforcing the provisions of Title I of the Employee Retirement Income Security Act (ERISA). ERISA governs employer-sponsored benefit plans, including health plans. Title I of ERISA contains rules for reporting and disclosure, vesting, participation, funding, fiduciary conduct, and civil enforcement.⁹

The **Internal Revenue Service (IRS)** (part of the Department of the Treasury) collects taxes and implements and enforces tax laws enacted by Congress. In addition to the payment of federal taxes, other aspects of health plans' formation and operations can involve the IRS. For example, a not-for-profit health plan that applies for tax-exempt status must comply with many rules and regulations set forth by the IRS.

Many other federal agencies impact various aspects of health plans. To mention a few:

- If a plan is charged with job discrimination or unfair labor practices, the Equal Employment Opportunity Commission (EEOC) or the National Labor Relations Board (NLRB) may become involved.
- A health plan that provides pharmacy benefits or contracts with pharmacy networks must ensure that the rules of the Food and Drug Administration (FDA) are complied with.
- The Federal Trade Commission (FTC) enforces a variety of antitrust and consumer protection laws.

Summary

At the state level, the main agency governing health plans is the insurance department, but other entities (such as the health department and the attorney general's office) regulate some aspects of

plans. Health plans that provide coverage to Medicare and/or Medicaid beneficiaries are supervised by the Centers for Medicare and Medicaid Services and its divisions. Plans that offer coverage under the Federal Employees Health Benefits Program are regulated by the Office of Personnel Management. Many other federal agencies have an impact on various aspects and operations of health plans.

Sources

¹ Centers for Medicare and Medicaid Services (CMS), “Center for Medicare,” 2016, https://www.cms.gov/About-CMS/Agency-Information/CMSLeadership/Office_CM.html

² Centers for Medicare and Medicaid Services (CMS), Center for Medicaid and CHIP Services, “About Us,” 2016, <https://www.medicaid.gov/about-us/index.html>

³ Centers for Medicare and Medicaid Services (CMS), “CMS Regional Offices,” 2016, <https://www.cms.gov/About-CMS/Agency-Information/RegionalOffices/index.html>

⁴ Centers for Medicare and Medicaid Services (CMS), “Center for Innovation,” 2016, <https://innovation.cms.gov/About/index.html>

⁵ Centers for Medicare and Medicaid Services (CMS), “Center for Consumer Information and Insurance Oversight,” 2016, <https://www.cms.gov/ccio/>

⁶ Centers for Medicare and Medicaid Services (CMS), “Center for Consumer Information and Insurance Oversight, About Us,” 2016, <https://www.cms.gov/ccio/>

⁷ Health Resources and Services Administration, “About HRSA,” 2016, <https://www.hrsa.gov/about/index.html>

⁸ Office of the Inspector General, Department of Health and Human Services, “About Us,” 2016 <https://oig.hhs.gov/about-oig/about-us/index.asp>

⁹ Department of Labor, Employee Benefits Security Administration, “History of EBSA and ERISA,” <https://www.dol.gov/agencies/ebsa/about-ebsa/about-us/history-of-ebsa-and-erisa>

7 State Regulation of Health Plans

Lesson Objectives

After completing this lesson, you should be able to:

- Describe the major provisions of the NAIC HMO Model Act
- Describe the types of state regulation that apply to PPOs, EPOs, POS products, TPAs, and UROs.
- Explain how the risk-based capital approach is used to set capital requirements for health plans.

In this lesson and the two that follow, we will look at how states regulate health plans. This lesson focuses on the licensing and oversight of different types of plans and organizations.

State Regulation of HMOs

In the 1970s many states had laws that governed health maintenance organizations (HMOs). But these laws were designed to regulate insurance companies, hospital and medical service corporations, or other entities, not health insurance plans.¹ They often constrained or restricted the development of HMOs. For example, some state laws required an HMO to be approved by a medical society, and the members of such societies often saw HMOs as a threat to their way of practicing medicine and put up roadblocks to deter their formation.

In 1972 the National Association of Insurance Commissioners (NAIC) adopted the **Health Maintenance Organization Model Act (HMO Model Act)**, a model law designed to regulate the licensure and operations of HMOs. This model has since been updated, most recently in 2003. It serves as the basis for the laws of many states. Since it is not possible to examine every state's HMO laws in this lesson, we will use the NAIC HMO Model Act as a framework for our discussion.

In the HMO Model Act, a "health maintenance organization" is defined as an entity that "undertakes to provide or arrange for the delivery of basic healthcare services to covered persons on a prepaid basis, except for a covered person's responsibility for copayments, coinsurance, or deductibles."² "Basic healthcare services" are defined as "the following medically necessary services: preventive care, emergency care, inpatient and outpatient hospital and physician care, diagnostic laboratory and diagnostic and therapeutic radiological services. It does not include mental health services or services for alcohol or drug abuse, dental or vision services, or long-term rehabilitation treatment."³ If an entity meets the definition of an HMO, it must comply with the licensing requirements and all other applicable requirements of the particular state's HMO statute.

Under the HMO Model Act and most state laws, an entity that wishes to operate as an HMO must obtain a **certificate of authority (COA)**, often called a **license**. A COA is issued by the state authority that regulates HMOs, and it asserts that all state requirements for the establishment of an HMO have been met. Generally, the purpose of licensing is to ensure that an HMO is a solid, dependable organization, which is fiscally sound and able to meet specified quality standards for healthcare delivery. Some states allow only domestic corporations to obtain a COA, while others grant it to foreign corporations as well. To obtain a certificate of authority, an organization must file an application form and additional information with the insurance department or another state agency.

Items to Be Submitted for HMO Certificate of Authority

Typically, the submission must include specific documents and information, including, but not limited to, the following:

- Organizational documents such as partnership agreements, trust agreements, or articles of incorporation and bylaws (depending on the organizational form under which the applicant operates)
- Information on the owners and ownership/affiliate structure of the organization
- Biographical information about the individuals who will be responsible for the HMO's day-to-day operations and copies of all contract forms the HMO will use for agreements with those individuals
- Current and historical financial statements (or, if not applicable, a list of assets representing the initial net worth of the entity) and a financial feasibility plan detailing projected enrollments, how the HMO will calculate premium rates, projected financial statements, and sources of working capital or other funding resources
- A description of the procedures or processes the HMO will follow to meet the protection against insolvency requirements in the HMO Model Act or state statute
- A description of the HMO's quality assurance program
- Contract forms the HMO will use for agreements with other parties providing services, such as healthcare providers and third-party administrators
- Copies of the evidence of coverage forms that the HMO will issue to enrollees
- Contract forms the HMO plans to use for group contracts with employers, unions, trustees, or other organizations
- A description or map of the geographic area in which the HMO proposes to operate
- A description of the proposed network adequacy standards that ensure adequacy, accessibility, and quality of healthcare
- A description of the internal and external grievance procedures the HMO will follow to investigate and resolve enrollee complaints
- Other information required by the state regulatory authority to determine whether to issue a COA

In many states the department of insurance and the department of health each have responsibilities for regulating HMOs.

- The insurance department may have responsibility for matters pertaining to solvency, financial statements, rate filings, group contract filings, evidence of coverage filings, benefit mandates, and member grievances.
- The health department may have responsibility for quality assurance, service area expansions, provider networks, and provider relations.

In other states, either the insurance department or the health department has responsibility for all HMO regulation, and in still others, this is the responsibility of a separate state agency, such as the Department of Managed Care in California.

The HMO Model Act authorizes the state insurance department to conduct an examination of the affairs of an HMO, its providers, and risk-bearing entities as often as is reasonably necessary, but at least once every five years. A state may accept the report of an examination made by another state. The insurance department is also authorized to take a number of actions, including suspending or revoking an HMO's certificate of authority, to enforce state laws and regulations and to protect the public.

In some states, HMOs that have received accreditation from an external accreditation organization are deemed to meet a variety of state requirements, including those concerning quality assurance and/or utilization review. A few states require accreditation. (For example, Florida requires HMOs to apply for accreditation within one year and be accredited within two years of receipt of their COA.) More than 35 states either require accreditation or deem accredited health plans in compliance with certain provisions of state law.⁴ Organizations from which health plans receive accreditation that may be accepted by states include NCQA, American Accreditation Healthcare Commission (The Commission/URAC-discussed later), and the Joint Commission on Accreditation of Healthcare Organizations (JCAHO).

Financial Requirements for HMOs

HMO laws seek to apply appropriate net worth or capital requirements to help ensure that HMO members are enrolled in financially viable entities with the resources to pay for their current and future healthcare needs. These requirements, along with provisions for financial reporting, accounting, liquidity, investment practices, and related matters, are often referred to as **financial standards**. HMO laws also seek to ensure that members are adequately protected in the event of **insolvency**, which occurs when an entity's liabilities exceed its assets or when it is unable to meet its financial obligations on time.

Adequate capital and surplus is essential to allow HMOs (as well as other types of health plans) to withstand losses incurred due to unexpected fluctuations in operating costs. If a health plan does not have a cushion to absorb such losses, its members may experience adverse consequences. These might include disruption of care, having to pay for medical expenses that should have been covered by the health plan (such as for emergency or specialty care out of the network), or the loss of a premium paid in advance.

The NAIC HMO Model Act requires an HMO seeking to obtain a COA to have an initial net worth of the greater of:

- the amount required under the state risk-based capital law (reflecting the NAIC Risk-Based Capital for Health Organizations Model Act, discussed below); or
- \$3,000,000; or
- (at the state insurance commissioner's discretion) a greater amount as indicated by the business plan and a projected risk-based capital calculation after the first full year of operation, based on the most current NAIC Health Annual Statement Blank.

Thereafter, the HMO must maintain a minimum net worth, as shown in the box below.

NAIC HMO Model Act: Ongoing Net Worth and Capital Requirements

- A health maintenance organization shall maintain a minimum net worth equal to the greater of \$2,500,000 or the amount necessary to maintain capital required pursuant to [the state law equivalent to the NAIC Risk-Based Capital for Health Organizations Model Act].
- The amount in paragraph A may be adjusted annually at the commissioner's discretion.

The NAIC offers the following alternative for states that have not adopted risk-based capital requirements:

- A health maintenance organization must maintain a minimum net worth equal to the greater of \$2,500,000; or an amount equal to the sum of:
 - (1) Eight percent (8%) of annual healthcare expenditures except those paid on a capitated basis or managed hospital payment basis as reported on the most recent financial statement filed with the commissioner
 - (2) Four percent (4%) of annual hospital expenditures paid on a managed hospital payment basis as reported on the most recent financial statement filed with the commissioner.

Source: National Association of Insurance Commissioners, Health Maintenance Organization Model Act, Section 17 (2003).

Net worth is an organization's **total admitted assets** minus its total liabilities. (The liabilities do not include fully subordinated debt.)

Assets are items of value owned by a company; an **admitted asset** is an asset whose full value can be reported on the assets page of a company's annual statement.⁵ The annual statement is a financial report that most health plans have to file to comply with state insurance regulations.

Risk-based capital (RBC) is a method of taking into account an organization's size, structure, and risk profile to set the minimum amount of capital needed for that organization to support its overall business operations (discussed later in this lesson).

Although financial standards provide a useful tool for state regulators to assess an organization's financial viability, they cannot completely eliminate the possibility of insolvency. To further protect HMO members, state regulators rely on other regulatory measures, such as:

- deposit requirements,
- requirements for HMOs to have a plan for handling insolvency,
- administrative supervision and receivership, and
- replacement coverage requirements.

Deposit Requirements

The HMO Model Act requires an HMO to place in trust with the state insurance commissioner a deposit that can be used to protect the interests of enrollees in case the HMO becomes financially impaired. At the discretion of the commissioner, the deposit can be placed with an organization or trustee acceptable to the commissioner. The deposit, regardless of where it is placed, must be in cash and/or securities and must at all times have a minimum value as specified by law. The amount deposited is treated as an admitted asset for purposes of determining the amount of the HMO's net worth.⁶ Sometimes an HMO must make an additional deposit in trust with the state insurance department to protect members from creditors in the event of the HMO's insolvency. The need for this deposit depends on the amount of the HMO's uncovered expenditures.

If an HMO becomes insolvent, in some cases a member will be liable for payment of healthcare services. Generally, network providers may not seek payment from members, under the hold harmless clause of their contract with the HMO. But if a member receives out-of-network care from a non-contracted provider (in an emergency, for instance), she may be liable for payment. Expenditures by an HMO that members would be liable for in the event of insolvency are **uncovered**; those they are not liable for are **covered**. If an HMO's uncovered expenditures exceed 10 percent of its total healthcare expenditures, it must place an **uncovered expenditures insolvency deposit** in trust with the state insurance department. If the HMO becomes insolvent, this money may be used by the commissioner on behalf of enrollees to pay claims for uncovered expenditures.⁷

A Plan for Insolvency

The HMO Model Act requires each HMO to have a plan for handling insolvency. If the HMO becomes insolvent, this plan must allow for the continuation of benefits for contract periods for which it has received premium payments. Benefits must also continue for members who are confined in an inpatient facility on the date of insolvency, until they are discharged. The state insurance department may require such a plan to include the following types of features:

- Insurance to cover the expenses to be paid for continued benefits
- Provisions in provider contracts requiring them to render services the HMO is obligated to provide following an insolvency
- Insolvency reserves

- Acceptable letters of credit

Administrative Supervision

If the state insurance commissioner determines, after a notice and a hearing, that the financial condition of an HMO is such that its continued operation might be hazardous to its enrollees, its creditors, or the general public, the commissioner may proceed with a number of interventions to protect against insolvency. These might include:

- monitoring a corrective plan developed by the company;
- suspending or limiting the writing of new business for a period of time, or reducing the volume of new business being accepted;
- reducing expenses by specified methods;
- taking over the management of the business; or
- selling the company or merging it with a financially sound company.

When an HMO is in hazardous financial condition, has failed to comply with insurance laws, or for other reasons, or by consensual agreement, the insurance commissioner may place an HMO under the state's **administrative supervision**. When the commissioner judges that an HMO's financial difficulties are so severe that more serious action is warranted, she may place the insurer in **receivership**. The commissioner, acting for a state court, takes control of and administers the HMO's assets and liabilities. **Rehabilitation** occurs when an insolvent HMO continues in existence under receivership. During this period, state authorities try to find ways to return the organization to normal operation. **Liquidation** occurs when all of the HMO's business and assets are transferred to other carriers, or when assets are sold to satisfy the HMO's outstanding obligations and the HMO's business is terminated.

Replacement Coverage

If an HMO becomes insolvent and is liquidated, provision is made to replace the coverage of the HMO's members.

- For a covered group, the commissioner can order all other carriers that participated in the group's most recent enrollment period to offer their plans to the insolvent HMO's group enrollees.⁸
- If no other carriers offered coverage to a group enrolled in the insolvent HMO, or if the commissioner determines that the other health plans do not have sufficient resources to assume responsibility for all of the group enrollees, the commissioner can allocate the insolvent HMO's group contracts among all HMOs that operate in the insolvent HMO's service area.⁹
- An insolvent HMO's non-group enrollees can also be allocated among all HMOs that operate within a portion of the HMO's service area.¹⁰

Other Requirements for HMOs

In addition to financial matters, state HMO laws address other concerns, which we will discuss in this section. These include:

- Healthcare delivery (including network adequacy, quality assurance programs, and grievance procedures)
- Enrollee contracts
- Filing and Reporting
- Disclosure

Network Adequacy

The HMO Model Act requires an HMO seeking to obtain a certificate of authority to file a description or map of the geographic area in which it proposes to operate and a list of the names, addresses, and license numbers of all providers in its network. Typically, state regulators examine this information to ensure that providers are located within a reasonable distance of all locations in the HMO's service area and that the HMO maintains adequate numbers of providers for the number of members enrolled.

With the growth of the health plan industry, the NAIC developed additional standards that were not contemplated in the original HMO Model Act. In 1996, the NAIC adopted the **Managed Care Network Adequacy Model Act** to establish network standards and to ensure the adequacy, accessibility, and quality of healthcare services offered under all health plans, including HMOs. This act was later renamed the **Health Benefit Plan Network Access and Adequacy Model Act** and revised during 2015 in light of concerns about narrow networks and the search for network adequacy standards for plans sold through exchanges under the Affordable Care Act.

Quality Assurance Program

A quality assurance program establishes procedures to ensure that healthcare services provided to enrollees are rendered under reasonable standards of quality of care, consistent with prevailing professionally recognized standards of medical practice. The purpose of a quality assurance program is to evaluate, maintain, and improve the quality of healthcare services delivered to covered persons.

The HMO Model Act also requires an HMO applying for a COA to file a description of its proposed program for quality assurance, which must comply with the state law equivalent of the NAIC Quality Assessment and Improvement Model Act. Under the HMO Model Act, an HMO's quality assurance program must include a written statement of goals, lines of authority, accountability, evaluation tools, data collection responsibilities, performance improvement activities, and an annual effectiveness review of the quality improvement program. The program must keep records of quality assurance program activities and make those records available to the commissioner of health or other authorized regulatory official. Before a COA is issued, the insurance department may confer with the health department to ensure that the HMO's proposed quality assurance program meets the appropriate standards.

The HMO Model Act requires an HMO applying for a COA to file a description of its proposed grievance procedures. Internal grievance procedures must comply with the state law equivalent of the NAIC Health Carrier Grievance Procedure Model Act. Procedures for independent external review of grievances must comply with the state law equivalent of the NAIC's Health Carrier Review Model Act.

Enrollee Contracts

Under the HMO Model Act, HMOs are required to provide each group and individual contract-holder with a document that specifies the benefits and services available to enrollees. This information is in the contract and may also be included in an **evidence of coverage (EOC)** (or **certificate of coverage**). For groups, the HMO typically provides a copy of the contract to the group contract-holder and an EOC to individual enrollees.

Individual HMO contracts must provide for a 10-day period in which the enrollee may examine and return the contract in exchange for a full premium refund. If an enrollee receives services during the 10-day period and returns the contract for a premium refund, the enrollee must pay for all services received.

An HMO's contract must contain a clear statement of the following information:

- The name and address of the HMO
- Eligibility requirements
- Benefits and services within the service area
- Emergency care benefits and services
- Out-of-area benefits and services, if any
- Copayments, coinsurance, deductibles, or other out-of-pocket expenses
- Limitations and exclusions
- Enrollee termination procedures
- Enrollee reinstatement procedures, if any
- Claims procedures
- Grievance procedures
- Procedures for requesting independent external review
- Utilization review procedures
- Continuation of coverage
- Conversion
- Extension of benefits, if any
- Coordination of benefits, if applicable

- Subrogation, if any
- Description of the service area
- Procedures for obtaining a provider directory
- The existence of a formulary and procedures for obtaining a copy of the formulary list (if any)
- Entire contract provision
- Term of coverage
- Cancellation
- Renewal
- Grace period
- Conformity with state law

Filing and Reporting

We have already discussed the information and documentation an HMO must submit with its application for a certificate of authority. In addition, in most states if an HMO wishes to make changes to its group contract and evidence of coverage forms, it must file the changes with the appropriate state agency. Some states also require HMOs to file changes in premium rates.

- Many states require **prior approval**. That is, an HMO is not permitted to use new forms (or rates, if rate filing is required) until the state agency reviews and approves the filing.
- Some states, however, operate on a **file and use** basis. This means that the HMO must submit a filing with the state at least 30 days prior to issuance or delivery, but prior approval is not required for the HMO to begin using the forms (or rates), although the regulator may later disapprove any forms or rates found to violate state laws.

HMOs must file with the state insurance department or other state agency an annual report detailing financial information and certain operational information. This annual report must be filed on a form approved by the state and must be verified by at least two of the HMO's principal officers. Under the HMO Model Act, HMOs also must file the following information each year:

- audited financial statements
- a list of the healthcare providers that have executed a contract agreeing to provide services to HMO enrollees

- a description of the HMO's grievance procedures, the total number of grievances handled, a compilation of the causes underlying those grievances, and a summary of the final disposition of the grievances

The HMO Model Act also gives the state the authority to require additional reports deemed necessary and appropriate, such as reports that provide information on quality assurance programs.

There is some variation by state in reporting requirements:

- Some states do not require an annual filing of a list of the HMO's contracted providers.
- Some states require that material business changes be reported more frequently than once a year. Such a filing may be required to update the state's records of the HMO's enrollee materials, including the handbooks it uses to describe coverage, provider lists, and sample member identification cards. Some states require regulatory approval of material business changes before the changes are allowed.
- An HMO may have to make a filing to obtain state approval to expand its service area.

Disclosure

We have noted that the HMO Model Act requires HMOs to provide enrollees with certain information about the HMO, its services, and its providers. Upon enrollment, an enrollee must receive a list of the HMO's healthcare providers. Any "material change" in the operation of an HMO affecting enrollees must be reported to enrollees within 30 days of the change.

For example, a major change in the provider network is considered a material change that must be reported to enrollees. By contrast, the termination of one medical care provider from the provider network would not qualify as a material change unless that provider is a primary care provider. When a primary care provider (PCP) is terminated from the network, the HMO must notify all enrollees who receive primary care from the terminated provider and help them transfer to another PCP.

More disclosure requirements:

- The HMO must help enrollees obtain information about HMO services and notify them where additional information on access to services can be found.
- The HMO must supply a toll-free telephone number that enrollees can use to contact the HMO.

Newly enacted consumer protection laws contain additional disclosure requirements intended to provide members with information about topics such as risk arrangements, plan financial information, and prescription drug formularies.

State Regulation of PPOs

Preferred provider organizations (PPOs) generally offer a network of contracted healthcare providers but cover services furnished by both network and non-network providers. However, PPOs generally provide financial incentives (for example, lower coinsurance or copayments) for members to use network providers. More Americans are enrolled in PPOs than any other health plan type.

A PPO may or may not bear risk. The American Association of Preferred Provider Organizations (AAPPO) makes this distinction:

- “The primary focus of a non-risk PPO is to contract with providers in a geographical area to form an interconnected network of providers and services. The non-risk PPO network leases and/or ‘rents’ its network for a fee to insurance companies, self-insured employers, union trusts, third-party administrators, business coalitions, and associations.”¹¹
- “A risk PPO (or insured PPO) has a similar structure, but unlike a non-risk PPO, it assumes the financial risk for an enrollee’s medical costs. Traditionally, insurance companies offer risk PPOs that include a benefit plan and network services, either provided by the risk PPO or leased from a non-risk PPO network.”¹²

State regulation of PPOs is difficult to summarize because PPOs are regulated in different ways depending on the state and on whether the PPO is risk-bearing.

- Many states require PPOs to be licensed, registered, or certified.
- States generally require that risk-bearing entities meet solvency standards. Frequently the solvency standards for risk-bearing PPOs are the same as for an indemnity insurance plan or an HMO.

The NAIC had a Preferred Provider Arrangements Model Act (PPA Model Act) that established minimum standards for preferred provider arrangements and the health benefit plans that include such arrangements. However, few states adopted this act and in 2004 it was dropped. Some states make sections of the state HMO laws applicable to PPOs, and some states have general “managed care” laws that apply to PPOs.

Non-risk bearing PPOs generally partner with an insurer or a self-funded employer group plan.

- If a PPO contracts with an insurer, the PPO is frequently subject to regulation as a downstream contractor of the insurer. That is, the state laws apply to the insurer and flow down to the non-risk PPO. For example, some states require that an insurer that contracts with a non-risk PPO submit documentation that the network is compliant with state requirements for network adequacy, timely claims processing, and grievances and appeals procedures.
- If a non-risk PPO contracts with a self-funded employer plan, state law may not apply, but the PPO must comply with requirements of the federal Employee Retirement Income Security Act (ERISA) (discussed in a later lesson)

Whether a PPO is regulated directly or as a downstream contractor of an insurer, some of its activities are governed by state laws. Some examples:

- Where PPOs perform utilization review, they may be subject to state laws regulating it.
- Where they pay claims on behalf of a self-insured employer, they may be regulated under state third-party administrator statutes.

- Where they act as an intermediate entity between an insurer and providers, they may be subject to state regulation of provider networks and credentialing.

At least six states have adopted versions of the **Rental Network Contract Arrangements Model Act** adopted in 2008 by the National Conference of Insurance Legislators. This act recognizes that frequently entities that are not insurers but solely provide provider networks are not regulated under state law. It requires all businesses that contract with providers to deliver healthcare services (known as “contracting entities”) to register with the applicable state.

The Model Act prohibits contracting entities from granting access to providers’ contractual discounts unless the applicable provider agreement specifically states that third-party use of the discounted rates is allowed. It obligates the third-party healthcare payers accessing the discounted rates to comply with all terms, limitations, and conditions of the agreement between the provider and contracting entity.

EPOs and POS Products

Exclusive Provider Organizations (EPOs)

An **exclusive provider organization (EPO)** can be thought of as a hybrid of a PPOs and an HMO. As in a PPO, generally no primary care provider or referrals are required (although prior authorization may be required for specified services). But as in an HMO, for non-emergency services to be covered, they must generally be furnished by a network provider. EPOs, like HMOs, are closed network health plans.

Like PPOs, EPOs may be risk bearing or non-risk bearing. In some instances, EPOs are created specifically for an employer, but most PPOs and national insurers offer EPO plans to employers using their existing PPO networks.¹³ They are frequently implemented by self-insured employers concerned with cost savings.¹⁴

In states that authorize EPOs, they are generally subject to similar regulation as PPOs. Where they are risk-bearing entities, they are subject to state solvency requirements.

- EPOs serving self-insured employers are generally subject to ERISA but not state laws.
- Other EPOs must generally comply with state laws regarding prompt payment, network adequacy, provider contracting, appeals, and utilization review.

Point-of-Service (POS) Products

A **point-of-service (POS) plan**, like an EPO, can be thought of as a hybrid of an HMO and a PPO, but the mix is different. As in an HMO, a primary care provider and referrals are usually required. But as in a PPO, a member can get care outside the network (with higher cost-sharing). There are two types of POS products:

- **Standalone POS product.** The HMO directly underwrites coverage of healthcare services rendered by providers outside its network, without the involvement of an insurer. Some states do not allow this, but others do, provided the HMO meets certain requirements.

- **Wraparound POS product.** The HMO arranges for coverage of non-network care through an insurance contract. If the HMO or an affiliated company is a licensed insurer, it can issue the contract; if not, a separate insurer can provide coverage for non-network care.

The financial viability of an HMO depends largely on its ability to manage costs by using network-based managed care techniques, and an HMO that does not want its financial condition impacted by “non-managed” non-network claims can offer a wraparound POS product, where permitted by law. Network benefits are subject to the laws that pertain to HMOs. Insured out-of-network benefits are subject to insurance laws.

Third-Party Administrators¹⁵

Third-party administrators (TPAs) are companies that provide various administrative services to insurers or self-funded employer health plans. TPAs often perform insurance functions such as claims processing, and the states have the authority to regulate these activities, and most states have adopted laws and/or regulations to do so.¹⁶

As we will discuss in more depth in a later lesson, for employer-sponsored health plans, especially self-funded plans, the federal Employee Retirement Income Security Act (ERISA) preempts some state laws. That is, such plans are exempt from certain state laws because they are regulated by ERISA instead. In some states, the state’s regulatory authority over TPAs that contract with self-funded plans has been challenged based on ERISA.

Courts have taken both sides in this controversy, but a recent U.S. Supreme Court decision may provide for broader preemption of state laws. The Court held that, for self-insured employer plans, a Vermont law that requires reporting of health plan claims, cost, and other information to a state database is preempted by ERISA.¹⁷ According to the Court, ERISA sets forth its own reporting and record retention requirements, and similar requirements from multiple states could “create wasteful administrative costs and threaten to subject plans to wide-ranging liability.”¹⁸

The **NAIC Third-Party Administrator Model Statute** is designed to regulate operations of third-party administrators, and many states have laws based on it. We will look at five areas of regulation:

- Certificate of authority
- Written agreements and records
- Payor responsibilities
- TPA responsibilities
- Suspension or revocation of the COA

Certificate of Authority (Licensing)

An organization that directly or indirectly underwrites; collects charges, collateral, or premiums from; or adjusts or settles claims on residents of the state in connection with life, annuity, health, stop-loss, or workers’ compensation coverage is considered a TPA for purposes of the TPA Model Statute. Exempted

are TPAs administering plans covered by ERISA, insurance companies acting as TPAs, and a few other entities. However, a TPA exempted by ERISA must still register with the state.

To obtain a certificate of authority, a TPA must submit a uniform application (developed by the NAIC) and provide basic organizational documents (such as articles of incorporation), bylaws, audited financial statements for the past two years, NAIC biographical data for individuals conducting the affairs of the organization, a business plan, and any additional information requested by the state. The TPA must also make available the written agreements it has entered into with health plans and employers. When evaluating an application for a TPA's COA, the insurance department will be concerned with ensuring that the TPA is solvent and that the individuals responsible for conducting its affairs are competent, trustworthy, financially responsible, and of good character.

What if a TPA is operating in a state but has a COA from another state?

- Some states allow such a TPA to operate without a COA if it provides services for 100 or fewer enrollees.
- Some states require a COA, but waive the application requirements if a TPA has a COA from another state with similar requirements.

Written Agreements and Records

A TPA's agreement with a health plan to provide administrative services must be in written form. To the extent that they pertain to the duties of the TPA, the health plan's benefits, premium rates, collateral and reimbursement procedures, and underwriting criteria must be agreed to in writing by the parties. Certain types of provisions may not be included in a TPA agreement. For example, the amount of the TPA's compensation may not be contingent on savings the TPA is able to realize from claims payments (so that the TPA is not induced to place its own financial interests above the interests of plan members).

The TPA must maintain the written agreement as a business record through the term of the agreement and five years thereafter. The TPA must maintain and make available to the health plan complete books and records of all transactions performed on behalf of the plan for at least five years after the termination of such an agreement. The insurance department has authority to examine these business records.

Payor Responsibilities

Even if an agreement between a health plan and a TPA specifies that the TPA will provide certain administrative services, the plan remains responsible for ensuring that it is administered properly. The plan also determines all premium rates, benefits, underwriting criteria, and claims payment procedures, and it must provide the TPA with written information on all such matters. If the TPA administers benefits for more than 100 policyholders of an insurer, the insurer must review the operations of the TPA at least semi-annually, with at least one review including an on-site audit.

TPA Responsibilities

A TPA must hold all funds it receives on behalf of a health plan in a fiduciary capacity. It must promptly remit all such funds to the proper parties, and it must periodically provide the plan with an accounting of all transactions it has performed on behalf of the plan. The TPA Model Law also requires a TPA to:

- Identify all charges that it collects from covered individuals.
- Disclose to the health plan all charges, fees, and commissions the TPA receives in connection with the services it provides to the plan.
- Promptly deliver to covered individuals any certificates, booklets, termination notices, or other written communications that it receives from the plan.

Reporting

A TPA is required to notify the state insurance department within 30 days of any material change in its ownership, control, or contact person, or any material change that might affect its qualification for a license. Each TPA is required to file with the insurance department an annual report, which includes information such as an audited financial report and the names and addresses of all payors that the TPA contracted with during the preceding year.

Suspension/Revocation of COA

The TPA Model Law requires the state insurance department to suspend or revoke a TPA's certificate of authority if the TPA is:

- financially unsound,
- using practices that are harmful to insured persons or the public, or
- has failed to pay any judgment rendered against it in the state within 60 days after that judgment became final.

In addition, the insurance department has discretionary authority to suspend or revoke a TPA's certificate of authority if, after notice and a hearing, the department finds that the TPA has engaged in certain specified activities such as violating the law or orders of the insurance department, refusing to be examined, or refusing to appropriately pay claims.

Utilization Review

We have looked at laws targeting different types of organizations—HMOs, PPOs, TPAs, etc. Another approach to regulating health plans is to enact laws focused on certain activities and functions. The NAIC has developed model laws in several areas, and many states have adopted or adapted them. These include models for:

- Credentialing
- Quality assurance
- Network adequacy
- Grievances and external review
- Utilization review

For a description of the model laws in these areas, go to the end of this lesson.

In this section we will explore regulation in one of these areas, utilization review.

Utilization Review (UR)

Utilization review (UR) is the evaluation of the medical necessity, efficiency, and/or appropriateness of healthcare services and treatment plans for a given patient. Utilization review may be undertaken by an in-house department of a health plan or an external entity. External entities that perform utilization review functions are called **utilization review organizations (UROs)**.

Typically, state laws impose requirements on personnel conducting utilization review.

- Almost all states require UR personnel to have training and experience in the field of medicine, subject of review. Many states require a healthcare provider conducting UR to be licensed to practice medicine in the state.¹⁹
- States also have laws governing the accessibility of UR personnel. For example, most states require that personnel be accessible by telephone during normal business hours, five days a week. Some states require the entity performing UR to establish a toll-free telephone line, staff after-hours coverage for review determinations, and/or meet certain response times for decisions.
- Some states have laws that ban the use of financial incentives that might induce UR personnel to deny certain services or reduce a length of stay in an inpatient facility, or that might otherwise impact a UR decision.²⁰

State laws also usually require an appeal procedure for adverse decisions or denials. Group health plans and health insurance issuers (with the exception of grandfathered plans) must comply with federal standards regarding determinations, internal appeals, and external appeals. Where state law is more stringent than federal law, organizations subject to state law must comply with the more stringent requirements.

Utilization review laws in some states require any entity that performs UR functions to be registered with the appropriate regulatory agency, such as the insurance department or health department. In other states, full certification is required, as discussed below. But in some states HMOs and other health plans are specifically excluded from registration or certification requirements. In states that require full certification, entities performing UR functions must meet certain requirements:

- The personnel of the entity must meet certain criteria with regard to experience, training, and education. Some states also require UR personnel to obtain reviewer licenses.
- Most states that require certification direct health plans and UROs to file a plan, confirming that the entity's personnel meet requirements and providing information about the services to be provided.

In general, states require certification to keep track of the entities performing UR services and to maintain an avenue for appeals of UR decisions. Licenses or certificates are usually valid for one to two

years and are renewable. State laws authorize officials to revoke an entity's license or certification and/or impose fines and penalties for noncompliance with UR laws.²¹

Some states allow entities conducting UR to submit evidence of their accreditation by a private accreditation organization instead of submitting an application for certification. The most prominent UR accrediting organizations are the American Accreditation Healthcare Commission (formerly URAC) and the National Commission on Quality Assurance (NCQA). Standards for accreditation focus on timely review of determinations, efficient and effective use of information to make determinations, maintenance of confidentiality standards, training and qualification of the UR staff, and the appeal process.

The Risk-Based Capital Approach to Financial Requirements

As we discussed earlier in this lesson, the HMO Model Act contains uniform solvency standards that all HMOs must meet to obtain and retain a license. However, the nature and volatility of business can vary significantly from one HMO to another. For instance, an HMO's capital and surplus requirements can be affected by factors such as out-of-network coverage provisions and provider compensation/risk-sharing arrangements. Moreover, a great diversity of organizations and arrangements have emerged over the years in the healthcare marketplace. As a result, a "one size fits all" approach for all plans has its limitations.

In response, in the early 1990s the NAIC began developing **risk-based capital (RBC)** formulas for all life and health insurance companies. Later, the organization created a separate RBC formula for all health insurers and health plans that accept risk. The most recent version of this is found in the NAIC's 2011 Risk-Based Capital for Health Organizations Model Act.

Risk-based capital (RBC) is a method of measuring the minimum amount of capital appropriate for an organization to support its overall business operations based on its size and risk profile.²² The greater the amount of risk a health plan takes, the greater the amount of capital it must hold. In this way, RBC limits the amount of risk an organization can accept. However, RBC is intended to be a minimum regulatory capital standard and not necessarily the full amount of capital that a health plan would want to hold to meet its safety and competitive objectives.

The RBC formula for health plans is a set of calculations, based on information in the health plan's annual financial report, that yields a target capital requirement for the organization. The health organization RBC formula assesses the risk profiles of specific health plans, gives credit for payment arrangements that reduce risk, and determines appropriate capital requirements based on these risk profiles.

The health organization RBC formula takes into account four types of risk:

- **Asset risk**—the risk of adverse fluctuations in the value of assets.
- **Underwriting risk**—the risk that premiums will not be sufficient to pay for services or claims.
- **Credit risk**—the risk that providers and plan intermediaries paid by capitation will not be able to provide the services contracted for, and the risk associated with recoverability of amounts due from reinsurers.

- **Business risk**—the general risk of conducting business, including the risk that actual expenses will exceed amounts budgeted.

Generally, underwriting risk is by far the largest single determinant of a company's total RBC requirement. Underwriting risk is primarily related to the amount and distribution of a health plan's premium income. A general rule of thumb is that the minimum RBC requirement for a health plan amounts to about 10 to 15 percent of its premium income.²³ As a health plan grows, the amount of risk-based capital related to underwriting also grows. However, if a health plan transfers some of the risk for the cost of healthcare to providers (for example, on a capitation basis), the plan's risk is reduced, and the RBC requirements are reduced commensurately.

Under the Risk-Based Capital for Health Organizations Model Act, regulators are to compare an organization's capital requirements (as determined by the health plan RBC formula) with the organization's **total adjusted capital** (its net worth as shown in the filing of its annual financial statements plus any other items specified in the formula instructions). If the organization's total adjusted capital is insufficient, regulatory intervention may be triggered, ranging from requiring the submission of a corrective action plan to placing the plan under the regulatory control of the appropriate state agency. In addition to serving as a tool for state regulators, the RBC formula provides start-up companies with the information needed to estimate initial RBC levels based on their operating projections for their first full year.

Summary

States license and regulate health plans of different types:

- HMOs must meet various requirements in financial matters, healthcare delivery (including network adequacy, quality assurance programs, and grievance procedures), enrollee contracts, filing and reporting, and disclosure.
- The regulation of PPOs differs depending on whether or not they bear risk. Risk-bearing PPOs must meet financial standards, while non-risk PPOs are often regulated as downstream contractors of insurers or employer plans.
- EPOs are regulated much like PPOs. The regulation of POS plans differs for standalone and wrap-around products. The activities of TPAs and entities performing utilization review are also subject to regulation.

Risk-based capital (RBC) is a method of measuring the minimum amount of capital appropriate for a health plan based on its size and risk profile.

NAIC Model Laws Regulating Health Plan Activities

In addition to model laws that target different types of health plans, the NAIC has developed a number of model laws to address various health plan activities. We briefly summarize some of these below.

In these model laws, the NAIC uses the term “health carrier.” The NAIC defines this term as “an entity subject to the insurance laws and regulations of this state or to the insurance commissioner’s jurisdiction that contracts or offers to contract to provide, deliver, arrange for, pay for, or reimburse any of the costs of healthcare services including a sickness and accident insurance company, a health maintenance organization, a non-profit hospital and health service corporation, or any other entity providing a plan of health insurance, health benefits, or healthcare services.”²⁴

Healthcare Credentialing Verification Model Act

All health carriers are required to establish a comprehensive credential verification program that verifies the credentials of all contracted healthcare professionals. Health carriers are required to establish written credential verification procedures that they disclose upon written request to any applying healthcare professionals. The act requires primary source verification for certain information such as licensure, liability coverage, hospital privileges, board certification, DEA registration, and education. Other information can be verified through secondary sources. Healthcare professionals must be recertified by obtaining primary source verification of specified information every three years. Health carriers must also allow providers to review and correct any information, including the source of the information, obtained by the health carrier as part of the credentialing process.

Quality Assessment and Improvement Model Act

This act requires health carriers that offer managed care plans to develop the systems necessary to measure and report on quality of healthcare services. All health carriers that offer managed care plans must have a quality assessment program and file a written description of the program with the insurance department. They must report to the appropriate licensure board any persistent and problematic pattern or care that leads to their termination or suspension of a contracting provider. In addition, health carriers offering closed plans or a combination plan with a closed component must have a quality improvement plan.

Health Benefit Plan Network Access and Adequacy Model Act

Under this act, health carriers offering a network plan must maintain a network that is sufficient in number and types of providers to ensure access to covered services without unreasonable delay. Health carriers must file with the insurance commissioner an access plan meeting the requirements of the act, and commissioners must determine network sufficiency under the criteria set forth in the act. Health carriers must meet general requirements pertaining to the provisions included in their intermediary and provider contracts. The act requires health carriers to provide electronic access to their provider directories and to update those directories at least monthly. This act also requires the filing of sample contract forms with the state regulatory agency and obtaining approval of any material contract changes. Recently added provisions of the act include those addressing non-contracted facility-based providers, which requires certain disclosures in non-emergency situations and provides for limitations on liability in emergencies, and provisions related to tiered provider networks.

Health Carrier Grievance Procedure Model Act

This act sets forth standards for internal review of member grievances. Under this act, health carriers must allow members to review relevant information pertaining to their grievance and submit written comments, documents, records, or other material relating to the request for benefits. Health carriers must develop written procedures for the expedited review of any grievance where the normal timeframe would jeopardize the life or health of the member. Health carriers must also use clinical peers, or peers “of the same or similar specialty” that typically manage the situation under review, to review adverse determinations related to utilization review. Grievances must be resolved within the timeframes set forth in the act. Health carriers may offer one level of review or may voluntarily offer a second level of internal review. The act provides that if the health carrier fails to follow the requirements for review or if an individual exhausts the carrier’s internal grievance procedure, the individual may appeal to an independent external review organization. Under federal law enacted as part of the Affordable Care Act, group health plans and health insurance issuers must comply with requirements for conduct of internal appeals similar to those of the NAIC Model Act. To the extent that state law imposes on health insurance issuers requirements that are stricter than those imposed under federal law, health insurance issuers must comply with those provisions of state law.

Uniform Health Carrier External Review Model Act

Health carriers must allow individuals to appeal adverse determinations or final adverse determinations to an independent external review organization. Independent review organizations must be accredited by a nationally recognized accreditation organization. This act sets forth standards for the conduct of standard and expedited external reviews including timelines for resolution of such reviews. It also sets forth standards for reviewing appeals of adverse determinations based on experimental or investigational status of the requested service. Under the Affordable Care Act, group health plans and health insurance issuers must comply with the applicable state external review process that, at a minimum, includes the consumer protections set forth in the Uniform External Review Model Act promulgated by the NAIC. If the state does not have such a process, group health plans and health insurance issuers must comply with the federal process, which is based on the NAIC Model Act.

Utilization Review and Benefit Determination Model Act

This act applies to health carriers and organizations that perform utilization review on their behalf. A health carrier that requires a request for benefits under a plan to be subject to utilization review must implement a written program that describes all review activities and procedures. The program must use documented clinical review criteria based on sound clinical evidence and evaluated periodically. Clinical peers must evaluate the clinical appropriateness of adverse determinations. Standard and expedited utilization review and benefit determinations must be made within the timeframes set forth in the act. Health carriers must provide written notice of adverse determinations with instructions for filing appeals and for requesting disclosure of the clinical rationale, including the clinical review criteria used to make the determination. Under this act, health carriers are required to cover emergency services needed to screen and stabilize a member, without prior authorization, if a prudent layperson acting reasonably would have believed that an emergency medical condition existed. Health carriers must also cover an emergency visit to a non-contracted provider if a prudent layperson would have believed that the delay in visiting a contracted provider would worsen the emergency.

Sources

¹ Jane Garwood et al., “State Regulation of Health Plans,” Supplement 6 of *Health Plan Law Manual*, vol. 1 (Gaithersburg, MD: Aspen Publishers, Inc., 1998), p. 23.

² National Association of Insurance Commissioners, “Health Maintenance Organization Model Act,” Section 3, 2003.

³ Ibid.

⁴ NCQA, “States Using NCQA Accreditation for Commercial Plans,” 2012, <http://www.ncqa.org/portals/0/Public%20Policy/WEB%20-%2001%20NCQA%20State%20Recognition%20Commercial%2030%202012.pdf>.

⁵ LOMA, *LOMA’s Glossary of Insurance Terms*, 2016.

⁶ Harriett E. Jones, *Regulatory Compliance: Insurance and Annuity Products* (Atlanta: LOMA, 1998), p. 122.

⁷ Ibid.

⁸ National Association of Insurance Commissioners, “Health Maintenance Organization Model Act,” Section 21.

⁹ Ibid.

¹⁰ Ibid.

¹¹ American Association of Preferred Provider Organizations, “Silent PPO White Paper,” 2010.

¹² Ibid.

¹³ Eric R. Wagner and Peter R. Kongstvedt, “Types of Health Insurers, Managed Care Organizations, and Integrated Healthcare Delivery Systems,” Chapter 2 in *The Essentials of Managed Care*, Sixth Edition (Peter Kongstvedt, ed.), 2012, p. 29.

¹⁴ Ibid.

¹⁵ This section excerpted and adapted from Harriet E. Jones, *Regulatory Compliance: Companies, Producers, and Operations*, 2002, pp. 63–67. Used with permission; all rights reserved.

¹⁶ Michael Bradford, “TPAs Battling Law, Competition,” *Business Insurance*, 22 Sept. 1997, p. 59.

¹⁷ *Gobeille v. Liberty Mutual Ins.*, 2016 WL 782861 (U.S. 2016), http://www.supremecourt.gov/opinions/15pdf/14-181_5426.pdf

¹⁸ Ibid.

¹⁹ Tom Wilder, “State Regulation of Managed Healthcare,” Chapter 28 in *The Essentials of Managed Care*, Sixth Edition (Peter Kongstvedt, ed.), 2012, p. 601.

²⁰ Bruce A. Johnson and Gerald A. Niederman, *Health Plan Legal Issues* (Englewood, CO: Medical Group Management Association, 1996), p. 185.

²¹ Ibid.

²² National Association of Insurance Commissioners, “Risk-Based Capital,” http://www.naic.org/cipr_topics/topic_risk_based_capital.htm.

²³ Centers for Medicare and Medicaid Services, Testimony of Jay C. Ripps to Consumer Operated and Oriented Plan (CO-OP) Advisory Board, January 13, 2011, www.cms.gov/CCIIO/Resources/Files/Downloads/j_ripps_testimony.pdf

²⁴ National Association of Insurance Commissioners, “NAIC Utilization Review and Benefit Determination Model Act,” Section 3, 2012.

8 State Regulation: Benefit Mandates and the Plan-Provider Relationship

Lesson Objectives

After completing this lesson, you should be able to:

- Explain what benefit mandates do and identify some of the most common state mandates.
- Discuss how the ACA and other federal laws affect and interact with state benefit mandates.
- Describe the regulation by states of various aspects of the health plan/provider relationship.
- Discuss the arguments for and against any willing provider laws.

In this lesson we will continue our exploration of state regulation of health plans. We will examine the state laws that mandate benefits and those that regulate the relationship between health plans and healthcare providers.

Benefit Mandates

Benefit mandates are laws that require health plans to cover certain healthcare services or the treatment of certain medical conditions. Over the years, the states have enacted a wide variety of benefit mandates. In most cases, this resulted from strong lobbying efforts by interested constituencies, such as healthcare providers or people with certain chronic diseases.

Some of the most common state mandates are for the following:

- emergency care
- mental health treatment
- alcohol and substance abuse treatment
- preventive services
- infertility treatment
- experimental treatments for cancer (such as bone marrow transplants)
- off-label uses for drugs to treat cancer and/or HIV/AIDS
- diabetes management
- services related to osteoporosis
- autism services

- cleft palate repair
- bariatric surgery
- breast reconstruction after a mastectomy or lumpectomy
- a hospital stay of a certain duration for certain conditions or procedures (such as maternity or mastectomy) (length-of-stay mandates)

States vary greatly in what mandates they impose and in how many mandates they have. In 2010 the number of mandates ranged from 13 in Idaho to 69 in Rhode Island.¹ A mandate may apply to one or all segments of the health insurance market (individual, small group, or large group) and to one or more health plan type (HMO, PPO, or POS). Two states may have the same or a similar mandate, but applicability differs.

Benefits are also mandated by some federal laws, notably the ACA.

- All health plans must generally comply with applicable federal benefit mandates.
- Health plans provided by the federal government and self-funded employer plans are generally exempt from state benefit mandates, but they are subject to mandates under the ACA and other federal laws.
- Most plans are subject to both state and federal mandates. They must generally meet the more rigorous standard.

Benefit mandates can increase health plans' costs, sometimes making it necessary to increase premiums.

Example: *The state of Arkizona requires health plans to cover a hospital stay of at least two days for a simple mastectomy. As a result, Smile Health Plan pays hospital charges it might not have paid if a hospital stay was determined not to be medically necessary. The plan also incurs some administrative costs: It must change plan literature, update its computerized claims payment system, amend all existing contracts with purchasers, and change its treatment guidelines and medical protocols to note this exception to its rules on medical necessity.*

The ACA recognizes the cost of state benefit mandates (although it includes many benefit mandates of its own). Under the ACA, if a state passes a benefit mandate that meets certain criteria, the state must pay the cost. This applies if the mandate:

- applies to individual and small group plans (sold on or off the exchanges)
- was passed after December 31, 2011, and
- exceeds the essential health benefits required by the ACA.

If such a mandate increases a plan's premium, the state must pay the premium cost attributable to the mandate.² Note that this rule does not apply to large plans, which are still subject to state mandates and must bear the cost of them.

States have reacted to this ACA rule in different ways:

- Some states pass mandates but exclude individual and small group plans.
- Other states specify that a mandate will not apply if it increases premiums.
- In addition, it seems likely that many states will simply refrain from enacting mandates, or at least more carefully consider their impact.

Over half the states have laws requiring the review of existing and/or proposed benefit mandates, in terms of their cost impact, their medical efficacy, and the need for the mandated service. For example, Colorado, Kentucky, and Oklahoma require an independent financial and social impact study for all proposed mandated benefits bills before they can be passed by the legislature.

Now let's look at some specific mandates enacted by states.

Mental Health Coverage

Almost every state has mental health coverage laws.

- Some laws mandate coverage of treatment for mental illness; some mandate parity (equal treatment) between coverage of mental illnesses and physical illnesses.
- Some laws mandate coverage of treatment of any type of mental illnesses; others mandate coverage of a subset of mental illnesses such as “severe disorders” or “biologically based illnesses” (such as schizophrenia, manic-depression, or bipolar disorder).
- Some states require coverage of substance abuse treatment; others specifically exclude this from the mandate.

The ACA includes mental health and substance abuse disorder treatment, including behavioral health treatment, among its essential health benefits, which individual and small group health plans must offer. However, the ACA does not require large group plans and grandfathered plans to cover mental health services.

Maternity and Mastectomy Length-of-Stay Mandates

The federal Newborns' and Mothers' Health Protection Act of 1996 (NMHPA) requires health plans to cover hospital stays for childbirth—at least 48 hours for normal deliveries and 96 hours for cesarean births. Many states also have maternity length-of-stay mandates that may be more stringent.

A number of states have mastectomy length-of-stay mandates.

- Some mandate a specific duration—for instance, in New Jersey health plans must cover 72 hours for a radical mastectomy and 48 hours for a simple mastectomy.
- In other states health plans must cover the hospital stay determined by the physician to be appropriate.

Emergency Care

In the past some HMOs denied coverage for services provided in an emergency room if it turned out that the illness or injury was not an actual medical emergency, as defined by the plan. State mandates for emergency care generally require health plans to cover emergency room services if a “prudent layperson” would have assumed the illness or injury to be an emergency. Beginning January 1, 2017, federal law will apply the prudent layperson standard to group and individual health plans.³

Other Mandates

Many state laws mandate coverage of certain preventive services, such as pap smears, mammograms, colorectal cancer screenings, and immunizations. The ACA now requires most health plans to cover a variety of preventive services without cost-sharing.

Diabetes can have many complications and can be very expensive to treat if left undetected or untreated for a long time. To both enhance patients’ quality of life and hold down the cost of their care, health plans have developed innovative strategies for the management of diabetes, often incorporating self-management. Many plans voluntarily include diabetes management, but nearly all states require coverage of equipment, supplies, and/or self-care training.⁴

In recent years many states have adopted laws requiring coverage of autism services. In 2015 alone seven states adopted such mandates, and as of December 1, 2015, 43 states and the District of Columbia have them.⁵

Many states have laws stipulating that when a certain type of provider (commonly podiatrists, nurse practitioners, clinical social workers, or chiropractors) furnishes a service that is covered by a health plan and is within the provider’s scope of licensure, the plan must provide coverage for it on the same terms as if it were furnished by another type of provider (such as a physician).

Some state laws address the use of formularies in pharmaceutical management, as is discussed in our lesson on pharmacy benefits.

Regulation of the Health Plan-Provider Relationship

The relationship between a health plan and an affiliated healthcare provider is based on the contract between the parties. However, states also have laws that regulate certain aspects of this relationship.

Any Willing Provider (AWP) Laws

Any willing provider laws allow any provider who meets a health plan’s terms and conditions of participation to become part of the plan’s network, or to receive payment from the plan for covered services provided to members whether the provider is part of the plan’s network or not. About half the states have some type of AWP law. Some states include a wide variety of providers, including physicians and pharmacists. Others have laws that apply only to pharmacists.

States vary in terms of when they consider that a provider is “willing” to participate in a plan’s network.

- Some require the provider to meet all the plan’s qualifications for participation, or to be appropriately licensed, reputable, and in good standing.

- Some go further and require the provider to comply with plan requirements relating to utilization review and the provision of cost-effective and clinically efficacious care.
- Some merely prohibit health plans from “unreasonably discriminating” against providers.

Proponents of any willing provider laws have included members of pharmacy, chiropractic, dental, home health, physical therapy, outpatient surgical, and clinical lab associations. They argue that selective contracting by health plans reduces the number of providers available in the network, decreases competition, and could sever long-standing relationships between patients and providers. They also contend that, in the absence of AWP laws, health plans limit access to certain types of providers.

Opponents of AWP laws include health plans, health plan associations, the NAIC, the National Governors Association, and many employers. They argue that these laws:

- Increase healthcare costs by eliminating incentives for providers to offer discounts in exchange for patient volume.
- Undercut quality initiatives by preventing plans from choosing and contracting with only the most qualified providers.
- Grant providers mandatory contracting privileges, not seen in any other industry or even elsewhere in the healthcare industry.

Direct Access Laws

Direct access laws give health plan members the right to see certain specialists in the plan’s network without a referral from a primary care provider (PCP). Most states require direct access to some type of provider. Most commonly, this applies to obstetricians and gynecologists, and beginning January 1, 2017, federal law will require this.⁶ But states have direct access laws for a variety of providers—for instance, Florida and Georgia for dermatologists, and New York for chiropractors.

Provider Contracts Provisions

Most states have laws requiring that certain provisions be included in, or omitted from, health plans’ participating provider contracts. Some states further require that template contracts be reviewed by the insurance department to confirm that such provisions are included or not.

Commonly required contract provisions:

- If the health plan does not pay a provider, the provider cannot seek payment from plan members (hold harmless clause).
- If a provider’s agreement with the plan is discontinued, the provider will continue treating certain patients at the contracted rate.
- The provider must grant the state department of insurance access to her records for purposes of auditing the health plan.

In a contract with a provider, a health plan *may not*:

- Require the provider to indemnify the plan for liability arising because of the plan's medical necessity decisions.
- Prohibit the provider from discussing a patient's full range of treatment options (no gag clause).
- Require the provider to participate in all products offered by the company at the discretion of the company (no all-products clause). For example, if a provider wishes to participate in a company's commercial health plan, the company could not require her to also participate in its Medicare and Medicaid plans.

Provider Due Process Laws

Provider due process laws require a health plan to disclose its criteria for selecting or deselecting providers. They also commonly require the plan to give an explanation of why a provider is deselected or rejected, and to give the provider a right to contest or challenge the decision.⁷

Provider Payment

Prompt payment. Every state has a "prompt payment" law that sets the timeframe within which a health plan must pay or deny a claim (generally from seven to 45 days). Most of these laws require the plan to pay a specified amount of interest on claims not paid within the required timeframe.

"Claw-backs." Another common state law provision is a limitation on the time during which a health plan can recover payment for claims it determines were incorrectly paid. For example, under Illinois law, 18 months after a claim is paid, the plan can no longer recover the amount paid, unless a formal finding of fraud or intentional misrepresentation has been made.

Incentives

A topic of considerable interest to consumers is the amount and type of any financial incentives offered to physicians related to their performance in meeting quality standards, the satisfaction rates of their patients, and their utilization of certain services. Health plans may offer incentives to minimize underutilization of some services (for instance, preventive services such as mammography or immunizations) or overutilization of others.

States frequently regulate physician incentive arrangements. There are two types of physician incentive laws:

- Some ban certain compensation arrangements that regulators believe may encourage providers to not refer members to specialists as necessary, or to not provide appropriate, medically necessary care.
- Other laws merely require the health plan to disclose to plan members any financial incentives available to participating providers.

Coverage Decisions

Some states have laws requiring health plans to disclose to both providers and patients the criteria that they use in making coverage decisions. Some common rules:

- Only physicians may be allowed to make adverse coverage decisions where the issue is medical necessity.
- The physician making the coverage decision must be in the same or similar specialty as the physician providing the care.
- The physician making the coverage decision must be licensed in the state where the patient resides.

Summary

Benefit mandates are laws that require health plans to provide coverage for certain healthcare services or for the treatment of certain medical conditions. States have a wide variety of mandates, for emergency care, preventive services, mental health and substance abuse treatment, diseases such as diabetes and osteoporosis, and other medical conditions and services. Self-funded employer plans and government plans are exempt from state mandates but subject to those of federal laws (notably the ACA); most plans are subject to both state and federal mandates.

States have laws regulating various aspects of the relationship between a health plan and its providers. These include:

- any willing provider laws giving any provider who meets a health plan's terms and conditions the right to serve its members,
- direct access laws giving plan members the right to see certain specialists without a referral,
- laws requiring or prohibiting certain provisions in the plan-provider contract, and
- laws guaranteeing due process for providers excluded from a network and prompt payment of their claims.

Sources

¹ Victoria Craig Bunce and J.P. Wiese, *Health Insurance Mandates in the States 2010*, Council for Affordable Health Insurance, 2010.

² Centers for Medicare and Medicaid Services, "Frequently Asked Questions on Essential Health Benefits," <https://www.cms.gov/CCIIO/Resources/Files/Downloads/ehb-faq-508.pdf>.

³ CFR Section 2590.715-2719A – Patient Protections, <https://www.law.cornell.edu/cfr/text/29/2590.715-2719A>.

⁴ Bunce and Wiese.

⁵ National Conference of State Legislatures, "Insurance Coverage for Autism," 2015, <http://www.ncsl.org/research/health/autism-and-insurance-coverage-state-laws.aspx>.

⁶ CFR Title 29, Part 2590 – Rules and Regulations for Group Health Plans, Section 2590.715-2719A Patient Protections, http://www.ecfr.gov/cgi-bin/text-idx?SID=b1df47d70023fee46c285c7131e94041&mc=true&node=se29.9.2590_1715_62719a&rgn=div8.

⁷ Alice G. Gosfield, *Guide to Key Legal Issues in Health Plan Quality* (New York: Faulkner & Gray, Inc., 1996), p. 147.

9 Other State Laws

Lesson Objectives

After completing this lesson, you should be able to:

- Identify the different types of state laws that affect health plans.
- Discuss state laws governing health plan sales and marketing, including sales representative licensing, advertising, and trade practices.
- Discuss state laws regulating the small group and individual health insurance market and how they interact with federal law.
- Identify the other state laws that impact health plans and state briefly what they require.

State Laws Affecting Health Plans

Health plans must comply with a variety of state laws. These include:

- Laws that apply to businesses of various sorts
- Laws related to healthcare
- General insurance laws
- Laws that target health plans or specific types of health plans

Some state laws applicable to all businesses affect health plans:

- In some states, general consumer protection laws apply to health plan claims payments.
- As employers, health plans are subject to laws that govern the workplace, from employment and hiring practices to health and safety standards.
- Health plans must comply with other laws governing diverse issues, such as civil rights, taxation, and zoning.

In this lesson, we will focus on state laws that have a substantial impact on health insurance and health plans. This includes laws in these areas:

- Healthcare
- Sales and marketing, including
 - Licensing of sales representatives
 - Advertising

- Trade Practices
- Small group and individual health coverage, including benefits, rating, and underwriting
- Antitrust issues
- Readability
- Reporting of data
- Privacy
- Claims and coordination of benefits
- Antifraud efforts
- Continuation and conversion
- States as purchasers

Healthcare-Related Laws

Since health plans deliver or arrange for delivery of healthcare, they are often governed by state laws related to healthcare. In our earlier discussion of the legal organization of health plans, we saw that they may be subject to state corporate practice of medicine laws. A number of other state laws may apply, such as those regulating:

- who may practice medicine,
- which facilities and individuals must be licensed,
- what types of services providers may deliver, and
- what fees providers may charge.

Two examples of state healthcare laws that affect health plans:

- Laws regulating payment rates for hospital services impact a health plan's ability to obtain discounts. For example, Maryland regulates the rates hospitals may charge, and New York requires hospitals to apply a surcharge for services rendered to health plans.
- States have laws requiring that before providers deliver certain types of healthcare they must secure the patient's consent. In managing healthcare through utilization review processes—such as precertification, case management, and telephone advice lines—a plan must be aware of each state's consent to treatment requirements.

Sales and Marketing

Typically, health plan products in the commercial market are sold by means of face-to-face interactions between a sales representative or agent and a prospective purchaser, often an employer. Health plan representatives also conduct marketing activities when recruiting providers, such as healthcare practitioners and hospitals. State laws regulate health plans' marketing and sales activities in various ways. For example, the HMO laws in some states require that an HMO's application for a certificate of authority include a description of the proposed method of marketing the plan. Health plans that violate these laws can face liability for damages, restitution, fines, or even criminal charges in cases of extreme misrepresentation or deception.

In the sections that follow, we will focus on three aspects of health plan sales and marketing:

- Sales representatives must meet all applicable licensing requirements. They must conduct business in accordance with all applicable laws and regulations and must not engage in any prohibited sales practices.
- Marketing and advertising material must be complete and accurate; may not be untrue, deceptive, or misleading; and must comply with state requirements, which are often very detailed.
- Unfair trade practices of various sorts must be avoided.

In this lesson we discuss state sales and marketing requirements; federal requirements are covered elsewhere in the course.

Licensing of Sales Representatives

States have licensing requirements that enable them to oversee the activities of individuals who engage in the sale of health plan products. As with other regulatory requirements, the specific licensing requirements vary from state to state but are similar in many respects. We'll use the **NAIC Producer Licensing Model Act** to illustrate state licensing requirements. Although not all states base their licensing laws on this Model Act, it has features that are common to licensing laws. In this section we will discuss:

- Who must be licensed?
- What are the licensing rules for residents and non-residents of a state?
- Who is eligible for a license?
- What is the duration of a producer license?

Who Must Be Licensed?

Sales representatives must be licensed in each state in which they sell, solicit, or negotiate insurance. The NAIC Model Act exempts certain individuals from the licensing requirements, including but not limited to:

- A health plan's officers, directors, and employees who do not sell, negotiate, or solicit insurance;
- Persons who assist in administering group health coverage and who do not receive commissions for their services; and
- Employers that provide employee benefit plans, trustees of employee trusts, and employees who administer such plans and who are not compensated directly or indirectly by the health plan or insurers that issued such plans.

A person (which can be an individual or a business entity) required to be licensed under the laws of a state to sell, solicit, or negotiate insurance is referred to as an **insurance producer**.

Resident and Non-Resident Licenses

Most insurance producers conduct their business in the state where they live; these producers must hold a **resident license** from the state. Individuals who do not live in the state but who wish to conduct business there must obtain a **nonresident license**.

State laws typically require applicants for a nonresident license to hold a similar license from another state and to be in good standing in that state. In other words, sales representatives usually must hold a resident's license from their home jurisdiction before being eligible to receive a nonresident license from another jurisdiction. Non-resident licenses are typically issued on a reciprocal basis. For instance, State A will award non-resident licenses to residents of State B, provided State B awards non-resident licenses to residents of State A on the same basis.

Who Is Eligible?

To be eligible for a license, individuals and business entities must meet requirements set by state law. For the requirements of the NAIC Producer Licensing Model Act, see below. Under the NAIC model, an individual must generally apply to take a written examination testing her knowledge of the lines of business for which the application is made, the duties and responsibilities of an insurance producer in the state, and the applicable laws and regulations.

An individual must also submit an application for licensure using the Uniform Application created under the Model Act. Once the insurance department receives a completed application and all applicable fees, the department may conduct whatever investigation it deems necessary before acting on that application. (Certain individuals are exempt from the application requirement, including those who were previously licensed for the same lines of business in another state, where their license is current or where the application is received within 90 days of cancellation of their license, as long as the individual was in good standing.)

Requirements for a nonresident license are generally the same as for a resident license. Some states that issue nonresident licenses impose a **countersignature requirement**. This means that applications from nonresident sales representatives must also be signed by an individual who holds a resident license.

The NAIC Producer Licensing Model Act: Licensing Requirements

For an individual:

- The applicant must submit the Uniform Application;
- Individual applicants must be at least 18 years old;
- The applicant cannot have committed any acts that are grounds for denial, suspension, or revocation, as set forth in the Act;
- Where required by the commissioner, the applicant must have completed a pre-licensing course of study for the line of business for which he/she is applying for licensure; and
- The applicant has paid the fees and passed the written examination in each line of business that they propose to sell.

For a business entity:

- The applicant must submit the Uniform Business Entity Application and pay any required fees; and
- The applicant must designate a licensed producer responsible for the business entity's compliance with the insurance laws, rules, and regulations of the state.

For a non-resident:

- The applicant must be currently licensed and in good standing in his/her state of residence;
- The applicant has submitted the application and paid the fees required by the state;
- The applicant has submitted the application that he/she submitted in his/her home state, or in lieu, a completed Uniform Application; and
- The applicant's home state awards non-resident producer licenses to residents of the state on the same basis.

Source: National Association of Insurance Commissioners, Producer Licensing Model Act, Sections 5 through 9, 2005.

Duration of License

Under the NAIC Model Act, as long as required renewal fees are paid and any continuing education requirements for individual resident producers are met, a producer's license continues for an indefinite time unless suspended or revoked. If the license lapses, the individual may reinstate it within 12 months without taking an exam. However, the individual must pay double the unpaid renewal fee.

The state insurance department has the authority to deny an application or suspend or revoke a license. Suspension or revocation can occur when an individual makes materially untrue statements in the license application, mishandles funds, violates an insurance law, or is convicted of a felony. In the event of an application denial or revocation, the insurance department notifies the sales representative of the reason for the action. The sales representative may request a hearing to appeal the action.

Advertising

General consumer protection statutes in many states regulate all types of advertisements, including those by health plans. Most states also have a variety of laws and regulations that specifically govern advertising of health coverage. The **NAIC Advertisements of Accident and Sickness Insurance Model Rule** provides a good framework for discussing the regulation of advertising for health coverage. Most states have adopted rules based on this NAIC model, although there are variations from state to state. The purpose of the NAIC model is to "assure the clear and truthful disclosure of the benefits, limitations, and exclusions of policies sold as accident and sickness insurance"¹ by means of guidelines and standards of conduct.

The NAIC model rule defines an advertisement to include:

- Printed and published material, audiovisual material, and descriptive literature of an insurer used in direct mail, newspapers, magazines, radio scripts, TV scripts, websites or other Internet displays or communications, other forms of electronic communications, billboards, and similar displays;
- Descriptive literature and sales aids of all kinds issued by an insurer, agent, producer, broker, or solicitor for presentation to members of the insurance-buying public such as circulars, leaflets, booklets, depictions, illustrations, form letters, and lead-generating devices of all kinds; and
- Prepared sales talks, presentations, and material for use by agents, brokers, producers, and solicitors whether prepared by the insurer or by the agent, broker, producer, or solicitor.²

The model rule requires health plans to "establish and at all times maintain a system of control over the content, form, and method of dissemination of all advertisements of its policies."³ It states that "all such advertisements, regardless of by whom written, created, designed, or presented, shall be the responsibility of the insurer whose policies are so advertised."⁴ Some states require health plans to maintain an advertising file that contains copies of all advertising pieces generated within a specified timeframe. This file must indicate whether each piece was reviewed for compliance with applicable requirements, how it was used, and whether it remains in use. Advertising materials are subject to regulatory market conduct examination.

Under the NAIC model rule, there are three types of advertisements, with different requirements applying to each.

- An **institutional advertisement** is designed to promote either the concept of health insurance or the insurer as a provider of health insurance. Such advertisements do not refer to any specific product.
- An **invitation to inquire** is designed to induce the audience to inquire further about a specific health policy. It may include a brief description of coverage, but states typically prohibit it from referring to the cost of coverage.
- An **invitation to contract** is any health policy advertisement other than an institutional advertisement or an invitation to inquire. Such an advertisement contains fairly detailed information about a specific health policy and usually must include the policy form number.

Because invitations to contract are designed to create consumer interest in purchasing a particular policy—such that a consumer's response to the ad is essentially an application for insurance—the states impose a number of specific disclosure requirements on such ads. For example, the advertisement must disclose any exceptions, reductions, or limitations.

- An **exception** eliminates coverage for a specific hazard. (For instance, charges for dental services are not covered.)
- A **reduction** reduces the amount of the benefit payable for a specified loss. (For instance, charges for dental exams are limited to \$40 per exam and not more than one exam in any calendar year.)
- A **limitation** restricts coverage in some other way. (For instance, charges for dental services, other than treatment due to an accident, are not covered.)

Exceptions, reductions, or limitations must generally be placed conspicuously and in close proximity to the statements to which they relate, or they must be put under prominent headings, such as "Exceptions," "Exclusions," "Conditions Not Covered," or "Limitations." Use of the words "only," "just," or "merely" to downplay the nature of limitations or cost-sharing features is prohibited. For example, an advertisement cannot state that "healthcare expenses are covered in full after you make a copayment of only \$15."

Some states have filing requirements for advertising material, which vary considerably by state.

- For instance, a state might require filing of all advertising material for HMO products, but not for other health plans. Another state might require filing for all small groups.
- Some states establish a timeframe for health plans to file certain types of advertising. For example, a health plan might be required to file each advertising piece at least 30 days prior to the date it intends to use the piece; if the insurance department does not reply by that date, the filing is deemed approved.

State advertising regulations also prohibit exaggerated claims. For instance, a health plan that requires members to file a claim form to be reimbursed for emergency care by a nonparticipating provider

cannot state that “you never have to file a claim form.” Most states require a health plan to disclose its legal entity name on all advertising, applications, policies, and claim forms. Most states prohibit advertisements that disparage other insurers or health plans, or advertisements that provide unfair or incomplete comparisons of plan provisions.

Unfair Trade Practices

Most states have a type of law typically known as an **unfair trade practices act**. Such a law defines certain practices as unfair and prohibits them in the business of insurance if they are committed flagrantly in conscious disregard of the law or so frequently as to indicate a general business practice. Such laws generally apply to all types of health plans, and to all individuals and legal entities involved in the sale or provision of insurance-related services, including sales representatives, insurers, third-party administrators. Many such state laws are based on the **NAIC Unfair Trade Practices Model Act**;⁵ to learn more, see below.

State unfair trade practices acts tend to contain broad prohibitions against certain types of practices, rather than attempting to identify every activity that is unfair. However, other state laws and regulations may prohibit certain specific acts, and these laws may reference a state’s unfair practices act, classifying those acts as unfair trade practices and making them subject to certain penalties.

States often prohibit offering inducements to enroll. For example, some states would not permit a health plan to give out exercise equipment to new members as part of a promotional campaign to increase membership. Unfair practices acts may apply to activities between health plans and providers. For instance, a health plan may be prohibited from discriminating against a provider based on the types of services typically rendered by the provider or the provider’s licensure if the provider is acting within the scope of that licensure.

NAIC Unfair Trade Practices Model Act

Section 4. Unfair Trade Practices Defined

Any of the following practices, if committed in violation of Section 3, are hereby defined as unfair trade practices in the business of insurance:

- A. Misrepresentations and false advertising of insurance policies;
- B. False information and advertising generally;
- C. Defamation;
- D. Boycott, coercion, or intimidation;
- E. False statements and entries;
- F. Stock operations and advisory board contracts (issuing or delivery stock or entering contracts of any kind offering returns and profits as an inducement to buy insurance);
- G. Unfair discrimination;
- H. Rebates (as an inducement for policies, with certain exceptions);
- I. Prohibited group enrollments;
- J. Failure to maintain marketing and performance records;
- K. Failure to maintain complaint handling procedures;
- L. Misrepresentation in insurance applications;
- M. Unfair financial planning practices;

- N. Failure to file or to certify information regarding the endorsement or sale of long-term care insurance; and
- O. Failure to provide claims history.

These practices are prohibited if committed (1) flagrantly and in conscious disregard of the Act or (2) so frequently as to indicate a general business practice.

Source: National Association of Insurance Commissioners, Unfair Trade Practices Model Act, Sections 3 and 4, 2004.

Small Group and Individual Health Coverage

Historically, each state regulated its small group and individual insurance markets. Since 1996 HIPAA has imposed federal requirements on matters such as guaranteed issue, renewability, discrimination based on health status, and exclusion of preexisting conditions. But states have continued to exercise substantial authority with regard to benefit design, underwriting, premium rating, and other issues.

The ACA further expanded federal regulation of small group and individual health coverage, sold both through and outside the exchanges. The ACA imposes requirements on benefit design, underwriting, and premium rating. However, states may still regulate individual and small group health coverage, to the extent that state requirements do not prevent compliance with federal law. In this section, we discuss state regulation of individual and small group insurance within the ACA framework.

Defining a Small Employer Group

Under federal law, a small employer is generally defined as one with an average of one through 50 employees on business days during the preceding calendar year. But states have the option of expanding this to up to 100 employees. In choosing whether to exercise this option, states must consider these arguments:

- Raising the limit to 100 increases the size of the risk pool for small employer groups, potentially decreasing costs.
- But including mid-size employers in the small group risk pool would give them less power to negotiate benefits and could potentially increase their costs.

To date, only a handful of states have chosen to increase the definition to 100 employees.

Uniform Benefit Design

Under the ACA, all insurers and health plans offering coverage in the individual and small group markets (other than grandfathered plans) must cover 10 categories of healthcare services, known as **essential health benefits (EHB)**. In addition, each state chooses a **benchmark** for health benefits based on health plans operating in the state. This benchmark can be based on the three largest small group plans, the three largest state employee health plans, the three largest federal employee health plan options, or the largest HMO offered in the state's commercial market. If a state does not choose a benchmark, it will be

the largest small group plan in the state. Generally, state mandated benefits (see lesson 8) will be included in the state's benchmark and be part of the essential health benefits in the state.

Restrictions on Premium Rating and Underwriting

Most states have laws regulating in some way medical underwriting and premium rating for health plans, often limiting the factors that can be considered. These state laws continue to apply only to the extent that they do not prevent application of ACA requirements. To recap ACA rules in this area:

- Health plans may not consider an individual's health status or medical history in offering or renewing coverage or impose preexisting condition exclusions.
- In setting the premium for an individual, plans can consider only age, tobacco use, family size, and geographic area. The oldest insured can be charged no more than three times the youngest, and tobacco users can be charged no more than 50 percent extra.

However, states can maintain more restrictive standards than the ACA. A few states require pure community rating. Others permit only smaller increases for age or tobacco use.

Most states require an insurer or health plan to obtain approval from the state before raising premium rates.⁶ Other states require only that the insurer file the new rates with the state insurance department or that the insurer file an actuarial certification regarding the rates. The ACA requires that, in any state, any proposed rate increase by individual or small group insurers at or above 10 percent must be scrutinized by independent experts to make sure it is justified. If CMS has determined that the state has an effective rate review program, that responsibility falls on the state; otherwise the review is done by the federal government. (As of 2016, almost all states were determined to have an effective program.)

State Antitrust Laws

Antitrust laws are designed to protect commerce from unlawful restraint of trade, price discrimination, price fixing, reduced competition, and monopolies. The health plan industry relies heavily on structural and operational integration, strategic partnerships, mergers, and acquisitions and so is affected by antitrust laws. Antitrust law also apply to the healthcare providers with which health plans collaborate. For example, antitrust law would prohibit competing hospitals from agreeing not to contract with a particular health plan unless the plan paid a certain price.

Under the McCarran-Ferguson Act, certain federal antitrust laws will not apply to the "business of insurance" as long as the state regulates in that area.⁷ However, courts are increasingly narrowing the definition of "the business of insurance." Moreover, the federal government has generally taken the position that mergers between health plans do not fall under "the business of insurance."

Most states have antitrust laws modeled on the federal Sherman Act and the Federal Trade Commission Act. State antitrust laws apply to all types of businesses, including health plans. However, many state laws include provisions calling for special treatment of health plans or healthcare providers.

- For example, Florida law contains an exemption for "the business of insurance" similar to federal law.

- A number of states have laws that exempt certain healthcare arrangements from “per se” scrutiny under the antitrust laws.
- Many states address antitrust issues through anti-competition provisions that are found in their insurance holding company laws.

State attorneys general enforce both federal and state antitrust laws. State insurance commissioners enforce insurance holding company laws. Private parties may be able to bring lawsuits under the antitrust laws. In looking at proposed mergers and acquisitions, regulators determine if a particular transaction would result in substantially reduced competition or a monopoly. Typically, they make this determination by looking at the market shares of the entities involved, as well as other factors, such as number of competitors in the market, volatility of ranking of market leaders, and ease of entry into and exit from the market.⁸

Other Areas of State Regulation

Readability

Many states have **readability laws**, also called **plain language laws** or **policy simplification laws**. The purpose is to simplify the language in the evidence of coverage and make it easier for health plan members to understand the plan provisions. Readability laws require plan documents to meet or exceed a minimum readability score. This score is determined by a formula that includes the average number of syllables per word and the average number of words per sentence. Typically, the health plan calculates and certifies the score when it submits forms to the state insurance department for approval.

Reporting Data

Many states have established what are known as “all payor claims databases.”⁹ In such states, health plans (and other entities) are required to report certain data to a state agency. This may include claims data, quality data, reports, and other member information. Generally, the states have timeframes, format requirements, and penalties for failure to comply.

***Example:** Minnesota requires the submission of enrollment, claims, and pricing information on a quarterly basis, in a specified electronic format.¹⁰*

Recently, in *Gobeille v. Liberty Mutual Ins. Co.*, the U.S. Supreme Court ruled that states cannot require self-funded employer health plans to submit information to all payor claims databases, because of the preemption of such state laws by ERISA.¹¹

Privacy

In doing business, health plans gather and hold a large amount of information about individuals. HIPAA and the Health Information Technology for Economic and Clinical Health (HITECH) Act impose obligations on health plans with regard to the confidentiality and security of personally identifiable health information. These laws regulate how health plans, their providers, and organizations performing functions on their behalf may use and disclose medical information. They also set forth individuals’ rights to access their own information and understand how it is used and disclosed.

However, HIPAA and HITECH generally serve only as a floor and do not preempt laws enacted by many states that offer more protection and rights to individuals. Many states have stricter limits than HIPAA about the use and disclosure of information on certain matters (such as HIV, mental health, substance abuse, or sexually transmitted diseases). Other state laws include stricter provisions related to other aspects of confidentiality and security. For example, Texas law goes beyond HIPAA or HITECH to expand the types of entities subject to confidentiality and security requirements, impose higher penalties for violation of the law, and set forth more stringent training requirements and shorter deadlines to notify individuals of security breaches.

Claims Settlement

Some state laws identify improper claims practices and prohibit insurers or health plans from engaging in them. Some laws apply to all insurance, and others to accident and health insurance. There is an **NAIC Model Unfair Claims Settlement Practices Act** for all insurance¹² as well as a model for life, health, and accident insurance.¹³ For more about the general model law, see below.

NAIC Model Unfair Claims Settlement Practices Act

Below are some acts that would be considered improper claims practices under the Act, if they are committed “flagrantly or in conscious disregard of this Claims Act” or so often as to appear to be a general business practice.

- Knowingly misrepresenting to claimants and insureds relevant facts or policy provisions;
- Failing to acknowledge with reasonable promptness communications about claims;
- Failing to adopt and implement reasonable standards for the prompt investigation and settlement of claims;
- Not attempting in good faith to effectuate prompt, fair, and equitable settlement of claims submitted in which liability has become reasonable clear;
- Compelling insureds or beneficiaries to institute suits to recover amounts due by offering substantially less than the amounts ultimately recovered in such suits;
- Refusing to pay claims without conducting a reasonable investigation;
- Failing to affirm or deny coverage of claims within reasonable time after having completed an investigation;
- Attempting to settle claims on the basis of an application that was materially altered without notice to, or knowledge or consent of, the insured;
- Making claims payments to an insured or beneficiary without indicating the coverage under which each payment is being made;
- Failing, in the case of claims denials or offers of compromise settlement, to promptly provide reasonable explanations of the basis for such actions; and

- Failing to provide forms necessary to present claims within fifteen (15) calendar days of a request with reasonable explanations regarding their use.

Source: National Association of Insurance Commissioners, Unfair Claims Settlement Practices Act, January 1997.

Coordination of Benefit

What happens when an expense incurred by a person is covered by more than one insurance policy or coverage? For instance, an expense might be covered by two group health plans, or a health plan and the medical benefit provisions of an automobile insurance policy.

- State **coordination of benefits (COB)** laws dictate which plan pays benefits first (that is, which is the primary payor) and which pays second (the secondary payor).
- The primary payor pays benefits as if the person had no other coverage. The secondary payor generally pays less than if it were the only coverage. The amount paid by the secondary payor is determined by the COB provision in its contract and/or according to the state's COB law, taking into account the benefits paid by the primary payor.

The **NAIC Coordination of Benefits Model Regulation** (most recently revised in 2013) sets forth rules about the order of benefits, the amount payable by the primary and secondary payors, and allowable expenses.¹⁴

Example: *Nancy is a member of an HMO, which does not cover out-of-network care except in an emergency. She is also covered by her husband's health plan, a PPO, which does cover out-of-network care. Nancy obtains care from a specialist not in her HMO's network, in a non-emergency situation.*

Under the NAIC model regulation, Nancy's HMO is the primary payor and the PPO the secondary payor. The HMO pays as if Nancy only had coverage under the HMO, which means it does not pay for the out-of-network care. The PPO is the secondary payor, and it pays for Nancy's care not covered by the primary payor, the HMO.

The model regulation also addresses situations in which a plan provides benefits in the form of services, not payments. In such instances, the reasonable cash value of each service is considered an allowable expense.

Antifraud Efforts

Because of the high cost of fraud to health plans, purchasers, and consumers, a number of states have enacted legislation designed to reduce fraud. Many such laws include requirements for health plans.

- Some states require plans to report all fraudulent activities that they discover.
- Some require plans to establish special anti-fraud units or systems.

- Some require plans to include a fraud warning on applications for coverage and/or on claim forms.

Further information on fraud and abuse can be found in the AHIP anti-fraud series of courses.

Continuation and Conversion

When person will no longer be eligible for a group health plan (because, for instance, his or his spouse's employment is ending), a **continuation law** allows the person to continue coverage. There are both federal and state continuation laws.

- The federal **Consolidated Omnibus Budget Reconciliation Act (COBRA)** applies to employers with 20 or more employees. The continuation coverage must be identical to the plan the employer provides to current employees. The individual is responsible for paying the full premium at the group rate (the employer is not required to continue contributing to the premium), and she may also be charged a small administrative fee.
- Several states also have continuation laws. They either apply to groups not subject to COBRA (fewer than 20 employees) or go beyond COBRA rules in requiring health plans to provide for continuation coverage.

A **conversion privilege** allows an individual to convert group coverage that is terminating to individual coverage. Many states have laws requiring health plans to offer conversion in some circumstances. However, under the ACA an individual who loses group coverage may enroll in an individual plan outside of the annual enrollment period, so state conversion laws have lost much of their relevance.

States as Purchasers

State governments also regulate health plans in their role as purchasers of healthcare.

- State governments employ thousands of workers for whom they purchase group health coverage. State employee health plans may be subject to unique bidding and contractual requirements that are specified in laws, regulations, or administrative guidelines.
- States also purchase healthcare for their Medicaid programs and Children's Health Insurance Program (CHIP).
- States arrange for workers' compensation coverage for employees in the state. These programs must comply with unique requirements under workers' compensation laws that are enforced by state departments of labor or similar agencies. (We discuss workers' compensation in more detail elsewhere in this course.)

State Laws in Historical Perspective

Many of the laws we have discussed in the last three lessons were enacted by states as part of a comprehensive package of legislation, commonly referred to as a "patient protection act" or a "consumer bill of rights." Many were enacted between 1995 and 2001, a period of some industry upheaval and consumer pushback against managed care.

Such legislation typically addressed issues such as grievance procedures, coverage of emergency room treatment, external review, provider credentialing, direct access to specialists, physician/patient communication, disclosure of financial incentives, provider contracting, point-of-service option mandates, qualification standards for utilization reviewers, network adequacy and performance, confidentiality of medical records, use of drug formularies, and continuity of healthcare coverage.

Many of these laws provided a basis for future federal legislation, such as the Affordable Care Act.

Summary

A wide variety of state laws affect health plans in various ways. These include general business laws, healthcare laws, insurance laws, and laws targeting health plans or certain types of health plans. These laws regulate many aspect of health plan operations:

- Sales and marketing, including licensing of sales representatives, advertising, and trade practices.
- Small group and individual health coverage, including benefits, rating, and underwriting.
- Antitrust issues, readability, reporting of data, privacy, claims, coordination of benefits, antifraud efforts, continuation of coverage, conversion, and state governments as purchasers of health care.

These state laws interact with the ACA and other federal laws in various ways.

Sources

¹ National Association of Insurance Commissioners, “Advertisements of Accident and Sickness Insurance with Interpretive Guidelines, NAIC Model Rule” (MDL-40), Section 1, April 1999.

² Id. at Section 3.

³ Id. at Section 2.

⁴ Id.

⁵ National Association of Insurance Commissioners, “Model Unfair Trade Practices Act” (MDL 880), January 2004.

⁶ Consumers’ Union, “50 State Overview: Statutes, Type of Rate Review, and Public Participation,” <http://consumersunion.org/wp-content/uploads/2014/06/AppendixA.pdf>.

⁷ 15 USC 1011 et seq. (The activity may not be an agreement to boycott, coerce, or intimidate, nor an act of boycott, coercion, or intimidation, to be exempt.)

⁸ Thomas A. Player and Steven B. Najjar, “State Antitrust Regulation of Insurance Company Take-Overs,” *Resource*, April 1998, p. 16.

⁹ For citations to state implementing legislation or regulations for all payor claims databases, see the APCD Council’s “APCD Legislation by State,” <https://www.apcdouncil.org/apcd-legislation-state>.

¹⁰ Minnesota Rules, part 4653.0100 through 4653.0600 (2009).

¹¹ *Gobeille v. Liberty Mutual Insurance Company*, 136 S. Ct. 936 (2016).

¹² National Association of Insurance Commissioners, “Unfair Claims Settlement Practices Act” (MDL 900), January 1997.

¹³ National Association of Insurance Commissioners, “Unfair Life, Accident, and Health Claims Settlement Practices Model Regulation” (MDL 903), July 1997.

¹⁴ National Association of Insurance Commissioners, “Coordination of Benefits Model Regulation” (MDL 120), October 2013.

10 Workers' Compensation

Lesson Objectives

After completing this lesson, you should be able to:

- Explain how state workers' compensation laws emerged and how they protect the interests of both employees and employers.
- Distinguish between compulsory and elective workers' compensation systems.
- Describe the ways in which employers may provide workers' compensation.
- Compare and contrast workers' compensation and health insurance coverage.
- Describe workers' compensation disability benefits.
- Identify ways employers, insurers, and states are addressing workers' compensation cost increases.
- Describe how managed care is used in workers' compensation.

Workers' Compensation: An Overview

Under state laws, employers are generally required to provide workers' compensation benefits for their employees. These benefits cover work-related injuries and illnesses—those arising out of or in the course of employment. Benefits are paid for medical expenses, lost earnings, and rehabilitation services. If death results from employment, a benefit may be paid to dependents.

Why do such laws exist? Before they were enacted in the early 20th century, employees who were injured on the job were not automatically entitled to receive compensation from their employers. An injured employee's only recourse was to sue his or her employer under common law, alleging that the employer had been negligent in fulfilling a duty. Relevant duties included to maintain a safe workplace, tools, and equipment; to develop, implement, and enforce safety rules and regulations; to warn employees of dangers in the workplace that they could not reasonably be aware of; and to provide an adequate number of competent employees for the work. However, under common law, the employer had defenses against allegations of negligence. An employee could not recover for an injury if the employer could show one of the following:

- Assumption of risk—the employee voluntarily entered the workplace, knew how unsafe the job was, and understood the risks in performing it.
- Contributory negligence—the employee's own negligence contributed to his injury.
- Fellow employee negligence—the injury resulted solely from the negligence of another employee.

Also, under common law, generally only the injured worker could bring such a lawsuit, not the worker's dependents in the case of death.¹

The system of state workers' compensation laws represents a compromise between the needs of employees and those of employers. On one hand, workers receive benefits to cover expenses resulting from a work-related injury or disease. On the other hand, employers are protected from lawsuits by workers. The **exclusive remedy doctrine** states that employees who are injured on the job are entitled to the benefits provided by workers' compensation but cannot sue their employers for any additional amounts. In exchange for this protection, though, an employer is not allowed to deny liability for work-related injuries and must provide benefits for them even if they are not the employer's fault. (There are a few exceptions in which employers are not required to pay benefits: injuries that are intentional or self-inflicted, caused by the worker's intoxication, or caused by the worker's willful violation of safety regulations.²)

Compulsory or Elective?

All states except one have compulsory workers' compensation; Texas has an elective system.

- In a **compulsory system**, employers must provide workers' compensation benefits for their employees. However, which employers and which employees are covered by this requirement can vary by state. For instance, small employers and certain categories of employees (such as domestic workers) may be exempt.
- In an **elective system**, an employer may choose not to participate in the workers' compensation system. However, a nonparticipating employer is open to large liability lawsuits. An employee can bring a negligence-based lawsuit, and the employer usually cannot use the common law defenses cited above (assumption of risk, contributory negligence, fellow-employee negligence).

Providing Workers' Compensation

An employer may provide workers' compensation benefits in four main ways:

- **Private insurance.** In the most common approach, the employer purchases workers' compensation coverage from a private-sector insurance company. An insurer can be licensed to sell workers' compensation coverage in one or more states. As in group health insurance, an insurer is not required to offer coverage to every employer that applies; each employer is underwritten to determine if its risk profile is attractive to the insurer and at what price.
- **Optional state coverage (competitive state funds).** In some states, employers may purchase coverage from a state program as well as a private insurer. However, the state program may not be able to meet all of an employer's needs. For instance, while an insurer may be able to provide a policy covering employees in multiple states, a state program may not.
- **Mandatory state coverage (monopolistic states).** Four states (North Dakota, Ohio, Washington, and Wyoming) require all workers' compensation coverage to be purchased from the state government.
- **Self-insurance.** Most states allow an employer to self-insure—that is, to pay workers' compensation benefits to its employees out of its own funds instead of purchasing coverage. An

employer must provide proof of its ability to pay benefits and meet other state laws and regulations (which of course vary by state). A number of states allow for group self-insurance; each state has its own rules on how groups are formed, who can purchase insurance through the group, and how the group must be managed. Self-insurance is not a viable option for all employers because of state financial and administrative requirements. However, it can reduce costs by allowing the employer to control claims management and to reap the benefits of measures cutting claims by improving workplace safety.

Other State Regulation

Workers' compensation is regulated by the states in other ways as well, and the laws vary widely from state to state. The administrative cost of ensuring compliance with regulations and keeping abreast of changes in them can be high, especially for insurers and employers operating in several states.

Most states that allow private health insurance plans to provide workers' compensation benefits have certification requirements to ensure that plans provide high-quality care. Employers and health plans have developed national accreditation standards for workers' compensation PPOs and utilization review organizations through the American Accreditation Health Care Commission (AAHCC/URAC). Several states now allow health plans to satisfy certification requirements with AAHCC/URAC accreditation.

Workers' Compensation and Health Insurance

Employers' workers' compensation programs and their group health insurance plans both pay benefits for the medical expenses of employees, but there are several important differences in the two coverages.

Medical Coverage Under Workers' Compensation and Group Health Insurance: Key Differences

Workers' Compensation	Group Health Insurance
Employers must provide coverage to most employees.	Some employees, such as part-time or temporary workers, may not be covered.
Coverage must be free for employees.	Employees usually pay at least a portion of the premium.
No employee cost-sharing (copayments or deductibles)	Cost-sharing common
State laws may set some limits and maximums on benefits.	The ACA prohibits annual and lifetime limits for most benefits.
Covers work-related injuries and illnesses only.	Covers work-related and non-work-related injuries and illnesses.
In addition to medical expenses, benefits for lost wages, rehabilitation, and death	Medical expenses only

Employees often have the right to choose their own healthcare providers (depending on state law).	Employees may have limited choice of providers (depending on the structure of the health plan).
---	---

Let's look at some of these differences in a bit more detail.

- **Eligibility.** In almost every state, employers are required to provide workers' compensation to all employees, regardless of the number of hours worked. (As noted earlier, in some states there are exceptions for a few categories such as domestic workers.) In contrast, group health plans typically limit coverage to full-time employees (now defined under the Affordable Care Act as working at least 30 hours a week or 130 per month³).
- **Employee costs.** Workers' compensation laws require employers to provide coverage at no cost to employees. There can be no employee contributions to premium and no cost-sharing such as deductibles and copayments.
- **Coverage limits.** The Affordable Care Act prohibits health plans from setting annual or lifetime dollar limits on most types of benefits.⁴ In contrast, workers' compensation coverage may include an annual or lifetime benefit maximum or limit benefits to a certain number of days or visits. However, such limits can only be established by state statutes—an insurer or employer may not set limits beyond state law.
- **Non-work-related injuries and illnesses.** Workers' compensation pays benefits only if an injury or disease is work-related—that is, it arose out of and in the course of employment. Group health plans pay benefits for both work-related and non-work-related conditions. (However, health plans typically do not pay duplicate benefits—they exclude expenses covered by workers' compensation. If a plan pays expenses that should have been covered by workers' compensation, the plan often seeks reimbursement through subrogation.⁵)
- **Non-medical benefits.** Health plans generally pay benefits only for medical expenses. Workers' compensation pays benefits to help cover lost earnings and sometimes for rehabilitation services. If an employee dies because of a work-related injury or illness, a death benefit may be paid to her dependents.
- **Choice of providers.** While many health plans limit members' choice of providers in some way, most state workers' compensation laws give employees a great deal of freedom in this regard. In some states employees have the right to use any provider they choose and to change providers whenever they like. Even in states that allow workers' compensation programs to have provider networks, employees may be able to opt out of the network.

Disability Benefits

Workers' compensation includes **disability benefits** (also called **indemnity benefits**), which replace an employee's earnings while she is unable to work because of a work-related injury or illness. For employers, disability benefits are a major component of workers' compensation, accounting for almost half of costs.⁶ For employees, these benefits are an important part of the safety net—as a source of

disability benefits, workers' compensation is surpassed in expenditures only by the federal Social Security Disability Insurance (SSDI) and Medicare.⁷

In workers' compensation, there are four classes of disability benefits:

- **Temporary total disability (TTD)** (the most common)—the employee is expected to recover and return to work but is unable to do any type of work while recovering.
- **Permanent total disability (PTD)**—the employee is unable to perform any kind of work for the remainder of his or her life.
- **Temporary partial disability (TPD)**—the employee can work but is unable to earn his or her usual wage until full recovery.
- **Permanent partial disability (PPD)**—the employee suffers an injury from which he or she will never recover, but the injury is such that the ability to work is only partially affected.

Waiting period. For workers' compensation disability benefits (but not medical benefits), there is a waiting period (similar to a deductible in other coverages). For a certain time (typically three to seven days) after disability is established, benefits are not payable. However, if the disability ends up lasting beyond a certain number of weeks (set by state law), benefits are paid for the waiting period.

Workers' Compensation Costs

While workers' compensation accounts for only a small portion of the total cost of providing employee health coverage, nonetheless employers pay billions of dollars a year for it. And medical expenditures have risen even more quickly under workers' compensation than under health insurance. Why?

- Because employees do not pay deductibles or copayments under workers' compensation, they have little incentive to use cost-effective providers.
- Workers' compensation now pays benefits for injuries and illnesses not covered in the past, such as carpal-tunnel syndrome and work-related stress disorders.

In addition, it is feared that workers' compensation disability costs may begin to rise. The population is aging, and older people tend to need more time off the job when they are injured or ill.

Efforts to Control Costs

Many employers have taken an active role in seeking to hold down workers' compensation costs.

- **Return-to-work programs.** The sooner an employee can return to work, even on a part-time or "light duty" basis, the lower the total disability payments. The development, implementation, and monitoring of programs to ease and hasten the return to work can significantly cut costs.
- **Safety programs.** An effective workplace safety program can cut the rate of accidents through the training of new employees (including proper lifting techniques and defensive driving) and the use of safety equipment (including personal protective equipment). Safety must become

part of the culture of the workplace, and personal accountability (of employees, supervisors, and management) is necessary to maintain a safe culture.

In addition, as we will discuss below, employers and insurers have worked to detect and prevent fraud in workers' compensation claims and to adopt managed care strategies and techniques to both cut costs and improve quality.

Fraud

Workers' compensation fraud costs insurers and employers billions of dollars a year. It can take several forms.

- **Non-work related conditions.** An employee may claim that her injury or illness is work-related when it is not, and apply for benefits under workers' compensation instead of health insurance. Why? A health plan may charge a large deductible and substantial copayments, while a workers' compensation program does not. A part-time employee may not be covered by an employer's health plan but is covered by workers' compensation.
- **Disability fraud.** An employee may claim to be unable to work when she really can. She may be truly disabled at first but remain on disability after she has recovered, or she may claim to have a total disability when she is only partially disabled and could work part-time or on light duty. Since under workers' compensation an employee often has the right to choose her own provider, she may seek out a physician willing to state that she is disabled.
- **Healthcare fraud.** Physicians may exaggerate the extent of an injury so they can provide (and charge for) treatments that are not really needed, or continue treating a patient after recovery. Since the employee is not paying a deductible or copayments and does not have to work, he may not object to or report the unnecessary care.

The Impact of the Affordable Care Act

While the Affordable Care Act focuses on health insurance and does not directly regulate workers' compensation, it may have an indirect impact. We have seen that employees sometimes seek benefits under workers' compensation for non-work-related conditions, either because workers' compensation is more generous than their health coverage or because they have no health coverage. With more workers receiving adequate health coverage under the ACA, the need to do this should be diminished, and one study suggests that as result there could be a shift of costs away from workers' compensation to health insurance.⁸

State Government Responses to Rising Costs

Like employers and insurers, state governments have taken steps to control workers' compensation costs.

- **Fee schedules** (usually based on Medicare and Medicaid schedules⁹) specify the maximum amounts healthcare providers may charge for certain services under workers' compensation. Fee schedules ensure that some providers do not charge substantially more than the norm, and they can limit annual increases.

- **Clinical practice guidelines** (treatment guidelines) for injuries covered by workers' compensation have been adopted by several states. Such guidelines are a utilization management and quality management mechanism designed to aid providers in making decisions about the most appropriate course of treatment for a specific case, and to avoid unnecessary and inappropriate services.

Managed Care

The managed care approach is actively used in workers' compensation to ensure quality of care, to hold down medical expenses, and (by hastening the return to work) reduce disability benefits.

- **Contracted providers.** Workers' compensation programs negotiate contracts with healthcare providers to deliver services to covered employees at discounted rates in return for a volume of patients. The performance of contracted providers can be measured in terms of high-quality, cost-effective care, as well as returning employees to work quickly and minimizing the potential for re-injury.
- **Utilization review** seeks to ensure that providers deliver the most appropriate services to a patient (not unnecessary and otherwise inappropriate care). It can include prospective review (before services are delivered, as in preadmission certifications and second opinion requirements) and concurrent review of ongoing treatment.
- **Case management.** A case coordinator (usually a nurse or other healthcare professional) can manage an employee's care to ensure that she is getting the services she needs and that overlapping and redundant services from different providers are avoided. In worker's compensation, **total disability management** is often practiced; this is a type of case management that develops treatment plans that emphasize the early return to work and include **return-to-work protocols**—detailed plans of medical treatment designed to facilitate the patient's resumption of employment. If a patient has partially recovered, a treatment plan might allow her to work limited hours or on a light duty basis while continuing treatment.
- **Occupational health services** are delivered by healthcare providers specializing in protecting the health and safety of employees at work, treating work-related injuries and illnesses, and facilitating employees' return to work. These providers are often more experienced than regular providers in treating certain conditions and more aware of different options for returning to work (such as light duty).

Issues in Managed Care

- **Out-of-network care.** As we have seen, while in some states workers' compensation laws allow employees to be directed to certain providers, in others employees have the right to seek care from any provider they choose. In the first group of states, managed care structures that require the use of participating providers (such as HMOs and EPOs) can be used in workers' compensation. In the second, PPOs must be used, and this is the most common structure in workers' compensation.
- **Incentives for in-network care.** In health insurance PPOs, members are given incentives to use network providers, including lower cost-sharing and out-of-pocket expenses. But in workers'

compensation, there are no copayments or deductibles. So employers and health plans can only encourage the use of network providers by such means as informing employees of the superior quality of in-network care.

The Integration of Workers' Compensation and Other Benefits

As we've seen, workers' compensation overlaps with both health insurance and disability income insurance, and some employers have sought to better integrate these coverages. An **integrated healthcare and disability plan** can combine in one program absence management, short-term disability, long-term disability, workers' compensation, and health coverage. This can have certain advantages: Employers can reduce administrative expenses with only one plan instead of several. And under an integrated plan, it is not necessary to devote time and resources to determining whether an illness or injury is work-related, since it will be covered whether it is or not.¹⁰

But there are also drawbacks to integration. It can require additional coordination between employer departments; for instance, health coverage may be overseen by the employee benefits department while workers' compensation is the responsibility of the risk management department. And although integration may produce administrative savings over the long run, the process of initially designing and implementing an integrated program can be expensive. Finally, until recently, few insurers offered products that combined health, disability, and workers' compensation coverage.

Summary

State laws generally require employers to provide workers' compensation benefits for their employees. These benefits cover injuries and illnesses arising out of or in the course of employment, and they pay for medical expenses, lost earnings, and rehabilitation services. An employer may provide workers' compensation by purchasing private insurance, participating in a state program (which may be optional or mandatory), or by self-insuring (self-funding) its own program.

Both employers' workers' compensation programs and their group health insurance plans pay benefits for the medical expenses of employees. But there are differences, the most important being that workers' compensation covers only work-related injuries, it is provided for part-time as well as full-time employees, and it does not charge employees premiums or cost-sharing.

Many employers have sought to hold down workers' compensation costs, by sponsoring return-to-work and safety programs, combating fraud, and adopting managed care strategies (including contracted providers, utilization review, case management, and the use of occupational health services). State governments have sought to control costs through fee schedules and clinical practice guidelines.

Sources

¹ Alan Pierce, "Workers' Compensation in the United States: The First 100 Years," March 14, 2011, <http://www.lexisnexis.com/legalnewsroom/workers-compensation/b/workers-compensation-centennial/archive/2011/03/14/workers-compensation-in-the-united-states-the-first-100-years.aspx>.

² Donna J. Popow, JD, CPCU, AIC, "Principles of Workers' Compensation Claims," 3rd Edition, American Institute for Chartered Property Casualty Underwriters, Insurance Institute of America, page 1.10.

³ Internal Revenue Service, "Identifying Full-time Employees," December 1, 2015, <https://www.irs.gov/Affordable-Care-Act/Employers/Identifying-Full-time-Employees>.

⁴ U.S. Department of Health & Human Services, <http://www.hhs.gov/healthcare/about-the-law/benefit-limits/index.html#>.

⁵ Patrick Connoles, AIS Health, "Health Insurers Tap Subrogation Vendors to Rein in Medical Costs, Help Tame Waste," November 10, 2014, <https://aishealth.com/archive/nhpw111014-02>.

⁶ National Academy of Social Insurance, "Workers' Compensation and Disability," <https://www.nasi.org/book/export/html/164>.

⁷ Ishita Sengupta and Virginia Reno, Social Security Administration, "Recent Trends in Workers' Compensation," Social Security Bulletin, Vol. 67, No. 1, 2007, <https://www.ssa.gov/policy/docs/ssb/v67n1/v67n1p17.html>.

⁸ Cognizant 20-20 Insights, "The Affordable Care Act and Its Impact on Workers' Compensation," July 2015, <http://www.cognizant.com/InsightsWhitepapers/the-affordable-care-act-and-its-impact-on-workers-compensation-codex1329.pdf>.

⁹ Barry Lipton, John Robertson, and Dan Corro, NCCI Holdings, Inc., "Medicare and Workers Compensation Medical Cost Containment," January 2010, https://www.ncci.com/Articles/Documents/II_Medicare-Research-Brief-Jan10.pdf.

¹⁰ Prudential Financial, "The Market for Integrated Health Care and Disability Management," January 2007, page 7, http://www.prudential.com/media/managed/102184_IHCDMRepnocrops.pdf.

Lesson #11/12 Pharmacy Benefit Managers

Lesson Objectives

After completing this lesson, you should be able to:

- Explain what pharmacy benefit managers (PBMs) do and how they operate.
- Describe PBM networks and discuss the laws that apply to them.
- Describe PBM formularies and the requirements of Medicare Part D.
- Define therapeutic, generic, and biosimilar substitution and discuss the laws that apply to them.
- Discuss issues in the transparency of PBMs' pricing processes and the laws that address them.

The pharmacy business and prescription drug benefits are regulated at the state level, with laws and regulations differing considerably from state to state. Some federal laws also apply, including the Affordable Care Act (ACA). In this lesson and the next, we will offer an overview of how this industry operates and discuss the main state and federal laws that govern it. In this lesson, we will focus on pharmacy benefit managers (PBMs) and their use of networks, formularies, and utilization management. In the next lesson we will look at pharmacies, prescribers, and drugs.

Trends in Pharmacy and Drug Benefits

The pharmacy business and prescription drug benefits have been heavily influenced by five key trends:

- The **rapid pace of innovation** in pharmaceuticals has made a broad array of new, highly effective compounds available to cure or alter the course of disease.
- New specialty drugs are very effective, but increasingly expensive. Their high cost, multiplied by the growing number of Americans with health insurance, has created **a costly system of pharmacy benefits**.
- The business of pharmacy has undergone a wave of **consolidation** and **integration** into networks to build efficiencies and leverage the value of increasingly larger data sets.
- As pharmacy providers have built the technical capabilities to collect **"big data"** and **sufficient scale** to analyze it meaningfully, they have been able to structure pharmacy benefits to drive adoption of best practices while also narrowing choices among equivalent therapeutic options in an effort to manage down pharmacy costs.
- The meaningful use of **electronic health records** and the ability to transfer protected health information efficiently and securely have accelerated individuals' access to care and encouraged better communication among clinicians.

Pharmacy Benefit Managers (PBMs)

The pharmacy business has evolved in response to broader changes in the U.S. healthcare environment, marked by an accelerating pace of drug discovery and practice innovation that has produced new, highly effective treatments for complex medical conditions. However, the discovery process, together with the growing number of individuals with prescription drug coverage, have converged to drive dramatic growth in pharmacy costs. The pharmacy benefit management (PBM) industry was born out of efforts to balance drug access with cost. Today, PBMs are involved in the majority of prescription drug transactions that take place every day.

PBM Functions

A PBM administers prescription drug benefits for a health insurance plan or employer. It uses a range of tools intended to increase clinical quality and appropriateness, provide decision support, and reduce cost. The PBM engages in the following activities:

- negotiating contracts with drug manufacturers and pharmacies,
- processing claims,
- developing and maintaining drug formularies (explained below),
- performing drug utilization reviews,
- managing clinical programs targeted to specific disease states, and,
- operating pharmacies, including mail-order and specialty pharmacies.

PBM Business Flow

The business flow of a PBM, its customer (a health plan or employer), pharmacies, and beneficiaries is shown in Figure 11.1.

Decisions on formulary design, cost-sharing for beneficiaries, and the size and scope of pharmacy networks are made on a contract-by-contract basis with the health insurance plan or employer.

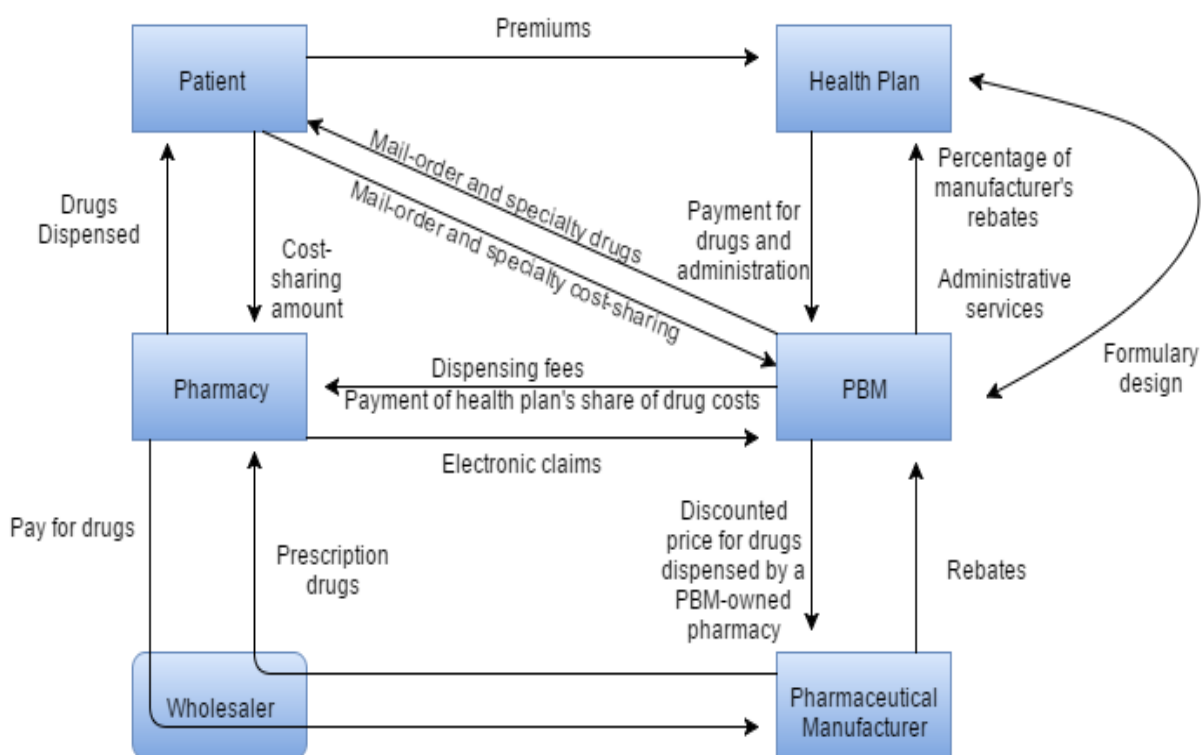
PBMs generate revenue in various ways.

- They negotiate rebates with drug-makers for preferred formulary placement for their drugs (which translates into broader market share for the manufacturer's products.)
- They receive fees for processing and dispensing drugs for plan sponsors and for operating their own mail-order and specialty pharmacies.
- They retain the margin (or "spread") between the amount they charge their customer (the health plan) to manage the benefit and the amount they pay pharmacies for dispensing prescriptions.

Pharmacies contract with PBMs to establish a payment rate for each prescription, plus a dispensing fee. Pharmacies collect patients' copayments and send them to the PBM. Some independent pharmacies still negotiate directly with wholesalers to purchase prescription drugs, but the number of independent pharmacies is declining.

Patients remit copayments (a fixed dollar amount) or coinsurance (a percent of the cost of the drug) to the pharmacy, and make monthly premium payments to their health plan for their prescription drug benefit.

Figure 11-1: PBM Business Flow



Source: Applied Policy, "Concerns Regarding the Pharmacy Benefit Management Industry," November 2015.
<http://www.ncpa.co/pdf/applied-policy-issue-brief.pdf>.

The Development of the PBM Industry

The first PBM was formed in 1968, and the market has developed rapidly since then through a series of ongoing acquisitions, consolidations, and divestitures. In the 1990s drug-makers saw a business opportunity in integrating PBMs into their distribution channels, and they began to acquire them. But the Federal Trade Commission (FTC) eventually forced manufacturers to sell the PBMs in an effort to eliminate the potential for conflicts of interest (where drug-makers might try to coordinate pricing policies, see competitors' pricing information, or favor their own drugs over others). More recently, drug store chains have sought to acquire PBMs, and this has raised antitrust concerns.

The advent of the Medicare prescription drug benefit (Medicare Part D) dramatically expanded the market opportunity for PBMs. By 2015, more than 39 million Medicare beneficiaries were enrolled in Part D prescription drug plans (PDPs), and half of those plans were administered by three large PBMs.¹

Today, PBMs manage pharmacy benefits for over 253 million Americans.² While more than 60 PBMs currently operate in the U.S., the three largest—Express Scripts®, CVSHealth®, and OptumRx®—dominate the market. Together, they cover more than 180 million lives in the United States, or roughly 78 percent of Americans whose pharmacy benefits are managed by a PBM.³

Pharmacy Networks

To satisfy the demands of their customers (health insurance plans and employers) to reduce the costs of pharmacy benefits without shifting so much cost to consumers that they leave the drug plan, PBMs have created pharmacy networks. These networks negotiate deep discounts on drug purchases from pharmaceutical manufacturers and drive pharmacies to agree to lower dispensing fees to remain in the network. (PBMs typically pay out-of-network pharmacies less than those in network, if they pay them anything at all.)

There are three types of pharmacy networks:

- **Open:** The consumer can choose any pharmacy that participates in the PBM's network. This is the most commonly used network design.
- **Preferred:** The consumer can choose any pharmacy, but she pays a lower copay at a pharmacy that has contracted with the PBM at a "preferred" rate. These preferred pharmacies accept lower dispensing fees from the PBM, and in exchange the PBM drives prescription volume to them. Almost one-third of all Medicare Part D prescription drug plans use the preferred pharmacy network design.⁴
- **Limited (restricted or narrow):** The consumer is encouraged or required to use a narrower network that includes only specifically designated pharmacies. Health insurance plans have been slow to adopt limited network models because of the inconvenience and potential disruption they can cause their members.

Research suggests that using preferred networks can lower prescription drug costs for consumers by an estimated 5 percent (compared to open networks); narrow networks can lower drug costs by an estimated 10 percent.⁵ However, many consumers want more accessibility, and intense competition among PBMs for customers ensures that PBMs will offer the types of network that consumers want.

Any Willing Provider Laws

When PBMs exclude pharmacies from their networks and steer patients elsewhere, they create significant incentives for pharmacies to compete aggressively for in-network contracts. However, regulators have been concerned that forming networks that are too narrow or restrictive may reduce access to care (since consumers may only fill their prescriptions at a limited number of in-network pharmacies).⁶ In response, federal and state governments have enacted laws to limit this selective contracting in order to protect appropriate access to care. **Any Willing Provider (AWP)** laws require PBMs to accept into their networks any pharmacy willing to accept their terms and conditions.

AWP and Medicare Part D

Federal regulators who oversee Medicare Part D require Part D plan sponsors to contract with any willing provider—that is, any pharmacy that meets the sponsor’s standard terms and conditions for conducting business. In addition, these terms and conditions must be “reasonable and relevant.”⁷ To comply with this requirement, a sponsor must make standard terms and conditions available for all Part D plans it offers. For those terms to be deemed reasonable and relevant, they must identify for the pharmacy the plan or plans to which the terms apply, and the offer must include language that obligates the Part D sponsor to list the pharmacy as an in-network provider for the plan once the pharmacy has accepted the terms and conditions.

The Part D Drug Benefit Manual states that the sponsor’s standard terms and conditions establish minimum requirements or “a ‘floor’ that all similarly situated pharmacies must abide by.”⁸ However, sponsors may modify some of their standard business terms and conditions to encourage any particularly important pharmacies in a given region to participate in the network. CMS retains authority to review all materials related to a sponsor’s compliance with the AWP requirement and may evaluate whether a sponsor’s standard terms and conditions are reasonable and relevant.

Freedom of Choice Laws

Freedom of Choice (FOC) laws compel PBMs to reimburse pharmacies for services rendered, even if the pharmacy is not in the PBM’s network. Under FOC laws, the PBM pays the pharmacy its negotiated rate. If the pharmacy’s charges are higher than the PBM’s standard rate for the service provided, the consumer is responsible for paying the balance of charges out of pocket.

More than 30 states have passed any willing provider or freedom of choice laws, though the specifics of these laws vary widely.

Managing the Prescription Drug Benefit

PBMs employ a number of standard tools to improve treatment outcomes for their members and to manage the cost of the drug benefits purchased by health plans or employers. A study published by the Federal Trade Commission in 2005 found that these tools can be effective. It reported that consumers covered by a PBM-administered drug plan paid significantly less for both brand name and generic drugs than consumers without prescription drug insurance.⁹ PBMs continue to use these familiar strategies to manage drug benefits today, but as the incremental value of these standard tools is depleted (that is, little or no excess or unnecessary cost remains), PBMs are beginning to explore new ways to offer broad, affordable access to innovative drugs as cost-effectively as possible.

Clinical Decision Support

Decision support tools present to clinicians evidence-based treatment guidelines on appropriate drug therapy for particular patients and their medical conditions. Guidelines are developed by medical professional specialty societies on the basis of the most robust clinical evidence available at the time. The societies endorse these guidelines, which are typically adopted by PBMs, pharmacy and therapeutics (P&T) committees, and other groups to inform coverage and payment policy.

Formularies

Simply put, a **formulary** is a list of drugs that a PBM agrees to pay for when they are prescribed for a member who meets specified criteria. More broadly, a formulary is a continually updated list of drugs and related information that represents the current clinical judgment of a committee of physicians, pharmacists, and other experts in the diagnosis, prophylaxis, or treatment of disease and the promotion of health. A formulary includes, but is not limited to, a list of prescription drugs and other associated medical devices or supplies, policies regarding drug use, and other related information. The formulary may include clinical decision-support tools and guidelines.

A **formulary system** is a standard process used to develop, review, and update policies regarding the use of drugs, therapies, and drug-related products. It identifies those that are most medically appropriate and cost-effective for a given patient population.¹⁰ Formularies are updated on a routine basis to account for the commercial availability of specific drugs and the rebates negotiated with manufacturers.

Formulary Structure

Formularies are structured into levels, or tiers. Generally, the higher the tier, the higher the cost to the PBM and the sponsoring health plan, and the higher the copay or coinsurance charged to the beneficiary.

- Covered generic drugs (preferred generics) are typically placed in Tier 1.
- Preferred brand drugs for which no generic equivalent is available are assigned to Tier 2.
- Non-preferred brands and specialty drugs may be assigned to Tier 3 or 4.
- High cost specialty drugs and biologics are typically assigned to the higher tiers (Tier 3, 4, or even 5), for which the beneficiary pays a higher portion of the cost.

Most health insurance plans define as **specialty drugs** high-cost drugs (such as those costing more than \$600 a month); some plans include any novel drugs that require special handling, monitoring, or administration, or are used to treat rare conditions.¹¹ Biologics are explained below.

Formulary structures are becoming more complex over time. The three-tier structure with a flat dollar copay has been the most common. But it is increasingly being replaced by four-tier designs, typically with a fourth tier for higher-cost specialty drugs (and with either copays or coinsurance).¹² Many plans, including Medicare Part D plans, employ a five-tier structure.

Updating the Formulary

PBMs update their formularies on a routine basis to account for changes in the commercial availability of brand and generic drugs. They also make modifications to reflect rebates negotiated with manufacturers. For instance, if a PBM negotiates an advantageous rebate with a drug-maker, the PBM might move the company's drug to a preferred tier on the formulary, reducing members' copays and making the drug more attractive to them.

Several PBMs are beginning to replace biologic drugs with less-expensive biosimilars (discussed below).¹³ As more biosimilars become available, PBMs are expected to use member incentives to drive their use (for instance, charging a higher copay for a biologic than for the corresponding biosimilar).

P&T Committees

Formularies are managed by pharmacy and therapeutics (P&T) committees, which generally include both physicians and pharmacists, as well as nurses, administrators, quality improvement managers, and other healthcare professionals and staff who participate in the formulary oversight process. The P&T committee evaluates trends in medication use, monitors and reports adverse drug events and medication errors, and may also develop clinical care plans and treatment guidelines for specific disease states (such as diabetes, hyperlipidemia, asthma, etc.). Committee members are typically required to disclose financial or other relationships with manufacturers, suppliers, and others that could result in potential conflicts of interest compromising unbiased, evidence-based formulary management.

P&T committees perform four major types of drug reviews.

- evaluation of new drugs for formulary placement;
- reevaluation of previous formulary decisions, either to add a drug on the basis of new evidence or remove a drug as a result of safety issues, relative effectiveness, or cost;
- review of therapeutic drug classes as a whole, for safety and effectiveness; and
- expedited reviews of newly approved medications for which no therapeutic equivalent is available or of existing drugs on formulary where significant safety concerns have been reported.

In addition to reviewing published clinical literature, P&T committees often consider expert opinion in the review process. They may also conduct pharmaco-economic or other financial evaluations of drug performance, including consideration of non-medication-related costs and any financial consequences to the pharmacy and to the organization as a whole.

Making Formulary Placement Decisions

Many P&T committees use a two-step process to make formulary decisions:

- They first focus on the clinical literature and the safety and effectiveness of a formulary candidate, in the absence of any economic data, as if cost were no object.
- If a drug clears this first hurdle, it is then evaluated from an economic perspective—that is, how the drug’s clinical effectiveness will impact the broader cost of care as compared to other available drugs or no drug therapy at all.

After a drug has successfully cleared these two steps, the PBM can then finalize negotiations with the drug manufacturer on a discounted acquisition price for the drug.

Decisions about whether a product is formulary “preferred” or “non-preferred” are typically made through negotiations between the PBM and manufacturer. If two or more drugs are therapeutically equivalent, the lower or least costly drug is generally preferred. Preferred drugs are more attractive to patients because they come with lower copays or coinsurance than non-preferred drugs.

Medicare Part D Formularies

PBMs contract with Medicare to manage Part D benefits under the direction of a Part D Pharmacy and Therapeutics (P&T) committee. Formularies are required to provide appropriate access to covered drugs, biologics, insulin, certain medical supplies, and vaccines included in broadly accepted treatment guidelines. To be approved by Medicare, a PBM's formulary must be consistent with best practice formularies in widespread use for treatment of medically accepted indications.¹⁴

Current rules require that a formulary cover at least two drugs (unless only one drug is available) for a particular category or class, as these are defined by United States Pharmacopeia (USP) or another body. The two drug minimum requirement must be met by providing two chemically distinct drugs, not two dosage forms of the same drug, or a brand and its generic. For particular categories or classes, more than two drugs may be required if additional drugs present unique and important therapeutic advantages in terms of safety and efficacy, and if their absence from the sponsor's formulary would substantially discourage enrollment by beneficiaries with certain disease states. A separate provision also requires plans to cover "substantially all" drugs in six drug classes: anticonvulsants, antidepressants, antineoplastics, antipsychotics, antiretrovirals, and immune-suppressants for transplant rejection. In addition, Part D sponsors must now also cover treatment of opioid dependence when medically necessary.¹⁵

The Centers for Medicare & Medicaid Services reviews the specific drugs, as well as the tiering and utilization management strategies employed in each formulary. It seeks to identify any strategies that are significantly different from common practices for managing drug benefits so that these can be evaluated before being approved. The PBM must limit the number of formulary changes made over the course of a plan year, provide notice of any such changes to beneficiaries and their physicians, limit changes in therapeutic classifications, and provide a transition process for new beneficiaries. For the 2016 plan year, CMS considered requiring PBMs to identify cost-sharing information for each drug on the formulary, but then decided not to. However, formularies must provide information on tiering, which should enable a consumer to determine the cost-sharing requirements for particular drugs using a plan's summary of benefits and coverage.¹⁶

Utilization Management

Utilization management programs are key components of a prescription drug benefit plan because they have direct influence on the dispensing process at the point of sale, as they provide financial incentives for patients to purchase lower-cost drugs when appropriate.

Prior Authorization

A drug benefit plan member may have to get authorization from her plan or PBM before obtaining certain drugs. This requirement for **prior authorization** may apply to brand-name drug when less expensive generic alternatives are available, drugs that have potentially dangerous side effects, drugs that can be harmful when combined with other drugs, and/or drugs that are frequently misused or abused. Examples of drugs that often require prior authorization include growth hormones, controlled substances, Retin-A, drugs for sleep disorders, proton pump inhibitors (for treatment of gastroesophageal reflux, or "GERD"), and drugs for weight loss/gain. If a specific drug does not receive prior authorization, it may still be dispensed with a valid prescription, but the patient is likely to have a higher copay or even be responsible to pay full retail price for the drug.

Step Therapy

In some cases a drug may be authorized only after another more conservative and/or less expensive “first-line” approach has been tried and failed, in what is called **step therapy**. Typically, the second-line therapy would continue to be covered only if clinical improvement can be documented.

Substitution of Lower Cost Drugs

PBMs may recommend or require that lower-cost drugs be dispensed in place of more expensive drugs when they are available and clinically appropriate as determined by the PBM’s P&T committee.

Therapeutic Substitution

Therapeutic substitution is the authorized exchange of therapeutic alternatives in accordance with previously established guidelines, policies, or protocols within a formulary system. It gives pharmacists the authority to use an alternative drug that is on formulary in place of a non-formulary drug, or a preferred drug in place of a non-preferred drug, without having to contact the prescriber. The kinds of drugs appropriate for therapeutic substitution are those that have different chemical structures but are expected to have similar therapeutic effects and safety profiles when administered to patients in therapeutically equivalent doses. A recent study found that therapeutic substitution might have saved an estimated \$73 billion in spending on branded drugs and an additional \$25 billion in consumer out-of-pocket costs over a two-year period.¹⁷

Generic Substitution

Some PBMs promote **generic substitution**—that is, they encourage or require pharmacies to dispense the generic version of a brand-name drug if a generic is available and both the prescriber and state law permit substitution. A generic drug has the same active ingredient, strength, dosage form, and route of administration as the brand-name drug (called the reference drug) and shares the safety and efficacy data of that drug. While they may be sold under different names than the reference brand, the Food and Drug Administration (FDA) considers generic drugs to be “bioequivalent” or “therapeutically equivalent” to the reference product.

State Laws on Generic Substitution

Most states have laws that allow, encourage, or require pharmacists to substitute generics for brand-name drugs, but the details vary widely from state to state. A few states follow a “negative formulary” approach—substitution is permitted for all drugs *except* those that appear on a list. Other states take a “positive formulary” approach—substitution is limited to the drugs on a list. Many states require the use of the FDA’s Orange Book, a guide for therapeutic equivalency, to determine whether generic substitution is permissible. Fourteen states and Puerto Rico *require* generic substitution if all prescription requirements are met.¹⁸

Several states have enacted interchange laws for narrow therapeutic index (NTI) drugs. Hawaii, for example, requires both practitioner and patient consent for pharmacists to substitute an equivalent generic product for any antiepileptic drug. The Boards of Pharmacy in Kentucky, North Carolina, and Pennsylvania all maintain a list of NTI drugs that are not to be substituted. Oklahoma law has taken an even stricter stance, stating that “at no time will a pharmacist substitute or cause to be substituted *any*

drug, medicine, chemical, or pharmaceutical preparation without the authority of the prescriber or purchaser.”¹⁹

Even when substitution is permitted, the prescriber has the option to specify the brand or supplier of the drug to be dispensed through a “dispense as written” order if this is considered clinically justified at the time of prescribing. This specification will almost certainly cause the patient to pay more out of pocket than she would for the generic equivalent. However, if the generic is permitted, the pharmacist selects the drug to be dispensed from among the generic equivalents for which the PBM has contracted.

Biosimilar Substitution

Biologics are substances used in medical treatment that are created from living organisms; they are much more complex than traditional chemically synthesized drugs. They include monoclonal antibodies, therapeutic proteins, immunomodulators, and growth factors. **Biosimilars** are similar to biologics and can sometimes be substituted for them. However, because biosimilars are manufactured from living organisms by programming cell lines, they are not identical and thus are not technically “generic biologics.” Rather, biosimilars are “highly similar to or interchangeable with” a reference product approved under a biologics license application (BLA).²⁰

A product is biosimilar if it is highly similar to the reference product notwithstanding minor differences in clinically inactive components, and if there are no clinically meaningful differences between the substituted product and the reference product in terms of safety, purity, and potency. Some biosimilars are also considered to be interchangeable if they can be expected to produce the same clinical result as the reference product in any given patient, and if there is no increased safety risk or diminished efficacy.²¹

The Biologics Price Competition and Innovation Act (BPCI)²² appears to allow PBMs to manage the cost of biological products by either excluding the more expensive reference product from the formulary, or by requiring a patient to use the less expensive biosimilar first, and only progress to the reference product if there is a lack of clinical efficacy or a drug-related adverse event. It appears likely that PBMs will also implement rigid prior authorization rules for some biologicals.

The FDA has created a Purple Book for biosimilars, similar to the Orange Book for generic drugs. The Purple Book—the *Lists of Licensed Biological Products with Reference Product Exclusivity and Biosimilarity or Interchangeability Evaluations*—will be the guide for prescribers and pharmacists regarding the substitution of biosimilars.

State Regulation of Biosimilar Substitution

State generic substitution laws do not apply to biosimilars; separate state statutes and regulations govern whether a biosimilar product may be substituted for a more expensive biologic. Some states have enacted laws in this area, and others are considering legislation. State laws vary, but there are several features and requirements that are frequently included:

- The product must be designated “interchangeable” by the FDA.
- The state must maintain a public list of interchangeable products.

- The prescriber must be able to require that the reference biologic be dispensed, by stipulating “dispense as written” or “brand medically necessary.”
- The prescriber and patient must be notified of the substitution. In some cases, states would require patient consent before biosimilar substitution.
- Pharmacist must explain to the patient the cost difference between the biologic and the biosimilar.
- The pharmacist and the physician must retain records of substitutions.
- The pharmacist would not be liable for dispensing an interchangeable biosimilar if she complied with state law.

Transparency

Federal and state regulators have sought greater oversight of the processes that PBMs use to establish drug prices and contract with dispensing pharmacies. PBMs have vigorously resisted these efforts, arguing that greater transparency would have an anti-competitive effect. Competing health plans would have access to others’ pricing information, and competing pharmacies would have access to others’ reimbursement calculations, allowing both to potentially fix prices. Despite these concerns, new regulations are being implemented at the federal and state levels.

Maximum Allowable Cost

When several formulations of the same generic drug are commercially available, a PBM may set a **maximum allowable cost (MAC)**, which is the highest unit price the PBM will pay for the generic.

***Example:** Three formulations of the same “Compound X” to treat male pattern baldness are commercially available, and their unit prices range from \$10 to \$40. The PBM sets the MAC at the lowest of these prices, \$10. This means that the PBM pays pharmacies no more than \$10 per unit of Compound X, regardless of what the pharmacy paid for it.*

PBMs maintain lists of MACs for various drugs. The methodologies used to create these lists are not always disclosed, and PBMs may amend MACs from time to time at their sole discretion.

PBMs assert that the MAC concept promotes competitive pricing. Different manufacturers will charge different amounts for interchangeable generic drugs. If a pharmacy purchases a higher-priced product, it may not make as much profit or may even lose money. But if it purchases generic drugs at a lower price, it will be more likely to make a profit.

MAC prices are driven by competitive factors, including how long the drug has been generic, how many manufacturers are making generic versions of the drug, how widely the generic drug is available for purchase, and whether there have been problems in manufacturing (such as access to basic ingredients or product recalls). Drugs that have been generic longer are more likely to face greater competition and, therefore, lower pharmacy-purchasing prices.

Transparency of Rebates

The terms of the contracts that PBMs negotiate with drug manufacturers are confidential, and this lack of transparency has raised questions about whether manufacturer rebates are actually being passed through to the health plans and employers purchasing the prescription drug benefits. For instance, some PBM contracts classify a rebate as a type of administrative fee that would not be passed through to the customer. Similarly, some PBM contracts define “brand” and “generic” categories loosely, and as a result, some drugs may be categorized incorrectly, affecting customers’ discounts.

Members of the Health Transformation Alliance seek to address this problem. This is a recently formed group of two dozen of the largest companies in the U.S., which is exploring ways to lower the cost of the prescription drug benefits they purchase. They plan to renegotiate contracts with their PBMs to eliminate the risk of any undisclosed drug company rebates being held back, by paying PBMs for the actual cost of the drugs plus a negotiated processing fee.

Federal Regulations

To promote transparency, the Affordable Care Act requires PBMs to provide health plans with the following information for each contract year:²³

- Retail vs. mail-order sales, as a percent of total volume;
- Percent of generic drugs dispensed when a generic is available, by pharmacy type (independent, chain, supermarket, and mass merchandiser);
- Aggregate amount and type of rebates, discounts, and negotiated price concessions that are attributable to patient utilization;
- Aggregate amount of rebates, discounts, and price concessions passed through to the plan sponsor;
- Total number of prescriptions dispensed; and
- Difference between the amount paid by the health plan to the PBM and the amount paid by the PBM to retail pharmacies and mail order pharmacies.

State Regulations

Some states have enacted laws to promote fair standards and transparency in pharmacy reimbursements, with a focus on MAC. A 2015 California law requires PBM-pharmacy contracts to identify data sources used to determine drug prices and to provide for an appeal process. PBMs are required to make available their most up-to-date MAC lists.²⁴ Florida requires PBMs to update MAC pricing information at least every seven days and to maintain a process to modify drug prices in a timely manner to remain consistent with the changes in market data used to calculate MAC.²⁵

Summary

Pharmacy benefit managers (PBMs) administer prescription drug benefits for health insurance plans and employers. They use a variety of tools and strategies to cut costs and enhance quality. Today PBMs handle the large majority of pharmacy benefits in the U.S.

PBMs maintain networks of pharmacies. This enables them to negotiate discounts from drug-makers and low dispensing fees from pharmacies, in exchange for a large volume of business. However, any willing provider (AWP) laws require PBMs to accept into their networks any pharmacy willing to accept their terms and conditions, and freedom of choice (FOC) laws require a PBM to reimburse a pharmacy for services rendered, even if the pharmacy is not in the PBM's network.

PBMs have formularies designed to encourage the use of lower-cost drugs. PBMs may promote the substitution of generics for brand-name drugs and biosimilars for biologic drugs, but both types of substitutions are subject to state laws, which vary considerably from state to state.

Some have sought greater transparency in PBM pricing. The ACA requires PBMs to disclose to health plans certain information about rebates, discounts, and price concessions. Some state laws impose requirements on PBMs regarding the prices they pay pharmacies for generic drugs (based on the PBM's maximum allowable cost, or MAC).

Sources

¹ J.F. Hoadley, J. Cubanski & P. Neuman, P. for The Kaiser Family Foundation, "Medicare's Part D Drug Benefit at 10 Years: The 2015 Marketplace and Key Trends, 2006-2015." <http://kff.org/medicare/report/medicare-part-d-at-ten-years-the-2015-marketplace-and-key-trends-2006-2015/>.

² Testimony of Mark Merritt, President and CEO of the Pharmaceutical Care Management Association (PCMA) before the U.S. House of Representatives Committee on Energy & Commerce, Subcommittee on Health, October 21, 2015. <http://www.pcmanet.org/images/stories/uploads/2015/pcma%20mark%20merritt%20mtm%20testimony%20e%20and%20c%20oct%202015.pdf>.

³ Health Strategies Group, "Research Agenda 2015: Pharmacy Benefit Managers." http://www.healthstrategies.com/sites/default/files//PBM_Research_Agenda_PBM_RA101513.pdf.

⁴ A.J. Fein, "The Narrow Network Revolution," *Drug Channels*, August 2, 2012. <http://www.drugchannels.net/2012/08/the-narrow-network-revolution.html>.

⁵ A. Fein for the Pharmaceutical Care Management Association (PCMA), "How Pharmacy Networks Could Save Medicare, Medicaid and Commercial Payers \$115 Billion," January 2013. http://spcma.org/images/PDFs/Visante_Pharmacy_Networks_Study_2013.pdf

⁶ J. Shepherd, "Selective Contracting in Prescription Drugs: The Benefits of Pharmacy Networks," *Minnesota Journal of Law, Science and Technology*, 2013.

⁷ 42 CFR §423.120(a)(8)(i).

⁸ Centers for Medicare & Medicaid Services, *Prescription Drug Benefit Manual*, §50.8.1 (2011).

⁹ Federal Trade Commission, "Pharmacy Benefit Managers: Ownership of Mail-Order Pharmacies," August 2005. https://www.ftc.gov/sites/default/files/documents/reports/pharmacy-benefit-managers-ownership-mail-order-pharmacies-federal-trade-commission-report/050906pharmbenefitrpt_0.pdf

¹⁰ American Society of Health-System Pharmacists, “AHIP Guidelines on the Pharmacy and Therapeutics Committee and the Formulary System,” *American Journal of Health-System Pharmacy*, Vol. 65, pp. 1272-83, 2008.

¹¹ The Pew Charitable Trusts, “Specialty Drugs and Health Care Costs,” November 16, 2015.
<http://www.pewtrusts.org/en/research-and-analysis/fact-sheets/2015/11/specialty-drugs-and-health-care-costs>

¹² Pharmacy Benefit Management Institute, “2015-2016 Prescription Drug Benefit Cost and Plan Design Report,” December 11, 2015. <https://www.pbmi.com/shop/prescription-drug-benefit-cost-and-plan-design-reports/2015-2016-prescription-drug-benefit-cost-plan-design-report/>

¹³ E. Silverman, “Pharmacy Benefits Managers Will Reduce Access to More Drugs Next Year,” *Pharmalot*, August 2, 2016. See: <https://www.statnews.com/pharmalot/2016/08/02/pharmacy-benefits-managers-restrict-drugs/>

¹⁴ Centers for Medicare & Medicaid Services, *Medicare Prescription Drug Benefit Manual*.
<https://www.cms.gov/Medicare/Prescription-Drug-Coverage/PrescriptionDrugCovContra/Downloads/Part-D-Benefits-Manual-Chapter-6.pdf>.

¹⁵ Ibid.

¹⁶ Ibid.

¹⁷ M.E. Johansen & C. Richardson, “Estimation of Potential Savings Through Therapeutic Substitution,” *JAMA Internal Medicine*, published online May 9, 2016. doi:10.1001/jamainternmed.2016.1704.

¹⁸ National Association of Boards of Pharmacy, *2014 Survey of Pharmacy Law*, pp. 67-70.

¹⁹ T. O'Shea, “Six Surprising Pharmacy Laws,” *Pharmacy Times*, July 2, 2015.
<http://www.pharmacytimes.com/contributor/timothy-o-shea/2015/07/6-surprising-pharmacy-laws>

²⁰ 42 USC §351(k).

²¹ 42 USC §262(k)(4).

²² 42 USC §1301 *et seq.*

²³ Affordable Care Act, Sec. 1150A Pharmacy Benefit Managers Transparency Requirements.

²⁴ AB 627 stats. 2015, ch. 74.

²⁵ Fla. Stat. 465.1862.

Lesson 12 Pharmacies and Pharmaceuticals

Lesson Objectives

After completing this lesson, you should be able to:

- Summarize regulations governing pharmacies (including internet and compounding pharmacies), and recent regulatory developments.
- Describe how electronic prescribing works and the rules that apply to it.
- Discuss some of the trends likely to shape the pharmacy business in the future.

In the last lesson we looked at how pharmacy benefit managers (PBMs) operate and how they seek in various ways to hold down costs while improving quality. In this second lesson on pharmacy benefits, we will examine the pharmacy business and the regulations that govern it, learn how physicians and others can prescribe electronically and the rules they must follow, and explore some trends that are changing the pharmacy industry.

Pharmacies

Changes in the Pharmacy Business

As healthcare changes in the United States, pharmacies are changing as well. No longer mere “pill counters,” pharmacists’ scope of practice is expanding in many states to give them greater authority to fill gaps in the delivery of non-critical primary care, advise patients on the management of medications and potential drug interactions, adjust dosing, and administer vaccinations without a physician’s order. In some states, pharmacists are authorized to help manage public health issues by dispensing naloxone, hormonal contraceptives, emergency contraception, and smoking cessation products without a patient-specific prescription. In the current environment where larger numbers of Americans have health insurance, but where there are increasingly too few physicians to manage patient care, pharmacists have emerged as “physician extenders.”¹ With broader implementation of robotics and technology to assist with routine drug dispensing functions, pharmacists are being freed up to provide increased patient-centered services.

Pharmacy business models are also changing. Independent and community-based retail pharmacies are increasingly being replaced by chain drugstores and lower-cost pharmacy service providers located in supermarkets or “big box” retailers. **Specialty pharmacies** have been developed to distribute and dispense high-cost drugs and biologics, many of which have specific storage and handling requirements and are administered by injection or infusion. **Mail-order and internet pharmacies** have also emerged, often tied to pharmacy benefit managers (PBMs) looking for less expensive alternatives for the routine refilling and delivery of prescription medications for chronic medical conditions. Other pharmacy models are associated with specific patient populations, such as pharmacies affiliated with nursing homes, long-term care facilities, hospitals, and clinics.

Finally, technological advances have given pharmacies access to large, searchable databases that warehouse information on prescribing patterns, trends in utilization of brand vs. generic drugs, and consumer data (such as rates of adherence with drug therapy for chronic medical conditions).

Pharmacies have immediate electronic access to information on consumers' prescription drug benefits and plan-specific formularies, enabling them to request prior authorization, process a claim, or report a denial of coverage and recommend alternate drug therapy quickly, efficiently, and cheaply.

State Regulation

The U.S. Food and Drug Administration (FDA) regulates virtually all commercial pharmaceutical manufacturing, but the states regulate drug dispensing through pharmacies. Every state has laws and regulations to guide pharmacy standards and requirements, address licensure for each type of pharmacy, and credential pharmacists and other employees. States may also establish requirements for securing drug storage and maintaining pharmacy records, and may approve formats for prescription pads and drug labels. States may set safety protocols related to the origins, authenticity, chain of custody, and expiration of prescription drug products. While the federal government maintains standards for the privacy and security of patient data, the states regulate protocols for data collection and transfer, the use of electronic prescribing, and the standards for connectivity between prescribers and pharmacies. Each state has a board of pharmacy that oversees standards and regulation.

Securing the Drug Supply Chain

To protect patient safety and reduce the sales of counterfeit or adulterated drug products, pharmacists are required to receive certain “track and trace” information from wholesalers and drug distributors and retain it for six years. Pharmacies must verify that a distributor is an authorized trading partner and capture drug transaction data. Pharmacies are required to have reporting protocols in place regarding potentially suspect products in pharmacy inventory.

Changing Roles and Changing Regulations

The National Association of Boards of Pharmacy (NABP) has observed that pharmacists trained and competent in drug therapy are vastly underutilized in most healthcare delivery systems, and the organization has encouraged state boards of pharmacy to expand the pharmacist's **scope of practice** (what a pharmacist is permitted to do under state law), within proper safeguards.² Some states have implemented new laws and updated existing laws and rules to allow pharmacists to initiate, modify, and terminate drug therapy in limited circumstances. Other states have expanded their collaborative practice guidelines and statewide protocols to allow pharmacists to be more actively involved in managing drug therapy.

For instance, in California pharmacists now have authority to furnish self-administered hormonal contraceptives (within certain restrictions), emergency contraceptives, nicotine replacement products, and prescription medications recommended for people traveling outside of the United States. Pharmacists may also order and interpret tests to monitor and manage the efficacy and toxicity of drug therapies and independently initiate and administer routine vaccinations.³ In a number of states, pharmacists may treat severe allergic reactions by injecting epinephrine or diphenhydramine. They may also dispense epinephrine auto-injectors to emergency medical personnel, lay rescuers, and schools.

Naloxone hydrochloride can be used for emergency treatment of drug overdoses. Over the last several years, the public health community has advocated expanding access to this drug by changing state law to permit it to be distributed outside of the traditional prescriber-patient relationship. To date, over half of the states and the District of Columbia have modified laws to increase access. Some states permit prescribers to write naloxone prescriptions for caregivers, and some allow “standing” or non-patient-

specific naloxone prescriptions. Nearly all of these laws provide civil immunity for the prescriber, and some extend limited immunity from criminal prosecution for a person who reports an overdose in good faith.⁴ Either through collaborative practice agreements or standing prescription orders, pharmacists may be authorized to dispense naloxone to at-risk patients and their caregivers without a patient-specific prescription; in such cases, they are required to follow a protocol specified by the state to identify eligible patients and caregivers and to ensure a clear understanding of how to administer the drug correctly in an emergency situation.

Internet Pharmacies

The internet has facilitated the easy, convenient, private, and inexpensive purchase of many prescription and non-prescription drugs and medical devices. But it has also opened the way to increased fraud, the unlicensed practice of pharmacy, and counterfeiting of medical products. Some internet pharmacies will prescribe and ship a medication after only a very limited medical consultation or even just a few questions answered by the customer. Others dispense prescription drugs without a prescription. Still others market products such as vitamins, herbs, or dietary supplements as prescription drugs. These practices of course have significant health and safety risks.⁵

Some internet pharmacies operate legally and ethically, but many do not. The NABP's VIPPS program (discussed below) has been collecting data on online sales to United States patients since 2008 and has found that the vast majority of sites reviewed do not operate in compliance with U.S. pharmacy laws.⁶ And when internet pharmacies are found to be engaging in illegal or improper activities, it can be very hard to shut them down or even find them.

State Regulation

Because pharmacies are regulated by the states, state attorneys general and licensing boards have been the primary prosecutors of internet pharmacies, seeking to enforce state pharmacy and consumer protection laws and codes of professional conduct. But their authority and effectiveness is often undermined by lack of territorial jurisdiction beyond a state's borders and the challenges of coordinating across multiple jurisdictions.

Federal Action

The FDA has established partnerships with several state bodies and the NABP to coordinate oversight and enforcement efforts. The NABP has created the Verified Internet Pharmacy Practice Sites (VIPPS) Program to certify internet pharmacies that operate legally and ethically. There are a few dozen VIPPS-certified online pharmacies, about a quarter of which are members-only pharmacy benefit programs. VIPPS is closed to all non-U.S. pharmacies that sell medications to persons in the U.S.⁷

Taking a somewhat different approach to the problem, the U.S. Department of Justice has brought charges against shippers (including FedEx and UPS) for distributing substances from unauthorized pharmacies. In another case, Google paid \$500 million to settle charges that it sold advertising to Canadian internet pharmacies that were marketing to Americans in violation of U.S. laws.⁸

The Federal Food, Drug, and Cosmetic Act⁹ prohibits the interstate shipment or importation from abroad of drugs that lack FDA approval (including foreign-made versions of U.S.-approved drugs). The FDA warns non-U.S. internet pharmacies that they may be violating this law and may advise foreign

government agencies and encourage them to take action.¹⁰ Shipping prescription drugs from other countries to consumers in the United States also violates some states' laws, because many states require licensing and registration of pharmacies that ship drug products directly to consumers.

Compounding Pharmacies

Many pharmacies are licensed to **compound** drugs—that is, to combine, mix, or alter ingredients of a drug. Compounding is done to tailor drugs to the needs of specific patients, based on prescriptions written by doctors or other prescribers. For instance, a person might have an allergy to a dye used in a medication, or elderly patients and children may need a liquid form of a drug because they cannot swallow a pill.

Regulation

Compounding can increase the risk that a dispensed drug will be super-potent or sub-potent, contaminated, or otherwise adulterated. Compounded drugs are not reviewed or approved by the FDA, so the agency cannot verify their safety or effectiveness. Concerns about the variable quality and integrity of compounded drugs led Congress to pass the Drug Quality and Security Act in 2013.¹¹ This law offers compounders of sterile drugs the option to register as an outsourcing facility. Registered facilities must comply with certain requirements, and in return they are exempt from certain federal rules on new drug approval, track and trace, and misbranding.

Under this Act, drugs must be compounded in compliance with **current good manufacturing practices (cGMP)** by or under the direct supervision of a licensed pharmacist in a registered facility. The facility must report certain information, including a list of all of the products it compounded during the previous six months, the source of the ingredients used to compound, and any compounded drug-related-adverse events. The FDA has issued draft guidance on registering and reporting for those entities that intend to register.¹²

Electronic Prescribing

Electronic prescribing (e-prescribing) is the computer-based electronic generation, transmission, and filling of a prescription, replacing paper and faxed prescriptions. This allows a prescriber to transmit a new prescription or renewal authorization electronically to a pharmacy. E-prescribing also make available eligibility, formulary, and medication history information to the medical professional, providing clinical decision support. Approximately 1.2 billion prescriptions were processed electronically in 2014; 98 percent of chain pharmacies and 88 percent of independent pharmacies are currently able to support e-prescribing.¹³

The E-Prescribing Process

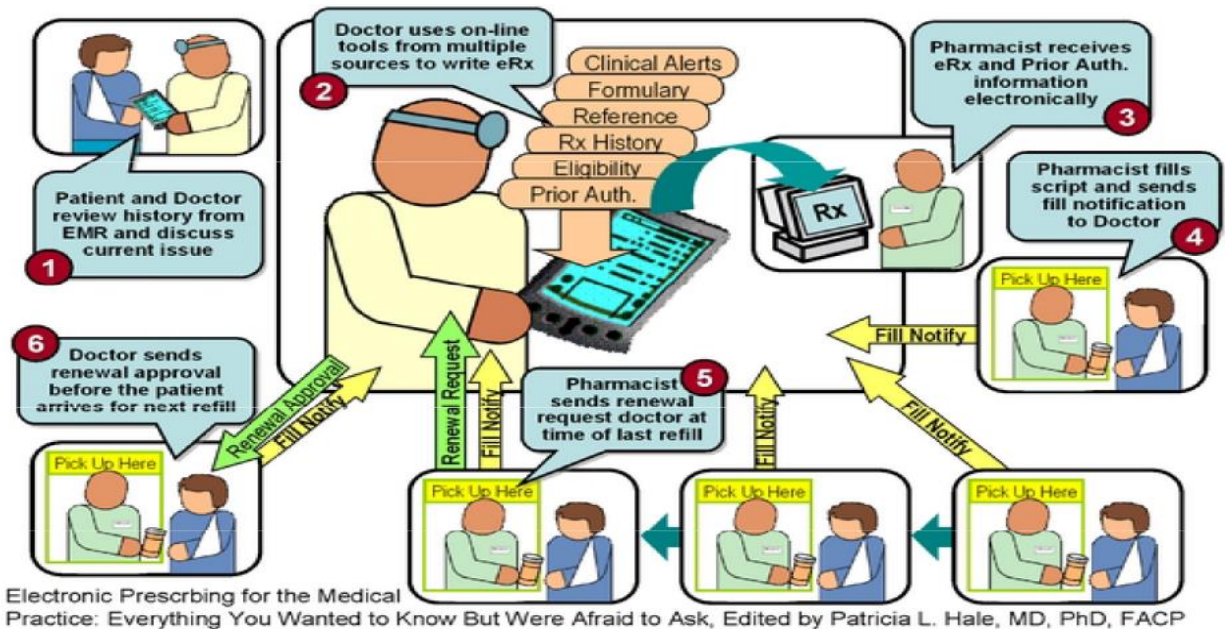
A typical e-prescribing transaction occurs as follows:¹⁴

- A prescriber signs into an e-prescribing system; a verification process authenticates his identity. The system then gives him access to functionality specific to his authorization level.

- The prescriber pulls up his patient's records in the practice management system and reviews her current medication list. He adds a new medication and selects the pharmacy where the prescription will be filled.
- This information is sent to a **transaction hub**. Information on patient eligibility, formulary, medication history, and fill status is sent from the hub back to the prescriber.
- The transaction hub is the common link between the prescriber, pharmacy, and PBM. The hub maintains a master patient index and a list of pharmacies, and it routes inquiries to the PBM and prescriptions to the pharmacy.
- The prescriber completes and authorizes the prescription, which is then transmitted to the pharmacy.
- The pharmacy receives prescription data from the transaction hub and provides confirmation back to the hub.
- The pharmacy confirms that prescription has been filled and may also notify the prescriber when the prescription has been picked up.

For a somewhat perspective on the e-prescribing process, see Figure 12-1.

Figure 12-1: Electronic Prescribing for the Medical Practice



Source: Healthcare Information and Management Systems Society (HIMSS), "Electronic Prescribing for the Medical Practice: Everything You Wanted to Know But Were Afraid to Ask," July 11, 2008.

<http://www.himss.org/electronic-prescribing-medical-practice-everything-you-wanted-know-were-afraid-ask-0>

Privacy—Federal Regulation

Transmitting prescriptions electronically raises concerns about the privacy and security of patient information. There are safeguards for protected health information (PHI), including electronically transmitted PHI (ePHI), under HIPAA and HITECH. In 2013 the HHS Office for Civil Rights issued regulations that expanded the privacy rights of patients and made changes in medical record privacy measures.¹⁵ Those changes apply to entities that receive and process ePHI (such as health plans, transaction hubs, pharmacies, and their subcontractors).

Under these regulations, pharmacies that process e-prescriptions must meet a minimum set of standards to protect the privacy of ePHI. While pharmacies may rely on their software vendors or service subcontractors to ensure that they are in compliance with these regulations, compliance is ultimately their own responsibility. The pharmacy must also implement internal controls to identify areas of potential security risk, such as hard drives, laptops, or other portable devices that may contain ePHI. Physical controls must be implemented to ensure that there is limited access to workstations that contain ePHI. Technical safeguards (such as unique user IDs and passwords for all individuals with access to ePHI) are required.

State Regulation

Every state permits e-prescribing, and a few are beginning to require it (either for controlled substances or for all drugs). States may impose regulations on the process for transferring prescriptions electronically, such as the type of electronic platform that must be used and the process for authenticating the identity of the prescriber (the “electronic signature”). Some states require additional safeguards—for instance, in New York both prescribers and pharmacists must have encrypted and encoded computer systems that are certified by the Drug Enforcement Agency (DEA) or some other organization as meeting federal standards.¹⁶

Drug Issues

Abbreviated Drug Approval

The Drug Price Competition and Patent Term Restoration Act (the Hatch Waxman Act) of 1984 created a new, abbreviated pathway for the regulatory approval of generic copies of already approved brand-name drugs.¹⁷ Under this law, manufacturers submit to the FDA an Abbreviated New Drug Application (aNDA) for a product that is identical to a brand drug in dosage form, strength, route of administration, quality, performance characteristics, and intended use. In a similar way, the Biologics Price Competition and Innovation Act (BPCI) opened an abbreviated approval pathway for products that are demonstrated to be “biosimilar” to or “interchangeable” with an FDA-licensed biological product. Manufacturers submit an Abbreviated Biologic License Application (aBLA).¹⁸

Once a biosimilar obtains FDA approval, the manufacturer must market the product at a lower cost than the reference product (the brand-name drug it replaces). Based on experience in Europe (where a similar approach was taken), despite this lower-price competition, brand-name biologics are expected to remain commercially viable, but it is likely that their prices will be driven down.

Pharmatization

Specialty drugs are expensive and account for a large portion of total drug spending. For instance, in 2014 fewer than 1 percent of all prescriptions were for specialty drugs, yet they added up to 32 percent of total expenditures.¹⁹ And the price of many specialty drugs continues to rise. One response is “**pharmatization**”—health plans change the injection or infusion of specialty drugs by a healthcare provider from a medical benefit to a pharmacy benefit. This allows a PBM to exercise better control over drug costs by comparing treatment options.

State Price Transparency Laws

Some states have enacted laws to try to contain drug costs by requiring greater transparency of pricing by drug manufacturers. Several states now require drug-makers to disclose the costs of research and development, production, and marketing. A few states limit the out-of-pocket payments of patients in private health plans. In 2016 Vermont enacted a law authorizing the state attorney general to require manufacturers of up to 15 drugs for which the wholesale acquisition cost has increased by more than 50 percent over the previous five years to provide justification for the cost increase.²⁰

Looking Ahead

We began the last lesson with a survey of the main trends that have been shaping the pharmacy business in recent years. Let's look now at what we can expect for the future.

- **Big data.** Larger data sets comprised of increasingly disparate types of data elements (including patient-reported data and data from health apps and social media) will be correlated with more traditional health data elements (such as blood pressure and lab test results). Complex data analytics will lead to expanded automation of routine care and a more personalized, predictive, and proactive engagement in specific patients' care through the adoption of "precision medicine."²¹
- **First-line drug therapy.** Whenever possible, patients will first use generic drugs and biosimilars; only when these products prove ineffective will they switch to brand-name drugs and biologics. High-cost branded specialty drugs will largely be used only for the treatment of well-defined patient populations.
- **Integration with healthcare.** Pharmacy services will continue to be integrated into cross-functional medical practices where patients will be cared for by multi-disciplinary teams of providers working together to deliver care. Pharmacy and PBM services will be integrated into local and regional healthcare delivery networks managed as medical homes, accountable care organizations (ACOs), and other newer approaches to delivering healthcare.
- **Role of the pharmacist.** Pharmacists will take on more responsibility for medical decision-making, medication therapy management, vaccination, preventive care, etc. Collaborative practice agreements and standing orders already permitted in many states will allow pharmacists to manage chronic disease, dispense and adjust medications, and perform other diagnostic and prescriptive services without first seeking authorization from a physician.
- **Risk-based contracting.** Pharmacies and PBMs will require manufacturers to offer arrangements in which prices will be tied to drug performance, the effectiveness of treatment, and health outcomes. This will occur first with higher-cost specialty drugs and biologics, but ultimately for other branded drugs and biologics as well.

Summary

The pharmacy business is changing, as pharmacists' scope of practice expands in many states, independent pharmacies are replaced by chain drugstores and large retailers, and technology gives pharmacists quick and easy access to more information.

The dispensing of drugs by pharmacies is largely regulated by the states, which have standards and requirements in several areas. The manufacture of drugs is regulated by the federal Food and Drug Administration (FDA). Hence, both state and federal laws and agencies play a role. Internet pharmacies, many of which do not operate in compliance with the law, are regulated by the states, but the effectiveness of state authorities has been undermined by jurisdictional problems. The federal government has responded to this problem by taking a number of actions. Compounding pharmacies must be licensed by the state, and they have the option of being registered with the FDA. Electronic

prescribing offers many advantages, but there are concerns about maintaining the privacy and security of information, and both state and federal regulations seek to address this issue.

Some trends that are likely to shape the pharmacy industry in the years to come include: “big data,” which will both increase automation of routine care and make possible more personalized care; the increased use of generics and biosimilars, especially as first-line therapy; the integration of pharmacy services with other healthcare services and organizations; the expansion of the scope of practice of pharmacists; and the linking of the drug prices paid by health plans and PBMs to performance and outcomes.

Sources

¹ S.G. Boodman, “Pharmacists Dispense Pills, Counsel Patients, Screen for Illness, Give Vaccines,” *The Washington Post*, January 12, 2010.

² National Association of Boards of Pharmacy (NABP), “Report of the Task Force on Pharmacist Prescriptive Authority, 2015. http://www.nabp.net/system/rich/rich_files/rich_files/000/001/232/original/report-taskforce-pharmacistprescriptiveauthority-final.pdf

³ CA stats, 2013, ch. 469, §4052(a)(10).

⁴ C. Davis, D. Webb & S. Burris, “Changing Law from Barrier to Facilitator of Opioid Overdose Prevention,” *The Journal of Law, Medicine, and Ethics*, vol. 41 (Suppl. 1), pp.33–36, 2013.

⁵ F. Upton, “Drugstores on the Net: The Benefits and Risks of Online Pharmacies,” before the U.S. House of Representatives Committee on Oversight and Investigations, Subcommittee on Commerce, 1999.

⁶ National Association of Boards of Pharmacy (NABP), “Internet Drug Outlet Identification Program Progress Report for State and Federal Regulators,” July 2015. http://s3.amazonaws.com/awarerx/rich/rich_files/rich_files/186/original/nabpidoireportjuly2015.pdf.

⁷ Ibid.

⁸ M. Lowery, “FedEx Indicted for Assisting Illegal Pharmacies,” *Drug Topics*, July 18, 2014. <http://drugtopics.modernmedicine.com/drugtopics/content/tags/fedex/fedex-indicted-assisting-illegal-pharmacies?page=full>

⁹ 21 USC §331.

¹⁰ J.M. Taylor, “Importation of Prescription Drugs,” Statement of John M. Taylor, III, Associate Commissioner of Regulatory Affairs, before the U.S. Senate Committee on Governmental Affairs, Permanent Subcommittee on Investigations, July 22, 2004. <http://www.fda.gov/NewsEvents/Testimony/ucm113635.htm>.

¹¹ 21 USC §353b *et seq.*

¹² U.S. Food and Drug Administration, “Registration of Human Drug Compounding Outsourcing Facilities under Section 503B of the FD&C Act: Guidance for Industry,” November 2014. <http://www.fda.gov/downloads/drugs/guidancecomplianceregulatoryinformation/guidances/ucm377051.pdf>

¹³ First Databank, “Electronic Prescribing,” 2016. <http://www.fdbhealth.com/~media/downloads/form%20required/us/issue%20brief%20-%20electronic%20prescribing.ashx>

¹⁴ Health Resources and Services Administration (HRSA), “Health Information Technology and Quality Improvement: How Does E-Prescribing Work?” <http://www.hrsa.gov/healthit/toolbox/HealthITAdoptiontoolbox/ElectronicPrescribing/epreswork.html>

¹⁵ 78 FR 5566.

¹⁶ New York State Education Department, “Electronic Transmittal of Prescriptions in New York State. Frequently Asked Questions.” <http://www.op.nysed.gov/prof/pharm/pharmelectrans.htm>.

¹⁷ 21 USC §355(j) *et seq.*

¹⁸ 42 USC §1301 *et seq.*

¹⁹ BlueCross Blue Shield Association, “The Growth in Specialty Drug Spending from 2013-2014: The Health of America Report,” April 2016. <http://www.bcbs.com/healthofamerica/Specialty-Rx-HoA-Report.pdf>

²⁰ 10 LSLR 12, Act 165. (2016).

²¹ National Institutes of Health, “Precision Medicine Initiative Cohort Program.” <https://www.nih.gov/precision-medicine-initiative-cohort-program>

13 Market Conduct Exams

Lesson Objectives

After completing this lesson, you should be able to:

- Describe a market conduct examination (MCE), including the business functions reviewed, the possible triggers, and the range of actions regulators may take.
- Summarize the steps a health plan should take to prepare for a market conduct exam.
- Describe how an onsite MCE typically works.
- Identify some of the most common findings of an MCE.

The State-Based Regulatory System for Insurers

Each state government includes an agency usually called the **department of insurance (DOI)**, or this agency may be part of a larger department that also regulates banking, securities, and other financial services. The head of the DOI is most often called the **commissioner**, but sometimes the director or superintendent. He or she may be appointed by the governor or by a state agency, or may be elected. If the position is appointed, the commissioner may change each time a new governor is elected. DOI management may or may not change with each new commissioner, but many staff members stay on, providing continuity. The functions and duties of the department remain the same, although priorities and interpretations may change.

As we have seen, some federal agencies, notably CMS, also regulate insurance and health plans. States have the primary enforcement authority over health insurers, but CMS also has responsibility to enforce regulations in the state, if needed. State DOIs and CMS work collaboratively to help ensure compliance with updated health insurance market regulations, including the ACA. If a state notifies CMS that it has found a potential violation and lacks either the statutory authority or the necessary resources to exercise enforcement, CMS will directly enforce the regulation. CMS's compliance enforcement tools include collecting and reviewing policy forms, performing other targeted market conduct examinations, and responding to customer complaints.¹

However, in this lesson we will focus on state regulation. Although state insurance regulation is similar across the U.S., enough variation exists to make an insurer's compliance work challenging.

Understanding the differences among state laws, codes, and regulations, and keeping up with the legislative and regulatory changes that occur each year can be an even bigger compliance challenge.

The **National Association of Insurance Commissioners (NAIC)** is participated in by all 50 states, five U.S. territories, and the District of Columbia. The commissioners and staff from each of the 56 jurisdictions work together to establish standards and coordinate their regulatory oversight. The NAIC as an organization employs over 400 experienced staff to help support the regulators and their work. The NAIC's stated goals are to:

- Protect the public interest;

- Promote competitive markets;
- Facilitate the fair and equitable treatment of insurance consumers;
- Promote the reliability, solvency, and financial solidity of insurance institutions;
- Support and improve state regulation of insurance.²

The NAIC functions through regulator committees and working groups, supported by NAIC staff, which address current and emerging industry issues. Model Laws are drafted in this way. Models are voted on by the NAIC membership at one of its three annual meetings, and then taken to legislatures in the 56 jurisdictions for enactment. The NAIC does not have any regulatory authority per se, but its positions and actions carry a great deal of weight because of the combined authority of its members and its work to improve standardized laws and regulations across the country.

Market Conduct and Examinations

Market Conduct can be defined as the operations and practices of an insurance company in the consumer marketplace. Insurance companies are highly regulated by state departments of insurance in how they market, underwrite, distribute or sell, issue, service, and pay claims on their insurance products. A primary commitment of insurance regulators is to protect the public, or consumers, in their states. Regulations are designed to accomplish that goal.

A **market conduct examination (MCE)** is an investigation or audit by state insurance regulators to evaluate the practices and operations of an insurer. The examination aims to establish that the company is conducting its insurance business fairly and responsibly and in compliance with the state's laws and regulations. MCEs were, at first, part of financial examinations, but because of increasing concern about market conduct issues, many states made them a separate function.

An MCE can be comprehensive, examining the entire company and including many areas of activity. Alternatively, the MCE can focus on or target one area, or even a specific business function (such as claims paid), or even a particular state law (such as newly enacted claim payment timing requirements). The 12 standard business functions reviewed in a comprehensive MCE are:

- Operations/management
- Complaint handling
- Marketing and sales
- Producer licensing
- Policyholder service
- Underwriting and rating
- Claims
- Grievance procedures

- Network adequacy
- Provider credentialing
- Quality assessment and improvement
- Utilization review
- External review

Additional business functions may be reviewed in examinations for the Affordable Care Act, Medicare supplement insurance, or long-term care insurance.

MCEs are directed by the DOI, and may be conducted by state employees or by contracted examiners managed by state regulators. The health plan is responsible for providing space and resources for the examiners and taking time to work with them, which may mean several months of business interruption and resource challenges while the examiners are onsite. Depending on the state, the health plan may be required to pay for the examiners' time and all related expenses.

Regulators conducting financial examinations of businesses are now focusing more on risk, and some aspects of this approach are being applied to market conduct exams. With a risk focus, an examiner looks not only at processes and procedures, consistent compliance, and a “snapshot” of how the company is doing, as in the past; she now seeks to determine whether both the day-to-day business decisions and the long-term strategic planning of the company are aligned with its risk tolerance and appetite. Members of the company's board of directors may be interviewed about their risk awareness, whether reporting to them is timely and transparent, and the way they make decisions about taking risks and budgeting for risk mitigation.

Examiners are also interested the sustainability of a company in the long term. Are the leaders and directors making informed choices about risk and controls that will help ensure that the company can continue to keep its promises to all stakeholders—members, providers, vendors and suppliers, investors, employees, leaders and board members, in compliance with insurance requirements?

A variety of factors may trigger an MCE—see below.

Possible Triggers for a Market Conduct Examination or Other State Regulatory Activity³

1. Complaints: Significant changes in the ratio of consumer complaints to sales activity in a relatively short period of time.
2. Premium: Significant increases/decreases in premium volume in the state or growth/decline in one or more lines of business without explanation from the company.
3. Lines of business: Significant changes in the company's book of business within the state.
4. Expansion: Rapid admission and growth in new states.

5. Compliance controls: High degree of reliance on third parties to perform company functions (such as managing general agents or third-party administrators).
6. Cyber: Significant problems with electronic data processing systems, potentially impacting the security or integrity of data for claims, underwriting, and financial systems.
7. Outside information: From other units of the same state insurance department, other jurisdictions, court records discovered, or media reports.

Like health plans, state DOIs feel economic pressure to accomplish their work in more cost-effective and efficient ways. And as regulation becomes more complex, the job of regulators becomes more demanding. Regulators have responded to these pressures in several ways, including:

- The use of technology
- Multi-state exams (discussed below)
- Use of the continuum of regulatory activities

The **continuum of regulatory activities** can be defined as a continuous, unified sequence of controls available for application by a regulator as needed and as deemed appropriate. These controls run from a simple phone call to a comprehensive onsite exam. By using the appropriate control for a situation, regulators can use time and resources effectively and efficiently. For a list of activities in the continuum, see below.

The Continuum of Regulatory Activities⁴

Please note that DOI action may be a combination of these options or may take a direction not listed here.

- DOI office-based information-gathering prior to any notification or contact with the company; may be internally gathered, requested from NAIC databases, or in conversation with other DOI staff.
- Contact with the regulated entity, possibly a phone call or email asking one or more questions about a potential issue.
- Scheduled telephone or in-person interview with the company's appointed liaison.
- Information requests to the company for targeted information; this may be a call for data or simply a request for responses on a topic or business function.
- Correspondence in a formal letter from the department, possibly including notification of an investigation or of an identified compliance violation trend of concern to the DOI.

- Request for documentation of specified policies and procedures in an area of concern to the DOI.
- Interrogatories, a formal set of questions requiring formal responses from the company, which may involve research and data-gathering on the company's part.
- Desk audits, where the regulator requires certain sets of files, reports, or other information to be sent to the DOI for review at the DOI offices.
- Company self-audit requests, to provide feedback on what the company may discover in a certain area of concern, which could result from a self-report previously provided by the company in this area.
- Voluntary compliance programs may be requested where the regulator identifies potential weaknesses in the compliance or governance processes of the company.
- Investigation of a company's agent(s) or business or market conduct, typically resulting from an incident reported or complaint filed with the DOI.
- A targeted examination can be called, typically in the insurer's offices, of a specific business function, business unit, or compliance concern.
- A comprehensive examination may extend to any and all 12 areas of standard business functions under the purview of DOI oversight.
- A multi-state (or multi-jurisdictional) cooperative examination also may extend to any of the 12 areas noted above; however, a multi-state exam is not typically comprehensive in nature, because of the extent of resources that would be required.
- Enforcement actions can include a simple fine, a large penalty, a written compliance plan with regular reports to the DOI, or even the most extreme action of withdrawing the authority of the company to do business in the state.
- Proposal of new statutes or regulations may result from an examination or investigation, where the regulator discovers a market conduct trend that had not previously been addressed by the DOI, in order to strengthen the state's compliance requirements.

In response to budgetary pressures and limited resources, states have joined together to conduct multi-state MCEs. This allows states to pool resources and share results, and it relieves health plans of the costs of multiple exams by different states. A working group at the NAIC helps coordinate multi-state exams. Typically, a "lead state" sends its examiners or contractors onsite for the exam. Other participating states may also assist onsite or may only provide documentation or input. All participating states receive the exam report and the company's response. All states are expected to sign off on any disciplinary action or compliance requirement. States share in any penalties levied, according to their share of the company's premiums.

Health plan concerns about MCEs may include:

- Examiners request a very broad range of information and materials (beyond the extent initially stated).
- Examiners remain onsite a very long time, resulting in cost run-ups.
- Contracted examiners lack knowledge about insurance and health plans, so that the plan has to “train” them at its expense.

Preparing for a Market Conduct Examination

Ongoing compliance preparation can begin at any time, and ongoing groundwork will be helpful when a health plan receives notification of an examination or other regulatory inquiry. One of the most important keys to preparation is for compliance staff and department leaders to work together as teams. Although the compliance staff may not know everything that is required to do the work of each department, they will know how to ensure that compliance matters are addressed in that work.

These compliance/department teams should implement various protocols including:

- Check procedures
- Check compliance
- Test controls
- Communicate risk awareness
- Document
- Self-report
- Relate to regulators
- Prepare leadership

Check procedures

Make sure written procedures and manuals are complete, accurate, and up-to-date, with revision dates duly noted. “Sticky-note procedures” are not sufficient to document your company’s compliance with any regulatory requirement.

Work with the compliance specialists to ensure that the procedures comply with:

- the domiciliary state of the health plan (where the legal company office is established) and
- variations of state laws in other admitted states (other states where the plan has members).

Example: Make sure the company’s definition of a complaint is not more restrictive than in the laws of the states it serves. The ratio of complaints to premium is a key consideration to regulators, and if

your company's complaint ratio is above or below that of similar companies in your marketplace, the DOI will view it as a red flag.

Business unit leaders are responsible for communicating the procedures, ensuring that staff members in each business function receive adequate training at the beginning of their employment, and making certain that communication and training are ongoing.

Check Compliance

Establish a system of compliance “quality audits” that mimic some of the MCE review standards. One approach is to recruit a compliance-minded person to perform a quality audit in a department she does not work in or with.

- The business unit under review provides the auditor with a checklist of what staff in the department should be doing, what their files should contain and not contain, and what their risk controls are, all according to their own documented procedures.
- The auditor may review files for accuracy and completeness, conduct interviews to determine that each staff member understands and follows procedures, or observe physical or electronic activities—or all three, depending on the best method to determine if procedures are consistently followed.
- At the end of the quality audit, the reviewer should not have any unanswered questions or incomplete information. If there are questions or data is lacking, these are reported as “findings,” just as in a regulatory MCE.

These compliance quality checks are very different from the financial or internal audits that are routinely performed. The focus is on whether or not the department is in compliance with its own policies and procedures, and whether or not its staff members apply those practices repeatedly and consistently. A rotating schedule of quality audits can be accomplished each quarter, cycling through all business units annually. Each “volunteer” auditor may be requested to commit a full day or more once a year to assist with these audits. so that the time away from daily responsibilities is minimized.

Test Controls

After a risk analysis is done within a department, controls will be documented, strengthened, and added, as necessary, to help mitigate each identified risk. Those controls may include, for instance, a quality second-check before paperwork goes out to a member or provider, use of a scoring system for correct data entry, or a rewards system for meeting claims-paying accuracy and deadline targets each month. Mitigation plans will be designed to include steps to be taken when certain triggers are encountered.

Example: *Action may be triggered when a red flag is noted in the claims department, indicating a potentially fraudulent claim. Steps may include the employee taking the suspicious transaction to a supervisor or team lead for a second review; the supervisor taking certain actions to document and move the review up another level, if fraud seems to be possible; and an investigator conducting a detailed review of the transaction.*

Example: Action may be triggered anywhere in the company when a suspicious email is received. The required step may simply be to forward that email to a technical help desk with an explanation of why it is suspect.

As a standing practice, the health plan will want to regularly test controls within each business unit. The unit leaders and compliance or legal staff may be able to design those tests. Some examples:

- “Planting” a red flag in a claims package can help determine if staff is well-trained in identification of suspicious claims.
- Sending suspicious emails out internally to test employees in identifying and forwarding them (rather than opening attachments or “approving” a potential spam communication) can help identify additional system security training needs.

Communicate Risk Awareness

Leaders and employees should understand their role in risk control in the health plan. Risk identification and reporting can be a regular topic in:

- Team-meeting agendas
- Leadership meetings
- All-staff meetings
- Annual employee surveys
- Compliance training opportunities
- Job descriptions
- Annual employee performance goals and reviews

Document

Throughout the time between MCEs, make sure each company standard practice is thoroughly documented. Retain the compliance quality audit reports. Memorialize the compliance audit schedule and findings, as well as reports to the company risk team and the Board Risk Committee. When there are findings, document what was changed or strengthened internally as a result of the findings.

Self-Report

If findings are significant, discuss with the legal department whether the issue is reportable to regulators of any or all states. Self-reporting violations to the DOIs will potentially lessen penalties or other disciplinary actions. Particularly helpful is submitting documentation of the improvements made to prevent future incidents and the remediation provided to members, providers, and other stakeholders impacted.

Relate to Regulators

Even when (or especially when) the health plan has nothing to report, compliance personnel will do well to reach out to regulators on a personal basis. Establishing a good working relationship with the DOI analysts, staff, and the commissioner will build a good reputation and a level of understanding and trust toward the health plan in the regulatory community.

Prepare Leadership

For health plan leaders as well as business units, the best preparation for an MCE is done intentionally and over the long term. Consistent, transparent reporting of company business activities to leaders will inform them reliably and in depth so that they are able to address MCE concerns when needed. With the new focus on risk, regulators engaged in financial examinations interview members of the board of directors as well as senior management. In particular, the Board Chair and the Audit Committee Chair should be prepared by compliance staff before any interviews with regulators, to ensure that they are comfortable in the interview situation and with the topics being discussed.

At this writing, a market conduct exam does not typically include such interviews with board members. However, becoming familiar with the questions financial regulators ask can help prepare leaders for what might come up in an MCE. For a list of such questions, see below. Understanding the purpose of these risk questions will help directors and senior leaders answer knowledgeably and thoughtfully, and their reasoned responses will assist regulators in assessing the effectiveness and competency of the health plan's governance.

Potential Questions for Health Plan Leadership from Regulators

- What is the involvement of the board of directors in the risk management function of the organization?
- What kind of risk management culture is demonstrated throughout the organization?
- What does the company culture indicate regarding the importance of risk management to the organization?
- How are risk tolerances and appetites defined and communicated throughout the organization?
- Does the insurer maintain appropriate policies outlining specific obligations of employees in dealing with risk?
- How are responsibilities for risk management functions delegated and monitored?
- How are existing risks identified, tracked, assessed, and managed?
- Does risk assessment take probability, potential impact, and time duration into account?
- How are emerging and/or prospective risks identified, tracked, assessed, and mitigated?

- How does the organization use the risk information it gathers to help determine its capital needs?
- Are internal models utilized and regularly updated to ensure appropriate risk management decisions?

The Onsite MCE

Before an MCE, the lead regulator, known as the examiner-in-charge (EIC), typically sends a letter to advise the health plan that an exam has been scheduled, when it is anticipated to begin, and whether it will be a comprehensive onsite exam, a targeted onsite exam, or a desk audit. The letter will communicate the general business areas on which the regulator will focus. A subsequent letter should detail the data to be examined, for what years, and include a listing of all records and data that must be furnished. It will also stipulate all office accommodations that will be needed while the examiners are onsite.

The health plan will appoint a staff member as liaison to the examiners. She should be well-connected throughout the business units or divisions, well-spoken and resolute, and have a solid grasp of all business functions of the company. The liaison will be the person through which the regulator's requests flow and the plan's responses flow back. The liaison knows which staff members to contact to get the correct and complete information, and will review the responses before submitting them to the regulator. She will keep a log of what was submitted and when, with a back-up copy archived. Some companies also designate an in-house counsel or legal department staff member to be part of the team that reviews responses and helps draft correspondence with the regulators.

Often, a senior manager will send a notification to all staff members that regulators will occupy a particular location in the offices for a specified or unspecified amount of time. The memo will remind employees not to fraternize with the regulators and to use special care when discussing confidential company or client information.

At the beginning of the MCE, the regulatory representatives and key health plan staff will meet to discuss the scope and anticipated length of the examination. This is the time for staff to ask questions and get clarification on any concerns the regulators may bring to the table. Regularly thereafter, typically weekly, at least the lead regulator and liaison will meet to discuss progress and concerns identified. After examiners have reviewed materials, the examiner-in-charge (EIC) will initiate a conference to present a summary of findings and discuss pertinent issues with key staff of the health plan, in particular the liaison.

Back in the DOI offices, the EIC will prepare a draft of the exam report for the company to review. The health plan has an opportunity to respond to or dispute findings. Additional materials may be submitted in support of the plan's position or a response letter addressing each of the findings may be submitted. Negotiations may ensue between the plan and the DOI about the extent or severity of findings. Since the findings that remain in the final report may be the basis of disciplinary action and/or a compliance plan, which could be very costly, the health plan has a strong interest in moderating the report.

After any negotiation between regulators and the health plan, the EIC submits the final report to the commissioner. Some states post examination reports on websites or make them available electronically. Some states require a plan to file a certified copy with the DOI of other states where they operate. If the report and resulting regulatory actions are severe, it could cause reputational damage and even provide grounds for litigation.

Common Findings of an Examination Report

Some of the issues most commonly identified in MCEs for health insurance companies:⁵

- Failure to properly acknowledge, pay, investigate, or deny claims within specified timeframes.
- Failure to adhere to advertising requirements.
- Failure to use licensed and/or appointed producers and to provide proper notification of producer appointments or terminations.
- Failure to provide required claims and underwriting disclosures, including those concerning coverage issues, policy replacements, and fraud warnings.
- Improper documentation of underwriting and claim files.
- Failure to adhere to required claims appeal processes, including timeframes and disclosures.
- Failure to pay claims properly in accordance with policy provisions.
- Failure to use compliant policy forms and/or maintain compliant plans.
- Noncompliant claim denial notices.
- Failure to issue correct explanations of benefits.

Regulators do not look for infrequent or even one-time occurrences of employee error, but rather systematic and seemingly intentional violations of state laws.

Claims payment activities appear several times in the list above. Failure to pay within timeframes may result from inaccurate or outdated records of state requirements for timeliness, payment approval/disapproval guidelines that do not fairly represent the contract of the claimant, or systematic disapproval of claims that puts undue requirements on claimants. Required disclosures may be inaccurate or may not be issued at all, or the appeals process may not be facilitated as is required by state laws.

Since the laws governing health insurance advertising are very restrictive, it is easy to see why advertising violations are common. Other issues are more administrative: not using current or approved policy or application forms, not maintaining agent licensing, and issuing incorrect explanations of benefits.

A common complaint of regulators conducting MCEs is improper documentation, incomplete files, or even missing files. Health plan staff members performing compliance quality reviews should determine when documentation is an issue and work to complete, find, or incorporate full documentation into files before regulators encounter them. It is also helpful to request data runs similar to those requested by examiners in the past to determine if the plan can easily comply with future requests.

Regulators also complain about waiting for answers to their questions, receiving incomplete or inaccurate answers, or getting no response at all. Failing to answer questions promptly can damage a health plan's relationships with regulators. The number of days to respond may not be specified in the rules and regulations, so health plan staff should ask the EIC about acceptable timeframes.

Enterprise Risk Management

Under enterprise governance and annual reporting laws instituted between 2012 and 2016, all U.S. insurance companies are required to establish and maintain an **enterprise risk management (ERM)** culture. ERM requires the identification of significant risks to insurer solvency as well as activities to help mitigate those risks. Previous governance models were less stringent in their requirements for documentation and reporting of risk decisions made by insurance companies, but now the charters, reports, and minutes that illustrate that risk framework are referenced or actually included in the annual reports.

Because health plans are typically rather complex organizations, sometimes comprised of many entities (such as providers, hospitals, insurers, and third-party administrators), establishing the ERM framework may also be complicated. While the insurer is the part of the health plan that is required to institute the ERM framework by DOI regulators, the health plan may choose to establish it over the whole organization so that governance will be coherent and scalable for every part of the health plan. The holding company of the health plan is also required to submit certain financial reports to its domiciliary state, which regulators expect to be consistent with the governance reports of the health plan.

If the risk governance framework is established at the health plan level, the health plan board of directors accepts accountability for oversight of risk decision-making throughout the plan, and in particular the insurance company. Therefore, the board must have directors who are competent to oversee the insurance company's risk decisions. So not only must the insurance company board (if it has a separate one) need to have competencies that are key to running a health insurance business, but the enterprise board will need the competence to ensure adequate focus and importance is placed on the insurer portion of the health plan.

Although the enterprise risk framework may be established at the health plan level, the insurance company will be identifying its own top risks (that could threaten solvency) and, additionally, instituting controls and monitoring to manage its own risks. Measures are put into place to track and mitigate these risks, and the entire register of risks with their measures and mitigation are documented. Transparent and frequent reporting is established within the insurer's structure and to the insurer's board (if separate from the plan board). Reports of a sufficiently detailed level are submitted to the health plan board of directors to enable them to effectively fulfill their duty of care to stakeholders by making or approving risk decisions that help ensure sustainability.

The day-to-day running of the business still falls to the senior leadership of the insurance company. If the insurer has its own board, that board will still provide oversight for the ERM program and the

specific risks of the insurer. When the risk framework is built at the enterprise level, it must still be visible and working at every level of the enterprise as appropriate to its functions.

NAIC Data

The NAIC has made great strides in the technological gathering of company reports and data so that regulators can compare similar companies. The **Insurance Regulatory Information System (IRIS)** is a proprietary collection of analytical tools and databases. States can use IRIS to find facts, statistics, records, and analytics for use in contrasting and comparing insurers. The NAIC collects data on:⁶

- Company financial statements
- Regulatory enforcement actions
- Market conduct examinations
- Financial examinations
- Market Conduct Annual Statement (MCAS) (currently only claims and underwriting Information)

IRIS can be used to conduct **market analysis**, in which companies are compared in different categories to find outliers and identify possible market conduct problems. Level 1 market analysis broadly compares companies of like size, with similar products, operating in the same jurisdictions, so the analyst might see anomalies. Data such as premium increase or decrease, reserves, overall financial condition, market conduct actions, and complaint ratios in the same lines of business are part of this analysis.

If an anomaly is identified, a Level 2 analysis may be conducted to assess the impact it may have on consumers, policyholders, and claimants. The Level 2 analysis helps state regulators prioritize their outlier companies to determine which of them warrant a closer look. The regulator can then choose an option from the continuum of regulatory activities. She might just telephone a compliance officer of the company if a good relationship has been established. The next step might be a data call, or the regulator could decide that the risk is high enough that a desk audit will be necessary.

Summary

A market conduct examination (MCE) is an investigation or audit by state insurance regulators of the practices and operations of an insurer in the consumer marketplace. The purpose is to establish that the insurer is conducting its business fairly and responsibly and in compliance with the state's laws and regulations. An MCE may look at 12 standard business functions, and regulators may take a range of actions in response to a variety of triggers.

To prepare for an MCE, health plan staff should check procedures, check compliance, test controls, communicate risk awareness, document, self-report, relate to regulators, and prepare leadership. The findings in an MCE final report may be the basis of disciplinary action and/or a compliance plan, which could be very costly for a health plan, so the plan has a strong interest in making sure the report is accurate and moderating its findings.

Sources

¹ Centers for Medicare & Medicaid Services, The Center for Consumer Information & Insurance Oversight, “Compliance and Enforcement,” <https://www.cms.gov/ccio/programs-and-initiatives/health-insurance-market-reforms/compliance.html>.

² National Association of Insurance Commissioners, “About the NAIC,” http://www.naic.org/index_about.htm.

³ National Association of Insurance Commissioners, “Market Regulation Handbook Examination Standards Summary,” http://www.naic.org/documents/prod_serv_marketreg_mes_hb.pdf.

⁴ Ibid.

⁵ “Top 10 market conduct issues for US insurers,” *Insurance Business America*, April 15, 2015. <http://www.ibamag.com/news/top-10-market-conduct-issues-for-us-insurers-22146.aspx>.

⁶ National Association of Insurance Commissioners, “Market Regulation Handbook Examination Standards Summary,” http://www.naic.org/documents/prod_serv_marketreg_mes_hb.pdf.

14 Overview of Federal Regulation

Lesson Objectives

After completing this lesson, you should be able to:

- Briefly discuss the impact of the HMO Act of 1973.
- Name the aspects of health coverage governed by the Health Insurance Portability and Accountability Act (HIPAA).
- Identify and explain the main provisions of the Affordable Care Act that affect health plan products and operations.
- Explain how ERISA and other federal laws provide protections to health plan members.
- Describe in general terms the Medicare and Medicaid programs.

Federal legislation and related regulations have a major impact on health plans, including the products they offer and how they conduct business. In this lesson we offer an overview of some of the main federal laws (including the HMO Act of 1973, HIPAA, the ACA, and ERISA) and programs (Medicare and Medicaid). Many of these topics will be explored in greater depth in the lessons that follow.

The HMO Act of 1973

The **Health Maintenance Organization Act of 1973** was enacted in an effort to control rising healthcare costs by encouraging the formation of HMOs.¹ The Act established the optional federally qualified HMO program. By meeting certain standards, an HMO can (if it chooses) become federally qualified and enjoy certain advantages. However, since the Act was passed, many of the advantages have been eliminated and fewer HMOs have sought federal qualification. As a result, while in the past this program played a substantial role in the growth of HMOs, it is less important today.

To be federally qualified, an HMO must:

- Offer a comprehensive basic benefits package.
- Enroll individuals eligible for group coverage without regard to health status.
- Set premiums using community rating (later modified to allow community rating by class), and charge only nominal copayments.
- Maintain a fiscally sound operation and adequate protections against the risk of insolvency.
- Operate an ongoing quality assurance program that meets CMS requirements, stresses outcomes, and provides for review by physicians and other health professionals.

Initially, HMOs that obtained federal qualification enjoyed three main advantages:

- Exemption from more stringent state laws for HMOs.
- Eligibility for federal grants and loans.
- A requirement that some employers offer employees an HMO option (if the employer has more than 25 employees, if it sponsors health insurance, and if an HMO is operating in the locality).

However, federal funding and the employer mandate have since been eliminated. On the other hand, there has been an expansion of the state laws that federally qualified HMOs are exempt from.

Some HMOs have viewed federal requirements as too burdensome, and since incentives have been eliminated, many HMOs today choose not to seek qualification. In response, various amendments to the HMO Act have been passed over the years to provide more flexibility in various areas, including community rating, open enrollment, and medical staffing. Some examples:

- An organization can now market health plan products separate and apart from its federally qualified HMO, and these products do not have to meet federal qualifications.
- An HMO can offer high-deductible plans.²
- An HMO can provide up to 10 percent of physician services through nonaffiliated physicians and charge reasonable deductibles for such services.
- The policymaking body of an HMO is no longer required to have HMO members make up one-third of its members or to include representation of underserved communities.

The Health Insurance Portability and Accountability Act (HIPAA)

The Health Insurance Portability and Accountability Act (HIPAA) of 1996 (Public Law 104-91) includes provisions that govern many aspects of health plans.

- Title I of HIPAA contains provisions designed to make health coverage more available and portable. It sets standards for access, portability, and renewability in both group and individual coverage.
- Title II of HIPAA includes Administrative Simplification provisions aimed at improving the efficiency and effectiveness of the healthcare system and ensuring the privacy and security of healthcare information.

HIPAA Title I: Access, Portability, and Renewability

Before HIPAA, many people feared that if they changed health plans (as when they changed jobs), the new plan might exclude coverage for a medical condition they already had. HIPAA placed restrictions on such exclusions. As we will discuss below, the ACA completely prohibits preexisting condition exclusions. But this ACA rule does not apply to **grandfathered health plans**—coverage that people enrolled in on or before March 23, 2010, that has not been changed in certain ways that reduce benefits or increase costs

to consumers.³ To these grandfathered plans, HIPAA rules still apply. Also, depending on what action is taken to repeal and replace the ACA, HIPAA rules may once again apply to a broad range of plans.

Under HIPAA, preexisting condition exclusions may apply only to conditions that were diagnosed or treated during the six months before enrollment, and coverage can be excluded for only 12 months after enrollment.⁴ Moreover, those changing from one health plan to another will likely have no preexisting condition exclusion at all. The rule is that if a person had prior creditable health coverage, the exclusion period is reduced by the length of time she had that coverage. So if a person had such coverage for at least 12 months, the period is completely eliminated.

Example: *Mary worked at ABC Company, where she was enrolled in a health plan for eight months. She moves to XYZ Corporation and enrolls in a health plan there. The new plan imposes a preexisting condition exclusion, but this is reduced by Mary's eight months of prior creditable coverage, so it only last four months. If Mary had been enrolled in the other plan at least 12 months, her exclusion period would have been completely eliminated.*

Creditable coverage includes a wide range of health coverage (an employer-sponsored plan, government employee coverage, an individual policy, Medicare, Medicaid, and other public benefit programs).⁵ To qualify for the prior creditable coverage rule, a person must maintain coverage continuously (that is, she must enroll in the new plan no more than 63 days after her old coverage ends).⁶ This prevents a person from waiting until she needs medical care and only then seeking to enroll in a plan. A preexisting condition exclusion cannot apply to a newborn, an adopted child, or a child under 18 placed for adoption, as long as the individual becomes covered under creditable coverage within 30 days of birth, adoption, or placement for adoption. In addition, preexisting exclusions cannot apply to pregnancies.⁷

In the group market, with limited exceptions, plans must be renewed regardless of the health status of any group member. In the individual market, insurers must also offer to renew policies, except in limited circumstances (such as when the insurer withdraws from the marketplace).

Title II: Administrative Simplification

HIPAA mandated the adoption of federal privacy protections for individually identifiable health information, commonly referred to as protected health information (PHI). To fulfill this mandate, HHS published a Privacy Rule, which:

- Sets national standards for the protection of PHI by health plans, healthcare clearinghouses, and healthcare providers who conduct transactions electronically,⁸ as well as certain business associates of these entities.⁹
- Requires safeguards to protect the privacy of PHI and sets limits and conditions on the uses and disclosures of PHI without patient authorization.
- Gives patients certain rights over their health information, including rights to examine and obtain a copy of their health records and request corrections.¹⁰

HIPAA also contains provisions to discourage fraud and other criminal offenses and to encourage the reporting of it.¹¹

The Affordable Care Act (ACA)

The Patient Protection and Affordable Care Act of 2010, more commonly called simply the Affordable Care Act (ACA), enacted comprehensive health coverage reforms and greatly expanded the federal role in regulating health plans. The ACA affects health plan products and policies in many areas, including the following:

Essential health benefits (EHBs)	Coverage tiers (“metal” plans)	Annual benefit limits
Lifetime benefit limits	Prohibition of preexisting condition exclusions	Limits on premium variation
Young adult coverage	Medical loss ratio (MLR) rules	Premium tax credits (APTC)
Cost-sharing reductions	Medicaid expansion	Individual mandate

Under the ACA, to be a **qualified health plan (QHP)**, a plan must provide **essential health benefits (EHBs)**—coverage of 10 broad categories of healthcare services. (The specific services covered in each category may vary based on state requirements.)

Essential Health Benefit Categories

Hospitalization	Prescription drugs
Ambulatory patient services (outpatient care)	Rehabilitative and habilitative services and devices
Emergency services	Laboratory services
Pregnancy, maternity, and newborn care	Preventive and wellness services and chronic disease management
Pediatric services	Mental health and substance use services

Note that dental and vision care are EHBs for children but not adults. Plans must include birth control (subject to limitations based on religious freedom objections) and breastfeeding coverage.

Health plans available in the ACA Marketplaces (exchanges) fall into four tiers based on how much of the cost of covered services the plan pays and how much the enrollee pays. These are commonly referred to as the “metal plans.”

Plan Tier	Plan Pays	Enrollee Pays
Bronze	60%	40%
Silver	70%	30%
Gold	80%	20%
Platinum	90%	10%

Premiums are generally lower for Bronze plans than other tiers.

An **annual benefit limit** is a maximum dollar amount that a health plan will pay in total benefits during a year for an individual. The ACA prohibits annual limits for essential health benefits. (However, this prohibition does not apply to grandfathered individual health plans.¹²) A **lifetime benefit limit** is a maximum dollar amount that a health plan will pay in total benefits during the entire time an individual is enrolled in the plan. The ACA prohibits lifetime limits on most benefits in all plan (including grandfathered plans).

Under the ACA, a health plan cannot deny a person coverage or charge her a higher premium based on her health. And the plan cannot impose a preexisting condition exclusion—that is, it cannot refuse to pay for essential health benefits for a medical condition the person had before she enrolled. However, this prohibition does not apply to grandfathered individual plans; these plans are governed by the HIPAA rules on preexisting conditions discussed above.

For non-grandfathered small group and individual health plans sold after 2013, premium rates may vary based only on these four factors:

- Age
- Tobacco use
- Family size
- Geography

For those 21 and older, the oldest person cannot be charged more than three times the youngest person. For those under 21, rates must be actuarially justified. Age bands can be used in rating. There must be a single age band for all those under 21, and another single age band for those over 63. In between, there can be one-year age bands. *For an excerpt of a rating age curve proposed by CMS, see below.*

Excerpt of Rating Age Curve Proposed by CMS

Age	Premium Ratio
0-20	0.635
21	1.00
40	1.278
50	1.786
60	2.714
64 and older	3.0

Other permitted rating factors:

- **Tobacco.** Tobacco users can be charged up to 50 percent more than others. (In practice, some younger smokers are charged less than 50 percent.)
- **Family size.** Health plans may charge a family more than an individual and may charge a larger family more than a small one. But in determining the premium for a family, no more than three children can be counted.
- **Geography.** States must establish rating areas based counties, three-digit zip codes, or metropolitan statistical areas (MSAs) along with at least one non-MSA.¹³

Before the ACA, when a child reached 19, her parents' health plan would usually stop covering her (with some exceptions for students). Under the ACA, if a health plan includes dependents, it must continue covering children up to age 26, even if the child gets married, starts or leaves school, or lives outside the parent's home.

Insurers and health plans must spend 80 percent (individual and small group coverage) or 85 percent (large groups) of the dollars they receive in premiums on medical care and healthcare quality improvement. Administrative costs (such as salaries and agent commissions) must come out of the remaining 15 or 20 percent. If an insurer or plan fails to meet this goal, it must pay a rebate to its customers.¹⁴

Some people are eligible for tax credits to help them pay the premiums of health plans bought through an ACA Marketplace. The credit is available to those with family income up to 400 percent of the federal poverty level (FPL), but the higher the income, the lower the credit. While most tax credits can be claimed only in filing an annual income tax return, this credit can be received in advance—called **Advanced Payments of the Premium Tax Credit (APTC)**. If a person chooses, amounts based on her estimated credit can be paid by the government to her health plan to cover part of her monthly premiums. Later, when she files her tax return, if she has received too much or too little in credit, adjustments can be made.¹⁵

Some people are also eligible for reductions in the cost-sharing (deductible, copayments, or coinsurance) they must pay. **Cost-Sharing Reductions (CSRs)** are generally available only to those with incomes up to 250 percent of the federal poverty level. To be eligible for CSRs, a person must be enrolled in a silver plan. (Note that a premium tax credit can be received for any metal category.¹⁶)

Medicaid is a federal-state health coverage program for the poor. Historically, it has served only certain categories of low-income people, such as children, pregnant women, very low-income parents, the disabled and elderly, and a few others. The ACA was designed to extend Medicaid to all poor people nationwide, regardless of category (including single people without children). Specifically, the ACA expands Medicaid eligibility to all those younger than 65 with family income at or below 133 percent of the FPL (in effect, 138 percent because of rules that disregard 5 percent of income). Medicaid is funded by both the federal government and the states, but the ACA provided that the federal government would pay nearly all costs of this expansion: 100 percent through 2016, 95 percent in 2017, and (should this component of the ACA survive), scaling down to 90 percent in 2020 and beyond.¹⁷

Historically, each state has chosen whether or not to participate in Medicaid (although all now do) and, within federal guidelines, has set its own eligibility rules. As written, the ACA required all states to participate in the Medicaid expansion; if they did not, they would lose all Medicaid funding. This led to court challenges that resulted in a 2012 Supreme Court ruling (*National Federation of Independent Business et al. vs. Sebelius*) that upheld the validity of the ACA but made Medicaid expansion voluntary for each state. As of this writing in January 2017, 31 states plus the District of Columbia have adopted the expansion of Medicaid.¹⁸ Whether this component of the ACA will survive is unknown.

As we have seen, under the ACA health plans cannot deny coverage to people with serious health conditions, charge them higher premiums, or impose a preexisting condition exclusion. The ACA's **individual mandate**, requiring every American (with a few exceptions) to have health coverage, is designed to prevent people from enrolling in a health plan only when they start to need healthcare, so that health plans cover only sick people, pay many claims, and must charge very high premiums. The individual mandate is intended to ensure that the population of insureds includes a mix of young and healthy people as well as the old and ill.

Under the individual mandate, each individual (of any age) must have **minimum essential coverage** throughout the year or pay a penalty when filing his federal income tax return. (Some exemptions apply.) Those claiming another person as a dependent must ensure that that person has coverage or pay a penalty for him or her.¹⁹ The penalty is a percentage of household income or a per-person amount (whichever is higher). For what counts as *minimum essential coverage*, see below.

Minimum essential coverage includes the following:²⁰

- Employer-sponsored coverage, including self-insured plans, COBRA coverage, and retiree coverage
- Individual coverage, including ACA Marketplace plans, student health plans, and catastrophic coverage (available to certain young adults)
- Medicare Part A and Medicare Advantage plans
- Most Medicaid coverage and Children's Health Insurance Program (CHIP) coverage

- Certain types of Veterans Administration coverage and most types of TRICARE coverage
- Other coverage recognized by the Secretary of HHS as minimum essential coverage

The Future of the ACA

The ACA was passed by Democrats on a strictly partisan vote. Since the Republicans retook control of Congress, they have voted many times to repeal it, but these bills have all been vetoed by President Obama. As of this writing, it is likely that the newly elected Republican Congress will repeal the ACA and such legislation will not be vetoed by President Trump. But it is very unclear what will replace the ACA. Moving forward, we will provide those enrolled in this course with periodic updates. We offer here some possibilities.

Several provisions of the ACA are likely to be repealed or greatly modified. These include the mandates requiring individuals to have health coverage and larger employers to sponsor coverage or pay fees. Both may be eliminated outright, or they may be kept but the fees/penalties reduced to a very small amount.

Some provisions of the ACA have proved highly popular and are likely to survive. Two examples:

- Giving young adults up to age 26 the right to continue coverage under their parents' health plan has increased coverage of this generally healthy demographic, and this has helped make the overall health insurance risk pool more viable.
- Preexisting condition exclusions are very unpopular, but retaining the prohibition on them would be problematic if (as expected) the individual mandate is repealed. This could result many people waiting until they became ill to buy coverage, creating a risk pool with a disproportionate number of sick people. This problem might be addressed by strict rules allowing people to enroll only during limited open enrollment periods. If this prohibition is repealed, the HIPAA restrictions discussed above would apply in many cases and may receive renewed attention.

Based on plans previously proposed, it is possible that replacement legislation will focus on offering tax credits or deductions to help cover the cost of health coverage. For example, individuals might be able to enroll—during a limited open enrollment period—in one of several health plans offered through an exchange. On their federal tax returns they could claim a deduction or credit for the cost of the coverage, up to a limit. Continuous enrollment might also be required, so that individuals could not enroll in a plan when they got sick, disenroll once they were well, and reenroll if they became ill once again.

Other Important Federal Laws and Programs

In the upcoming lessons we will be covering the following topics in federal involvement in health plans:

- ERISA and related legislation
- Wellness programs
- Medicare

- Medicaid

Let's take a few minutes to preview each of these.

ERISA and Related Laws

The Employee Retirement Income Security Act of 1974 (ERISA) is a federal law that sets minimum standards for employee benefit plans, including both retirement and health plans, in order to provide protection for those covered by these plans. ERISA:

- Requires plans to provide participants with information about plan features and funding.
- Specifies fiduciary responsibilities for those who manage and control plan assets.
- Requires a grievance and appeals process for plan participants seeking benefits.
- Gives participants the right to sue for benefits and breaches of fiduciary duty.

Responsibility for the interpretation and enforcement of ERISA is shared among the Department of Labor, Department of the Treasury, and the Pension Benefit Guaranty Corporation.

ERISA does not require employers to sponsor health coverage for employees, but if an employer does so it must meet ERISA requirements and standards.²¹ Both fully insured and self-funded (self-insured) employer health plans are covered by ERISA, but they are treated somewhat differently. Since the passage of the ACA, many employers have set up self-funded plans, increasing the importance of ERISA.

A number of other federal laws (some enacted as amendments to ERISA) expand protections for health plan members. These include the following:

- The **Consolidated Omnibus Budget Reconciliation Act (COBRA)** gives some workers and their families the right to continue their health coverage for a limited time, at their own expense, after leaving a sponsoring employer (or after certain other events).
- As previously discussed, **HIPAA** imposes restrictions on preexisting condition exclusions.
- Under the **Newborns' and Mothers' Health Protection Act**, health plans must cover a hospital stay for childbirth for at least 48 hours following a vaginal delivery and 96 hours following a cesarean. The attending physician may decide, after consultation with the mother, to discharge the mother or the newborn earlier, but he or she cannot be offered any incentives to do so.²²
- Under the **Mental Health Parity and Addiction Equity Act of 2008 (MHPAEA)** a health plan is not required to provide benefits for mental health or substance abuse services, but if it does so, it may not impose less favorable limits on such benefits than it does on medical benefits. MHPAEA may become more important if the ACA provisions that effectively extend mental health and substance use benefits (by incorporating them into the essential health benefits package) is eliminated.

- Under the **Women’s Health and Cancer Rights Act of 1998 (WHCRA)**, group health plans covering mastectomies must cover reconstruction of the affected breast, reconstruction of the other breast to produce a symmetrical appearance, prostheses, and complications.²³

Further information on ERISA can be found in Lesson 15.

Wellness Programs

Wellness programs help people take action to maintain and improve their health, such as exercising more, losing weight, and quitting smoking. Many employers sponsor programs that give employees incentives to participate in activities or achieve goals. Wellness programs can run afoul of the law in two ways:

- The privacy of health and genetic information is not protected.
- Incentives discriminate against those with disabilities.

The relevant laws include the American with Disabilities Act (ADA), the Genetic Information Nondiscrimination Act (GINA), HIPAA, and the Affordable Care Act.

Further information on wellness programs and related legal issues can be found in Lesson 16.

Medicare

Medicare is a federal health coverage program. It primarily serves those 65 and older, but also people with severe, long-term disabilities, kidney failure, or Lou Gehrig’s disease. The program includes Medicare Part A (inpatient hospital care and related services, and hospice) and Part B (doctors’ office visits and a variety of other outpatient healthcare services).

Private-sector health plans provide Part A and Part B coverage (and often other benefits such as vision care and wellness programs) through the **Medicare Advantage (MA)** program (Medicare Part C). Beneficiaries may enroll in an MA plan or Original Medicare (traditional Parts A and B). Despite changes in how MA plans are compensated by Medicare under the ACA, they have continued to be popular. Private health plans also sponsor **Medicare Part D prescription drug plans**. Beneficiaries have the option of enrolling in a Part D PDP for an additional monthly premium. Some MA plans include Part D coverage (MA-PD plans).

Further information on Medicare can be found in Lesson 17 and on Medicare Advantage in Lesson 18.

For some years the Medicare Sustainable Growth Rate (SGR) was the methodology used to increase payments to physicians participating in Medicare to account for rises in healthcare costs. But the SGR resulted in very low increases or even cutbacks. In response, Congress passed the Medicare Access and CHIP Reauthorization Act (MACRA), which repeals the SGR and replaces it with an approach called the Quality Payment Program. The objectives of the new system are to emphasize quality of care, not quantity; to encourage better coordination among providers; and to prioritize wellness and prevention.

The Quality Payment Program is designed to reward high-quality care, through two avenues:

- **Advanced Alternative Payment Models** give physicians the opportunity to be paid more for delivering better care and for making investments that support patients.
- Under the **Merit-based Incentive Payment System**, physicians can participate in organizations that get paid primarily for keeping individuals healthy. For example, a physician might be part of an accountable care organization (ACO), where clinicians come together to coordinate the care of patients. When clinicians achieve better health results and reduce care costs, they receive a portion of the savings.

Medicaid

As we mentioned earlier, Medicaid is a federal-state program that pays for healthcare for some low-income people. Each state establishes and administers its own Medicaid program and determines the type, amount, duration, and scope of services offered, within broad federal guidelines. Federal law requires states to provide certain mandatory benefits (such as inpatient and outpatient hospital services), and states may also choose to provide additional “optional” benefits (such as case management).

Some very poor elderly or disabled individuals are eligible for both Medicare and Medicaid. For these **dual eligibles**, Medicare is the primary payer and Medicaid the secondary payer. In other words, if a service is covered by both Medicare and Medicaid, Medicare pays. An expense not covered by Medicare (such as deductibles and copayments) is paid by Medicaid.

Medicare pays for nursing home care and home health care as part of the treatment of an illness or injury (typically for a few weeks after a hospital stay). Those who need such services indefinitely because of a chronic physical or cognitive impairment (long-term care) may be able to rely on Medicaid. However, to qualify, they must generally spend almost all their assets and income on their care.

Further information on Medicaid can be found in Lessons 19 and 20.

Summary

The Health Maintenance Organization Act of 1973 sets standards for federally qualified HMOs. The Health Insurance Portability and Accountability Act (HIPAA) includes provisions designed to promote access and portability of health coverage, administrative modernization and efficiency in the health industry, and privacy and security of health information. The Affordable Care Act establishes rules for health plans in a wide range of areas, including benefits and premium rating, and provides subsidies to help consumers afford coverage. ERISA and several other federal laws afford a variety of protections to health plan members. Medicare provides health coverage for the elderly and disabled; Medicaid pays for healthcare needed by some poor people.

Sources

- ¹ 42 USC Section 300e – Requirements of Health Maintenance Organizations, <https://www.law.cornell.edu/uscode/text/42/300e>
- ² HIPAA, Title I, Sec. 193, <https://www.gpo.gov/fdsys/pkg/PLAW-104publ191/html/PLAW-104publ191.htm>
- ³ Department of Health and Human Services, “Preexisting Conditions,” <https://www.hhs.gov/healthcare/about-the-law/pre-existing-conditions/index.html>
- ⁴ 29 USC Section 1181(a)(2), Increased Portability through Limitation on Preexisting Condition Exclusions, https://www.law.cornell.edu/uscode/text/29/1181?qt-us_code_temp_noupdates=0#qt-us_code_temp_noupdates
- ⁵ 29 USC Sec. 1181(c)(1).
- ⁶ 29 USC Sec. 1181(c)(2)(A).
- ⁷ 29 USC Sec. 1181(d)]
- ⁸ Department of Health and Human Services, “Health Information Privacy – HIPAA for Professionals,” <https://www.hhs.gov/hipaa/for-professionals/inces.html>; 45 CFR 160.102, Applicability, <https://www.law.cornell.edu/cfr/text/45/160.102>
- ⁹ 45 CFR 160.103, Definitions, <https://www.law.cornell.edu/cfr/text/45/160.103>
- ¹⁰ Department of Health and Human Services, “Health Information Privacy,” The HIPAA Privacy Rule, <https://www.hhs.gov/hipaa/for-professionals/privacy/>
- ¹¹ 42 USC Section 1320a-7, Exclusion of Certain Individuals and Entities from Participation in Medicare and State Health Care Programs, <https://www.law.cornell.edu/uscode/text/42/1320a-7>; 42 USC Section 1395b-5, Beneficiary Incentive Programs, <https://www.law.cornell.edu/uscode/text/42/1395b-5>
- ¹² HealthCare.gov, “Health insurance Rights and Protections,” <https://www.healthcare.gov/health-care-law-protections-lifetime-and-yearly-limits/>
- ¹³ Centers for Medicare and Medicaid Services, “Overview: Final Rule for Health Insurance Market Reforms,” <https://www.cms.gov/CCIIO/Resources/Files/Downloads/market-rules-technical-summary-2-27-2013.pdf>
- ¹⁴ Centers for Medicare and Medicaid Services, “MLR Final Rule,” <https://www.cms.gov/CCIIO/Resources/Fact-Sheets-and-FAQs/mlrfinalrule.html>
- ¹⁵ Internal Revenue Service, “Questions and Answers on the Premium Tax Credit,” Oct. 21, 2016, <https://www.irs.gov/affordable-care-act/individuals-and-families/questions-and-answers-on-the-premium-tax-credit>
- ¹⁶ HealthCare.gov, “Saving Money on Health Insurance,” <https://healthcare.gov/lower-costs/save-on-out-of-pocket-costs/>
- ¹⁷ Department of Health and Human Services, “Medicaid Expansion Under the Affordable Care Act Leads to Better Care and Improves Financial Well-Being for New Enrollees” (Press Release), June 21, 2016, <https://www.hhs.gov/about/news/2016/06/21/medicaid-expansion-under-aca-leads-better-care-and-improves-financial-well-being-for-new-enrollees.html>
- ¹⁸ The Henry J. Kaiser Family Foundation, “Status of State Action on Medicaid Expansion Decision, as of January 1, 2017,” <http://kff.org/health-reform/state-indicator/state-activity-around-expanding-medicaid-under-the-affordable-care-act/?currentTimeframe=0>

¹⁹ Internal Revenue Service, “Questions and Answers on the Individual Shared Responsibility Provision,” <https://www.irs.gov/affordable-care-act/individuals-and-families/questions-and-answers-on-the-individual-shared-responsibility-provision>

²⁰ Ibid.

²¹ Department of Labor, “Health Plans & Benefits: ERISA,” <https://www.dol.gov/general/topic/health-plans/erisa>

²² Department of Labor, “Newborns’ and Mothers’ Health Protection Act FAQs,” <https://www.dol.gov/agencies/ebsa/about-ebsa/our-activities/resource-center/faqs/nmhp>

²³ Department of Labor, “Fact Sheet: Women’s Health and Cancer Rights Act,” <https://www.dol.gov/agencies/ebsa/about-ebsa/our-activities/resource-center/fact-sheets/whcra>

15 ERISA and Health Plans

Lesson Objectives

After completing this lesson, you should be able to:

- State what health plans are subject to ERISA and when ERISA generally preempts state laws.
- Summarize ERISA's requirements of health plans for fiduciaries, documentation, reporting, and disclosure.
- Describe ERISA's requirements for benefits and claim procedures.
- Discuss some of the litigation issues involving ERISA

Introduction

For the past several years, discussions of health plans have tended to focus on the Affordable Care Act, and it is true that this is landmark legislation affecting all Americans. But the ACA comprises only one of the many layers of statutes and regulations affecting health plans (albeit a thick layer). The fundamental federal law governing employer-sponsored group health plans (as well as many other employee benefit plans) is the **Employee Retirement Income Security Act (ERISA)**.¹ Few federal laws have a greater impact on the operations of health plans than ERISA.

ERISA does not directly regulate health insurers; rather, it regulates employers. Specifically, it regulates most employer-sponsored employee benefit plans, and health insurers sell their health plan products to these plans. As a result, ERISA affects the nature, design, and administration of these products. In addition, ERISA determines what state laws can be applied to such products and what legal challenges can be made to their administration. Consequently, employers and other plan sponsors, health insurance issuers, claims administrators, and others dealing with group health plans must have a working knowledge of ERISA in order to lawfully establish, design, and operate employer-sponsored group health plans.

What Plans Are Governed by ERISA?

ERISA applies to a wide range of employee benefit plans. These include **employee retirement plans** and **employee welfare plans**, and employee welfare plans include health plans. Specifically, under ERISA an employee welfare plan is:

...any plan, fund, or program which [is] established or maintained by an employer or by an employee organization, or by both, to the extent that such plan, fund, or program was established or is maintained for the purpose of providing for its participants or their beneficiaries, through the purchase of insurance or otherwise, (A) medical, surgical, or hospital care or benefits, or benefits in the event of sickness, accident, disability, death, or unemployment, or vacation benefits, apprenticeship or other training programs, or day care centers, scholarship funds, or prepaid legal services.²

ERISA defines medical care to include the “diagnosis, cure, mitigation, treatment, or prevention of disease, or amounts paid for the purpose of affecting any structure or function of the body,” as well as amounts paid for insurance coverage for such items.³ This means that ERISA applies not only to traditional group health plans but also to other plans or programs that provide medical benefits. These can include:

- dental and vision care plans
- health flexible spending accounts (FSAs)
- certain wellness programs (programs that provide flu shots, biometric screenings, etc.)
- onsite medical clinics (but not programs that treat only minor injuries or illnesses or render first aid)
- executive physical programs
- employee assistance programs (EAPs)

As noted in the definition above, to be subject to ERISA, a plan, fund, or program must be established or maintained by an employer or by an employee organization. If not, even if the benefits constitute medical care, the plan, fund, or program is not covered by ERISA. Note that governmental and religious employers are generally not subject to ERISA.

An employee benefit plan is *not* subject to ERISA if these requirements are met:

- The plan is fully insured, not self-funded (see discussion below).
- Participation in the plan by employees is completely voluntary.
- Employees pay 100 percent of costs, and no contributions are made by the employer or an employee organization. (However, the employer may collect employees’ premiums through payroll deductions.)
- The employer does not endorse the plan. (However, the employer may permit the insurer to publicize the plan.)
- The employer receives no consideration in connection with the plan, other than reasonable compensation for the administrative costs of payroll deductions.

An employer sponsoring a health plan may **fully insure** the plan (pay premiums to an insurer, which pays benefits to employees) or **self-fund (self-insure)** the plan (pay benefits to employees out of its own funds). Both fully insured and self-funded plans are covered by ERISA, but they are treated differently in several ways, as we will discuss. Historically, most employer health plans have been fully insured, but today a growing number are self-funded. There are advantages and disadvantages of self-funding—for a discussion, see “Should an Employer Self-Fund an Employee Health Plan?” at the end of this lesson.

ERISA has general requirements that apply to all employee welfare plans. It also has a number of requirements that apply only to health plans, including specific notice requirements, more stringent

claims and appeals procedures, and broader disclosure requirements. And, of course, group health plans are subject to other federal laws and regulations (HIPAA, COBRA, etc.), as discussed elsewhere in this course.

Preemption

In general, ERISA makes the regulation of employee benefit plans a federal responsibility and frees such plans from state regulation. But in drafting ERISA, Congress did not want to divest the states of their traditional power to regulate insurance, and it included language designed to preserve this power. As a result, while employee benefit plans are generally exempt from state regulation, they are not exempt from all state laws, and courts have sometimes ruled that a state law still applies and sometimes that it does not. In this section we offer a summary of this complex issue.

The Preemption Issue

ERISA's **preemption clause** provides that ERISA supersedes any and all state laws insofar as they relate to any employee benefit plan subject to ERISA (with exceptions, noted below)—that is, ERISA preempts such state laws.⁴ ERISA preempts not only state laws that are specifically designed to regulate employee benefit plans but also those that may only indirectly affect such plans.⁵ However, courts have found that state laws of general applicability that only tangentially affect ERISA-covered plans are not preempted.⁶

ERISA's **savings clause** preserves from preemption any state law to the extent that the law regulates the "business of insurance" (or banking or securities)—in other words, a state law regulating the business of insurance applies to employee benefit plans.⁷ However, ERISA's **deemer clause** states that an employee benefit plan "shall not be deemed to be an insurance company or other insurer, bank, trust company, or investment company or to be engaged in the business of insurance or banking for the purposes of any law of any state purporting to regulate insurance companies, insurance contracts, banks, trust companies, or investment companies."⁸

What do these two clauses mean? That self-funded employee benefit plans but not fully insured plans are exempt from state regulation of insurance.⁹

- A self-funded plan is not insurance and so is exempt from state insurance laws under the deemer clause.
- A fully insured employee benefit plan is not itself insurance, but it uses insurance to provide benefits to employees, and this insurance is governed by state insurance laws, under the saving clause.

The Supreme Court has determined that this difference in treatment of self-funded and fully insured plans was the intent of Congress and is valid.¹⁰

Defining the "Business of Insurance"

Since a fully insured employee benefit plan is subject to state laws that regulate the "business of insurance," it is important to clarify what that means. In *Pilot Life Insurance Co. v. Dedeaux*, the U.S. Supreme Court used several questions to decide whether a law regulates the business of insurance:¹¹

- Is the law specifically directed toward the insurance industry?

- Does the practice the law regulates have the effect of transferring or spreading a policyholder's risk, is it an integral part of the policy relationship between the insured and the insurer, and is it limited to entities within the insurance industry?

However, more recently, in *Kentucky Association of Health Plans, Inc. v. Miller*, the Supreme Court took a different approach, asking these two questions: Is the law limited to the insurance industry? Does the law have a substantial effect on the risk pooling arrangement between the insurer and the insured?¹²

The Battle over Preemption

States often enact and attempt to enforce laws that regulate employee benefit plans, and employers and health plans ask the courts to exempt them from such laws on the grounds of ERISA preemption. Laws that affect employer health plans only indirectly are often allowed to stand. For instance, a New York law imposing surcharges on hospital bills increases the benefit costs of an employer's health plan, but the affect was indirect and the law was not preempted.¹³

Increasingly, state insurance authorities are attempting to apply insurance laws and regulations directly to noninsurance products of health plans as well as to integrated delivery systems and other entities that have contracted to provide services to employer health plans. For instance, state laws may add reporting obligations for plans, including self-funded group health plans. To read more about such a law in Vermont and the challenge to it, see "An ERISA Preemption Case in Vermont" at the end of this lesson.

Self-Funded Plans

Many self-funded plans have **stop-loss (excess-loss) insurance**, which provides reimbursement to the plan sponsor if benefits paid exceed a certain threshold. This can be on an individual or aggregate basis (or both).

- **Individual.** If any single plan participant receives benefits above an annual dollar amount, the plan is reimbursed for benefits above that level.
- **Aggregate.** If all plan participants combined receive benefits above a certain annual dollar amount, the plan is reimbursed for benefits above that level.

Example: ABC Company has a self-funded health plan, but it purchases stop-loss insurance from High Plains Insurance Company. The policy works on an individual basis. If any one plan participant makes claims of more than \$50,000 during a year, High Plains reimburses ABC for any amount paid in excess of \$50,000.

Example: XYZ Company also has a self-funded health plan, and it also purchases stop-loss insurance from High Plains, but on an aggregate basis. If all participants combined make claims of more than \$600,000 during a year, High Plains reimburses XYZ for the amount paid in excess of \$600,000.

If a plan has stop-loss insurance, is it still considered self-funded for purposes of ERISA? Generally, it is, provided these two conditions are met:

- Plan participants must have no right to claim benefits directly from the stop-loss insurer. The employer must bear the obligation to pay all claims, although it may be reimbursed by the

insurer. If participants can make claims directly to the stop-loss insurer, there is an insurance contract between the participants and the insurer, and the plan is subject to state insurance regulation.

- The threshold for stop-loss coverage must be high enough that the employer bears substantial risk for the payment of claims. If the threshold is very low, the stop-loss insurer effectively pays most claims, and the plan is considered insured, not self-funded.

Although some courts have found that self-funded plans with low-threshold stop-loss insurance are insured plans,¹⁴ and the U.S. Department of Labor (DOL) seems to agree,¹⁵ some state laws taking this position have been held by other courts to be invalid.¹⁶ In response, some states sought to prevent this arrangement by prohibiting insurers from selling such insurance. The National Association of Insurance Commissioners (NAIC) has adopted a model law that prohibits the sale of stop-loss insurance with an annual individual threshold below \$20,000. The aggregate threshold for groups of 50 or fewer must be at least the greater of \$4,000 times the number of group members, 120 percent of expected claims, or \$20,000. For groups of 51 or more, the aggregate threshold must be at least 110 percent of expected claims.¹⁷

ERISA Requirements for Fiduciaries, Documentation, Reporting, and Disclosure

Fiduciaries

Every ERISA plan must “provide for one or more named fiduciaries who jointly and severally shall have authority to control and manage the operation and administration of the plan.”¹⁸ These persons have fiduciary duties under ERISA.

- In her role as fiduciary, a person must act solely in the interests of plan participants and beneficiaries, providing benefits to them and defraying reasonable expenses of administering the plan.¹⁹
- A fiduciary must act in accordance with plan terms and the Plan Document (discussed below), unless these conflict with ERISA, and with the care, skill, and diligence that a prudent person familiar with such matters would use in a similar enterprise.²⁰
- If a plan is self-funded, a fiduciary must ensure that plan investments are diversified to minimize the risk of large losses.²¹

Often, a plan fiduciary is also an officer of the sponsoring employer. This raises questions: When is she wearing her “fiduciary hat” and thus required to act solely in the interest of plan participants and beneficiaries? And when is she wearing her “corporate hat” and thus allowed to act in the best interests of the employer?

The distinction is not always clear, but in one area it is. The authority to amend or terminate a plan is considered a “settlor function” or business decision of the plan sponsor. This means that a person is not acting as a fiduciary when making decisions on such matters.²² For instance, an employer might amend its plan to eliminate certain coverages, and although this action may not be in the best interest of plan participants, it does not violate a fiduciary’s obligations under ERISA.

A plan fiduciary may not engage in certain **prohibited transactions**.²³

- A fiduciary may not cause a plan to engage in certain transactions (such as the sale or lease of property or the extension of credit) between the plan and a **party in interest** (a plan fiduciary, a person who performs services for the plan, or someone related to a fiduciary or service provider).²⁴ Unless advance approval is obtained from the Department of Labor (DOL), this type of transaction is prohibited without regard to the economic benefits of the transaction to the plan.
- A fiduciary may not act in certain conflict-of-interest situations or receive compensation from a third party in connection with a transaction involving the assets of the plan. She may not engage in **self-dealing**—essentially, using her authority to have the plan hire herself (or a related party) to perform additional services for a fee.²⁵

Documentation, Reporting, and Disclosure

ERISA has a number of documentation, reporting, and disclosure requirements that apply to all employee benefit plans. Over the years, due to amendments to ERISA and the passage of other laws, these requirements have grown more complex, particularly for health plans. However, this is a critical area of compliance; failure to comply with requirements could lead to inconsistent plan administration, civil monetary penalties, excise taxes, and less favorable standards of review in court.

To comply with ERISA, a plan must draft a number of documents:

- Plan Document
- Summary Plan Description (SPD)
- Summary of Material Modifications (SMM)
- Annual Return (IRS Form 5500)

In addition, the ACA requires a Summary of Benefits and Coverage (SBC).

The Plan Document

Every employee benefit plan must be set forth in a written Plan Document (or documents) that details the operative provisions governing benefits under the plan.²⁶ For fully insured plans, the employer might maintain a simple Plan Document that describes certain of the plan's rules but otherwise refers to the insurance or health plan contract for the description of plan benefits. Many employers have multiple employee benefit plans (for health, dental, vision care, and disability), and they may opt to use a "wraparound" Plan Document that creates a single ERISA plan. A Plan Document and a Summary Plan Description may be combined into one document.

ERISA requires employer health plans to be in compliance with HIPAA, and HIPAA requires that the Plan Document address the measures the plan and plan sponsors will take to safeguard protected health information.²⁷ Plan fiduciaries must implement these measures or risk fiduciary duty violations under ERISA, as well as violations of the HIPAA privacy and security regulations.

The Plan Document must specify a procedure for amending the plan, and fiduciaries must follow this procedure.²⁸ If, for instance, the Plan Document states that the plan may be amended only by action of the company's Board of Directors, failure to obtain Board approval could result in an amendment being determined to be invalid.

The Summary Plan Description (SPD)

A Summary Plan Description (SPD) is a booklet that describes the provisions of a plan in lay language. The term "Summary Plan Description" is often used loosely to refer generally to plan materials, but it has a specific meaning with specific requirements. The DOL, various federal statutes and regulations, and court decisions prescribe the types of information that must be included in an SPD, as well as disclosure requirements²⁹. Therefore, employers need to be sure they are referring to the correct documents—certificates of coverage, benefit summaries, schedules of benefits, and other benefit descriptions typically are *not* SPDs.

For fully insured plans, employers often use the booklet published by the insurer as the basis for the Summary Plan Description, although the employer generally has to add certain administrative information to comply with DOL and other requirements. A health plan may contain detailed benefit schedules that are difficult to summarize; in such cases, instead of repeating the benefit schedules, the SPD may provide a general description of the types of benefits provided and inform participants that the complete schedules are available for review.³⁰

Employers with self-funded plans typically work with third-party administrators (TPAs), consultants, and counsel to prepare the SPD. Often such SPDs are based on insurance certificates the TPAs use with insured plans.

As mentioned, ERISA requires employer health plans to comply with HIPAA, and HIPAA requires SPDs to include certain information:

- The SPD must state whether an insurer, health plan, or TPA is responsible for the financing or administration of the plan; show the name and address of the insurer, health plan, or TPA; and indicate to what extent, if any, the benefits under the plan are guaranteed under an insurance or HMO contract. The SPD must indicate any services provided by the insurer, health plan, or TPA under an administrative services only (ASO) contract.
- The SPD must include information about the HIPAA requirements for length of hospital stay for childbirth.
- The SPD must disclose the office of the DOL from which plan participants can obtain assistance regarding their rights under HIPAA.

The ACA requires certain disclosures, and these could be included in an SPD. For instance, plan participants must be informed of their rights in choosing a primary care provider and their right to obtain obstetrical or gynecological care without prior authorization.

The SPD must be distributed to participants within 120 days after the plan is adopted or made effective. An employee who becomes a participant after the adoption or effective date of the plan must receive the SPD within 90 days of becoming a participant.

Summary of Material Modifications (SMM)

A new SPD does not have to be issued every time there is a change in a plan that affects the information in it. In general, even if there are changes, a new SPD must be issued only every five years (if no changes, every 10 years). However, when changes (plan amendments) are made, the employer must publish a Summary of Material Modifications (SMM), which explains the changes and acts as a supplement to the SPD until a revised SPD is distributed.³¹ Or, if it wishes, the employer may at any time publish an updated SPD rather than an SMM.

The Summary of Material Modifications must be distributed to participants within 210 days after the close of the plan year in which the plan amendment is adopted.³² However, if there is a “*material reduction* in covered services or benefits provided under a health plan,” the employer must furnish a SMM no later than 60 days after the adoption of the reduction (or, alternatively, the employer can furnish SMMs at regular intervals of not more than 90 days).

DOL regulations describe a *material reduction* as a modification that “would be considered by the average plan participant to be an important reduction in covered services or benefits under the plan.”³³ This might include the elimination of a benefit, an increase in a deductible or copayment, or the reduction of an HMO service area.

The SPD and the SMM should accurately reflect the terms of the plan as set forth in the Plan Document. But what if there is a discrepancy? Some courts have held that, if the SPD or SMM provides for benefits and the Plan Document does not, the SPD might govern, particularly if an individual can show that she relied on it.³⁴ More recently, however, the Supreme Court held that, in general, ERISA did not intend the SPD or SMM to be formal terms of the plan and so the descriptions of benefits in them cannot be enforced.³⁵ But the Court did allow that discrepancies between the SPD and the Plan Document could support claims for relief. Because of this issue, employers should seek to avoid such discrepancies. One way to do this is to have one document instead of a separate SPD and Plan Document.³⁶

Annual Return (IRS Form 5500)

Each year, the administrator of an ERISA-covered plan must file an Annual Return (Form 5500) with the IRS, which passes on a copy to the DOL. Some plans are exempt (such as those with fewer than 100 participants).³⁷ If multiple plans are combined under one wraparound Plan Document, only one Form 5500 need be filed for all plans. Form 5500 requests information such as plan year, plan number, employer identification number, type of coverage provided, approximate number of plan participants, and financial information. It is due the last day of the seventh month after the close of the plan year, with extensions possible. It is not distributed to plan participants, but it must be provided to a participant within 30 days of a written request.³⁸

An ACA Requirement: Summary of Benefits and Coverage (SBC)

The focus of this lesson is ERISA, but we will also mention a document required of all health plans by the ACA. The plan sponsor or administrator must provide the Summary of Benefits and Coverage to new participants.³⁹ The SBC is intended to make it easier for consumers to compare and contrast health plans, so it must be no more than eight pages, follow certain formatting rules, and contain certain information. It must describe the coverage and cost of the plan, including whether it provides minimum essential coverage and whether it pays less than 60 percent of the total cost of benefits. If there is a

change in a plan that will affect the SBC, the plan must notify participants at least 60 days before the change becomes effective.

The ACA requires large employers to offer health coverage that meets certain standards to full-time employees or pay a penalty. To facilitate the enforcement of this requirement, employers to which it applies must report certain information to the IRS about whether and to what extent they offer coverage (using Forms 1094-C and 1095-C). There is a similar requirement for health insurance issuers (Forms 1094-B and 1095-B).

Requirements for Benefits and Claims

ERISA Requirements for Benefits

ERISA has detailed requirements for the benefits of retirement plans, but not for employee welfare plans (including health plans). However, ERISA-covered health plans must comply with a few rules:

- If a plan provides coverage for dependent natural children, it must provide the same coverage for adoptive children.⁴⁰
- A plan must comply with a court order providing for health coverage for a child of an employee.⁴¹
- A plan may not reduce its coverage of pediatric vaccines below the level it provided as of May 1, 1993.⁴²
- A plan sponsored by an employer with 20 or more employees must offer COBRA continuation coverage if employment ends (as discussed elsewhere in this course).⁴³
- A plan must comply with the requirements of HIPAA.

State Mandates and ACA Requirements

Many states require health plans to provide benefits for certain medical conditions. These **state mandates** apply to fully insured plans but not to self-funded plans. As discussed elsewhere in the course, the ACA imposes a number of benefit requirements on group health plans. For instance, certain preventive services must be provided with no cost-sharing,⁴⁴ there can be no annual or lifetime dollar limits on essential health benefits,⁴⁵ and preexisting condition exclusions and limitations are not allowed.⁴⁶

The ACA's Employer Shared Responsibility provisions have in effect established minimum levels of coverage for the health plans of larger employers. Employers with 50 or more full-time employees or full-time equivalents must offer coverage that meets the following criteria or pay a substantial penalty:

- Minimum essential coverage (MEC) must be offered to at least 95 percent of full-time employees.
- Offered coverage must be affordable, meaning that the employee's premium contribution for self-only coverage does not exceed 9.5 percent of her household income (as adjusted).

- Coverage must have a minimum value—the plan must pay at least 60 percent of the total cost of covered services, and participants' cost-sharing cannot be more than 40 percent of total costs.⁴⁷

ERISA and Retiree Health Benefits

Although it is becoming much less common, some employers provide health coverage to retired employees. What if the employer wants to reduce benefits for retirees, modify their coverage in some other way, or even discontinue it? Does the employer have the right to do so? There have been numerous lawsuits, and court decisions have been mixed, but in recent years a consensus seems to be emerging. Where the employer has, in the Plan Document, explicitly reserved to itself the authority to amend or terminate the plan, the employer's exercise of that right has been upheld.⁴⁸ Where the employer has not done this, or where the language is ambiguous, the courts seek to ascertain the parties' intent when creating the plan. (Did the employer or, in the case of collective bargaining, the parties to the contract intend to create vested benefit rights that cannot thereafter be modified by the employer?) To avoid problems, employers would be advised to keep retiree plans separate from plans for active employees, with separate Plan Documents, reporting, and vendor contracts.

Claim Procedures

For all types of employee benefit plans, the plan must establish a procedure whereby a participant may challenge a denial of her claim for benefits.⁴⁹ In general, a claim should be answered in writing, with an explanation of the reasons for the decision and references to pertinent plan provisions, within 90 days. An appeal of a denied claim may be filed with the plan fiduciary, and it must be answered in writing (again with an explanation of the reasons and reference to plan provisions) within 60 days. In certain cases, the 90-day and 60-day periods can be extended, provided the participant is notified before the end of the initial period.⁵⁰

For health plans (and disability plans) these timeframes are shortened in some cases.⁵¹ For instance, health plans must respond to urgent care claims within 72 hours. There are also ACA requirements; for instance, self-funded plans now must make external review procedures available.⁵²

Litigation

ERISA and Civil Action

Parties involved in an ERISA plan can take civil action (bring a lawsuit) to protect their rights and interests. ERISA authorizes a plan participant, beneficiary, or fiduciary, or the Secretary of DOL to bring a variety of civil actions, including for breach of fiduciary duty.⁵³ A participant, beneficiary, or fiduciary may bring an action to prevent an act or practice that violates ERISA or the terms of the plan, or to force an action.⁵⁴ The DOL has the authority to seek civil penalties for a range of ERISA violations.⁵⁵ A plan may bring action against a fiduciary for breach of fiduciary duty. In this section we will look at several issues that often result in litigation and what the courts have decided.

If a plan brings action against a fiduciary, the fiduciary may have pay monetary damages.⁵⁶ However, plan participants and beneficiaries who sue a plan can generally obtain only **equitable relief**—that is, the court may order the plan to do or not do something, such as pay a benefit—but not monetary damages.⁵⁷ And when ERISA preempts state laws, plaintiffs may not bring action based on those laws, which may allow for monetary damages.⁵⁸ Finally, most (but not all) courts have held that ERISA does

not provide a right to a jury trial. In this legal climate, the courts have struggled to provide meaningful remedies to participants and beneficiaries.

Denied Claims

If a plan participant brings a lawsuit against an employee benefit plan over a denied claim, a question arises about how the court should consider the case:

- Should the court use a **de novo** standard of review? That is, should the court look at the case “as new,” independently review and weigh the evidence, and make its decision accordingly, with no deference to the decision already made by the plan administrator?
- Or should an **arbitrary and capricious** standard be used? Should the court review the evidence but overturn the plan administrator’s decision only if that decision is considered arbitrary and capricious, representing a clearly unreasonable interpretation of the plan?

In *Firestone Tire & Rubber Co. v. Bruch*, the U.S. Supreme Court determined that the de novo standard should be used, *unless* the Plan Document grants to the plan administrator or other fiduciary the discretionary authority to determine eligibility for benefits or to construe the terms of the plan, in which case the arbitrary and capricious standard is to be used.⁵⁹ In other words, a plan sponsor can obtain the standard more favorable to itself simply by adding language to the Plan Document.

However, this language must be quite explicit and specific. In a number of decisions, courts have applied the de novo standard despite broad language in the Plan Document giving authority to the plan administrator. The arbitrary and capricious standard will apply only if the administrator is given the power to construe uncertain terms or if it is stated that the administrator’s eligibility and benefit determinations are to be given deference.⁶⁰ For instance, in *Michael Reese Hospital & Medical Center v. Solo Cup Employee Health Benefit Plan*, even though the Plan Document gave the administrator the authority to control and manage the operation and administration of the plan, it was held that the plan demonstrated insufficient intent to grant to the administrator discretionary authority to determine eligibility or construe the terms of the plan.⁶¹

Another aspect of this issue: Suppose the fiduciary of a self-funded plan is also an officer of the sponsoring employer; since the employer pays benefits out of its own funds, a denial of benefits favors the employer, and the fiduciary can be said to have a conflict of interest. In such cases, courts generally apply a standard of review less deferential to the plan administrator than the arbitrary and capricious standard.⁶² The Supreme Court has held, in essence, that while a conflict of interest alone does not require a *de novo* standard, but it must be weighed as a factor in applying the abuse of discretion standard of review.⁶³ The Court did not provide any guidance on how to weigh factors, but it did provide examples of some relevant factors, such as a history of biased claims administration.

Preemption

In much of the litigation involving ERISA plans, the key issue is whether the state laws on which a civil action is based are preempted by ERISA. Recall that state laws that affect ERISA plans at least somewhat directly tend to be preempted, while those that have general application and impact ERISA plans only tangentially are not. Therefore, if a lawsuit is based on a state law that addresses an integral part of the administration of ERISA plans, the law will generally be preempted and the suit dismissed. We will look at several issues in which this comes into play. To read more about cases that illustrate the issues

discussed below, see “Court Cases: Actions Against ERISA Plans and Preemption” at the end of this lesson.

Utilization Review

If an ERISA plan participant is adversely affected by a utilization review decision by the plan, can she sue the plan under state law? A number of court decisions have held that UR decisions, even when they involve medical decisions, are an integral part of the administration of ERISA plans. Consequently, the relevant state laws cannot be said to affect ERISA plans only tangentially, and so they are preempted by ERISA, and a lawsuit cannot be brought against an ERISA plan based on them. However, in *Pegram v. Herdrich*, the Supreme Court looked more closely at managed care decision-making and decided that ERISA governs decisions about eligibility for benefits but not treatment.⁶⁴

Credentialing

A health plan has a duty to conduct a reasonable investigation of the qualifications and competence of the providers to whom it refers patients.⁶⁵ If an individual is not properly cared for by a provider to which she was referred by an ERISA plan, can she sue the plan on the grounds that the plan did not do its duty in this regard? Most (but not all) courts have held that such claims relate to the administration of an ERISA plan and so are preempted by ERISA.⁶⁶

Vicarious Liability for Medical Malpractice

If a health plan refers a participant to a healthcare provider and the provider commits medical malpractice, is the plan liable? That is, does it have **vicarious liability** (secondary liability for the actions of another party)? Some have claimed that the plan has such liability because the participant reasonably relied on actions or representations of the plan, which “held out” the negligent provider as its employee or agent.⁶⁷ However, it is also argued that ERISA preempts such claims. The courts have disagreed on this question. Some courts have held that such actions do not involve the administration of benefits or the level or quality of benefits provided under plan, but merely allege negligence by a physician and an agency relationship between the physician and plan. Therefore, the claim does not sufficiently relate to the administration of the plan to warrant ERISA preemption.⁶⁸ But other courts have reasoned that such claims concern the delivery of benefits and require the examination of the terms of the plan to determine the quality and quantity of services required of physicians and the relationship between the plan and the physicians. Therefore, the claim does sufficiently relate to the administration of the plan and the laws on which it is based are preempted by ERISA.⁶⁹ However, two U.S. Circuit Court decisions indicate that the trend may be against preemption.

Representations by Health Plans to Providers

It may happen that a healthcare provider performs a service for an individual after checking with a health plan to ascertain whether the individual and the service were covered by the plan. Later, the plan states that the individual or service were not covered. Can the provider sue the plan on the grounds of misrepresentation? Will such an action be preempted by ERISA? It depends. Most courts have held that if the provider is suing on its own behalf, the suit is not preempted by ERISA, but if the provider is acting as the assignee of the plan participant, preemption may occur.

Any Willing Provider Laws

State any willing provider (AWP) statutes generally require a health plan to include any provider in its network who meets the network's general qualifications and is willing to be compensated at its payment rate. If a network has a contract to serve an ERISA plan, is the network exempt because the AWP law is preempted by ERISA? If the AWP law is considered to govern the "business of insurance," then under ERISA's savings clause it is not preempted for fully insured plans. (However, self-funded plans are still preempted, under the deeming clause.) In *Kentucky Association of Health Plans, Inc. v. Miller*, the Supreme Court concluded that Kentucky's AWP law regulated the business of insurance and so was protected by the savings clause from preemption and applied to fully insured ERISA plans.⁷⁰ Moreover, as discussed earlier, the Court in this decision established a new test for determining what constitutes the regulation of insurance, and under this new test, other courts have reversed prior decisions and concluded that some state AWP laws are not preempted by ERISA because they constitute the regulation of insurance.⁷¹

Summary

The federal Employee Retirement Income Security Act (ERISA) regulates employee benefit plans, including health plans. Both fully insured and self-funded plans are covered by ERISA, but they are treated somewhat differently. ERISA preempts state laws that at least somewhat directly affect employee benefit plans. Therefore, ERISA determines what state laws can be applied to employer-sponsored health plans and what legal challenges can be made to their administration. In much of the litigation involving ERISA plans, the key issue is whether the state law or laws on which a civil action is based are preempted by ERISA. If the law addresses what is considered an integral part of the administration of ERISA plans, it will generally be preempted.

ERISA plans must be run by fiduciaries, who have certain duties under ERISA. ERISA plans must fulfill requirements for documentation, reporting, and disclosure, including generating a Plan Document, a Summary Plan Description, a Summary of Material Modifications (SMM), and an Annual Return. (In addition, the ACA requires a Summary of Benefits and Coverage (SBC).) ERISA does not have detailed requirements for health plan benefits, but there are a few rules for benefits and claim and appeal procedures that plans must follow.

Should an Employer Self-Fund an Employee Health Plan?

Advantages of Self-Funding

- **Exemption from state-mandated benefits.** A self-funded health plan is not subject to state laws mandating certain benefits, so the employer has more control over the benefits provided by the plan. (However, now that employers must comply with ACA requirements for minimum essential coverage and minimum value, this advantage is somewhat diminished.)
- **Cost savings.** Although the employer will pay for benefits, it will not pay premiums to an insurer, and costs may be lower. A self-funded plan is not subject to state premium taxes.

Disadvantages

- **Greater risk.** The biggest disadvantage of self-funding is the assumption of greater risk. Claims may be very high in some years.
- **Cash flow and budgeting.** A period of high claims can have an adverse effect on cash flows for which it can be difficult to plan. The budgeting process is complicated because healthcare outlays will vary from year to year and throughout each year.
- **Administrative work and costs.** Even with a claims administrator, the employer must exercise strong oversight. Failure to implement an efficient administrative system can result in a breach of fiduciary duty under ERISA.
- **Employee relations.** When claims are denied under a fully insured plan, employees tend to blame the insurer; under a self-funded plan, they sometimes blame the employer.

Fluctuating Claims

The biggest mistake some employers make when they consider self-funding is to focus entirely on the potential savings from a year of low claims, when they will retain the unspent reserved funds. Obviously, years of low claims can be followed by years of high claims. Self-funding should be approached as a long-term strategy with the expectation that good and bad years should average out, but also with the recognition that the company needs to be prepared for those years where claims may be very high.

Risk

An employer's risk for claims and its ability to rely on stop-loss coverage depends on the quality, clarity, and consistency of the applicable insurance, plan documents, and administrative agreements, as well as the employer's adherence to certain administrative duties and obligations. But while some of the financial risk of self-funding can be addressed by good drafting and administration, it cannot be eliminated. Even with the most carefully drafted plan documents, administrative services agreements, and stop-loss policies, uncontrollable circumstances may result in more claims than expected. Employers need to carefully consider and discuss the financial risks of self-funding internally, as well as with financial advisors, accountants, and insurance brokers.

Other Considerations

Beyond financial risk, we offer here a non-exhaustive list of key issues employers should consider in deciding whether to move to a self-funded arrangement for providing health plan benefits to employees.

Assess the independence and quality of the broker. Employers working with a broker need to ensure that the broker is examining all options objectively and making appropriate recommendations, and be mindful that some brokers may be compensated for directing their customers a certain way and to certain administrators. At a minimum, employers need to be aware of where the broker's affinities lie in weighing any advice.

Examine past experience under the insured plan. With a consultant or broker's assistance, employers should review the experience of the current plan to help to gauge what future claims might look like. Examples of questions to ask include: In the past five years, what was the annual total dollar value of claims? What percentage was due to one-time incidents as opposed to chronic or catastrophic illness? What does the workforce look like on an aggregate basis for purposes of estimating the kinds of expenses plan participants would be more likely to incur? Employers must recognize, of course, that as with financial investments, past experience cannot guarantee future results, but this exercise should help provide some insights into plan experience and help in planning.

Select a claims administrator. Employers must carefully vet the vendor that will process claims for both medical and prescription drug benefits. This vetting process needs to consider a wide range of issues, not just fees. These issues include provider networks, experience and reputation in processing claims and in customer service, transparency in the determination of discounts and rebates (particularly in the case of pharmacy benefit managers), attention to compliance, willingness to coordinate with related service providers, performance metrics, and provision of related services (COBRA administration, disease management, wellness programs, fraud prevention, etc.).

Assess an administrator's services agreement. As part of selecting the TPA, employers also may want to have a look at the services agreement, including provisions such as:

- Employer's role (if any) in the claims/appeals administration process
- Employer's exposure in the event of the claims administrator's negligence
- Indemnity and limitation on liability provisions
- Employer's funding obligation and funding account structure
- Employer's right to audit the administrator's performance
- Subrogation responsibilities and procedures
- Termination provisions—right to terminate, early termination fees, and run-out period for claims processing following termination
- Safeguarding of protected health information and breach response

Examine plan documents. The claims administrator likely will provide a specimen Plan Document/Summary Plan Description. It is critical that this document be reviewed thoroughly to ensure that it contains all of the legally required notices and disclosures and that the plan covers only those expenses the company intends to cover under the plan. In most cases, this will be the employer's responsibility. Note, however, that this review often happens too late in the process, leaving plan administrators unable to provide updated documents in a timely fashion.

Evaluate Stop-Loss Coverage Options. Stop-loss coverage can be triggered by either an individual's high dollar claim or by surpassing a predetermined aggregate claim amount. Employers should assess this coverage with their brokers. Key provisions can include:

- A "Monthly accommodation rider" or "monthly aggregate" provisions would protect the employer if there are a lot of high claims early in a year, before enough money has been set aside to cover them. The stop-loss insurer would advance funds to cover the claims.
- A "terminal liability rider" would protect the company if it were to switch back to being fully insured, by requiring the stop-loss carrier to continue paying claims incurred before the switch but not filed by employees before the year ends.

Be aware of potential discrimination issues. The ACA, the Americans with Disabilities Act, Title VII of the Civil Rights Act, and other federal legislation prohibit discrimination on the basis of race, color, national origin, sex, age, or disability in certain health programs or activities. Employers and their administrators must be careful to avoid violations of these laws. In addition, if a self-funded plan is found to favor highly compensated individuals with respect to eligibility or benefits, there could be adverse tax consequences for those individuals.⁷²

An ERISA Preemption Case in Vermont

A Vermont statute sought to create a resource—an “all-payer claims database”—for insurers, employers, providers, and the state to examine health care utilization, expenditures, and performance.⁷³ To create this database, the law required covered entities (which included self-funded group health plans and any third-party administrators) to provide information such as healthcare costs, prices, quality, utilization, and health insurance claims and enrollment data. Reporting intervals could be as often as monthly, and the failure to comply could expose covered entities to penalties as high as \$2,000 per day and disqualification of administrators from performing services in the state.

Liberty Mutual sponsors a self-funded group health plan which provides health benefits to over 80,000 individuals across the United States. Concerned about the burden Vermont’s law placed on its self-funded group health plan, as well as its fiduciary obligations to maintain the confidentiality of sensitive plan information that could be made available to entities with access to the database, Liberty Mutual challenged Vermont’s law, arguing it was preempted by ERISA.

In holding that ERISA preempted the Vermont law, the U.S. Supreme Court made clear that when core plan administrative functions such as reporting are at stake, state laws like the one in Vermont will not survive ERISA preemption. This was the case despite arguments that the law’s purpose was to further the public good. Additionally, the state’s argument that its law had little, if any, economic impact did little to persuade the Court. It reasoned that employer-sponsored plans should not have to wait until they are burdened by multiple state laws with inconsistent obligations resulting in growing costs before seeking protection under the preemption doctrine.

Court Cases: Actions Against ERISA Plans and Preemption

Utilization Review: *Tolton v. American Biodyne, Inc.*⁷⁴

The co-administrators of the estate of a mental patient who had committed suicide sued, among others, the patient's ERISA plan administrator, the plan's mental health utilization review company, and the psychologists performing utilization review on behalf of the utilization review company, alleging that the plan administrator wrongfully denied benefits for inpatient psychiatric care based on the utilization review company's refusal to authorize such care. Plaintiffs' claims based on state law included wrongful death, improper refusal to authorize benefits, medical malpractice, and insurance bad faith. The U.S. Court of Appeals for the Sixth Circuit held that such claims clearly related to the ERISA plan and so were preempted by ERISA.

Credentialing: *Kearney v. U.S. Healthcare, Inc.*⁷⁵

The plaintiff filed wrongful death and survival claims against an HMO, alleging that the decedent's primary care physician failed to properly diagnose the decedent's condition or to refer the decedent to a hospital for specialized treatment. Plaintiff claimed that the HMO breached its contract to provide needed specialized care by limiting or discouraging the use of specialists, hospitalization, and state of the art diagnostic procedures; misrepresented the primary care physician's competence; and was negligent in selecting and supervising the primary care physician. Plaintiff also claimed that the HMO was vicariously liable for the malpractice of the primary care physician. The court held that a "claim that an operator or administrator of a plan failed to use due care in selecting those with whom it contracted to perform services relates to the manner in which benefits are administered or provided and is preempted."⁷⁶ The court dismissed the plaintiff's claims of negligent selection (as well as the claim for misrepresentation and breach of contract). (However, the court held that plaintiff's claim that the HMO was vicariously liable for the malpractice of the primary care physician on ostensible agency grounds was not preempted.)

Trends in Vicarious Liability for Medical Malpractice

The U.S. Court of Appeals for the Tenth Circuit in the case of *Pacificare of Oklahoma, Inc. v. Burrage* identified four categories of laws that related to an employee benefit plan: laws that regulated the type of benefits or terms of ERISA plans; laws that created reporting, disclosure, funding, or vesting requirements for ERISA plans; laws that provided rules for the calculation of the amount of benefits to be paid under ERISA plans; and laws and common law rules that provided remedies for misconduct growing out of the administration of ERISA plans.⁷⁷ The Tenth Circuit held that a claim that an HMO was vicariously liable for the malpractice of one of its primary care physicians did not involve the administration of benefits or the level or quality of benefits promised by the plan; it merely alleged negligent care by the physician and an agency relationship between the physician and the HMO. The court pointed out that ERISA would not preempt the malpractice claim against the physician and concluded that ERISA should similarly not preempt the vicarious liability claim against the HMO if the HMO held the physician out as its agent. Reference to the ERISA plan to resolve the agency issue did not "implicate the concerns of ERISA preemption."⁷⁸

In *Dukes v. U.S. Healthcare, Inc.*, the U.S. Court of Appeals for the Third Circuit addressed the issue of whether vicarious liability malpractice claims against the defendant HMO could be removed from the state court to federal court.⁷⁹ The lower federal courts had allowed such a removal and then had

dismissed the claims, holding that they were preempted by ERISA. The Third Circuit reversed, noting that not all claims preempted by ERISA were subject to removal. The court held that removal was improper where plaintiffs were merely attacking the quality of the benefits received and were not claiming that the ERISA plans had erroneously withheld benefits that were due or were not seeking to enforce rights under the terms of their respective plans or to clarify rights to future benefits. The court expressly distinguished the situation where the HMO denied benefits in its utilization review role.

Subsequent Circuit Court decisions, however, have held a participant's vicarious liability claims against a plan administrator alleging provider malpractice to be preempted. The court pointed to the participant's claim that the physician's negligent treatment stemmed from a UR determination that treatment was not medically necessary. Additionally, the court reasoned that the administrator and physician's agency relationship existed solely on account of the plan, and malpractice had to be determined by reference to the plan.⁸⁰ Similarly, the Supreme Court held that a state law denial of treatment claim is really a claim for benefits under ERISA when the relationship between HMOs or other managed care organizations and insureds is solely administrative and the plan is merely making a determination about whether the treatment is covered under the terms of the plan.⁸¹ The Court determined such claims are preempted by ERISA, thus rejecting the line of cases that have found "mixed eligibility/treatment" to survive preemption).

Allegation of Misrepresentation by a Health Plan to a Provider: *Memorial Hospital System v. Northbrook Life Insurance Company*

In this influential case, the plaintiff hospital had treated a patient after the hospital had called the employer of the patient's husband and verified that the patient had coverage for the hospital care under a group insurance policy issued and administered by the defendant.⁸² Subsequently, the patient and her husband assigned their benefits to the hospital. Upon the hospital's request for payment, however, the defendant (the insurer) informed the hospital that the patient had not been eligible for benefits on the date of her hospitalization and denied benefits. The hospital subsequently sued, alleging that the employer acted as the defendant's agent in verifying coverage and asserting state law causes of deceptive and unfair trade practices under the Texas insurance code, breach of contract, negligent misrepresentation, and equitable estoppel.

The lower court dismissed the hospital's claims for breach of contract and deceptive and unfair trade practices, finding that those claims were brought by the hospital in a derivative capacity, were related to a claim for benefits under an ERISA plan, and were consequently preempted. The lower court reached the opposite conclusion with respect to the claims of misrepresentation and estoppel, however, finding that those claims were based on the hospital's independent position as a third-party healthcare provider, were not related to the ERISA plan, and thus "were not caught in the broad net of ERISA preemption."⁸³

The hospital appealed the dismissal of the deceptive and unfair trade practices claim, alleging that the lower court had mischaracterized it as a derivative claim for benefits. The U.S. Court of Appeals for the Fifth Circuit agreed, holding that it was merely a Texas codification of the common law doctrine of negligent misrepresentation. Nevertheless, the Fifth Circuit felt "compelled to enter the preemption thicket" and determine whether such cause of action was sufficiently related to the employee benefit plan at issue so as to be preempted.⁸⁴ The court ultimately held that it was not sufficiently related to be preempted.

It is worthwhile, however, to examine in some detail the court's analysis of the policy issues involved in determining whether to preempt a provider's claims based on an erroneous verification of a patient's eligibility and coverage because the same reasoning has been adopted by courts in subsequent cases. Before determining the legal issue of preemption, the court found it necessary to examine the "commercial realities" of the hospital's position as a healthcare provider.

The scenario depicted in Memorial's appeal is one that is reenacted each day across the country. A patient in need of medical care requests admission to a hospital (or seeks treatment from a physician). The costs of medical care are high, and many providers have only limited budget allocations for indigent care and for losses from patient nonpayment. Naturally, the provider wants to know if payment reasonably can be expected. Thus, one of the first steps in accepting a patient for treatment is to determine a financial source for the cost of care to be provided.⁸⁵

The court recognized that it was customary practice for a provider to communicate with plan agents to verify eligibility and coverage and viewed the issue of a mistaken verification of eligibility and coverage by such plan agents as solely one of allocation of risks, namely, whether the risk of nonpayment of the provider's costs "should remain with the provider or be shifted to the insurance company, which through its agents misrepresented to the provider the patient's coverage under the plan," noting that enforcing "the allocation of risks between commercial entities that conduct business in a state is a classically important state interest."⁸⁶

The court remained unconvinced that "either the commercial scenario described above, or its state law vindication, raises any issue concerning the matters that Congress intended to be regulated exclusively by ERISA."⁸⁷ Moreover, the court was unpersuaded that "insulating plan fiduciaries from the consequences of their commercial dealings with third-party providers would further any of ERISA's goals."⁸⁸

According to the court, a one-time recovery by the hospital against the insurer or its putative agent, the employer, would not affect the ongoing administration or obligations of an ERISA plan. In addition, the court found that, if providers were held to have no recourse under either ERISA or state law in situations where there was no coverage under the express terms of the plan but providers had relied on assurances that there was such coverage, providers would be understandably reluctant to accept the risk of nonpayment and might require upfront payment by beneficiaries or impose other inconveniences before treatment would be rendered: "This does not serve, but rather directly defeats, the purpose of Congress in enacting ERISA."⁸⁹

Finally, noting that a healthcare provider did not have independent standing to seek redress under ERISA, the Fifth Circuit found that, although employees had received protection under ERISA in exchange for certain rights to sue under previous federal and state law, the plaintiff and "the countless other healthcare providers were not a party to this bargain."⁹⁰ The court stated that it could not believe that "Congress intended the preemptive scope of ERISA to shield welfare plan fiduciaries from the consequences of their acts toward non-ERISA healthcare providers when a cause of action based on such conduct would not relate to the terms or conditions of a welfare plan, nor affect—or affect only tangentially—the ongoing administration of the plan."⁹¹ The court concluded that ERISA's "preemption provision designed to prevent state interference with federal control of ERISA plans does not require the creation of a fully insulated legal world that excludes these plans from regulation of any purely local transaction."⁹² Other courts have adopted the reasoning of the *Memorial Hospital System* case and

refused to insulate ERISA plans, their sponsors, or their administrators from liability when they mistakenly verify eligibility or coverage to a healthcare provider.⁹³

Sources

¹ 29 U.S.C. §1001 et seq.

² ERISA Sec. 3(1)29 U.S.C. §1002(1) (emphasis added).

³ ERISA Sec. 733(a)(2).

⁴ 29 U.S.C. §1144(a).

⁵ FMC Corp. v. Holliday, 498 U.S. 52, 58 (1990). See also Ingersoll-Rand Co. v. McClendon, 498 U.S. 133 (1990); United Wire, Metal & Mach. Health & Welfare Fund v. Morristown Memorial Hosp., 995 F.2d 1179 (3d Cir.), cert. denied, 114 S. Ct. 382- 83 (1993) (a state law may be preempted even though it has no direct nexus with ERISA plans if its effect is to dictate or restrict the choices of ERISA plans with regard to their benefits, structure, reporting, and administration).

⁶ FMC Corp., 498 U.S. at 58; Shaw, 463 U.S. at 100 n. 21 (some state actions may affect employee benefit plans in “too tenuous, remote, or peripheral a manner to warrant a finding that the law ‘relates to’ the plan”); Mackey v. Lanier Collection Agency & Serv., Inc., 486 U.S. 825 (1988).

⁷ 29 U.S.C. §1144(b)(2)(A).

⁸ 29 U.S.C. §1144(b)(2)(B).

⁹ FMC Corp., *supra*.

¹⁰ Id. (“Our interpretation of the deemer clause makes clear that if a plan is insured, a State may regulate it indirectly through regulation of its insurer and its insurer’s insurance contracts; if the plan is uninsured, the State may not regulate it.”)

¹¹ Pilot Life Insurance Co. v. Dedeaux, 481 U.S. 41, 48-9 (1987).

¹² Kentucky Association of Health Plans, Inc. v. Miller, 123 S.Ct. 1471 (2003).

¹³ N.Y. State Conference of Blue Cross & Blue Shield Plans v. Travelers Ins. Co., 514 U.S. 645 (1995).

¹⁴ Brown v. Granatelli, 897 F.2d 1351 (5th. Cir.), cert. denied, 498 U.S. 848 (1990).

¹⁵ U.S. Department of Labor, Technical Release No. 2014-01. <https://www.dol.gov/agencies/ebsa/employers-and-advisers/guidance/technical-releases/14-01>

¹⁶ American Medical Security, Inc. v. Bartlett, 111 F.3d 358 (4th Cir. 1997).

¹⁷ National Association of Insurance Commissioners, Stop-Loss Insurance Model Act (July 2002) at <http://www.naic.org/store/free/MDL-92.pdf>.

¹⁸ 29 U.S.C §1102(a).

¹⁹ 29 U.S.C. §1104.

²⁰ Id.

²¹ Id.

²² U.S. Department of Labor, Advisory Opinion 2003-04A (Mar. 26, 2003).

²³ 29 U.S.C. §1106.

²⁴ 29 U.S.C. §1002(14).

²⁵ 29 C.F.R. §2550.408b-2.

²⁶ 29 U.S.C. §1102.

²⁷ 45 C.F.R. 164.504(f).

²⁸ 29 U.S.C. §1102.

²⁹ 29 U.S.C. §1022; 29 C.F.R. §2520. 104b-2.

³⁰ 29 C.F.R. §2520.102-3(j).

³¹ 29 U.S.C. §1024.

³² *Id.*

³³ 29 C.F.R. 2520.104b-3(d)(3).

³⁴ *Aiken v. Policy Management Sys. Corp.*, 13 F.3d 138 (4th. Cir. 1993); *Edwards v. State Farm Mut. Auto. Ins. Co.*, 851 F.2d 134 (6th Cir. 1988).

³⁵ *CIGNA Corp. v. Amara*, 131 S. Ct. 1866 (2011).

³⁶ *Board. of Trustees. v. Moore*, No. 14-4048, 2015 U.S. App. LEXIS 14945 (6th Cir. Aug. 25, 2015).

³⁷ 29 U.S.C. §1024.

³⁸ 29 U.S.C §1024(b)(4).

³⁹ PHS §2715; U.S. Department of Labor guidance at: <https://www.dol.gov/agencies/ebsa/laws-and-regulations/laws/affordable-care-act/for-employers-and-advisers/summary-of-benefits>

⁴⁰ 29 U.S.C. §1169.

⁴¹ *Id.*

⁴² *Id.*

⁴³ 29 U.S.C. §§1161–1168.

⁴⁴ PHS §2713.

⁴⁵ PHS §2711.

⁴⁶ PHS §2704.

⁴⁷ IRC §§36B(c)(2)(C)(i) and (ii), respectively.

⁴⁸ *Hughes v. 3M Retiree Medical Plan*, 281 F.3d 786 (8th Cir. 2002); *Boyer v. Douglas Components Corp.*, 986 F.2d 999 (6th Cir. 1993).

⁴⁹ 29 U.S.C. §1133.

⁵⁰ 29 C.F.R. §2560.503-1.

⁵¹ *Id.*

⁵² PHS §2719.

⁵³ 29 U.S.C. §1132(a)(2).

⁵⁴ 29 U.S.C. §1132(a)(3).

⁵⁵ 29 U.S.C. 1132(c)(2).

⁵⁶ 29 U.S.C. § 1109.

⁵⁷ 473 U.S. 134 (1985) and 113 S. Ct. 2063 (1993).

⁵⁸ *Pilot Life Insurance Co. v. Dedeaux*, 481 U.S. 41 (1987).

⁵⁹ *Firestone Tire & Rubber Co. v. Bruch*, 489 U.S. 101 (1989).

⁶⁰ *Fuller v. CBT Corp.*, 905 F.2d 1055 (7th Cir. 1990).

⁶¹ 899 F.2d 639 (7th Cir. 1990).

⁶² *Brown v. Blue Cross & Blue Shield of Ala., Inc.*, 898 F.2d 1556 (11th Cir. 1990), cert. denied sub nom; *Blue Cross & Blue Shield v. Brown*, 498 U.S. 1040 (1991).

⁶³ *Metro. Life Ins. Co v. Glenn*, 128 S. Ct. 2343 (2008).

⁶⁴ 530 US 211 (2000).

⁶⁵ *Johnson v. Misericordia Community Hosp.*, 99 Wis.2d 708, 301 N.W.2d 156 (1981); *Harrell v. Total Health Care, Inc.*, No. W.D. 39809. 1989 W.L. 153066 (Mo. Ct. App. Apr. 25, 1989), aff'd on other grounds, 781 S.W.2d 58 (Mo. 1989); *McClellan v. Health Maintenance Org.*, 413 Pa. Super. 128, 604 A.2d 1053, appeal denied sub nom. *Health Maintenance Org. v. McClellan*, 532 Pa. 664, 616 A.2d 985 (1992).

⁶⁶ *Butler v. Wu*, 853 F.Supp. 125 (D.N.J. 1994); *Elsesser v. Hosp. of the Philadelphia College of Osteopathic Medicine*, 802 F.Supp. 1286 (E.D. Pa. 1992); *Altieri v. CIGNA Dental Health, Inc.*, 753 F.Supp. 61 (D. Conn. 1990).

⁶⁷ *Raglin v. HMO Illinois, Inc.*, 230 Ill. App. 3d 642, 595 N.E.2d 153 (1992); *McClellan v. Health Maintenance Org.*, 413 Pa. Super. 128, 604 A.2d 1053, appeal denied sub nom; *Health Maintenance Org. v. McClellan*, 532 Pa. 664, 616 A.2d 985 (1992); *Boyd v. Albert Einstein Medical Center*, 377 Pa. Super. 609, 547 A.2d 1229 (1988); *Decker v. Saini*, No. 88361768 N.H., 1991 W.L. 277590 (Mich. Cir. Ct. Sept. 17, 1991).

⁶⁸ *Jackson v. Roseman*, 878 F.Supp. 820 (D. Md. 1995); *Kearney v. U.S. Healthcare, Inc.*, 859 F.Supp. 182 (E.D. Pa. 1994); *Haas v. Group Health Plan, Inc.*, 875 F.Supp. 544 (S.D. Ill. 1994); *Dearmas v. Av-Med, Inc.*, 865 F.Supp. 816 (S.D. Fla. 1994); *Paterno v. Albuerno*, 855 F.Supp. 1263 (S.D. Fla. 1994); *Elsesser v. Hosp. of the Philadelphia College of Osteopathic Medicine*, 802 F.Supp. 1286 (E.D. Pa. 1992); *Independence HMO, Inc. v. Smith*, 733 F.Supp. 983 (E.D. Pa. 1990); *Pickett v. CIGNA Healthplan of Texas, Inc.*, 742 F.Supp. 946 (S.D. Tex. 1990); *McClellan v. Health Maintenance Org.*, 413 Pa. Super. 128, 604 A.2d 1053, appeal denied sub nom. *Health Maintenance Org. v. McClellan*, 532 Pa. 664, 616 A.2d 985 (1992).

⁶⁹ *Pomeroy v. John Hopkins Medical Servs., Inc.*, 868 F.Supp. 110 (D. Md. 1994); *Nealy v. U.S. Healthcare HMO*, 844 F.Supp. 966 (S.D.N.Y. 1994); *Butler v. Wu*, 853 F.Supp. 125 (D.N.J. 1994); *Ricci v. Gooberman*, 840 F.Supp. 316 (D.N.J. 1993); *Altieri v. CIGNA Dental Health, Inc.*, 753 F.Supp. 61 (D. Conn. 1990).

⁷⁰ 538 U.S. 329 (2003).

⁷¹ *Prudential Ins. Co. of Am. v. Nat'l Park Med. Ctr., Inc.*, 413 F.3d 897 (8th Cir. 2005).

⁷² IRC §105(h).

⁷³ *Gobeille v. Liberty Mutual Ins. Co.*, 136 S.Ct. 496 (2015).

⁷⁴ 48 F.3d 937 (6th Cir. 1995).

⁷⁵ 859 F.Supp. 182 (E.D. Pa. 1994).

⁷⁶ *Id.* at 187.

⁷⁷ 59 F.3d 151 (10th Cir. 1995).

⁷⁸ *Id.* at 155.

⁷⁹ 57 F.3d 350 (3d Cir. 1995), cert. denied sub nom. *Healthcare Inc. v. Dukes*, 116 S. Ct. 546 (1995).

⁸⁰ *Jass v Prudential Health Care Plan*, 88 F3d 1482 (7th Cir. 1996). See also *Hull v Fallon*, 188 F3d 939 (8th Cir. 1999).

⁸¹ *Aetna Health Inc. v. Davila*, 542 US 200 (2004).

⁸² 904 F.2d 236 (5th Cir. 1990), reh'g denied, No. 89-2513, 1990 U.S. App. LEXIS 13227 (5th Cir. July 16, 1990).

⁸³ *Id.* at 239.

⁸⁴ *Id.* at 244.

⁸⁵ *Id.* at 246.

⁸⁶ *Id.*

⁸⁷ *Id.* at 247.

⁸⁸ *Id.*

⁸⁹ *Id.* at 247–48.

⁹⁰ *Id.* at 249.

⁹¹ *Id.* at 250.

⁹² *Id.*

⁹³ *Toomajian v. Insight Global, Inc.*, 2014 WL 3650232 (D. Mass. 2014); *Hall v. NewMarket Corp.*, 2009 WL 5217026 (S.D. Miss. 2009); *S. Alaska Carpenters Health & Sec. Trust Fund v. Jones*, 2008 WL 612880 (Alaska 2008); *Omega Hosp., LLC v. Aetna Life Ins. Co.*, 2008 WL 4747864 (E.D. La. 2008); *The Meadows v. Employers Health Ins.*, 47 F.3d 1006 (9th Cir. 1995); *Lordmann Enters., Inc. v. Equicor, Inc.*, 32 F.3d 1529 (11th Cir. 1994), cert. denied sub nom. *Equicor, Inc. v. Lordmann Enters.*, 116 S. Ct. 335 (1995); *Hospice of Metro Denver, Inc. v. Group Health Ins. of Okla., Inc.*, 944 F.2d 752 (10th Cir. 1991); *Rehabilitation Inst. v. Group Adm’rs. Ltd.*, 844 F.Supp. 1275 (N.D. Ill. 1994); *Hoag Memorial Hosp. v. Health Plan Adm’rs*, 820 F.Supp. 1232 (C.D. Cal. 1993); *Gaston Memorial Hosp. Home Health Servs., Inc. v. Bridgestone/ Firestone, Inc.*, 830 F.Supp. 287, 830 (W.D.N.C. 1993); *Suburban Hosp., Inc. v. Sampson*, 807 F.Supp. 31 (D. Md. 1992).

16 Wellness Programs, Part I

Lesson Objectives

After completing this lesson, you should be able to:

- State what wellness programs are subject to HIPAA nondiscrimination rules, as modified by the ACA.
- Explain when incentives offered by wellness programs do and do not violate these rules.
- Discuss the impact of HIPAA and HITECH privacy and security rules on wellness programs.

Many health plans and employers sponsor wellness programs, which are designed to promote the health of individuals in various ways (and result in lower healthcare costs). They make available and encourage participation in such programs as:

- weight-loss, smoking cessation, and fitness;
- screening for health problems, such as health risk assessments (HRAs); and
- disease management for those with chronic conditions (such as diabetes).

Wellness programs are subject to various laws and regulations.

- In part 1 of this lesson we will discuss HIPAA and ACA rules on nondiscrimination and privacy, along with privacy rules of the Health Information Technology for Economic and Clinical Health (HITECH) Act.
- In part 2 we will examine other laws, including the Genetic Information Nondiscrimination Act (GINA) and the Americans with Disabilities Act (ADA).

HIPAA/ACA Nondiscrimination Rules and Wellness Programs

HIPAA prohibits group health plans from discriminating against individuals based on health factors with regard to:

- Eligibility—including enrollment, effective date of coverage, and waiting periods
- Benefits—including premiums, premium contributions, cost-sharing, and covered expenses

For purposes of HIPAA nondiscrimination rules, “health factors” are defined broadly to include:

- Health status
- Physical or mental condition
- Receipt of health care

- Medical history
- Claims experience
- Genetic information
- Disability
- Evidence of insurability (including factors such as dangerous occupations or activities)
- Plus any other health factor stipulated by the Secretary of HHS.

How does this affect wellness programs? If a wellness program is considered a group health plan or part of a group health plan, it is subject to HIPAA nondiscrimination rules. The focus of HIPAA nondiscrimination rules for wellness programs is incentives. Wellness programs often offer employees or members incentives to participate in certain activities or meet certain criteria. Those who do so may pay a lower premium for health coverage, be charged lower cost-sharing, or receive some other reward. The rules primarily seek to ensure that such incentives do not unfairly discriminate against individuals based on their health.

HIPAA wellness program rules were issued as regulations in 2006, but in 2009 the ACA codified these rules into statute, while also making some significant changes. In 2013, the Departments of Treasury, Labor, and Health and Human Services finalized regulations implementing the ACA's wellness program provisions, which are effective for plan years beginning on and after January 1, 2014. In the discussion that follows, we speak of HIPAA rules as modified by the ACA. (However, it is important to keep in mind that other laws and regulations may also apply to wellness programs, and a program in compliance with HIPAA/ACA may violate another law, as we will discuss.)

Application of Nondiscrimination Rules

We have said that a wellness program that is considered a group health plan or part of a group health plan is subject to HIPAA nondiscrimination rules. So to determine whether these rules apply to a particular program, we must ask three questions:

- Is the program considered a wellness program under HIPAA?
- Is the program part of a group health plan?
- Is the program itself considered a group health plan?

Wellness programs take many forms and may be called by other names. Generally speaking, if a plan or program covers such things as disease management, smoking cessation, or a diagnostic testing program for early detection of health problems, it will be considered a wellness program.¹

In determining if a wellness program is part of a group health plan, a key question is: Do the program's incentives relate to the health plan? If not, HIPAA rules likely do not apply.²

Example: An employer institutes a simple wellness program that offers every employee a \$50 coupon to join a local gym. Employees' decisions to join or not have no impact on their eligibility for the em-

employer's group health plan or their premiums or cost-sharing. This program is not part of the health plan.

Example: *Another employer offers employees a 10 percent discount on their group health plan premiums if they join a local gym. This program would likely be considered part of the health plan.*

A wellness program may itself be considered a group health plan (and subject to the rules) if it provides medical services. For instance, employer-sponsored stand-alone biometric screening programs and flu shot programs may be considered group health plans, even if they are not a part of the comprehensive health plan sponsored by the employer.³

Discrimination and Incentives

If a wellness program is subject to HIPAA nondiscrimination rules, we must next ask: Does the program unlawfully discriminate against individuals based on a health factor? In general, if a program offers an incentive (a reward or penalty) related to health coverage to individuals who meet a standard related to a health factor, it discriminates based on health.

Examples: *Four employer-sponsored group health plans include wellness programs. Plan A extends eligibility for health coverage only to employees who meet a certain health standard. Plan B extends coverage to all but offers lower premiums to those who meet this standard. Plan C charges those who do not meet the standard a premium surcharge. Plan D offers those who meet the standard lower copayments. These are all cases of impermissible discrimination based on health.*

However, while a wellness program *may not* generally offer incentives based on health standards, it *may* offer incentives based on voluntary participation.

- **Examples of health standards:** A person's cholesterol, weight, or blood pressure must be at certain levels for her to win the incentive (not permissible).
- **Examples of participation:** A person must join a gym, attend smoking cessation classes, or complete a health risk assessment (HRA) (without questions on family medical history) to receive the incentive (permissible).

Such participatory wellness programs must be available to all similarly situated individuals, regardless of health status.

Example: *Mainline, Inc. establishes a participatory wellness program. Incentives are offered to all employees who complete a 10-question health risk assessment. This does not violate HIPAA/ACA nondiscrimination rules.*

Example: *Sideline, Inc. also has a wellness program, and employees who achieve a cholesterol level under 200 get a 10 percent reduction in their group health plan premium. This incentive is based not on participation, but on health standards, and so violates the rules.*

Moreover, a wellness program can offer incentives based on health standards in two circumstances:

- The discrimination is benign.

- The program satisfies additional rules (under what is called the **Wellness Program Exception** to HIPAA nondiscrimination rules).

Benign discrimination does not disadvantage those in ill health—on the contrary, it favors them. An incentive is offered only to individuals suffering from specific adverse health factors—for instance, those with diabetes, heart disease, or cancer. Benign discrimination is permitted.

***Examples:** For those diagnosed with cancer, copayments are waived for specialized drugs. Diabetics are offered \$300 toward a gym membership if they complete specified blood tests twice annually.*

The Wellness Program Exception

If a wellness program offers incentives based on health standards, it will still be compliant with HIPAA/ACA nondiscrimination rules if it meets the requirements of the Wellness Program Exception.

- These requirements are designed to ensure that incentives are made available to all, by giving those who cannot meet a health standard an alternative standard that they can reasonably expect to meet.
- Rules also limit the amount of incentives and impose other requirements.

The Wellness Program Exception distinguishes two types of programs:

- **Participatory programs** (discussed above)
- **Health-contingent programs** (with incentives based on health standards)

Health-contingent programs are the focus of the Wellness Program Exception and are divided into two subtypes:

- **Activity-only programs** require an individual to perform or complete an activity related to a health factor to obtain an incentive. But they do not require her to attain or maintain a specific health outcome.
- **Outcome-based programs** require an individual to attain or maintain a specific health outcome to obtain an incentive.

How is an activity-only program different from the participation-based programs we discussed above? Distinguishing the two can be challenging.

***Examples:** To obtain an incentive, an individual must attend a monthly nutrition seminar—this is participatory. But if the individual must participate in a dieting program, this is likely activity-only.*

***Examples:** To receive an incentive, an individual must join a fitness center—nothing more. This is a participatory program. But suppose the individual must participate in an exercise program at the fitness center—this will likely be considered an activity-only program, even if the program does not require that participants achieve any outcomes while participating in the exercise program.*

Outcome-based programs are typically structured in two tiers:

- In **tier one** if the individual meets a certain standard (a measurement, test, or screening), she earns the incentive. If she does not, she goes to tier two.
- In **tier two** the individual can take additional steps to earn the same incentive. This might be meeting with a health coach, taking a health or fitness course, participating in a walking or exercise program, or complying with a healthcare provider's plan of care.

Wellness Program Exception Requirements

To offer health-based incentives under the Wellness Program Exception, a program must meet requirements in these areas:

- Reasonable design
- Uniform availability (alternatives)
- Incentive limitation
- Annual qualification

To meet the reasonable design requirement, a program must:

- provide a reasonable chance of improving health or preventing disease,
- not be overly burdensome,
- not be a subterfuge for health factor discrimination, and
- not be highly suspect in the method chosen.

The determination of whether these criteria are met is based on the facts and circumstances in each case.

Uniform Availability (Alternatives)

Programs must make an incentive available to all by providing a reasonable alternative to the health standard that must be met to obtain it.

- **Activity-only programs** must provide an alternative standard or a waiver for individuals who have difficulty meeting the regular standard because of a medical condition, or because it is medically inadvisable for them to do so.
- **Outcome-based programs** must provide a reasonable alternative to any individual who does not meet an initial standard based on a measurement, test, or screening related to a health factor.

The regulations provide flexibility on reasonable alternatives, but there are some rules:

- For treatment of addiction (tobacco, food, etc.), the individual should be allowed multiple attempts at meeting alternatives.
- For education programs, the wellness program must pay for and provide the alternative education program or assist the individual in finding an alternative program.
- For diet programs, the wellness program must pay for the alternative program but not the cost of food.
- The time commitments required to meet the alternative must be reasonable.
- The recommendations of an individual's personal physician must be taken into account if the physician determines that the wellness standard is medically inappropriate.

What if the reasonable alternative a program offers to a health standard is another health standard? The program must go through the same analysis it did for the initial standard, and it may need to make a second reasonable alternative available. This is true for both activity-only and outcome-based standards. In addition, for outcome-based standards only:

- If the alternative is a different level of the same standard, the program must provide additional time to comply, taking into account the individual's circumstances.
- The program must allow the individual to comply with his physician's recommendations as a second alternative, but only if the physician joins in the request. Individuals can inject their physician's recommendations at any time, and physicians can adjust recommendations any time, as long as medically appropriate.

Does a program have to automatically grant an individual an alternative standard or waiver?

- For activity-only programs, no. The employee can be asked to verify the need for the alternative.
- An outcome-based program must offer the alternative to any individual who does not meet the initial standard.

Must a program create a standardized approach in crafting alternatives? No. A program can take either a standardized or an individualized approach. But it is helpful to have some alternatives in mind when the program is being rolled out.

Materials describing the program must disclose the possibility of an alternative or a waiver to a standard. The regulations provide sample language for a notice:

"Your health plan is committed to helping you achieve your best health. Rewards for participating in a wellness program are available to all employees. If you think you might be unable to meet a standard for a reward under this wellness program, you might qualify for an opportunity to earn the same reward by different means. Contact us at [insert contact information] and we will work with you (and, if you wish, with your doctor) to find a wellness program with the same reward that is right for you in light of your health status."

Incentive Limitation

The total dollar amount of incentives may not exceed 30 percent of the health plan premium (employer and employee share combined).⁴ An exception: If a program is designed to prevent or reduce tobacco use, incentives may be as high as 50 percent of the premium.

Example: *A wellness program offers Mary a reduction in her health plan premium equal to 10 percent of the total premium if she reaches a cholesterol target, and another 10 percent if she achieves a blood pressure target. The combined amount, 20 percent, is under the 30 percent cap.*

Now suppose the reduction is 20 percent for each, for a total of 40 percent. This exceeds the 30 percent cap and is not permissible.

Now let's say the total is 40 percent, but a tobacco reduction program accounts for 20 percent. This is permissible, because the cap rises to 50 percent.

Note that these percentage caps apply only to the health-contingent incentives governed by the Wellness Program Exception, not to participatory incentives.

Example: *A wellness program offers two health-contingent incentives each worth 10 percent of the health plan premium, but there is also a 15 percent reduction for all employees who join a gym. This totals 35 percent, above the 30 percent cap, but the 15 percent does not count because it is participatory, not health-contingent.*

We have learned that, under HIPAA/ACA nondiscrimination rules, if tobacco reduction is included in a wellness program, incentives may be as high as 50 percent of the health plan premium. And for participatory (not health-contingent) incentives, there is no incentive cap. But note that if the Americans with Disabilities Act (ADA) applies—essentially, if incentives are offered for employees to respond to disability-related inquiries (such as an HRA) or undergo a medical exam (such as biometric screening)—an ADA limit of 30 percent applies, even if tobacco reduction is included or if the incentives are participatory. We will study ADA rules in Part 2 of this lesson.

Finally, under the Wellness Program Exception, a program must give eligible individuals the opportunity to qualify for an incentive at least once a year.

Caution: Other Laws May Apply

If a program satisfies the HIPAA/ACA wellness program rules, or if it is not subject to those rules, this does not necessarily mean the program is lawful. Various other laws may apply. These include the Genetic Information Nondiscrimination Act (GINA), the Americans with Disabilities Act (ADA), Equal Employment Opportunity Commission (EEOC) regulations, certain state laws (often referred to as lifestyle laws), the Internal Revenue Code, HIPAA Privacy Rules, and the Age Discrimination in Employment Act (ADEA).

Example: *An employer adopts a policy that any employee who smokes will be fired. This does not relate to the group health plan, so the HIPAA/ACA wellness program rules do not apply. But other laws, such as state laws prohibiting discrimination against employees who participate in lawful activities, may apply.*

One issue that has arisen in relation to wellness plans is “disparate impact.” EEOC rules prohibit unintentional discrimination—that is, even if policies or practices are intended to be neutral, an employer may not use them if they have a disparate impact on employees in legally protected groups. It is unclear whether some wellness programs violate this rule. But employee representatives have raised concerns that outcome-based wellness programs could have an unlawful disparate impact by making health coverage more expensive for some legally protected groups, such as individuals with disabilities, racial minorities, women, and older workers. AARP has recently challenged wellness programs on the grounds that they discriminate against older workers because failure to participate can result in substantially higher health plan premiums.

Privacy and Security

HIPAA

HIPAA privacy and security rules are designed to protect the health information of individuals, including participants in certain employer-sponsored wellness programs. HIPAA basically establishes a firewall to prevent an employee’s individual health information gathered as part of a wellness program from being accessed by the employer. An employer can use such information for plan administration purposes if a number of requirements are met. As a general rule, an employer may obtain aggregated health information about its workforce but not health information about individual employees.

Employers that are actively involved in the operation of their wellness programs need to, among other things, maintain health data in secure files separate from the more general records normally maintained by human resource departments. Employers that contract with a third-party administrator to manage a wellness program need to ensure that the TPA has agreed to the terms of a HIPAA-compliant **business associate** agreement. These agreements require the TPA (as the employer’s business associate) to take a number of steps to safeguard protected health information, including conducting security risk assessments, adopting written policies, and training its workforce in these policies. Should a security breach occur, business associates must follow a series of rules, including notifying the individuals impacted.

Why are HIPAA privacy rules important? They give employees the assurance that information will not be shared with the employer or others. Few people would like their employer (or their fellow employees) to know, for example, that they have three to four drinks every evening, or that they have a sexually transmitted disease. Privacy protections encourage employees to honestly answer questions about lifestyle and health status and promote participation in a wellness program.

Small Employers and Health Risk Assessments (HRAs)

The information provided by employees in HRAs is protected by HIPAA privacy and security rules. In most cases employers can access aggregated data about their employees as a group, which can help them better target wellness benefits and programs. However, in firms with few employees, the identities of individuals may be easily discerned from aggregated data; such employers may need to rely on third-party specialists that aggregate data from other firms of similar size and engaged in similar businesses. They may also need to have more stringent policies and training to ensure that information is not accessed or used for an improper purpose.

The HITECH Act

The **Health Information Technology for Economic and Clinical Health (HITECH) Act** places increased responsibilities on TPAs that establish and manage wellness programs for employers. We mentioned above that such TPAs have been subject to HIPAA privacy and security rules as a matter of contract, through their business associate agreements; under HITECH, they are now directly liable for violations of many of the HIPAA rules that apply to health plans.

HITECH Security Breach Notification Rules: A Quick Summary

What is a security breach? Subject to certain limited exceptions, a security breach occurs when there is access, acquisition, use, or disclosure of unsecured protected health information (PHI).

What must be done if there is a breach? Individuals whose information has been compromised by the breach must be notified. This notification must take place without unreasonable delay, but not later than 60 days following the discovery of the breach. In addition, the Department of Health and Human Services (HHS) [or in some cases the Federal Trade Commission (FTC)] must be notified as well as the media.

Are there any exceptions? There are exceptions, such as if the PHI could not be retained by the individual who acquired it. Also, notification may not be required if it can be shown after consideration of a number of factors that there was a low probability of compromise.

Summary

HIPAA nondiscrimination rules, as modified by the ACA, apply to wellness programs that are part of group health plans or considered group health plans themselves. Many of these programs offer incentives to employees to participate in certain activities or achieve specified health standards (such as cholesterol levels below a threshold). The rules seek primarily to ensure that these incentives do not unfairly disadvantage individuals with health problems. Programs must meet various requirements, such as offering alternatives to those who are unable to meet a health-based standard to earn an incentive.

HIPAA privacy and security rules (supplemented by the HITECH Act) protect the individual health information collected and used by wellness programs from access by employers and others.

Even if a wellness program complies with HIPAA/ACA rules, it may violate other laws, as we will discuss in the second part of this lesson.

Sources

¹ U.S. Department of Labor (DOL), “Field Assistance Bulletin Number 2008-02,” February 14, 2008.

² Ibid.

³ Ibid.

⁴ Affordable Care Act, Section 1201, Subtitle C (Amendment to the Public Health Service Act, Section 2705).

16 Wellness Programs, Part II

Lesson Objectives

After completing this lesson, you should be able to:

- List the main actions related to employment and health coverage that the Genetic Information Nondiscrimination Act (GINA) prohibits.
- Summarize GINA rules for health risk assessments (HRAs), including spousal participation and inducements to participate.
- Explain the rules of the Americans with Disabilities Act (ADA) for wellness programs with regard to reasonable accommodation and disability-related inquiries and medical examinations.
- State what wellness programs are subject to ERISA and what the main advantages and requirements of ERISA are for such programs.
- State what state lifestyle discrimination laws prohibit.

In Part 1 of this lesson we looked at the impact on wellness programs of HIPAA nondiscrimination rules, as modified by the ACA, as well as HIPAA and HITECH privacy and security rules. In Part 2 we will consider:

- The Genetic Information Nondiscrimination Act of 2008 (GINA)
- The Americans with Disabilities Act of 1990 (ADA)
- The Employee Retirement Income Security Act of 1974 (ERISA)
- State lifestyle discrimination laws

(Note that, as mentioned in Part 1, other laws—such as Title VII of the Civil Rights Act and the Age Discrimination in Employment Act—may also apply to wellness programs.)

In 2016 the Equal Employment Opportunity Commission (EEOC) finalized regulations under GINA¹ and the ADA² to address certain aspects of employer-sponsored wellness programs. The EEOC's rulemaking was particularly concerned with whether employees' participation in disability-related inquiries and medical examinations by employers was voluntary, and with inducements for employees' spouses or other family members to disclose current or past medical conditions on health risk assessments (HRAs). The EEOC is also attempting to align its interpretation of the ADA and GINA with the regulation of wellness programs under the ACA and HIPAA. The final ADA and GINA regulations were issued along with FAQs concerning ADA³ and FAQs concerning GINA⁴. Our discussion in this lesson will incorporate these regulations.

The Genetic Information Nondiscrimination Act (GINA)

Genetic science is increasingly used in treating disease, and this has both benefits and risks.

- Testing for a genetic propensity toward a particular disease can enable individuals to take actions and adopt lifestyles that lower their risk of actually developing the disease.
- For those suffering from a disease, the effectiveness of treatment can be enhanced by tailoring it to their genetic makeup.
- However, such information might be used against people as they seek employment or health coverage.

The **Genetic Information Nondiscrimination Act (GINA)** prohibits discrimination based on genetic information and restrict acquisition and disclosure of such information.⁵ It was intended to allay fears that undergoing genetic testing or participating in research studies might adversely affect a person's employment or health coverage. The EEOC defines "genetic information" as information about an individual's genetic tests and the genetic tests of an individual's family members, as well as information about the manifestation of a disease or disorder in an individual's family members (family medical history).⁶

Two parts of GINA have a significant impact on wellness programs:

- Title I—focused on preventing discrimination in health coverage
- Title II—focused on preventing discrimination in the employment setting

GINA Title I sets forth rules relating to genetic discrimination in health coverage. The regulatory requirements are administered jointly by the U.S. Departments of Health and Human Services (HHS), Labor, and the Treasury. In general, the provisions of Title I prohibit genetic discrimination in group health plans, individual insurance products, and Medicare supplement (Medigap) markets. Under Title I rules:

- Eligibility for insurance coverage or benefits cannot be based on genetic information.
- Premiums may not be set based on genetic information.
- The ability of health plans to request or require an individual to undergo genetic testing or to access her genetic information is limited and restricted.

In addition, as directed by the GINA statute, HHS revised the HIPAA privacy regulations to clarify that genetic information is "protected health information." Health plans covered under these HIPAA rules may not use or disclose genetic information for underwriting purposes, which include determining eligibility, setting premiums, applying any preexisting condition exclusions, and any other activities related to the creation, renewal, or replacement of a contract of health insurance or health benefits.

Under GINA Title I, a health plan may not:

- Collect genetic information for underwriting purposes

- Collect genetic information before or in connection with plan enrollment
- Vary premiums or contributions for an entire group based on genetic information of an individual

GINA Title II protects job applicants, current and former employees, labor union members, and apprentices and trainees from discrimination based on their genetic information. It prohibits employers and other entities (such as third-party administrators of wellness programs) from requesting, requiring, purchasing, using, or disclosing genetic information except in limited circumstances. The EEOC is charged with the interpretation and administration of Title II and issues the rules and guidelines for this section of the law.

In this section we will discuss some of the most significant issues that arise in relation to GINA and wellness programs:

- Information for medical treatment
- Health risk assessments (HRAs)
- Spousal and Dependent Participation in HRAs
- Disease management
- Confidentiality and security

Information for Medical Treatment

In narrow circumstances, an insurer or health plan is permitted to request, collect, and use genetic information to determine the medical necessity of a treatment and the best possible approaches of treatment.

Example: *Sally suspects she may be at risk for breast cancer, since her mother died of it. She is advised by her physician to be tested for a genetic trait highly correlated with the disease. The test indicates the presence of the gene, Sally is found to have symptoms, and a treatment is recommended. Her health plan may request the test results as evidence of the medical appropriateness of the recommended treatment.*

Health Risk Assessments (HRAs)

Regulations implementing GINA Titles I and II clarify when and how health risk assessments (HRAs) can be used.⁷

- HRAs requesting genetic information may be given to an individual only after she is enrolled in a health plan or product, not before.
- Completion of an HRA must be completely voluntary.
- No financial or other incentive can be offered for providing genetic information. There may be an incentive for answering nongenetic questions on an HRA, provided it is made clear both that

the incentive will be granted even if the individual does not answer the genetic questions and which questions are considered genetic. (However, for nongenetic questions the Americans with Disabilities Act (ADA)—discussed below—may apply.)

Example: *ABC Company offers \$100 to employees who complete an HRA. The HRA contains 100 questions, the last 20 concerning genetic information. The instructions make clear that the employee can choose not to respond to this section and will still receive the payment.*

An individual's family medical history can be an important predictor of the diseases she has a genetic propensity for. That is, it is more likely that an individual will develop certain diseases such as diabetes, cancer, and possibly obesity if a parent or other close relative suffered from the disease. Many wellness programs wish to have this kind of information so that they can help participants avoid a disease, mitigate its onset, or control it once it develops. But under GINA, family medical history is considered genetic information. So if an HRA contains questions on family medical history (such as "Did your father have heart disease?" or "Do you have a family history of colon cancer?"), it is requesting genetic information.

If an employee's spouse participates in an HRA and is asked for genetic information about himself or herself, this is considered family medical history (and therefore genetic information) for the employee. However, information about a spouse's height, weight, exercise regimes, present or past tobacco use, or nicotine levels in the blood is not considered genetic information.

Spousal and Dependent Participation in HRAs

Under EEOC regulations for GINA Title II:

- **For an employee's children**, neither genetic information nor information about manifestation of diseases or disorders may be collected.
- **For an employee's spouse**, genetic information cannot be collected, but information on manifestation of diseases or disorders can be requested as part of an HRA.

Moreover, employers may offer limited inducements for spouses to provide information on manifestation of diseases or disorders, if certain requirements are met. The rules on such inducements are effective for plan years beginning on or after January 1, 2017. (For actions occurring before this date, the EEOC may consider earlier regulations and interpretations.) They apply to all employer-sponsored wellness programs that request genetic information, whether part of a group health plan or not.

A wellness program asking spouses about manifestation of disease or disorder must meet the following requirements:

- The program must be reasonably designed.
- The employer must obtain written authorization from the spouse.
- The incentive may not exceed a certain level.

The wellness program must have a reasonable chance of improving the health of, or preventing disease in, participating individuals, and must not be overly burdensome, a subterfuge for violating Title II of

GINA or other laws prohibiting employment discrimination, or highly suspect in the method chosen to promote health or prevent disease.

The spouse must provide prior knowing, voluntary, and written authorization, and that authorization must describe (among other things) confidentiality protections and restrictions on the disclosure of genetic information (discussed below).

The inducement for a spouse to provide information about manifestation of disease or disorder can be no greater than 30 percent of the total cost (employee and employer contribution) for an employee's self-only health coverage.

Employers may not deny access to health insurance or any package of health benefits to an employee and/or his or her family members, or retaliate against an employee, based on a spouse's refusal to provide information about his or her manifestation of disease or disorder to an employer-sponsored wellness program.

Disease Management

Some wellness programs include disease management (DM) programs, which offer participants education about a disease and recommend actions to control it or limit its progress.

- Wellness programs can use aggregated data, including family history and other genetic information, for the employee population as a whole to identify common needs and address them.
- But it cannot use such information to identify individual employees.

***Example:** Aggregated data from HRAs indicate that many of ABC's workers have a genetic propensity to diabetes, and in response the company establishes a disease management program for that disease. This is permissible. But ABC cannot use individual genetic information to identify employees with a higher likelihood of developing diabetes so that it can encourage them to join the DM program.*

Confidentiality and Security

GINA Title II confidentiality and security protections include the following:

- Genetic information must be maintained in medical files (including electronic files) that are separate from personnel files.
- Authorization is required for certain disclosures.
- Information is available only for purposes of healthcare or genetic services and is not disclosed to the employer except in aggregate terms.
- Individually identifiable genetic information may be provided only to the individual receiving services and the licensed healthcare professionals or board certified genetic counselors involved in providing those services.

The Americans with Disabilities Act (ADA)

The **Americans with Disabilities Act (ADA)** prohibits discrimination against a qualified individual on the basis of a disability. The ADA is fairly broad in scope. It covers employee hiring, advancement, discharge, and compensation, and it applies to job procedures and other terms, conditions, and privileges of employment.⁸ The act was first enacted in 1990 but amended in 2008 to further broaden its scope and make it easier for individuals to seek protection under its provisions. The provisions of the ADA, like those of GINA, are administered and interpreted by the EEOC.

Who is considered disabled under the ADA? Again, the scope of the act is fairly broad. A person with a disability means any person who has a physical or mental impairment that substantially limits one or more major life activities.

- Major life activities include but are not limited to walking, seeing, hearing, and speaking.
- Impairments include loss of limb, speech impediments, muscular dystrophy, visual impairments, and learning disabilities, but also diseases such as cancer, cerebral palsy, and diabetes.
- An individual with a record of such a limitation or who is regarded as having such impairment is also considered disabled.

The ADA includes two provisions that can have an impact on wellness programs:

- The reasonable accommodation requirement
- Disability-related inquiries and medical examinations

Reasonable Accommodation

In wellness programs, the standards that must be met or the activities that must be performed to earn an incentive are designed to be attainable by most working people, but they may not be possible for a person with a disability. If this is the case, the program must make reasonable accommodation for disabled employees. Typically, this involves working with an employee to develop a reasonable alternative that satisfies the goals of both the individual and the program.⁹

Example: *Rock Creek Hardware's wellness program offers partial payment of gym fees for employees who participate in an organized exercise program. Because of a physical disability, Melanie cannot participate in most such programs, but the wellness program manager works with her to find a water aerobics class she can take. This meets the goals of the program while taking Melanie's disability into account.*

As we learned in Part 1 of this lesson, wellness programs subject to HIPAA/ACA rules that have incentives based on health standards (health-contingent programs) must offer a reasonable alternative. A program that complies with HIPAA/ACA rules in this area generally meets ADA requirements. But remember that this HIPAA/ACA rule applies only to health-contingent programs, not participatory programs. A participatory program must meet the ADA requirement—that is, if a disabled person has difficulty meeting a participation-based criterion to earn an incentive, the program must offer a reasonable accommodation.

Disability-Related Inquiries and Medical Examinations

The ADA restricts disability-related inquiries and medical examinations. They are prohibited **before** an offer of employment. They are permitted (as a general rule) **after** a conditional offer of employment if they are:

- job-related and consistent with business necessity and
- voluntary—that is, employees are neither required to answer nor penalized for not doing so.

Note that this component of the ADA offers protection to all employees, disabled or not. A person—with or without a disability—cannot be required to undergo a medical exam before being hired.

What is a disability-related inquiry? It is a question or series of questions likely to elicit information about a disability, such as:

- How often do you feel depressed?
- Do you suffer from asthma?
- Have you been diagnosed with cancer?
- Do you monitor your blood sugar level on a regular basis?
- How many prescription drugs are you taking?

As noted, a disability-related inquiry or medical exam is permissible if it is voluntary.

Example: *At a company-sponsored health fair, employees are offered the opportunity to take tests to measure blood pressure and body mass index (BMI). Whether or not they take these tests has absolutely no impact on the health insurance premiums they pay, and there are no other rewards or punishments for doing so or not. This is permissible under the ADA.*

The ADA rules for disability-related inquiries and medical exams affect wellness programs primarily in relation to HRAs. These rules must be considered in crafting HRA questions. But since they allow voluntary participation, information can often be gathered that can be used to enhance program effectiveness.

As stated, an employer cannot require an employee to respond to disability-related inquiries or undergo a medical exam, nor penalize her for not doing so. But can it offer incentives to those who do so as part of a wellness program? Yes, within limits and if certain requirements are met.

- The wellness program must be reasonably designed.
- A confidentiality notice may be needed.
- The incentive may not exceed a cap (30 percent of the health plan premium in most situations).

To meet the reasonable design requirement, a wellness program cannot:

- require an overly burdensome amount of time for participation,
- involve unreasonably intrusive procedures,
- be a subterfuge for violating the ADA or other laws prohibiting employment discrimination, or
- require employees to incur significant costs for medical examinations.

A program may ask employees to provide medical information in an HRA in order to use aggregated data information to design programs.

An employer must provide clear notice to employees of what medical information will be obtained, how it will be used, who will receive it, and the restrictions on disclosure. However, the company may use an existing notice or a program description to deliver this information.

For programs using disability-related inquiries or medical exams, there is a limit on the total value of incentives: 30 percent of the premium of the employer's health plan (both the employer and employee contribution) for self-only coverage. There are no exceptions for de minimus (very small) or in-kind inducements.

If the employer sponsors more than one health plan, the percentage is based on the lowest-cost plan. If the employer sponsors no health plan, it is based on the cost for a 40-year-old non-smoker of the second-lowest-cost Silver Plan available through the exchange in the employer's principal place of business.

Recall from Part I of this lesson the limits on wellness program incentives under HIPAA/ACA nondiscrimination rules:

- For a health-contingent program (with incentives based on health standards), the limit is 30 of the health plan premium.
- For a health-contingent program that includes tobacco reduction, the limit is 50 percent.
- For a participatory program (with incentives based on voluntary participation, such as joining a gym), there is no limit.

However, for all of these programs, if incentives are offered for employees to respond to disability-related inquiries (such as an HRA) or undergo a medical exam (such as biometric screening), the ADA limit of 30 percent applies.

Example: Under Big Clown Toy Company's wellness program, employees can receive a premium discount for participating in an HRA. Under HIPAA/ACA nondiscrimination rules, this is considered a participatory program (not a health-contingent program), so it is not subject to any incentive limit. But under the ADA, an HRA is a disability-related inquiry, so a 30 percent cap applies.

Example: Sweet Sue Toy Company has a health-contingent wellness program that includes tobacco cessation. Under HIPAA/ACA rules, a cap of 50 percent applies. But the program requires participants

to undergo testing to confirm that they are not using tobacco. This is considered a medical examination, so the program is subject to the ADA and its 30 percent cap.

These rules apply to all wellness programs that offer incentives for responding to disability-related inquiries or undergoing medical exams, not just those offered in connection with the employer's group health plan. The rules are effective for plan years beginning on and after January 1, 2017.

Note that the EEOC's final regulations make clear that, although incentives may be offered if the requirements above are met, employers may not require employees to participate in a program using disability-related inquiries and medical exams as a condition for health plan eligibility, despite a recent federal district court decision allowing such a plan design.¹⁰ Employers also cannot prevent access to a benefit based on participation in a disability-related inquiry or medical exam.

Confidentiality

The EEOC requires that employers receive and disclose employee medical information from a wellness program only in aggregate form (not information on individuals), except as necessary to administer the program or as otherwise permitted under the ADA.

Programs in compliance with HIPAA privacy and security rules are also likely in compliance with ADA confidentiality requirements. However, the EEOC advises that individuals who handle medical information that is part of an employee health program should not be responsible for making decisions related to employment, such as hiring, termination, or discipline. Employers typically avoid this problem by using third-party vendors, but they should take steps to make sure that the vendor has appropriate privacy policies for ensuring the confidentiality of medical information.

Employee Retirement Income Security Act (ERISA)

The **Employee Retirement Income Security Act of 1974 (ERISA)** governs employee benefit plans, including many wellness programs:

- As a general rule, a wellness program that is part of an employer-sponsored group health plan is subject to ERISA.
- A program that is not part of an employer's health plan but offers medical services or products of some sort (such as nicotine patches) may itself be considered an employer health plan and so subject to ERISA.¹¹

Wellness programs subject to ERISA must comply with its nondiscrimination rules and its reporting requirements. For instance, they must file Form 5500¹² (if there are over 100 participants) and provide a Summary Plan Description. On the other hand, since employee benefit plans are regulated at the federal level under ERISA, many state laws do not apply to them. (These laws are "preempted" by ERISA). For instance, a state lifestyle discrimination law (described next) might not apply.

State Lifestyle Discrimination Laws

Many states have **lifestyle discrimination laws** that protect employees against discrimination for engaging, while away from work, in lawful activities or using lawful products even if these activities or

products are unhealthy. These laws can come into play when an employer provides disincentives for such behavior.

Many employers have sought to lower their healthcare costs by reducing the number of smokers in their workforce. In most jurisdictions, an employer can ban smoking at the workplace, but if one seeks to control employee behavior away from work, it may be subject to litigation. However, an employer that offers incentives to quit smoking, especially as part of a wellness program subject to ERISA, would likely be on firmer legal ground.

In some states it may be possible to refuse to hire smokers; in others, a smoker who is not hired may be able to claim discrimination based state law. Or a person might file an action under the ADA, claiming to be a disabled person suffering from nicotine addiction. The bottom line: Before instituting bans on legal behaviors and activities off the job, check with legal counsel to know where your state stands and any applicable federal laws or regulations that may apply.

Summary

The Genetic Information Nondiscrimination Act (GINA) includes rules on requests by wellness programs for genetic information of employees or their spouses, usually as part of a health risk assessment.

The Americans with Disabilities Act (ADA) requires wellness programs to make reasonable accommodation if a disabled person is unable to meet a standard or engage in an activity to earn an incentive. It also establishes rules for programs making disability-related inquiries about employees or requesting medical examinations.

Many wellness programs are subject to the Employee Retirement Income Security Act (ERISA); they must comply with ERISA rules, but they are exempted from many state laws.

State lifestyle discrimination laws are designed to prevent employers from discriminating against employees who engage in unhealthy but legal behavior (such as smoking) while away from work.

Sources

¹ Federal Register, “Genetic Information Nondiscrimination Act, Final Rule by the EEOC,” May 17, 2016, <https://www.federalregister.gov/articles/2016/05/17/2016-11557/genetic-information-nondiscrimination-act>.

² Federal Register, “Regulations Under the Americans with Disabilities Act, Final Rule by the EEOC,” May 17, 2016, <https://www.federalregister.gov/articles/2016/05/17/2016-11558/regulations-under-the-americans-with-disabilities-act>.

³ U.S. Equal Employment Opportunity Commission, “EEOC’s Final Rule on Employer Wellness Programs and Title I of the Americans with Disabilities Act,” <https://www.eeoc.gov/laws/regulations/qanda-ada-wellness-final-rule.cfm>.

⁴ Ibid.

⁵ National Human Genome Research Institute (NHGRI), “Genetic Information Nondiscrimination Act,” <http://www.genome.gov/24519851>.

⁶ U.S. Equal Employment Opportunity Commission, “Genetic Information Discrimination,” www.eeoc.gov/laws/types/genetic.cfm.

⁷ U.S. Equal Employment Opportunity Commission, “EEOC’s Final Rule on Employer Wellness Programs and Title I of the Americans with Disabilities Act” <https://www.eeoc.gov/laws/regulations/ganda-ada-wellness-final-rule.cfm> See also 29 CFR Part 1625, Regulations Under the Genetic Information Nondiscrimination Act of 2008, Federal Register, Vol. 75, No. 216 (November 9, 2010).

⁸ EEOC Compliance Manual, Chapter 3, ADA Issues, Section 1.

⁹ EEOC, 42 USC Section 12102(1). See also 29 CFR 1630.2.

¹⁰ *EEOC v. Flambeau, Inc.*, W.D. Wis., No. 3:14-cv-00638 (December 31, 2015).

¹¹ Readers may find the Department of Labor checklist of help in determining ERISA status, www.dol.gov/ebsa/pfd/fab2008-2.pdf.

¹² Form 5500 Series was jointly developed by the DOL, IRS, and the Pension Benefit Guaranty Corporation so employee benefit plans could utilize the forms to satisfy annual reporting requirements under Title I and Title IV of ERISA and under the Internal Revenue Code.

17 Medicare Overview

Lesson Objectives

After completing this lesson, you should be able to:

- State who is eligible for Medicare and who pays premiums for different parts of Medicare.
- Summarize the benefits paid by Medicare in different areas and the cost-sharing required.
- Identify the health-related expenses that Medicare does not cover and those that may be covered by a Medicare supplement insurance policy.
- Explain how Medicare Part D prescription drug coverage works.
- Explain what Medicare Advantage plans are and what coverage they provide.

This is the first of two lessons on Medicare. This lesson introduces the basics of the Medicare program and its various parts. The next lesson examines key factors that health insurance plans must consider as they seek to participate in Medicare Part C—that is, to provide coverage to Medicare beneficiaries through Medicare Advantage (MA) plans.

Medicare

Medicare is a federal program that provides healthcare benefits to seniors (65 and older); people with severe, long-term disabilities; and a few others. It was established in 1965 under the Social Security Act as part of President Lyndon Johnson's Great Society program.¹ Medicare provides extensive benefits but does not cover all health-related expenses, and beneficiaries must pay significant deductibles and coinsurance. Many Medicare beneficiaries buy private Medicare supplement (Medigap) insurance to cover some of these cost-sharing payments.

An alternative to Original Medicare is the Medicare Advantage (MA) program, under which private-sector health plans provide coverage to Medicare beneficiaries. Medicare beneficiaries may also enroll in Medicare Part D prescription drug coverage (either in addition to Original Medicare or as part of Medicare Advantage coverage).

Over 55 million Americans receive health coverage through Medicare, and enrollment is expected to grow as the population ages and members of the large post-World War II Baby Boom cohort increasingly enter their retirement years.²

In this lesson, we will look at Original Medicare and Medigap insurance. We will also provide an overview of Medicare Advantage and Medicare Part D.

Medicare Programs

Traditionally, Medicare consisted of two parts (together referred to as **Original Medicare**):

- **Medicare Part A** (Hospital Insurance, or HI) covers inpatient care, including stays in hospitals, rehabilitative facilities, skilled nursing facilities, and psychiatric facilities, as well as related services.³
- **Medicare Part B** (Supplementary Medicare Insurance, or SMI) covers physician services, outpatient hospital care, clinical lab and diagnostic tests, and some other medical services not covered by Part A.⁴

More recently **Medicare Part C**—the **Medicare Advantage (MA)** program—was created.⁵ Under this program, Medicare beneficiaries can opt to receive Part A and Part B coverage, as well as some other benefits, from Medicare-approved private-sector health insurance plans.

For many years, Medicare did not cover outpatient prescription drugs. This changed in 2006 when the Medicare Prescription Drug, Improvement, and Modernization Act established **Medicare Part D**.⁶ Medicare beneficiaries who want to obtain Part D coverage have the option of enrolling in a private-sector prescription drug plan (PDP) or an MA plan with drug benefits (an MA-PD plan). Those who enroll in Part D must pay an additional premium, but the program has proved to be popular with seniors.

Medicare Eligibility, Cost, and Enrollment

Medicare Part A

Medicare Part A coverage is generally provided without charge to citizens of the United States (and certain resident aliens) who are 65 or older and fall into one of these categories:

- persons receiving or eligible for Social Security benefits;
- persons receiving or eligible for retirement benefits from the Railroad Retirement Board (RRB); and
- most persons participating in retirement programs for federal, state, or local government employees instead of Social Security.

Individuals 65 or older who do not fall into one of these categories may still enroll in Medicare Part A, but they must pay a monthly premium. As a general rule, those who paid into the Medicare system while they were working by means of FICA payroll deductions for at least a minimal amount of time (or whose spouses did so) do not pay a premium, while those who did not must pay.

Examples: *Julie has just turned 65. She has worked and paid FICA payroll deductions for many years. She is eligible for Medicare Part A and will pay no premiums.*

Rod has also just turned 65, but he inherited income-producing investments and has never worked or paid FICA deductions. He can enroll in Medicare Part A but will have to pay premiums.

Medicare Part A coverage is also available to disabled persons under 65 who meet certain criteria.

- They must qualify for Social Security or RRB disability benefits, which means that their disability must be total—that is, so severe that they are unable to engage in any substantially gainful work.

- The disability must be long-term—they must have received Social Security or RRB disability benefits for at least 24 months.

Because of these restrictive conditions, many disabled persons do not qualify. Medicare Part A coverage is also extended to persons of any age who suffer permanent kidney failure (end-stage renal disease, or ESRD) or amyotrophic lateral sclerosis (ALS, or Lou Gehrig's disease).

Medicare Part B

All U.S. citizens (and certain resident aliens) 65 or older have the option of enrolling in Medicare Part B. Persons with disabilities, end-stage renal disease (ESRD), or ALS who are eligible for Medicare Part A are also eligible for Part B. Because Part B covers important health services (such as outpatient doctor visits) not covered by Part A, most individuals who are enrolled in Part A choose Part B as well.

All Part B enrollees pay a monthly premium. Those with higher incomes—single persons with modified adjusted gross income (MAGI) over \$85,000 and couples with MAGI over \$170,000—pay a higher premium than other beneficiaries, and the higher the income, the higher the premium. Under the ACA the income level above which beneficiaries must pay a higher-than-standard Part B premium will be frozen from 2011 through 2019, so that as incomes rise more beneficiaries will pay the higher premium. Part B premiums may be deducted from Social Security checks, Railroad Retirement checks, or Office of Personnel Management (OPM) checks.

Premiums for Medicare Part B (2016)

Modified Adjusted Gross Income (MAGI)		Monthly Premium
Individual	Couple	
\$85,000 or below	\$170,000 or below	\$104.90 or \$121.80*
\$85,001 - \$107,000	\$170,001 - \$214,000	\$170.50
\$107,001 - \$160,000	\$214,001 - \$320,000	\$243.60
\$160,001 - \$214,000	\$320,001 - \$428,000	\$316.70
Above \$214,000	Above \$428,000	\$389.80

**For most people, the standard premium for Part B is \$104.90 in 2016. The standard premium is \$121.80 for individuals who enroll in Part B for the first time in 2016; do not receive Social Security benefits; or are billed directly for Part B premiums (that is, do not have premiums taken out of their Social Security check).*

Medicare Advantage and Medicare Part D

Individuals participating in both Medicare Part A and Part B have the option of enrolling in a Medicare Advantage (MA) plan, and they may also enroll in Medicare Part D. Both typically charge additional premiums (although in some cases an MA plan may be available without added cost).

Enrollment

People who receive Social Security or RRB retirement benefits are automatically enrolled in Medicare Part A when they turn 65. Disabled persons are automatically enrolled in Part A after they have received Social Security or RRB disability benefits for 24 months. Both of these groups are also enrolled in Part B unless they indicate they want to opt out. Others eligible for Medicare must sign up for Part A and Part B if they want it. This includes those who are 65 or older but still working and not receiving Social Security (as many are since full Social Security benefits are not available until at least age 66).

***Examples:** Maria chooses to begin receiving Social Security retirement benefits at age 62. When she turns 65, she is automatically enrolled in Medicare Parts A, and also in Part B unless she opts out.*

Dan has just turned 65, but he plans to keep working and delay receiving Social Security for a few more years, in order to obtain a larger monthly benefit. If he wants to begin Medicare coverage, he must enroll.

Individuals who do not enroll in Part B when they first become eligible may have to pay a **late enrollment penalty**, in the form of higher premiums, if they enroll later. Those who wait because they are covered by an employer-sponsored health plan do not have to pay this penalty provided they enroll within eight months after their employment or coverage ends. This also applies to those who must pay a Part A premium—they must pay a late enrollment penalty if they do not enroll when they are first eligible (but are also exempt from the penalty if they maintain employer coverage). For those subject to the late enrollment penalty, the Part A and Part B premiums are each increased 10 percent for every 12-month period the individual waits to enroll.

Those who are not automatically enrolled in Part A or Part B can enroll:

- during an **initial enrollment period** (consisting of the month they turn 65, the three months before, and the three months after) or
- later during an **annual open enrollment period** (every year from January 1 to March 31 for Original Medicare).⁷

Persons 65 and older who have employment-based group health insurance can enroll in Part A or Part B at any time while they are still covered by the group plan and up to eight months after their employment ends or their coverage ends (whichever comes first).

Benefits: Medicare Part A

Inpatient Hospital Care

Medicare Part A pays for hospital care—a semiprivate room, nursing care, inpatient drugs, meals, and other medically necessary services and supplies provided in the hospital. Benefits are based on a **benefit period**, which continues until the beneficiary has not received hospital or nursing home care for 60 days in a row.

***Example:** Danielle, a Medicare beneficiary, is released from the hospital and later readmitted. If she has received hospital or nursing home care during the last 60 days before readmission, she is still in*

the same benefit period; if she has not, she starts a new benefit period (with, for instance, a new deductible).

Beneficiaries pay a deductible per benefit period (\$1,288 in 2016, adjusted annually). After 60 days in the hospital, they must make substantial daily copayments (\$322 in 2016). Benefits are limited to 90 days per benefit period. However, each beneficiary has an additional 60 days of hospital coverage per lifetime (referred to as lifetime reserve days), which she can use if the 90-day limit is exceeded. But for lifetime reserve days there is a higher daily copayment (\$644 in 2016). Beneficiaries must pay all costs beyond 150 days. Inpatient mental health care is limited to 190 days per lifetime.

Example: *Doris, a Medicare beneficiary, enters the hospital in 2016. She pays the \$1,288 deductible but makes no other payments for the first 60 days. But beginning on day 61, she must pay a \$322 copayment each day. After 90 days she has exhausted her regular benefits for this benefit period. But she can still use her lifetime reserve days, although she has to pay \$644 per day. After 150 days, she has exhausted her 60 lifetime reserve days, and Medicare pays no more benefits.*

Doris finally leaves the hospital, but six months later she is admitted again. Because she was not in a hospital or nursing home for over 60 days, her previous benefit period ended and a new benefit period begins. She again pays the \$1,288 deductible, and if she stays for more than 60 days, she must make daily copayments. And if she stays for more than 90 days, she will receive no more benefits, since she has exhausted her 60 lifetime reserve days.

Skilled Nursing Facility Care

Medicare Part A pays for care in a skilled nursing facility (a semiprivate room, nursing care, therapy, meals, and other services and supplies), but only if certain restrictive conditions are met:

- The facility must be Medicare-certified.
- The skilled nursing facility stay must occur within 30 days after an inpatient hospitalization of three or more consecutive days.
- Skilled nursing care or skilled rehabilitative services must be necessary to treat a medical condition that is related to the beneficiary's hospital stay or started while she was in the skilled nursing facility for a hospital-related condition.

Medicare covers skilled nursing facility care only if it is medically necessary. Until recently, this meant only to the extent it was necessary to improve the relevant condition. For instance, if skilled nursing facility care was required to help a person recover from a stroke, the medical necessity requirement was met and benefits paid. But if it was needed by someone with an impairment that was not expected to improve, this requirement was not met.

But a 2013 court decision in response to a class action suit changed this.⁸ Now, even if a person is not expected to recover or improve, as long as the services of a healthcare professional are required to maintain an individual's condition or prevent deterioration of it, she can continue to receive Medicare skilled nursing facility benefits. However, Medicare does not pay for the nursing home care of the many people who need only personal care (help with activities of daily living such as bathing and dressing) or only supervision because of a cognitive condition.

Medicare pays for up to 100 days of skilled nursing facility care per benefit period.

- Some people recover from an acute injury or illness after a few weeks and do not need the full 100 days.
- Others do not recover but stop needing skilled care—they need only personal care to cope with what has become a chronic impairment. Their benefits also end.

If Medicare coverage continues beyond 20 days, the beneficiary must make a daily copayment (\$161 in 2016). After 100 days in the same benefit period, all benefits end. If an individual does not receive benefits for over 60 consecutive days, a new benefit period begins and she is again eligible for up to 100 days. However, to qualify she will again have to have a three-day hospitalization within the last 30 days.

Example: *Sam, a Medicare beneficiary, has a major stroke. After two weeks in the hospital, he is transferred to a skilled nursing facility, where he receives speech and physical therapy and nursing care. Because Sam needs skilled care as part of the treatment of his medical condition Medicare pays benefits.*

For the first 20 days of Sam's nursing home stay, Medicare pays all of the \$300 daily charge. Beginning on day 21, Sam pays \$161 (2016) per day, and Medicare pays the remaining \$139. After 30 days, Sam no longer needs rehabilitation and skilled nursing care to treat his condition and so no longer qualifies for Medicare benefits.

However, Sam does need help with dressing and bathing (personal care) and is likely to need it indefinitely. Unfortunately, he will have to pay for this care himself (either out of his own funds or by means of a private long-term care insurance policy).

Home Health Care

Medicare Part A and Part B each pay benefits for home health care, providing seamless coverage. (We will discuss Part B benefits below.) Part A pays benefits only if certain conditions are met:

- Skilled nursing care or physical, speech, or occupational therapy is medically necessary. Medical necessity is defined as for skilled nursing facility care, in keeping with the recent court decision discussed above. A need for only personal or supervisory care is not sufficient.
- Intermittent (not full-time) care is needed.
- Care is needed within 14 days after a stay of at least three days in a hospital (or in a skilled nursing facility following hospitalization).
- The beneficiary is homebound.
- The home health agency is Medicare-certified.

Medicare pays 100 percent of approved costs. For Part A, there is a limit of 100 visits per benefit period (but coverage can continue under Part B). As with skilled nursing facility care, if a person no longer has a medical necessity for skilled care, benefits end.

Example: *Isabel, like Sam, has a major stroke. When she is released from the hospital, she goes home, where she receives rehabilitation services two days a week. Isabel qualifies for Medicare Part A home health care benefits because her doctor certifies that intermittent therapy at home is necessary for the treatment of her condition (and she meets the other requirements described above). Part A benefits will end after 100 visits, but benefits may continue under Part B.*

Hospice

Hospice care is nursing care, pain relief, symptom management, physical therapy, and supportive medical and social services for terminally ill individuals.⁹ It may take place in the person's home, a hospice facility, or another type of facility.

Medicare Part A pays for hospice care for those with life expectancy of six months or less.

- If care is provided in a facility where the person lives (such as a nursing home or assisted living residence), Medicare covers only hospice care, not room and board.
- Medicare offers some respite care benefits, which allow a family caregiver to take a break while the terminally ill individual stays in a facility or receives care at home from paid personnel.
- The costs of hospice care are fully covered by Original Medicare, except for a copayment of no more than \$5 for each outpatient drug prescription for pain relief or symptom control and a 5 percent coinsurance for inpatient respite care.¹⁰

In general, to qualify for Medicare hospice benefits, a person must choose to end curative treatment of his terminal illness (such as radiation for cancer) and instead receive palliative (comfort) care. However, the Medicare Choices Model, established by the ACA, waives this requirement. This model, which is scheduled to run through 2020, will test whether this approach can improve care and enhance patient satisfaction while at the same time reducing costs.¹¹ Moreover, while receiving hospice care, if an individual needs care for a condition not related to his terminal illness (for example, he falls out of bed and breaks a leg), Medicare will pay for that care under its usual coverage standards.

Benefits: Medicare Part B

Medicare Part B covers a wide array of services and supplies. These include:

- Physician and surgeon services
- Services provided by non-physician practitioners, such as certified nurse anesthetists, clinical psychologists, and nurse practitioners
- Outpatient clinic services, including same-day surgery, and ambulatory surgical center services in a Medicare-approved facility
- Emergency room and ambulance services
- Laboratory tests, x-rays, and other diagnostic radiology services

- Physical and occupational therapy and speech pathology services furnished on an outpatient basis
- Radiation therapy and outpatient renal dialysis
- Approved durable medical equipment (DME) for home use, such as oxygen equipment, wheelchairs and walkers, prosthetic devices, and surgical dressings, splints, and casts (from Medicare-approved providers)
- Certain vaccines
- A few drugs (those that are usually not self-administered, certain anticancer drugs, and immunosuppressive drugs after a transplant)

In the past Medicare coverage of preventive care was fairly limited, but it has been expanded in recent years. Covered are a one-time “Welcome to Medicare” physical exam and certain screening tests, including bone mass and cardiovascular screenings, screenings for colorectal cancer and diabetes, mammograms, Pap tests. The ACA added annual wellness visits. The ACA also eliminated cost-sharing for most (but not all) preventive care services.

Medicare Part B, like Part A, pays home health care benefits. It covers care for beneficiaries who meet one of these conditions:

- They meet all the requirements discussed above for Part A benefits except for a recent hospitalization.
- They have used up their 100 visits under Part A. If this happens and the individual still meets the requirements for home health care (including medical necessity) benefits continue without interruption under Part B. (Note also that for the few people who have only Part A, the 100-visit limit does not apply.)

Cost-Sharing

For most Part B services and products there is an annual deductible (\$166 in 2016) and 20 percent coinsurance. As noted, the ACA eliminated cost-sharing for most preventive care. Under both Part A and Part B, beneficiaries must pay for the first three pints of blood (if it is not donated by a blood bank, which is often the case).

Other Medicare Considerations

Generally, beneficiaries do not have to file claims with Medicare. The provider charges the beneficiary any deductible, coinsurance, or copayment amounts due; bills Medicare for the remaining amount; and is paid directly by Medicare. Medicare pays no more for a health care service than the **Medicare-approved amount**, the charge that Medicare determines is reasonable for the service. Most healthcare providers are “participating” providers who accept **assignment**—that is, they accept the Medicare-approved amount as full payment. These providers are always directly paid by Medicare.

A “**nonparticipating**” provider generally does not accept assignment and is allowed to charge more than the Medicare-approved amount. The beneficiary must pay the difference (**excess charge**). However, a

nonparticipating provider may not charge more than 115 percent of the Medicare-approved amount for a Medicare covered service.¹² In addition, some doctors formally opt out of the Medicare program. They do not submit any claims to Medicare, and Medicare beneficiaries who go to them must pay for services out of their own pockets, even if a service is covered by Medicare. Opt-out doctors are not subject to Medicare limits on billing—they can charge whatever they want.

Dual Eligibles

People who qualify for both Medicare and Medicaid (the federal-state health coverage program for the poor) are referred to as **dual eligibles**. Medicare is the first payer and Medicaid the second—that is, Medicare first pays benefits, then Medicaid pays any expenses not covered by Medicare but covered by Medicaid. Medicaid also generally pays Medicare premiums and cost-sharing (copayments and deductibles) for dual eligibles. If a state Medicaid program does not pay Medicare cost-sharing, the dual eligible individual is not responsible for it.

Summary of Medicare Coverage

Medicare Part A covers the following types of care (with limitations):

- **Inpatient hospital care**—subject to a deductible, daily copayments after 60 days, and a limit of 90 days per benefit period (except for 60 lifetime reserve days).
- **Skilled nursing facility care.** Skilled nursing care or therapy must be medically necessary, and other conditions must be met. Subject to a limit of 100 days per benefit period, with daily copayments after day 20.
- **Home health care.** Intermittent skilled care at home must be medically necessary, and other conditions must be met. For Part A coverage there is a requirement for a prior hospital stay and a limit of 100 visits (but see Part B below).
- **Hospice care**—provided to terminally ill persons, with only small copayment and coinsurance amounts for a few items.

Medicare Part B covers physician services, outpatient care, emergency room care and ambulance, laboratory work, durable medical equipment, and many other services and supplies. Most of these are subject to an annual deductible and coinsurance. Part B covers home health care for those who qualify under Part A except for a prior hospital stay or who have used up their 100 visits under Part A.

What Does Original Medicare NOT Cover?

Medicare Parts A and B do not provide for all healthcare needs.

- **Prescription drugs.** Original Medicare does not cover most outpatient drugs (except for hospice care). Beneficiaries can obtain drug coverage by enrolling in Medicare Part D (but even this program does not pay all drug costs).
- **Long-term care.** Medicare provides some benefits for nursing home care and home health care, but they are available only while a person needs skilled nursing care or therapy to treat a

medical condition. They are not available for extended periods to those who need only personal care because of a chronic physical impairment or supervision because of a cognitive impairment.

- **Other services and products.** With a few exceptions, Original Medicare does not pay benefits for dental care and dentures, vision care and eyeglasses, hearing aids, acupuncture, cosmetic surgery, healthcare delivered outside the United States, and some other health-related services and products.
- **Cost-sharing.** For a serious or extended illness, the deductibles, coinsurance, and copayments required by Medicare can add up to a substantial amount.

Medicare Supplement (Medigap) Insurance

Medicare supplement insurance policies are offered by private-sector insurers. They fill some of the gaps in Original Medicare coverage and so are commonly called **Medigap** policies. To be eligible for Medigap insurance, an individual must be enrolled in Original Medicare (both Part A and Part B). Those enrolled in a Medicare Advantage (MA) plan may not buy a Medigap policy because many of the benefits provided by Medigap are already covered by an MA plan.

The main purpose of Medigap policies is to cover some of the cost-sharing required by Medicare.

- All Medigap policies cover hospital daily copayments and all or part of Part B coinsurance.
- Some policies also cover all or part of the hospital deductible, the Part B annual deductible, and/or all or part of skilled nursing facility daily copayments.

Some Medigap policies also provide benefits for a few healthcare expenses not covered by Medicare.

- **Part B excess charges.** As mentioned previously, if a healthcare provider does not accept assignment and the provider's actual charge is greater than the Medicare-approved amount, the beneficiary must normally pay the difference (the excess charge). A Medigap policy with excess charge benefits covers this expense.
- **Foreign travel emergency.** The Medigap benefit pays 80 percent of the cost of emergency medical care needed during foreign travel (after a \$250 deductible), up to a \$50,000 lifetime maximum.
- **Prescription drugs.** Some older policies offer limited outpatient prescription drug coverage. However, since the establishment of Medicare Part D the sale of new Medigap policies with drug benefits has been prohibited. Old policies can continue to provide these benefits, as long as the policyholder does not enroll in Medicare Part D.

Medigap Regulation and Standardization

Medigap policies are subject to extensive government regulation. Standard Medigap plans, each with a certain combination of benefits, have been established, and insurers are required to offer only these plans. These standard plans are identified by letters A through N (with not all letters currently sold). (Note that different standard plans are offered in Massachusetts, Minnesota, and Wisconsin.) In addition, all insurers must use the same standardized format, language, and definitions in describing the

benefits of these plans. This standardization is intended to make it easier for consumers to compare plans and premiums. Since insurers offer the same standard plans, they compete on the basis of price and customer service. For a description and comparison of the standard Medigap plans, click here: <https://www.medicare.gov/supplement-other-insurance/compare-medigap/compare-medigap.html>

Medigap Enrollment

If a person applies for a Medigap policy during her Medigap open enrollment period, **guaranteed issue** applies—that is, the insurer cannot subject the individual to medical underwriting, deny coverage, make the individual wait to obtain coverage, or charge more than the standard premium. A person's open enrollment period begins the first day of the first month in which she is both 65 or older and enrolled in Medicare Part B, and it lasts for six months.

***Example:** John retires at age 67 and enrolls in Medicare Part B in April. His Medigap open enrollment period begins April 1 and lasts till the end of September.*

An individual is entitled to guaranteed issue after her open enrollment period in certain circumstances.

- A person does not enroll in Medigap at first because she has employer-sponsored health coverage, but she applies later.
- Medigap coverage ends through no fault of the policyholder, and she wishes to buy a new policy.

***Example:** Mildred bought a Medigap policy from Big City Insurer several years ago, but Big City has decided to no longer provide Medigap coverage. Mildred can obtain similar coverage from another insurer on a guaranteed issue basis.*

If a person is not entitled to guaranteed issue, an insurer has the right to conduct underwriting, decline to offer coverage, or charge a higher-than-standard premium.

Further information on Medicare Supplement Insurance (Medigap) may be found in the AHIP course entitled Supplemental and Voluntary Products.

Medicare Part D

The Medicare Modernization Act (MMA) of 2003¹³ established a prescription drug benefit program, Medicare Part D, which became effective in 2006. Part D is available to all those enrolled in Medicare, but beneficiaries choose whether or not to participate. Those who do pay an additional monthly premium. Medicare Part D coverage is offered by private-sector prescription drug plans (PDPs) and through Medicare Advantage plans that include Part D coverage (MA-PD plans). Typically, there is an annual deductible and per-prescription cost-sharing.

A person with Original Medicare can generally enroll in any PDP operating in his area. He enrolls in the PDP of his choice, receives benefits from that PDP, and pays premiums to the PDP (or has premiums deducted from his Social Security check and paid to the PDP). Those in Original Medicare generally have several PDPs to choose from, increasing the likelihood that each person will find a plan that meets his needs and circumstances. Different PDPs charge different deductibles and cost-sharing amounts, cover different drugs (in addition to those required by Medicare), require participants to use different

pharmacy networks, and charge different monthly premiums. This competition is designed to drive PDPs to offer attractive benefit packages. A person in a Medicare Advantage plan offering Part D coverage may generally only receive Part D through that plan and cannot enroll in a PDP.

Part D Benefits

While PDPs vary in the benefit packages they offer, all plans must provide a minimal level of benefits. Specifically, Medicare has established a standard Part D benefit, and while a PDP may offer a different benefit structure, the package must be at least actuarially equivalent to the standard. Medicare also specifies certain drugs that all PDPs must cover.

As Medicare Part D was originally designed, enrollees received benefits until the total amount received reached a certain level; then they received no more benefits. But if their total drug expenditures reached a stipulated higher level, catastrophic coverage kicked in and benefits resumed. This coverage gap proved to be unpopular, and it was addressed by the ACA. The coverage gap is being gradually reduced and will be eliminated by 2020, through a combination of discounts that drug manufacturers are required to offer and increased benefits paid by Medicare.

Medicare Advantage

Medicare Advantage (Medicare Part C) is an alternative to Original Medicare. Those entitled to Medicare can choose to receive coverage through Original Medicare, or (in most areas of the country) they have the option to instead enroll in a Medicare Advantage (MA) plan, a Medicare-approved private-sector health insurance plan. The Medicare program pays an MA plan a fixed amount each month for each person it serves, and the plan in turn provides Medicare coverage.

An MA plan must provide all Part A and Part B benefits except hospice. Most MA plans provide additional benefits that fill some of the gaps in Medicare coverage, like a Medigap policy. Some MA plans provide benefits that go beyond Original Medicare or Medigap, such as for routine dental care, hearing aids, and eyeglasses. And as noted above, some MA plans offer Medicare Part D coverage.

MA plan enrollees pay the Medicare Part B premium, and they also pay their MA plan a monthly premium (which covers any benefits beyond Medicare Parts A and B that the plan provides). MA plans also charge cost-sharing for some services. The extent of coverage beyond Original Medicare, premium rates, and cost-sharing payments vary from plan to plan.

There are several types of Medicare Advantage plans, reflecting the kinds of health insurance plans in the marketplace today. These include health maintenance organizations (HMOs), preferred provider organizations (PPOs), high-deductible plans accompanied by a medical savings account (MSA plans), and what are referred to as private fee-for-service (PFFS) plans. In addition, there are special needs plans (SNPs) that serve certain populations. These include people with chronic conditions (such as diabetes or heart disease), Medicare/Medicaid dual eligibles, and nursing home residents.

To enroll in an MA plan, a person must be eligible for Medicare Parts A and B and live in the service area covered by the MA plan. A person can enroll in an MA plan when he first becomes eligible for Medicare, and there is also an annual enrollment period during which a person can switch from Original Medicare to an MA plan, from an MA plan to Original Medicare, or from one MA plan to another.

Because MA plans fill some of the same gaps in Medicare coverage as Medigap policies, having both MA coverage and a Medigap policy would result in the duplication of benefits. Therefore, insurers may sell Medigap policies to Original Medicare enrollees and not to MA participants. Medicare beneficiaries who want supplementary benefits must choose either Original Medicare plus a Medigap policy or an MA plan.

The ACA includes provisions designed to reduce government expenditures on Medicare Advantage and to make MA plans operate more efficiently and provide high-quality services.

- The ACA calls for reduced payments to MA plans—the law as passed required cutbacks beginning in 2012.
- Bonuses (higher payments) are given to MA plans that meet certain quality standards.
- The highest-quality MA plans (those with a five-star rating) are granted the advantage of being able to enroll beneficiaries at any time during the year, not just during the annual enrollment period.

Summary

Medicare is a federal program that provides health benefits to individuals 65 and older; those with severe, long-term disabilities; and a few with certain diseases. Medicare consists of four parts:

- Medicare Part A—coverage of inpatient hospital care and hospice care and limited benefits for nursing home and home health care when restrictive conditions are met.
- Medicare Part B—coverage of physician services, outpatient care, and many other medical services.
- Medicare Part C (Medicare Advantage)—program under which private-sector health plans provide Part A and Part B coverage and often some other benefits (including prescription drug coverage).
- Medicare Part D—prescription drug coverage

Generally, those who paid into the Medicare system while working receive Medicare Part A at no charge. Those who choose to enroll in Part B, Part D, or Medicare Advantage pay an additional premium. Beneficiaries must pay significant deductibles, coinsurance, and copayments, and many buy private Medicare supplement (Medigap) insurance to cover some of these costs for Parts A and B. Not covered are most dental, vision, and hearing services and products, as well as long-term care.

Additional Resources

These publications are available from the Centers for Medicare and Medicaid Services:

- A brief history of Medicare, <http://www.cms.gov/History>.

- *Medicare & Medicaid: Milestones 1937-2015* (CMS publication), July 2105
<https://www.cms.gov/About-CMS/Agency-Information/History/Downloads/Medicare-and-Medicaid-Milestones-1937-2015.pdf>.

Sources

¹ Social Security Act, Title XVIII – Health Insurance for the Aged and Disabled,
http://www.ssa.gov/OP_Home/ssact/title18/1800.htm.

² Centers for Medicare & Medicaid Services, “On Its 50th Anniversary More Than 55 Million Americans Covered by Medicare,” July 28, 2015, <https://www.cms.gov/Newsroom/MediaReleaseDatabase/Press-releases/2015-Press-releases-items/2015-07-28.html>.

³ Social Security Act, Title XVIII, Section 1811- Description of Program.

⁴ Social Security Act, Title XVIII, Section 1831- Establishment of Supplementary Medical Insurance Program for the Aged and Disabled.

⁵ The longstanding program of providing Medicare services through private health plans paid on an at-risk basis was renamed the Medicare Advantage program and recodified under Part C of Title XVIII of the Social Security Act under the Balanced Budget Act of 1997.

⁶ Social Security Act, Title XVIII, Section 1860D-1, Eligibility, Enrollment, and Information.

⁷ Centers for Medicare & Medicaid Services, Medicare.gov, “When Can I Sign Up for Part A and Part B?”
<https://www.medicare.gov/sign-up-change-plans/get-parts-a-and-b/when-sign-up-parts-a-and-b/when-sign-up-parts-a-and-b.html>.

⁸ *Jimmo v. Sebelius*,
http://scholar.google.com/scholar_case?case=3191060390956184947&q=jimmo+v.+sebelius&hl=en&as_sdt=20006&as_vis=1; see also Centers for Medicare & Medicaid Services, “Settlement Agreement Fact Sheet” (explaining the case and its implications), <http://www.cms.gov/Medicare/Medicare-Fee-for-Service-Payment/SNFPSP/Downloads/Jimmo-FactSheet.pdf>

⁹ Centers for Medicare & Medicaid Services, Medicare.gov, “Your Medicare Coverage: Hospice and Respite Care,”
<https://www.medicare.gov/coverage/hospice-and-respite-care.html>.

¹⁰ Ibid.

¹¹ Melanie Evans, “CMS to Test Concurrent Coverage of Hospice and Curative Care,” *Modern Healthcare*, July 20, 2015, <http://www.modernhealthcare.com/article/20150720/NEWS/150729994>; Virgil Dickson, “CMS Launches Hospice Demo,” *Modern Healthcare*, March 22, 2014, <http://www.modernhealthcare.com/article/20140322/MAGAZINE/303229969>

¹² Centers for Medicare & Medicaid Services, Medicare.gov, “Costs and Assignment,”
<https://www.medicare.gov/your-medicare-costs/part-a-costs/assignment/costs-and-assignment.html>.

¹³ U.S. Government Publishing Office, Medicare Prescription Drug, Improvement, and Modernization Act of 2003,
<https://www.gpo.gov/fdsys/pkg/PLAW-108publ173/pdf/PLAW-108publ173.pdf>.

18 Medicare and Health Plans

Lesson Objectives

After completing this lesson, you should be able to:

- Describe the types of Medicare Advantage plans.
- Explain how Medicare compensation of MA plans is determined, including bids, benchmarks, rebates, and risk adjustment.
- Identify the main rules that apply to MA plans in the areas of eligibility, enrollment, benefits, appeals and grievances, networks, and marketing.

Of the more than 55 million Americans who receive health coverage through the Medicare program, approximately one-third get their coverage from a private-sector health insurance plan, through a program known as **Medicare Part C** or, more commonly, **Medicare Advantage (MA)**.¹ Under the MA program, health plans contract with the federal government, through the Centers for Medicare & Medicaid Services (CMS), to provide Medicare benefits.² Enrollment in both Medicare and Medicare Advantage is expected to grow as the population ages and members of the large post-World War II Baby Boom cohort increasingly enter their retirement years.³

To provide some historical background, as you recall from the last lesson, Medicare originally consisted of two parts: Part A (hospital insurance) and Part B (supplementary medical insurance). In 1997 the Balanced Budget Act (Public Law 105-33) established Medicare Part C, then called Medicare + Choice. In 2003 the Medicare Prescription Drug, Improvement, and Modernization Act (MMA) (Public Law 108-173) modified this program and renamed it Medicare Advantage. The MMA also established the fourth part of Medicare, Part D, a voluntary prescription drug benefit program, which may be provided through either private-sector prescription drug plans (PDPs) or a Medicare Advantage plan.⁴

Types of MA Plans

In terms of structure, there are four types of Medicare Advantage (MA) plans:

- health maintenance organizations (HMOs) (with or without a point-of-service option)
- preferred provider organizations (PPOs)
- private fee-for-service (PFFS) plans
- Medicare savings account (MSA) plans

In addition, there are special needs plans (SNPs), which can be either HMOs or PPOs.

HMOs and PPOs are referred to as **coordinated care plans**. In some coordinated plans (such as HMOs), members must generally use network providers to receive benefits; in others (such as PPOs), members

can use out-of-network providers but pay a higher share of costs if they do. Most MA enrollees are in coordinated care plans.⁵

Health Maintenance Organization (HMO)

An MA HMO may or may not have a point-of-service (POS) option. If there is no POS, plan members can use only the hospitals, doctors, and other healthcare providers that have agreements with the plan (that is, that are in the HMO's network). There are exceptions for emergency care, urgently needed services, and dialysis when a member is temporarily out of the service area, as well as for services that are required under Medicare but cannot be provided in the plan's network. A member may also need to obtain a referral from her primary care physician (PCP) before seeing a specialist.⁶

If an HMO has a point-of-service option, members can go outside the network to obtain certain services. However, the HMO can limit the types of services available out-of-network, cap the amount of out-of-network coverage available, and impose referral requirements.

Preferred Provider Organization (PPO)

Like an HMO, a PPO contracts with healthcare providers that agree to provide services to plan members for a specified level of reimbursement, and these providers form the PPO's network. But unlike an HMO, a PPO must pay benefits for all covered services, whether they are obtained in or out of the network, and a PPO may not require referrals for out-of-network providers. In other words, PPO members can obtain covered services from any non-network provider eligible to accept Medicare payment. However, if a member goes out of network she will generally pay a higher portion of costs. There is an exception when a covered service is not available in the PPO's network—in this case, the member will pay the regular in-network cost-sharing for out-of-network care.

Private Fee-for-Service (PFFS) Plan

An MA private fee-for-service plan may or may not have a network of providers. A PFFS plan operating in a service area with two or more MA plans with networks (known as a network area) must maintain a network that meets MA access-to-care requirements.⁷ In other areas non-employer PFFS plans may not have a network.

The contracted network providers of a PFFS plan may be required to serve plan members for a negotiated payment rate, but the plan must pay both network and non-network providers on a fee-for-service basis and cannot put them at financial risk. If there is no network, the PFFS plan must pay providers at least as much as they would receive under Original Medicare fee-for-service. A plan may not vary the rates paid to providers based on utilization of services, except to increase payment for furnishing preventive and screening services.

The members of a PFFS plan can go, without a referral, to any provider eligible for Medicare payment who is willing to accept the plan's terms and conditions of payment, whether or not the plan has a contract with the provider. Non-contracted providers can decide whether to accept the PFFS plan's terms and conditions of payment each time they see a plan member. If a non-contracted provider furnishes non-emergency services to a PFFS member, the provider is deemed to have accepted the terms and conditions of the plan for that episode of care.

Medicare Savings Account (MSA) Plan

Medicare savings account (MSA) plans are similar to the combination of a high-deductible health plan and a health savings account (HSA) used by many non-Medicare insureds. A beneficiary is enrolled in an MA plan with a high annual deductible. The amount varies by plan, within a maximum set each year by CMS. Until this deductible is satisfied, the plan pays no benefits (except for certain preventive services, which must be covered with no cost-sharing). However, for each enrollee money is contributed to an account, and the enrollee can draw on this account to pay for medical expenses before the deductible is met. (Unlike with an HSA, the individual cannot contribute to the account herself.) The amount in the account also varies from plan to plan, but it is somewhat below the annual deductible; if the account is exhausted, the enrollee will be responsible for all costs until the deductible is met. But once the deductible is met, the plan is generally responsible for all additional costs.

***Example:** Juliet is enrolled in an MSA plan. Her annual deductible is \$3,000; \$1,500 is deposited in her account. The plan will pay nothing until she has satisfied the deductible. If Juliet has medical expenses, she can pay them out of her account. But if she incurs more than \$1,500 in expenses, she will have to pay for them out of her own pocket. However, if she reaches \$3,000 in covered expenses, she satisfies her deductible, and the plan will pay all additional costs.*

Special Needs Plan (SNP)

While most MA plans must generally accept all Medicare beneficiaries who apply, an MA special needs plan (SNP) is designed for and limited to people who meet certain criteria. These include those with certain chronic conditions (C-SNPs), those needing institutional care (I-SNPs), and Medicare-Medicaid dual eligibles (D-SNPs). An SNP can be either an HMO or PPO. Organizations wishing to offer an SNP must get special approval from CMS; in addition to meeting all the general requirements for MA plans, they must have an approved model of care that sets forth how they will manage and coordinate care and promote quality of care for the special population served.

Compensation of MA Plans

In return for providing beneficiaries with Medicare Part A and Part B coverage, MA plans are paid a fixed monthly amount for each beneficiary enrolled (a **capitation** amount), regardless of how many services the plan delivers. The amount an MA plan is paid is based principally on the bid that it submits to CMS, but this bid amount may be adjusted, and the plan may receive a rebate that it can use to reduce member costs or add benefits. In this section, we will offer a summary of the complex process of determining how much compensation a plan will receive.

CMS Guidance

Each year, before health plans submit bids for the following year, CMS distributes annual guidance. This includes the following:

- The Advance Notice of Methodological Changes for Medicare Advantage (MA) Capitation Rates, Part C and Part D Payment Policies and Draft Call Letter is issued in mid-February for the following year. CMS requests comments from health plans on the proposed payment policies set forth in this document.

- The Announcement of Calendar Year Medicare Advantage Capitation Rates and Medicare Advantage and Part D Payment Policies and Final Call Letter is issued on the first Monday of April.

For instance, for the 2017 plan year the Advance Notice was issued on February 19, 2016, and the Final Call Letter was issued on April 4, 2016.

The Bid

A health insurance plan seeking to offer (or to continue offering) MA coverage must submit a bid to CMS. The bid is an aggregate monthly per capita amount based on the plan's estimated costs of providing Medicare Part A and Part B benefits. If an organization seeks to offer different MA plans, it must submit a bid for each plan. Bids for the following plan year must be submitted no later than the first Monday of June of the current year. (For instance, Wellness Health submits its bid on Monday, June 5, 2017, to provide coverage in the 2018 plan year.) CMS reviews each bid to make sure it meets requirements and may approve or reject a bid (but before doing so, CMS may negotiate with the plan).⁸

The Benchmark

CMS establishes a **benchmark** for each county or region. This is the maximum capitation amount that CMS will pay an MA plan to provide Part A and Part B coverage.

The benchmark for a county is calculated in this way: The per capita spending for Medicare Part A and Part B for each county in the U.S. is estimated, and counties are grouped into quartiles according to spending (the top 25 percent, the next 25 percent, and so on). Then, for each county, the per capita spending amount is adjusted, using a different multiplier for each quartile (see table below). The result is the benchmark.

**Medicare Advantage Benchmark Calculation:
Adjustment to Estimate of County Medicare Parts A and B Spending**

Quartile in Medicare A & B Spending	Benchmark Equals This Percent of Estimated Spending
4 th (top 25 percent)	95 percent
3 rd (51 st to 75 th percentiles)	100 percent
2 nd (26 th to 50 th percentiles)	107.5 percent
1 st (bottom 25 percent)	115 percent

Example: For Big Mountain County, the per capita spending on Medicare Parts A and B is estimated to be \$800. Big Mountain is in the second quartile in terms of spending, so the benchmark of MA plans operating in the county will be 107.5 percent of the base amount, or \$860.

In this way, the benchmark is raised for counties with lower spending. This is designed to enhance the financial viability of MA plans in parts of the country with lower Medicare costs, which typically have lower MA penetration.

Adjustment of Benchmark for Quality

As part of CMS's efforts to promote and reward quality in MA plans, it has created a **star rating system**. CMS gives each MA plan a rating based on the plan's performance in five categories:

- Staying healthy: screenings, tests, and vaccines
- Managing chronic conditions (such as diabetes)
- Member experience with the health plan
- Member complaints, problems obtaining services, and improvement in the plan's performance
- Health plan customer service⁹

The ratings take the form of stars:

- 1 Star – Poor performance
- 2 Stars – Below average performance
- 3 stars – Average performance
- 4 stars – Above average performance
- 5 stars – Excellent performance

(For more information on the star rating system see, "Primer: The Medicare Advantage Star Rating System" by Tara O'Neill at <https://www.americanactionforum.org>.)

CMS gives quality bonuses to plans with high star ratings. In effect, these bonuses raise the benchmark that applies to the plan. This allows a plan to:

- submit a higher bid while remaining under the benchmark, thereby increasing the capitation amount it receives;

and/or

- qualify for a rebate, enabling the plan to lower costs to its members or offer them additional benefits, which makes the plan more attractive to consumers.

The quality bonus system works like this:

- All new MA plans are given a bonus of 3.5 percent, resulting in a benchmark of 103.5 percent of the standard benchmark for the county or region.
- Plans that are not new with four, four-and-a-half, or five stars earn a bonus of 5 percent, for a benchmark of 105 percent of the standard.
- Non-new plans with less than four stars (or that do not report on quality measures) earn no bonus. The standard benchmark applies.¹⁰

The Rebate

Once a benchmark is established for a plan, it is compared to the plan's bid amount. It is rare that the bid is above the benchmark, but if it is, CMS pays the plan the benchmark capitation amount. The plan can make up the costs not covered by this amount by charging higher premiums, but of course this will make the plan less attractive to potential members.

If, as is more common, a plan's bid is below the applicable benchmark, CMS pays the plan the benchmark amount plus a rebate. If a plan receives a rebate, it must use it for two purposes:

- reducing cost-sharing or premiums or
- providing additional benefits not included in Medicare Parts A and B—for instance, benefits for dental, hearing, and vision care.¹¹

Lower premiums or cost-sharing or extra benefits will of course make a plan more competitive.

Calculation of the Rebate

The difference between the bid and the benchmark is referred to as “savings,” and the rebate amount is a percentage of the savings. This percentage is higher for higher quality star ratings (another way CMS rewards plan quality).¹²

Rebate Calculation Based on a Health Plan's Quality Star Rating

Quality Star Rating	Percentage of the Difference Between the Bid and the Benchmark That Is Returned to the Plan as a Rebate
4.5 and 5 stars	70 percent
3.5 and 4 stars	65 percent
3 stars or fewer	50 percent

Risk Adjustment

Capitation payments to MA plans are risk adjusted—that is, CMS pays the plan a higher capitation amount for enrollees at high risk of needing extensive medical care.¹³ There are adjustments for age and sex, but the most significant adjustments are for health conditions of the enrollee.

Example: George has a serious and chronic health condition, making it likely he will need many healthcare services. George enrolls in the Wellness Health MA plan. CMS will pay Wellness a higher capitation payment for George.

Risk Adjustment

For a useful summary of the risk adjustment program, see “Medicare Advantage Risk Adjustment Program: Quick Reference Guide,” Excellus, at [https://www.excellusbcbs.com/wps/wcm/connect/19f64b88-dfc5-44b7-abb8-6567edee4672/Medicare%2BAdvantage%2BRisk%2BAdj%2BBrochure%2B\(EX\).pdf?MOD=AJPERES&CACHEID=19f64b88-dfc5-44b7-abb8-6567edee4672](https://www.excellusbcbs.com/wps/wcm/connect/19f64b88-dfc5-44b7-abb8-6567edee4672/Medicare%2BAdvantage%2BRisk%2BAdj%2BBrochure%2B(EX).pdf?MOD=AJPERES&CACHEID=19f64b88-dfc5-44b7-abb8-6567edee4672).

See also the chapter on risk adjustment in the CMS *Medicare Managed Care Manual* at <https://www.cms.gov/Regulations-and-Guidance/Guidance/Manuals/Downloads/mc86c07.pdf>.

Rules for MA Plans

Medicare Advantage plans are closely regulated by CMS, whose rules are set forth in the *Medicare Managed Care Manual* and pertinent regulations. In this section we will highlight the main rules for MA plans in these areas:

- Eligibility
- Enrollment
- Benefits
- Provider networks
- Reconsideration, appeals, and grievances
- Marketing

Eligibility

An individual is eligible to enroll in an MA plan if she:

- is entitled to Medicare Part A and enrolled in Part B;
- lives in the service area of the MA plan (with some exceptions—for instance, a person who intends to move into the area may be eligible); and
- is not already enrolled in another MA plan.

A person cannot be ineligible based on her health, except for some persons with end-stage renal disease (ESRD). Variations of these rules apply to those seeking to enroll in an MA Special Needs Plan or Employer Group Health Plan, as well as those with ESRD.¹⁴

Enrollment

An individual (or her legal representative) must complete an enrollment request, including all required information, and agree to abide by the rules of the plan. MA plans must accept the enrollment applications of eligible individuals during these periods:

- **Annual Election Period (AEP)**—the Fall open enrollment period, from October 15 through December 7 each year.¹⁵
- **Initial Coverage Election Period (ICEP)**—the period during which an individual who is newly eligible for MA may enroll.
- **Special Election Periods (SEPs)**. An individual can enroll in a plan or change her plan if certain circumstances arise.

The ICEP and SEPs are explained below.

The Initial Coverage Election Period (ICEP)

The ICEP begins three months before the day the individual first becomes entitled to *both* Medicare Part A and Medicare Part B (commonly the individual's 65th birthday). It ends on the *later* of:

- the last day of the month preceding the day the individual becomes entitled to both Part A and Part B, or
- the last day of the individual's Part B initial enrollment period.

When is the Part B initial enrollment period? It is a seven-month period beginning three months before the month the individual first meets the eligibility requirements for Part B and ending three months after that month.¹⁶

Example. *On August 10 Marion turns 65 and meets the eligibility requirements for Medicare Part B. Marion's Part B initial enrollment period includes the three months before August (May, June, and July), August, and the following three months (September, October, and November).*

Marion's ICEP for enrolling in an MA plan begins on July 10 (one month before her 65th birthday) and ends on November 30 (when her Part B initial enrollment period ends).

Special Election Periods (SEPs)

A person is eligible for a special election period in a number of circumstances, including the following:

- She moves and so now lives outside of her current plan's service area or has new plan options.
- The MA plan she is enrolled in terminates operations, violates its contract, or significantly changes its provider network (see below).
- She loses employer-sponsored group coverage.

In addition, when a person enrolls in an MA plan shortly after turning 65, she can switch from MA to Original Medicare at any time during the first 12 months (called **SEP 65**). This allows beneficiaries to try out Medicare Advantage and leave it if they are not satisfied (perhaps because they find out that their long-time primary care physician is not in the plan's network).¹⁷ Finally, Medicare-Medicaid dual eligibles have an ongoing SEP—they can enroll in or change plans at any time.

Depending on the cause of the SEP, an individual may switch from one MA plan to another MA plan, from Original Medicare to an MA plan, or from an MA plan to Original Medicare. The length of an SEP (how long the person has to make the change) also varies according to the cause, but it is usually ample.¹⁸

Recently, CMS has recognized that when an MA plan makes significant changes in its provider network outside the routine contract initiation and renewal cycles, this can cause difficulties for enrollees. CMS may now establish a SEP under which enrollees can switch to another MA plan or Original Medicare if it determines, on a case-by-case basis, that there has been a significant network change. CMS determines (again, on a case-by-case basis), whether the SEP will include only enrollees who have been affected by the network change, or also those who may be affected by it.¹⁹

Benefits

In general, MA plans must cover all healthcare services that are covered under Original Medicare (Part A and Part B). Some MA plans also provide benefits for additional services, such as dental, hearing, and vision care. Many MA plans, known as Medicare Advantage Prescription Drug (MA-PD) plans, include Part D prescription drug benefits.

CMS sets limits (adjusted annually) on the total out-of-pocket expenses that MA plan members can be required to pay, as well as limits on cost-sharing specific to certain benefits. Certain preventive benefits must be covered without cost-sharing. But within these restrictions, CMS allows MA plans a degree of flexibility in designing their benefit package. For instance, a plan may set limits on utilization of certain services, such as covering only a limited number of physical therapy sessions following an orthopedic procedure.

Example of Benefit Package Design: Wellness MA plan covers all healthcare services covered under Medicare Parts A and B, plus preventive dental services. There is a \$20 copayment for most physician visits and 10 percent coinsurance for non-emergency hospital care, subject to an annual cap on member cost-sharing of \$6,700.

An MA plan member may request clarification as to what benefits she is entitled to and what cost-sharing she is responsible for, and she may request a determination of whether a benefit will be paid and how much. The plan must respond to such requests according to CMS rules.²⁰

- **Standard request.** The plan must notify the member as expeditiously as her health condition requires but in no more than **14 calendar days**.
- **Expedited request.** Sometimes members request an expedited determination before obtaining a service, perhaps because of the very serious nature of the illness. In this case, the plan must notify the enrollee as expeditiously as her health condition requires but in no more than **72 hours**.

The timeframe for both of these requests can be extended once for an additional 14 calendar days, if this is requested by the enrollee or if it is justified and in the enrollee's interest because of the need for medical evidence from a noncontract provider or extraordinary, exigent, or other non-routine circumstances.

For providers that are contracted with an MA plan, the timeframe for payment of a claim is established by the contract. For non-contracting providers, “clean” (non-problematic) claims must be paid within 30 days, and all other claims within 60 days.

Provider Networks

An MA plan’s provider network must meet certain requirements intended to ensure that the plan’s members have access to the providers and care they need. In addition, an MA plan is generally financially responsible for care provided outside the network when a member is temporarily out of the plan’s service area and requires ambulance services, emergency care, post-stabilization services, or renal dialysis, or he unexpectedly and urgently needs services.²¹ Access to care must be provided in a culturally competent manner, including to those with limited English proficiency.²²

Reconsideration, Appeals, and Grievances

If an MA plan member disagrees with a benefit determination, he can appeal the decision.

- He first appeals to his MA plan (**first-level appeal** or **reconsideration**).²³
- If the first-level appeal decision is against the member, the case is automatically forwarded to an **independent review entity (IRE)** outside the MA plan, for further consideration.²⁴
- If the member disagrees with the decision of the IRE, and if the amount in dispute is high enough, he may be able to continue to appeal. He may request a hearing before an administrative law judge (ALJ), and if he is unhappy with the ALJ’s decision, he may request review by the Medicare Appeals Council (MAC). If the MAC denies review or the member is unhappy with the MAC’s decision, he may go to the courts for judicial review.²⁵

A **grievance** differs from an appeal. It is a complaint or dispute, *not about a benefit determination*, but rather about a plan’s operations, activities, or behavior, regardless of whether remedial action is requested.²⁶ An MA plan must provide a grievance process for its members, and this process must include a notice to a member no later than 30 days after the receipt of the grievance.²⁷ (Regulations and the *Medicare Managed Care Manual* provide detailed guidance.)

{IT Developer: See if we can pick-up the flow chart provided by CMS at this link}

<https://www.cms.gov/Medicare/Appeals-and-Grievances/MMCAG/Downloads/Managed-Care-Appeals-Flow-Chart-.pdf>

Marketing

In relation to the sales and marketing of MA plans, CMS is very concerned about the welfare of consumers and has imposed a number of rules and requirements. Of course, high-pressure sales tactics and bait-and-switch techniques will not be tolerated, but in addition many aspects of marketing are regulated.²⁸ A full discussion of the rules is beyond the scope of this course, but we will highlight some of the most important points.

CMS requirements apply to the activities of a plan’s employees and of its downstream contractors, including independent agents or brokers, third-party marketing organizations (TMOs), and other similar organizations that contribute to the steering of a potential enrollee toward a specific plan or limited

number of plans, or that may receive compensation directly or indirectly from a plan for marketing activities.²⁹

Marketing Materials

CMS guidelines stipulate what must be included in MA plans' marketing materials, what is permissible advertising, and even what font sizes and form numbers must be used. CMS's definition of marketing materials is broad, and includes the following:

- **General audience promotional materials**, such as brochures and leaflets, direct mail materials, and advertising in newspapers, magazines, television, radio, billboards, yellow pages, or the Internet, including materials circulated by physicians, other providers, or third-party entities;
- **Materials for marketing representatives**, such as scripts, outlines, slides, and charts for telemarketing or other presentations;
- **Materials and communications to plan members**, including membership rules, subscriber agreements, enrollee handbooks, wallet cards, and instructions to enrollees; communications to enrollees about contractual changes, such as changes in providers, premiums, benefits, and plan procedures; and communications related to membership activities.

MA plans must file marketing materials with CMS.

- **CMS-sanctioned "file and use" materials** (general advertising that does not include specific benefit and plan premium information) must be filed within five days before use or distribution, and can be used or distributed five days after filing.
- **CMS-developed model materials without modification** must be filed 10 days before use or distribution; if no decision is received after 10 days, the materials are deemed approved.
- **Customized materials** developed by a plan must be filed 45 days in advance of use or distribution; if no decision is received after 45 days, the materials are deemed approved.³⁰

Marketing materials must be made available in any language that is the primary language of more than 5 percent of the population of a plan's service area.³¹ (And regardless of their service area, MA plans must have the ability to serve non-English speakers through their call centers.³²)

Agents and Brokers

There are also extensive rules for and oversight of the activities of those representing plan products to the public, such as agents and brokers. The responsibilities of MA plans in this area include ensuring that agents and brokers representing their products are licensed by the appropriate state authorities and that they fulfill annual training requirements. (More information about these rules and training requirements is provided by AHIP in its *Medicare + Fraud, Waste, and Abuse (FWA) Training*.)

Ongoing Compliance

MA plans must maintain effective compliance and fraud, waste, and abuse (FWA) programs. In order to ensure that such compliance is taking place, CMS maintains an ongoing oversight and audit system.³³ A

full discussion of compliance is beyond the scope of this course, but AHIP offers additional resources and courses in this area.

Summary

There are several types of Medicare Advantage (MA) plans, including health maintenance organizations (HMOs), which may or may not have a point-of-service (POS) option; preferred provider organizations (PPOs); and private fee-for-service (PFFS) plans. Medicare savings account (MSA) plans are similar to combinations of high-deductible health plans and health savings accounts used in the non-Medicare market. Special needs plans (SNPs), which can be either HMOs or PPOs, serve certain populations.

MA plans are paid a fixed monthly amount for each beneficiary enrolled, regardless of how many services the plan delivers. This capitation amount is determined in this way: A plan submits a bid to CMS. A benchmark is set for the plan's county or region (based on Medicare Part A and Part B spending, but adjusted to give lower-spending areas a higher benchmark). This benchmark is increased for plans with high star quality ratings. If a plan's bid is below the benchmark that applies to it, the plan will receive some of the difference as a rebate (with high-quality plans receiving a higher percentage). The plan can use that rebate to lower member costs or add benefits, making the plan more attractive to consumers. Finally, the capitation amount may be adjusted for individuals at high risk of needing extensive healthcare.

With some exceptions, those who are entitled to Medicare Part A and enrolled in Part B and live in the service area of an MA plan are eligible to enroll in the plan. Individuals can enroll when they first become eligible, during the annual open enrollment period, and in certain other circumstances. MA plans must provide at least the coverage of Original Medicare, and some offer additional benefits. MA plans must allow members to appeal benefit determinations and file grievances. CMS closely regulates the sale and marketing of MA plans, including the activities of agents and brokers.

Sources

¹ Centers for Medicare & Medicaid Services, "On Its 50th Anniversary More Than 55 Million Americans Covered by Medicare," July 28, 2015. <https://www.cms.gov/Newsroom/MediaReleaseDatabase/Press-releases/2015-Press-releases-items/2015-07-28.html>.

² Social Security Act, Title XVIII, Part C, Section 1851, et seq., 42 USC Section 1395w-21, et seq.

³ Centers for Medicare & Medicaid Services, "On Its 50th Anniversary More Than 55 Million Americans Covered by Medicare," July 28, 2015. <https://www.cms.gov/Newsroom/MediaReleaseDatabase/Press-releases/2015-Press-releases-items/2015-07-28.html>.

⁴ Social Security Administration, "Annual Statistical Supplement, 2011 – Medicare Program Description and Legislative History." <https://www.ssa.gov/policy/docs/statcomps/supplement/2011/medicare.html>.

⁵ 42 CFR Section 422.4 – Types of MA Plans; *Medicare Managed Care Manual*, Chapter 1.

⁶ Centers for Medicare & Medicaid Services, "Glossary." <https://www.medicare.gov/find-a-plan/staticpages/glossary/planfinder-glossary.aspx?let=H>.

⁷ 42 CFR Section 422.114(a)(3) – Access to Services Under an MA Private Fee-for-Service Plan. <https://www.law.cornell.edu/cfr/text/42/422.114>.

⁸ 42 CFR Section 422.254(a)(1) – Submission of Bids. <https://www.law.cornell.edu/cfr/text/42/422.254>.

⁹ Centers for Medicare & Medicaid Services, “Star Rating System.” <https://www.cms.gov/Outreach-and-Education/Training/CMSNationalTrainingProgram/Downloads/2013-5-Star-Enrollment-Period-Job-Aid.pdf>.

¹⁰ Social Security Act, Title XVIII, Part C, Section 1853, 42 USC Section 1395w-23. See also, *Primer: The Medicare Advantage Star Rating System*, Tara O’Neill, pp. 4-5. <https://www.americanactionforum.org>.

¹¹ 42 CFR 422.304(a)(3) – Monthly Payments: Payment of Rebate for Plans with Bids Below Benchmark. <https://www.cornell.edu/cfr/text/42/422.304>]

¹² Centers for Medicare & Medicaid Services, “Advance Notice of Methodological Changes for Calendar Year (CY) 2016 for Medicare Advantage Capitation Rates, Part C and Part D Payment Policies and 2016 Call Letter,” February 20, 2015, pg. 15.

¹³ *Ibid.*, p. 19.

¹⁴ Centers for Medicare & Medicaid Services, *Medicare Managed Care Manual*, Chapter 2 – Medicare Advantage Enrollment and Disenrollment (Revised August 14, 2014), Section 20; 42 CFR 422.50 – Eligibility to Elect an MA Plan. <https://www.cornell.edu/cfr/text/42/422.50>.

¹⁵ Centers for Medicare & Medicaid Services, Medicare.gov, “When Can I Join a Health or Drug Plan?” <https://www.medicare.gov/sign-up-change-plans/when-can-i-join-a-health-or-drug-plan/when-can-i-join-a-health-or-drug-plan.html#collapse-3190>.

¹⁶ 42 CFR 4017.14 – Initial Enrollment Period. <https://www.law.cornell.edu/cfr/text/42/407.14>.

¹⁷ Centers for Medicare & Medicaid Services, *Medicare Managed Care Manual*, Chapter 2, Section 30.4.5 – SEPs for Beneficiaries Age 65 (SEP 65).

¹⁸ Centers for Medicare & Medicaid Services, *Medicare Managed Care Manual*, Chapter 2 – Medicare Advantage Enrollment and Disenrollment (Revised August 14, 2014), Section 30 – Election Periods and Effective Dates; 42 CFR 422.62 – Election of Coverage Under an MA plan; and 42 CFR 422.68 – Effective Dates of Coverage and Change of Coverage.

¹⁹ Centers for Medicare & Medicaid Services, *Medicare Managed Care Manual*, Chapter 2, Section 30.4.6 – SEP for Significant Change in Provider Network.

²⁰ Centers for Medicare & Medicaid Services, *Medicare Managed Care Manual*, Chapter 13, Medicare Managed Care Beneficiary Grievances, Organizational Determinations, and Appeals Applicable to Medicare Advantage Plans, Cost Plans, and Health Care Prepayment Plans (HCPPs), (revised April 20, 2012), Sections 30-60; 42 CFR Sections 422.520; 422.568; 422.572.

²¹ 42 CFR 422.113. <https://www.law.cornell.edu/cfr/text/42/422.113>.

²² 42 CFR 422.112. <https://www.law.cornell.edu/cfr/text/42/422.112>.

²³ Centers for Medicare & Medicaid Services, *Medicare Managed Care Manual*, Chapter 13, Medicare Managed Care Beneficiary Grievances, Organizational Determinations, and Appeals Applicable to Medicare Advantage Plans, Cost Plans, and Health Care Prepayment Plans (HCPPs), (Collectively referred to as Medicare Health Plans (revised 4/20/12), Sections 30-60; 42 CFR Sections 422.520; 422.568; 422.572.

²⁴ Centers for Medicare & Medicaid Services, *Medicare Managed Care Manual*, Chapter 13, Section 90 – Reconsiderations by Independent Review Entity; 42 CFR Sec. 422.590 – Timeframes and Responsibility for Reconsiderations. <https://www.law.cornell.edu/cfr/text/42/422.590>.

²⁵ Centers for Medicare & Medicaid Services, *Medicare Managed Care Manual*, Chapter 13, Sections 90 through 120. <https://www.cms.gov/Regulations-and-Guidance/Guidance/Manuals/downloads/mc86c13.pdf>.

²⁶ 42 CFR 422.561 – Definitions.

²⁷ 42 CFR 422.564 – Grievance Procedures. <https://www.law.cornell.edu/cfr/text/42/422.564>.

²⁸ Centers for Medicare & Medicaid Services, *Medicare Managed Care Manual*, Chapter 3 – Medicare Marketing Guidelines (June 10, 2016). <https://www.cms.gov/Medicare/Health-Plans/ManagedCareMarketing/Downloads/2017MedicareMarketingGuidelines2.pdf>; 42 CFR Section 422, Subpart V – Medicare Advantage Marketing, <https://www.law.cornell.edu/cfr/text/42/part-422/subpart-V>; 42 CFR 423m Subpart V – Part D Marketing Requirements, <https://www.law.cornell.edu/cfr/text/42/part-423/subpart-V>.

²⁹ Centers for Medicare & Medicaid Services, *Medicare Managed Care Manual*, Chapter 3, Section 10, Introduction. <https://www.cms.gov/Medicare/Health-Plans/ManagedCareMarketing/Downloads/2017MedicareMarketingGuidelines2.pdf>.

³⁰ Centers for Medicare & Medicaid Services, *Medicare Managed Care Manual*, Chapter 3, Section 90, The Marketing Review Process. <https://www.cms.gov/Medicare/Health-Plans/ManagedCareMarketing/Downloads/2017MedicareMarketingGuidelines2.pdf>.

³¹ Centers for Medicare & Medicaid Services, *Medicare Managed Care Manual*, Chapter 3, Medicare Marketing Guidelines, Section 30.5 – Requirements Pertaining to Non-English Speaking Populations; 42 CFR Section 422.2264e – Guidelines for CMS Review, <https://www.law.cornell.edu/cfr/text/42/422.2264>.

³² Centers for Medicare & Medicaid Services, *Medicare Managed Care Manual*, Chapter 3, Medicare Marketing Guidelines, Section 30.5 – Requirements Pertaining to Non-English Speaking Populations.

³³ 42 CFR 422.503(b)(4)(vi) – General Provisions. <https://www.law.cornell.edu/cfr/text/42/422.503>]

19 Medicaid Medical Coverage

Lesson Objectives

After completing this lesson, you should be able to:

- Summarize who is eligible for Medicaid medical coverage and the most important eligibility criteria.
- Describe the main changes to Medicaid eligibility made by the ACA.
- Contrast the eligibility requirements for ACA populations and for the elderly and disabled and the medically needy.
- State the purpose and briefly describe the Children's Health Insurance Program (CHIP) and Basic Health Programs (BHPs).

What Is Medicaid?

Medicaid is a government program that provides health coverage to some poor people. Historically, Medicaid has covered low-income individuals who fall into certain categories, such as children and pregnant women, a few parents, and the disabled and elderly. The Affordable Care Act has expanded Medicaid to all adults with income below a certain level, even if they do not fall into one of these categories—but this change has not been adopted by all states. In addition, some people who are not initially poor can qualify for Medicaid payment of nursing home care and home health care (long-term care) if they spend almost all their assets and income on care.

Medicaid was created by Congress in 1965 as part of President Lyndon Johnson's Great Society program and codified under Title XIX of the Social Security Act. It is a joint **federal-state program**. Both the federal government and the states contribute funding. Each state operates its own program and sets its own rules within broad federal guidelines; that is, within those guidelines, each state establishes its own eligibility standards; determines the type, amount, duration, and scope of services covered; sets the rate of payment for services; and administers its own program. Medicaid was originally rather limited in scope, but since its founding many states have used federal matching funds to provide health coverage to previously uninsured populations.¹ Medicaid is now the single largest source of health coverage in the United States; together with the Children's Health Insurance Program (CHIP), it provides health coverage to over 72.5 million Americans.²

Medicaid is an **entitlement program**. This means that anyone who meets the eligibility criteria has the right to enroll in core Medicaid programs. A state cannot limit enrollment or establish waiting lists for such programs because of budgetary concerns. (But note that some states provide

coverage to certain populations that do not qualify under regular Medicaid rules, under waivers to those rules granted by the federal government, and for these waiver programs enrollment or funding caps may be allowed.)

In this lesson, we will examine Medicaid coverage of medical care for the poor. In the next lesson we will look at Medicaid coverage of long-term care, for which the rules are quite different.

Eligibility

Persons applying for Medicaid must meet both basic requirements (related to citizenship or immigration status and state residency) and financial requirements. They must provide documentation demonstrating that they meet these requirements. While the basic requirements are the same for all Medicaid programs, the financial requirements differ based on the category into which applicants fall.

Citizenship/Immigration Status

To qualify for Medicaid, a person must be either a U.S. citizen or a legal immigrant. In general, immigrants must have lived legally in the U.S. for at least five years. However, states have the option of providing coverage to children and/or pregnant women during their first five years in the country, and more than half do so.³ Also, Medicaid may pay for emergency medical care for legal immigrants in their first five years and for illegal immigrants.⁴

State Residency

An individual must be a resident of the state where she is applying for Medicaid. However, a state may not require a person to have lived in the state for a certain amount of time before becoming eligible. An individual is generally considered a resident of a state if she is currently living there and has the intention of remaining there. Residency can be demonstrated by voter registration, automobile registration, tax filing, or simply receiving mail at an address.

Categories

Traditionally, to qualify for Medicaid, a person has to fall into an eligible category and meet the financial requirements for that category. People who do are referred to as **categorically needy**. Under federal law, all states must provide coverage to people in **mandatory categories**. There are many of these, but the most important are:

- Children through age 18 in families with income below 133 percent of the federal poverty level (FPL);
- Pregnant women with incomes below 133 percent of the FPL;

- A few adults in families with dependent children and very low income; and
- Disabled and elderly persons who receive Supplemental Security Income. (To qualify for SSI, a person's income must be very low—usually well below the poverty level.)

States may also choose to provide coverage for **optional categories**, and the federal government will provide some of the funding. Optional categories include people in the mandatory categories with somewhat higher income. For instance, a state may provide coverage to pregnant women with income up to 185 percent of the FPL or to disabled and elderly people with income above the SSI level but below the FPL. Other optional categories include people suffering from specific medical conditions, such as women needing treatment for breast or cervical cancer and individuals with tuberculosis.⁵

In addition, some states offer coverage to the **medically needy**. These are people who do not meet the requirements of a mandatory or optional category, but have high medical expenses not covered by health insurance.

The Affordable Care Act substantially revises the financial eligibility rules for most categories. However, most of these changes do not apply to the elderly and disabled and the medically needy. In the discussion that follows, we will look first at **ACA populations** (those covered by the new rules) and then at the elderly and disabled and the medically needy (who largely follow the traditional rules).

ACA Populations

Simplification and Standardization

For ACA populations, the financial eligibility rules have been simplified and standardized in several ways:

- **Income limits.** As explained above, states are required to provide coverage to those in certain categories with income below certain levels. Previously, these levels varied somewhat according to category; now, the level for all ACA categories is at least 133 percent of the FPL. However, at the option of each state, this level may be set higher for some categories, allowing more people to qualify.⁶
- **Countable income.** The ACA mandated for all states the same method of calculating a person's countable income for Medicaid eligibility purposes (as described below).
- **Resource limits.** Medicaid refers to a person's assets (bank accounts, investments, real estate, etc.) as **resources**. Some states have had resource limits for some categories—that is, if a person has more resources than the limit, she cannot qualify for Medicaid, regardless of her income. For ACA populations, these resource tests have been eliminated.

Medicaid Expansion

The ACA also expands Medicaid eligibility to all working-age low-income people, whether they fall into one of the traditional categories or not. Specifically, if a person is between the ages of 19 and 65, is not on Medicare, and has income below 133 percent of the FPL, even if she is not disabled or a parent or pregnant, she qualifies. However, the U.S. Supreme Court has held that the federal government cannot obligate states to expand their Medicaid programs in this way, and each state decides for itself whether to do so.⁷ Currently, about two-thirds of the states have adopted the expansion, although some large states (including Texas and Florida) have not.⁸

Counting Income

As mentioned, the ACA has simplified and standardized the way income is calculated for Medicaid eligibility purposes.

- For ACA populations, income is now based on Modified Adjusted Gross Income (MAGI). This is the figure used in the ACA marketplaces to determine eligibility for premium and cost-sharing subsidies, and using it for Medicaid eligibility as well makes it easier for consumers to use the marketplaces to find the best coverage for them (which may be a private health plan or Medicaid).
- Before the ACA, most state Medicaid programs calculated an applicant's income by adding up income from all sources and then subtracting allowances for certain expenses, such as child care. (For example, an individual might have total monthly income of \$1,200, minus an allowance of \$50, for countable income of \$1,150.) The rules for allowances varied significantly across states. Under ACA rules, instead of various allowances, in all states applicants are allowed to disregard an amount equal to 5 percent of the FPL—that is, this amount is not counted in determining income for Medicaid purposes. (This explains why it is often reported that Medicaid eligibility has been expanded to 138 percent of the FPL—the threshold is 133 percent plus a 5 percent disregard, for a total of 138 percent.)

Example: Jonathan, a single man, has Modified Adjusted Gross Income (MAGI) of \$16,000. This is more than 133 percent of the FPL (\$15,800 in 2016). But of Jonathan's income, \$594 (5 percent of the FPL) is disregarded, leaving him with countable income of \$15,406. This is below \$15,800, so Jonathan qualifies.

What Is MAGI?

Modified Adjusted Gross Income (MAGI) is not a line on a person's federal income tax return. It is a separate calculation; the guidelines are provided on IRS Form 8962 and its instructions. Generally, MAGI is a taxpayer's adjusted gross income (AGI) according to the Internal Revenue Code (IRC), plus three additional kinds of income, if any:

- non-taxable Social Security benefits,

- tax-exempt interest, and
- untaxed foreign income.

Further information on MAGI can be found at <http://www.healthreformbeyondthebasics.org/key-facts-income-definitions-for-marketplace-and-medicaid-coverage/>

Elderly and Disabled and Medically Needy

Elderly and Disabled

Medicaid eligibility rules have not changed under the ACA for the elderly (65 and older) and the disabled.

Establishing Disability

To qualify for Medicaid based on disability, a person must be unable to engage in substantial gainful activity (SGA) because of a physical or mental impairment that is expected to last at least a year or result in death. Social Security Disability Insurance (SSDI), Medicare, and the Supplemental Security Income (SSI) program use the same criteria, so a person receiving disability benefits from any of those programs automatically qualifies as disabled under Medicaid. Others must submit medical evidence of their disability. (Note that in states that have adopted the Medicaid expansion, a person under 65 with income below 133 percent of the FPL can qualify for Medicaid on that basis, without proving disability. However, if a disabled person is on Medicare, she is not eligible for the expansion and must qualify based on the rules in this section.)

Income

Income limits for the elderly and disabled are extremely low, so that only very poor people qualify. A person may qualify in three ways:

- **SSI recipient.** Supplemental Security Income (SSI) provides a minimal income to very poor people. SSI payments are set to bring total income up to the federal benefit rate (FBR) (\$733 a month in 2016). In most states, SSI recipients are automatically eligible for Medicaid; in some states, they must apply, but the criteria are generally the same as for SSI (and cannot be more restrictive).⁹

***Example:** Jane has countable monthly income of \$170. She receives \$563 from SSI, giving her total income of \$733 (the FBR).*

- **Income above SSI level but below FPL.** States have the option of extending Medicaid to elderly and disabled persons with income too high for SSI but at or below the FPL (\$990 monthly for an individual in 2016). Slightly less than half of the states do so.¹⁰
- **Medically needy**—in some states, as discussed below.

Counting Income

Unlike for ACA populations, for the elderly and disabled the old way of calculating countable income is used—the methodology of the SSI program. One begins with gross income and subtracts standard deductions. A distinction is made between unearned income (such as Social Security or pension payments) and earned income (wages from work). The first \$20 of income (earned or unearned) is deducted. Next, the first \$65 of earned income and half of all remaining earned income is deducted.

***Example:** Shirley's only monthly income is \$425 she receives for looking after a neighbor's child. The first \$20 of her income is subtracted, then \$65 of her earned income. This leaves \$340 of earned income, of which half is deducted, leaving countable income of \$170.*

Medically Needy

Under a medically needy program, a person who does not meet the requirements for a Medicaid category (who is not “categorically needy”) but has high medical expenses that are not covered by health insurance may be able to have some of these expenses paid by Medicaid. About two-thirds of the states have medically needy programs. States that have such programs must include children and pregnant women in them; they may also include the elderly and disabled, and all but one do so.¹¹ For both groups, traditional methods of counting income are used.

The calculation and payment of benefits to the medically needy are complicated, but in essence a person must spend all her income above her state's medically needy income standard on her care, and Medicaid pays remaining expenses. The income standard in most states is very low, about half the FPL.¹² Payments are typically made on a one-, three-, or six-month cycle.

***Example:** Judy has countable monthly income of \$1,700; her state's medically needy standard is \$400. Judy pays \$1,300 (her income above the standard) for her care, and Medicaid pays the rest of her medical expenses. However, Judy's state is on a three-month cycle. This means that Judy must first pay \$3,900 for her care (three times her monthly obligation), then Medicaid pays remaining expenses for the rest of the three-month period. If at the end of the three months, Judy still needs care, another cycle starts; she again pays \$3,900 and Medicaid pays the rest.*

Resources

While resource limits have been eliminated for ACA populations, they still exist for the elderly and disabled and the medically needy. Typically, the resource limit is \$2,000 for an individual and \$3,000 for a married couple. Persons with resources above these levels cannot qualify; they may be able to spend down their assets until these limits are reached and then qualify. Some resources are not counted, such as a home, a car, personal property, and some burial funds.

Resource limits do not play a large role in determining eligibility for Medicaid medical coverage, since people with such low incomes rarely have significant assets. On the other hand, as we will see in the next lesson, resource limits are quite important in Medicaid long-term care coverage, which is sought by many middle class people with large LTC expenses.

Other Eligibility Considerations

Medicare

A low-income Medicare beneficiary may also qualify for Medicaid, as an elderly or disabled person or on a medically needy basis. For such **dual eligibles**, Medicare is the first payer and Medicaid the second—that is, Medicare first pays benefits, then Medicaid pays any expenses not covered by Medicare but covered by Medicaid. Medicaid also generally pays Medicare premiums and cost-sharing (copayments and deductibles) for dual eligibles. Those who do not qualify for regular Medicaid coverage may be eligible for one of three **Medicare Savings Programs**, which pay Medicare premiums and sometimes cost-sharing.¹³

- **Qualified Medicare Beneficiaries (QMBs)** have income at or below the FPL. Medicaid pays Medicare premiums and cost-sharing.
- **Specified Low-Income Medicare Beneficiaries (SLMBs)** have income between 100 and 120 percent of the FPL. Medicaid pays Medicare premiums.
- **Qualifying Individuals (QIs)** have income between 120 and 135 percent of the FPL. Medicaid pays the Medicare Part B premium. However, for this program funding and enrollment are capped.

There are also resource limits for these programs, but they are higher than for regular Medicaid.

Private Health Coverage

In general, if an individual has private health insurance, that does not disqualify her from Medicaid. However, Medicaid will pay only expenses not covered by the private insurance.

Example: *Jake has health insurance from his employer; however, it has large deductibles and copayments. He also qualifies for Medicaid under the expansion group. Medicaid will pay for his cost-sharing as well as healthcare expenses not covered by his insurance but covered by Medicaid.*

When Does Coverage Begin?

Medicaid coverage can begin as early as the third month before application, if the applicant would have been eligible if she had applied earlier.

Example: *Rita starts needing medical care in April and applies for Medicaid in September. Her financial situation has not changed over these months, so Medicaid coverage is retroactive to June, and any expenses she incurred from June through August are covered.*

Annual Redetermination

Once a person qualifies for Medicaid, she does not automatically remain eligible. At least every 12 months, a state Medicaid program must look at a recipient's situation and determine if she is still eligible, and the recipient must demonstrate that she still meets all requirements. A person's income might increase, making her no longer eligible. However, ACA provisions encourage states to simplify the redetermination process. In particular, states should seek to avoid people losing eligibility simply because they have not filled out a form, by using electronic matching and prepopulated forms.

Benefits

For those who qualify, Medicaid coverage is extensive, and cost-sharing is generally minimal. All states must cover certain healthcare services (mandatory benefits); states may also cover other services (optional benefits).

- The most important mandatory benefits include inpatient and outpatient hospital care, physician services, laboratory and x-ray services, medical or surgical services by a dentist, and family planning.¹⁴
- Major optional benefits include services by optometrists, chiropractors, podiatrists, and certain other practitioners; dental services (non-medical/surgical) and dentures; physical, occupational, and speech therapy; eyeglasses; prosthetic devices; and transportation to medical care.¹⁵ Prescription drugs are an optional benefit, but they are covered by all states.

Note that these lists do not include long-term care (nursing home care and various home and community-based services), which we will discuss in the next lesson.

Two Related Programs

Before we leave Medicaid medical coverage, we must briefly describe two other programs that provide health coverage to the poor.

The Children's Health Insurance Program (CHIP)

Closely related to Medicaid is the Children's Health Insurance Program (CHIP), initiated by the Balanced Budget Act of 1997 (BBA) (Public Law 105-33). CHIP provides health coverage to children (up to age 19) in families with income too high to qualify for Medicaid but often too low to afford private health insurance (for instance, a family with income around 200 percent of the FPL). Like Medicaid, CHIP is a federal-state program; both levels of government contribute funding, and states run their own programs and set their own rules within federal guidelines. And as with Medicaid ACA populations, for CHIP applicants MAGI is used, and there are no resource tests. However, unlike Medicaid, CHIP is not an entitlement program—states are not required to accept all who meet the requirements; they may limit the number of enrollees and establish waiting lists. A state may elect to provide CHIP coverage through its Medicaid program, a separate state program, or a combined program. (Most states take a separate or combined approach.¹⁶) The Medicare Access and CHIP Reauthorization Act of 2015 (MACRA) (Public Law 114-10) has extended federal CHIP funding through September 30, 2017; additional legislative action will be needed to continue the program after that date. Further information on CHIP may be found at HealthCare.gov <https://www.healthcare.gov/medicaid-chip/childrens-health-insurance-program/>.

Basic Health Programs (BHPs)

The ACA seeks to expand health coverage in various ways, such as extending Medicaid and making insurance available through marketplaces. Section 1331 of the law is designed to do this by allowing states to run Basic Health Programs (BHPs). BHPs are intended to provide public health coverage to people with income somewhat above Medicaid eligibility levels. Their main purpose is to give low-wage earners continuity of coverage and of relationships with their healthcare providers. Because of fluctuating earnings, many low-income individuals often find themselves churned between private insurance (sometimes obtained through an ACA marketplace) and Medicaid. When this happens, the physicians and other healthcare professionals with whom they have built relationships are often no longer available in their new provider networks. BHPs are aimed at stopping this from happening, and one suggested way for accomplishing this is through jointly administered BHP and Medicaid programs.

To qualify for a BHP, an individual must:

- have income above 133 percent but below 200 percent of the FPL,
- be younger than 65 and ineligible for Medicare,
- not have access to affordable employer-sponsored health coverage, and

- legally reside in the United States and the state in which application for the BHP is made.

The income range for BHPs was set at 133 percent to 200 percent of the FPL on the assumption that those with income at or below 133 percent would qualify for the Medicaid expansion. However, as we have seen, some states have not adopted the expansion, and in those states some people with income below 133 percent of the FPL are ineligible for either Medicaid or a BHP. (Note also that slightly different rules apply to some legal aliens.)

A BHP may charge premiums. States are allowed to support BHPs by drawing on a portion of federal dollars that would otherwise go toward subsidizing the purchase of private coverage through the ACA marketplaces. However, implementation has been slow. As of this writing, Minnesota and New York are the only states that have inaugurated BHPs. Successful program results in these states are likely to result in other states following their lead, as Medicaid expansion and other changes brought about by the ACA are digested.¹⁷

Summary

Medicaid is a federal-state program that provides health coverage to some poor people. It is now the single largest source of health coverage in the United States. To qualify for Medicaid, a person must be a U.S. citizen or a legal immigrant and a resident of the state where she is applying. Those eligible include:

- people in certain categories, such as children, pregnant women, some parents, and elderly and disabled persons;
- in states that have adopted the ACA's Medicaid expansion, all working-age adults with income below 133 percent of the federal poverty level; and
- in some states, the medically needy—people who do not meet the requirements for a Medicaid category but have high medical expenses.

In addition to expanding eligibility, the ACA standardized Medicaid's treatment of income, based on MAGI and an income disregard equal to 5 percent of the federal poverty level. The ACA also eliminated resource limits. However, these changes do not apply to the elderly and disabled nor to the medically needy; for them, the traditional rules for counting income (based on the deduction of allowances) and resource tests still apply. For those who qualify, Medicaid coverage is extensive, and cost-sharing is generally minimal.

Two other programs also serve low-income people. The Children's Health Insurance Program (CHIP) provides health coverage to children in families with income too high to qualify for Medicaid but often too low to afford private health insurance. Basic Health Programs (BHPs) are intended for those with income somewhat above Medicaid eligibility levels and are intended to give low-wage earners continuity of coverage and of relationships with their healthcare providers.

Sources

- ¹ Kaiser Family Foundation, “Medicaid Timeline,” accessed on August 15, 2016 at: <http://kff.org/medicaid/timeline/medicaid-timeline/>.
- ² Kaiser Family Foundation, “Total Monthly Medicaid and CHIP Enrollment,” May 2016. <http://kff.org/health-reform/state-indicator/total-monthly-medicand-and-chip-enrollment/>.
- ³ Centers for Medicare & Medicaid Services, Medicaid.gov, “Eligibility,” accessed August 15, 2016 at: <https://www.medicaid.gov/medicaid-chip-program-information/by-topics/eligibility/eligibility.html>.
- ⁴ The Personal Responsibility and Work Opportunity Reconciliation Act of 1996 (Public Law 104-193).
- ⁵ Social Security Act, Section 1902(a)(10)(A)(ii)(XVIII) and 1902(aa); Social Security Act, Section 1902(a)(10)(A)(ii)(XII) and 1902(z).
- ⁶ Kaiser Family Foundation, “Medicaid and the Uninsured,” May 2012. <https://kaiserfamilyfoundation.files.wordpress.com/2013/01/8204-02.pdf>
- ⁷ National Federation of Independent Business et al. v. Sebelius, Secretary of Health and Human Services, et al., 132 S. Ct. 2566 (2012).
- ⁸ Kaiser Family Foundation, “State Health Facts,” updated July 7, 2016. <http://kff.org/health-reform/state-indicator/state-activity-around-expanding-medicand-under-the-affordable-care-act/>.
- ⁹ Social Security Administration, *Program Operations Manual System (POMS)*, Section SI01715.010. <https://secure.ssa.gov/poms.nsf/lnx/0501715010>
- ¹⁰ Kaiser Family Foundation, “Medicaid Financial Eligibility for Seniors and People with Disabilities in 2015.” <http://kff.org/report-section/medicaid-financial-eligibility-for-seniors-and-people-with-disabilities-in-2015-report>.
- ¹¹ Ibid.
- ¹² Kaiser Family Foundation, “Medicaid Eligibility Through the Medically Needy Pathway,” 2015. <http://kff.org/other/state-indicator/medicaid-eligibility-through-the-medically-needy-pathway/>
- ¹³ Centers for Medicare & Medicaid Services, “2015 Medicare Savings Program (MSP) Income Limits.” <https://www.cms.gov/Outreach-and-Education/Training/CMSNationalTrainingProgram/Downloads/2015-Medicare-Savings-Program-Job-Aid.pdf>
- ¹⁴ Centers for Medicare & Medicaid Services, Medicaid.gov, “Benefits,” accessed August 15, 2016 at: <https://www.medicaid.gov/medicaid-chip-program-information/by-topics/benefits/medicaid-benefits.html>
- ¹⁵ Ibid.
- ¹⁶ Centers for Medicare & Medicaid Services, Medicaid.gov, “CHIP/ Benefits,” accessed August 15, 2016 at: <https://www.medicaid.gov/chip/benefits/chip-benefits.html>
- ¹⁷ Centers for Medicare & Medicaid Services, Medicaid.gov, “Basic Health Program,” accessed August 15, 2016 at: <https://www.medicaid.gov/basic-health-program/basic-health-program.html>.

20 Medicaid LTC Coverage

Lesson Objectives

After completing this lesson, you should be able to:

- Explain the asset criteria a person must meet to qualify for Medicaid coverage of long-term care.
- Explain how income is treated for Medicaid long-term care coverage.
- Summarize the rules that apply when one spouse applies for Medicaid LTC coverage and the other does not.
- Describe the LTC services that are covered by Medicaid.
- Discuss the role of managed care in Medicaid.

In the last lesson we examined Medicaid coverage of medical care for low-income people. Now we turn to Medicaid coverage of long-term care (LTC).

Long-Term Care

First, let's clarify what we mean by "long-term care" by contrasting it with medical care. Medical care treats illnesses and injuries and seeks to restore normal functioning. It is delivered primarily by healthcare professionals and usually takes place in a hospital, clinic, or physician's office. In contrast, long-term care does not try to cure a condition, but rather help a person cope with a permanent or long-term loss of functioning. It involves supporting someone who is no longer able to perform activities of daily living (ADLs) like dressing and bathing, or supervising someone with a cognitive condition such as Alzheimer's disease. It takes place in a nursing home, the impaired person's home, an assisted living residence, or a community setting (such as an adult day center). LTC may be delivered by healthcare professionals, but more often it is provided by nonprofessionals such as home health aides or family members.

The Medicaid programs for medical care and long-term care differ in important ways. As we have learned, medical coverage is limited to low-income people; in contrast, many middle-class people rely on Medicaid to pay for long-term care. However, to do so, they must spend almost all their assets and most of their income on their care.

Physical Criteria

To meet the physical criteria for Medicaid LTC coverage, a person must, essentially, be in need of long-term care. She must be unable to take care of herself without help—specifically, she must have a physical (functional) impairment, meaning she is unable to perform activities of daily living without substantial assistance. Alternatively, she may be fully functional physically but need supervision because of a serious cognitive impairment (such as Alzheimer's).

Financial Criteria

An applicant must meet tests related to both assets (called “resources” by Medicaid) and income. In the section that follows, we will discuss the rules for assets and income when a single person applies for Medicaid LTC, or when both spouses of a married couple are applying. There are different rules when one spouse is applying and the other is not, discussed below under “Spousal Impoverishment.”

Assets (Resources)

Essentially, the total value of certain kinds of assets cannot be greater than the **asset eligibility limit**. This limit varies by state but is generally around \$2,000 for an individual and \$3,000 for a married couple (both applying).

Countable and Noncountable Assets

An applicant’s assets can be divided into two categories:

- **countable assets** are those that are considered in determining whether a person exceeds the asset eligibility limit, and
- **noncountable assets** (also called exempt assets) are those that are not.

Countable assets include:

- liquid assets such as cash, savings and checking accounts, and certificates of deposit;
- stocks and bonds;
- individual retirement accounts (IRAs) and other retirement funds;
- trusts (depending on the exact language and individual’s named interest, this may include trust income, trust corpus, or both);
- the cash surrender value of life insurance policies with a combined face value greater than \$1,500;
- items that may be converted into cash, including vacation homes and second vehicles; and
- in some cases, the applicant’s home (see below for further details).

Noncountable assets include:

- household goods and personal effects, such as furniture and clothing;
- one automobile, regardless of its value, if its primary use is the day-to-day transportation of a household member;
- the cash surrender value of life insurance policies with a combined face value of less than \$1,500;

- burial plots for the applicant and his or her immediate family, as well as burial funds for the applicant and his or her spouse; and
- the applicant's home in some instances (discussed below).

If an applicant's countable assets exceed her state's eligibility limit, she must spend her countable assets on her care until she has no more than the eligibility limit. Illiquid assets, such as houses and vehicles, must be sold and the proceeds spent on care. This is called "**spending down**." (Note that it is also permissible to use assets to pay living expenses, including mortgage and car payments, utilities, and credit card bills, and one can prepay funeral expenses.¹)

The Home

Medicaid's treatment of a person's home (primary residence) is complex. The rules can be summarized as follows:²

- If the applicant is living in the home and is applying for Medicaid medical coverage (not long-term care coverage) the home is noncountable, regardless of value.
- If the applicant is applying for Medicaid LTC coverage, the home is countable if the equity value is more than \$552,000 (or \$828,000 at the option of the state) (2016, adjusted annually), unless a spouse, dependent child, or disabled child lives in it.
- If the applicant leaves her home permanently to live in a nursing home (or other institutional setting), the home is countable, regardless of value, unless a spouse, dependent child, or disabled child lives in it.

Income

With regard to income, a state may take one of two approaches, or some states use both.

The Income Cap Approach

In the **income cap approach**, the state sets a **special income level (SIL)** that can be no higher than three times the Supplemental Security Income (SSI) Federal Benefit Rate (FBR). In 2016 the SSI FBR is \$733, so the SIL cannot be higher than \$2,199.³ Those with gross monthly income above their state's SIL are not eligible for Medicaid LTC coverage. However, in some states they may be able to qualify by establishing an irrevocable trust into which an income stream is directed (a so-called **Miller trust**). The income directed to a Miller trust may have to be spent on care (as discussed below).

***Example:** Harry lives in an income cap state with a SIL of \$2,199. He receives a pension of \$1,000 and Social Security benefits of \$1,500, for total monthly gross income of \$2,500, exceeding the SIL. So he establishes a Miller trust and directs his pension check to the trust. This lowers his income to \$1,500, below the SIL, and he qualifies for Medicaid. But for Medicaid purposes the \$1,000 pension payment in the trust is included in his other income, most of which he will have to spend on his care.*

The Medically Needy Approach

In the **medically needy approach**, the state Medicaid program compares an applicant's medical and LTC costs to her income. The methodology is similar to that for medical coverage (discussed in the last lesson), but applicants do not have to spend money on care before they can qualify—qualification is based on projected LTC expenses.

***Example:** Carol lives in a medically needy state with an income standard of \$325. Her gross monthly income is \$1,250, so her care must cost at least \$925 for her to qualify. Her nursing home charges \$5,000 monthly, so she easily qualifies.*

Spending Income on Care

Whether a person qualifies on an income cap or a medically needy basis, once she is on Medicaid she must contribute to the cost of her care out of her income.

- A Medicaid recipient in a nursing home must spend almost all of her income on her care. She may retain only a small personal needs allowance (usually between \$30 and \$90 per month, depending on the state) to cover such items as toiletries and reading material. She can also pay health insurance premiums out of her income.⁴
- A person receiving **home and community-based services (HCBS)**—long-term care at home or a site such as an adult day center—can keep more of her income, to enable her to pay for food, housing, etc. She retains a **maintenance needs allowance**, which ranges by state from \$625 to three times the SSI FBR (\$2,199 in 2016), or even higher in some states that allow Miller trusts.⁵ And there is a trend in states toward increasing this allowance, to encourage people to stay at home instead of moving into a nursing home, where care costs the state more.

To clarify how this works, let's return to our examples above:

***Example:** Carol has monthly income of \$1,250 and lives in a nursing home. She can keep \$60 for personal needs, and she can pay her Medicare Part B and Part D premiums, but the rest of her income goes to pay the nursing home, with Medicaid paying the balance.*

***Example:** Harry has income of \$1,500 plus \$1,000 that goes to a Miller trust. If he enters a nursing home he will have to spend all \$2,500 on his care except for a \$70 personal needs allowance and his Medicare premiums. If he is cared for at home, he will be able to keep more.*

Supplemental Needs Trust

A **supplemental needs trust** may be established by family members of a Medicaid recipient to enhance her quality of life. Money from the trust can be used for care not covered by Medicaid as well as living expenses that the person cannot afford, given the small Medicaid allowances. This might include dental work not covered by Medicaid, haircuts and beauty salon services, non-food grocery items, travel expenses to visit family members, and cell phone and Internet service. These trusts contain language that clearly specifies that funds are to be used to *supplement*, *not supplant* benefits otherwise available. If properly established, such trusts generally will not be considered a countable asset for Medicaid eligibility purposes.

Spousal Impoverishment

The Medicaid eligibility rules explained above give rise to a question: If one of the spouses of a married couple needs long-term care and applies for Medicaid, must almost all of the assets and income of the couple be spent on his or her care, leaving the spouse who does not need care with little to live on? Medicaid's **spousal impoverishment** rules are designed to ensure that the spouse who remains at home (called the **community spouse**) while the other spouse enters a nursing home retains a reasonable amount of financial resources.⁶ States must also apply these rules when a person does not enter a facility but needs home or community-based care,⁷ but for the sake of simplicity, we will refer here only to the "nursing home spouse." These rules are complex, and for the purposes of this lesson we will summarize them.

Income

If a community spouse has income of his or her own, the community spouse retains that income and does not have to spend it on his or her spouse's nursing home care. If the community spouse has little or no income of his own, some of the income of the nursing home spouse is set aside for his use and is not required to be spent on care. Specifically, the spousal impoverishment rules provide for a **minimum monthly maintenance needs allowance (MMMNA)** for the community spouse. In 2016 the amount of the MMMNA ranges from \$2,002.50 to \$2,980.50, depending on the community spouse's actual housing costs.⁸

The MMMNA works in this way:

- If the community spouse has no income of his own, he receives the entire MMMNA amount from the income of the nursing home spouse.
- If the community spouse does have some income of his own, he receives an amount from the income of the nursing home spouse necessary to bring his income up to the MMMNA level.
- If the community spouse's income is greater than the MMMNA, he receives nothing from the income of the nursing home spouse.

Example: Jane and Ted Smith are married. Ted enters a nursing home, and the Smiths apply for Medicaid. The applicable MMMNA is \$2,500.

First, let's suppose Jane has no income of her own. Jane can keep \$2,500 of Ted's income, the MMMNA.

Now, let's say that Jane receives \$1,000 per month from a trust left by a relative but has no other income of her own. Jane is allowed to keep \$1,500 of Ted's monthly income, which when added to her trust income of \$1,000 gives her a total monthly income at the MMMNA level of \$2,500.

Finally, what if Jane has \$3,000 in monthly income of her own? What happens? She can keep all of the \$3,000, but she receives none of Ted's income.

Assets

In general, a couple must spend all their countable assets above the eligibility limit on the care of the nursing home spouse, except for a **protected resource amount (PRA)** reserved for the community spouse. (All noncountable assets, including their home, are of course retained by the couple.) The amount of the PRA is set by each state within federal guidelines. All states must allow the community spouse to retain countable assets up to a minimum amount (\$23,844 in 2016). States have the option of also allowing the community spouse to keep half the assets in excess of the minimum, up to a certain limit. This limit cannot be higher than a federal maximum (\$119,220 in 2016), but it may be less than that maximum. For example, a state might let the community spouse retain half the assets up to \$50,000.⁹ In addition, if the entire income of the nursing home spouse is allotted to the community spouse and this is insufficient to bring the community spouse's income up to the MMMNA level, the community spouse may retain enough of the couple's assets to generate enough income to reach the MMMNA level.

Other Asset Considerations

Long-Term Care Partnerships

Most states have **long-term care partnerships**, programs that give people an incentive to buy private long-term care insurance (LTCI) so that the state Medicaid program will be less likely to have to pay their LTC costs. Essentially, if a person buys a partnership-qualified LTCI policy but later needs Medicaid because she has used up the benefits of her policy, she does not have to spend all her countable assets to qualify. In addition to whatever amount she could normally keep, she can retain an amount of countable assets equal to the total benefits she received from her LTCI policy.

***Example:** Mary buys an LTCI policy meeting the requirements of her state's LTC partnership. Several years later, she needs LTC and receives benefits from the policy. But she eventually uses up all \$300,000 in benefits payable by the policy. She then applies for Medicaid. Because she received benefits from the partnership policy, she can keep \$300,000 of countable assets she would normally have had to spend on care. And she can pass these assets on to her heirs.*

(Additional information on long-term care and ways of financing it can be found in the AHIP Long-Term Care series of courses.)

Transfers of Assets

Some people believe that, if they ever need long-term care, they can give their assets to family members, thereby reducing their countable assets to the Medicaid eligibility limit and qualifying for Medicaid. This is generally not the case. If assets are transferred within five years before a person applies, the person does not receive Medicaid coverage during a penalty period (with some exceptions). The duration of this period is set so that the person must spend on his care roughly the amount of assets he transferred.

***Example:** Jason gives assets worth \$200,000 to his daughter. A year later, he goes into a nursing home and applies for Medicaid. He will not receive Medicaid coverage during a penalty period. The length of this period is set so that Jason will have to spend roughly \$200,000 on his care before Medicaid begins paying.*

Estate Recovery

When a person who had Medicaid LTC coverage dies, the state seeks to recover from her estate the amount it paid for her long-term care. The estate may include the person's home and other noncountable assets. However, there are limits on estate recovery: Assets protected under an LTC partnership are also protected from estate recovery, as is the estate of a person who is survived by a spouse, child under age 21, or a blind or disabled child of any age.

Coverage

Medicaid coverage of long-term care varies from state to state, but federal guidelines require all states to provide certain coverage.

- All states are required to pay for nursing home care, and they must also pay for home healthcare for those who are **nursing home eligible** (those who would need nursing home care if they did not receive home health care).¹⁰
- Although federal guidelines do not require it, all states also cover home and community-based services for certain other Medicaid recipients.
- A few states pay for long-term care received in an assisted living residence.

As these rules indicate, Medicaid has had a bias toward facility care as opposed to HCBS. But this is changing, thanks to legislation, judicial decisions, and administrative flexibility, as well as the preference of most people to stay in their homes if possible. Many Medicaid recipients now have options beyond nursing homes, and HCBS now accounts for just over 50 percent of the **long-term services and supports (LTSS)** expenditures for the elderly and disabled.¹¹

However, HCBS coverage largely remains a state option. States seeking to offer HCBS coverage have done so largely through waivers to the regular Medicaid rules granted under Section 1915(c) of the Social Security Act. Under these 1915(c) waivers, the federal government authorizes a state to provide Medicaid services that are non-medical in nature, such as personal care services, adult day health services, adult day care services, habitation services, and respite care. The exact mix and nature of the services varies from state to state. Section 1915(c) waiver programs are not considered an entitlement—that is, Medicaid recipients have no right to enroll, and a state may manage its budget by capping enrollments. For this reason, these waivers remain a primary vehicle for providing HCBS.

The Affordable Care Act provides an alternative way for states to provide HCBS through amended Section 1915(i) of the Social Security Act. This option differs from 1915(c) waivers in two key ways:

- **Broader population / earlier intervention.** While the services that a state may offer are the same as under a Section 1915(c) waiver, they can be offered to more people. Recipients do not need to demonstrate the need for an institutional level of care—they can have a lower level of need. This allows a state to step in earlier and possibly prevent the need for facility care.
- **Lack of enrollment cap.** Unlike under a Section 1915(c) waiver, enrollment in a program initiated under Section 1915(i) cannot be capped or subject to a waiting list—it must be available to all who qualify. This means that a state will not have as much control of

expenditures; however, a state can limit and project enrollments and costs by making a program open only to specific populations.

The ACA also amended Section 1915 of the Social Security Act to create the Community First Choice (CFC) program (Section 1915(k)). CFC gives state Medicaid programs the option to include attendant care service and supports for those who need an institutional level of care. Like 1915(i) programs, these programs can have no enrollment cap.¹²

Medicaid and Health Plans

Medicaid—both medical and long-term care coverage—was historically a fee-for-service program, with little involvement by private-sector health plans. But this has changed dramatically. As of July 2014, over 75 percent of all Medicaid recipients received some or all services through a managed care delivery system. And by January, 2014, 20 states had operationalized Medicaid managed long-term services and supports (MLTSS).¹³ In the one-year period from 2013 to 2014, MLTSS enrollment grew 175 percent.

CMS has recognized the widespread and growing use of managed care delivery systems in Medicaid programs and responded by revising its managed care regulations. A new Final Rule effective as of April, 2016¹⁴ is broad in scope and aligns Medicaid rules with those of other major programs such as the Medicare Advantage plans we have studied. The rule addresses payment provisions, program integrity, and quality of care.

Key Legislative, Judicial, and Policy Events

In the table below, we list and briefly describe some of the laws, court decisions, and administrative actions that have affected Medicaid LTC coverage. Note that in addition to older adults and persons with disabilities, two other populations have received long-term services and supports through Medicaid: individuals with development disabilities and those with serious emotional disturbances or serious mental illness. Also, Medicaid home and community-based services support not just the elderly but individuals of all ages with a range of disabilities and chronic conditions, including those with spinal cord injuries due to automobile or sports accidents, severe cases of multiple sclerosis, cognitive disabilities (including dementia or the early stages of Alzheimer’s disease), intellectual or development disabilities (such as Down’s syndrome), and behavioral health issues.¹⁵

Key Legislative, Judicial, and Policy Events in Medicaid LTC Coverage¹⁶

Year	Event	Summary
1981	Omnibus Budget Reconciliation Act (OBRA-81)	Established SSA Section 1915(c) waivers to cover HCBS for the elderly and individuals with disabilities who would otherwise require Medicaid-covered hospital, nursing facility, or intermediate care facilities.
1982	Tax Equity and Fiscal Responsibility Act (TEFRA)	Gave states the option to extend Medicaid coverage to children with disabilities living at home who qualify for

		institutional services but are not otherwise eligible for Medicaid.
1987	Nursing Home Reform Act (part of OBRA 87)	Established more stringent quality requirements for nursing facility services. Made it easier for states to use 1915(c) waivers to provide HCBS to people with developmental disabilities (DD).
1990	Americans with Disabilities Act (ADA)	Specified broad protections against discrimination based on disability, including discrimination in services covered by public programs such as Medicaid.
1994	“Cold bed” rule repeal by CMS (then HCFA)	Administratively repealed a rule that required states to demonstrate a reduction in institutional capacity for each Section 1915(c) waiver enrollee.
1997	Balanced Budget Act of 1997	Established Medicaid Buy-In program enabling states to cover working individuals with disabilities with incomes up to 250% of the Federal Poverty Level.
1999	Olmstead vs. L.C.	Clarified decision that unnecessary institutional isolation of a person with a disability is a form of discrimination under ADA, requiring states to provide alternatives when practicable.
2005	Deficit Reduction Act (DRA)	Expanded states’ options to provide HCBS under Section 1915(i) of the SSA. Authorized the Money Follows the Person (MFP) demonstration to help Medicaid enrollees transition from institutional services to HCBS.
2010	Affordable Care Act	Created the Balancing Incentive Program, with financial incentives through FY 2015 to states to increase percentage of Medicaid spending on HCBS. Amended SSA Section 1915(i) to allow states to target services to specific populations and increase income

		standards. Created (under SSA Section 1915 (k)) Community First Choice, a new option that allows states to offer attendant services and supports.
--	--	--

Summary

Medicaid provides coverage of long-term care, but this differs from Medicaid medical coverage in important ways. Middle-income people can qualify, but they must spend their assets on their care until they have little left, and while on Medicaid they must spend most of their income on care. Spousal impoverishment rules ensure that a spouse not receiving Medicaid retains a reasonable amount of a married couple’s financial resources. Medicaid has traditionally focused on facility care, but there has been a major shift to home and community-based services (HCBS). Similarly, Medicaid has historically been a fee-for-service program, but managed care delivery systems are now playing a major role.

Sources

¹ 42 US Code Section 1382b – Resources, Subsection (a)(2)(B).

² 42 US Code Section 1382b – Resources.

³ Social Security Administration, SSI Federal Payment Amounts, <https://www.ssa.gov/oact/COLA/SSIAMts.html>]

⁴ 42 C.F.R. 435.832.

⁵ AARP Public Policy Institute, “Access to Long-Term Services and Supports,” Table A6 at http://assets.aarp.org/rgcenter/ppi/ltc/i44-access-ltss_revised.pdf.

⁶ 42 US Code Section 1396r-5 – Treatment of Income and Resources for Certain Institutionalized Spouses.

⁷ Section 2404 of the Affordable Care Act adopted a provision requiring use of spousal impoverishment rules when determining eligibility for HCBS. The provision is effective for the five-year period beginning January 2014. See a further discussion in CMS, SMD# 15-001 at <https://www.medicaid.gov/federal-policy-guidance/downloads/SMD050715.pdf>.

⁸ Centers for Medicare & Medicaid Services, Medicaid.gov, “SSI and Spousal Impoverishment Standards,” <https://www.medicaid.gov/medicaid-chip-program-information/by-topics/eligibility/downloads/2016-ssi-and-spousal-impoverishment-standards.pdf>]

⁹ Ibid.

¹⁰ Centers for Medicare & Medicaid Services, Medicaid.gov, “Nursing Facilities,” <https://www.medicaid.gov/medicaid-chip-program-information/by-topics/delivery-systems/institutional-care/nursing-facilities-nf.html>

¹¹ Centers for Medicare & Medicaid Services, Medicaid.gov, “Long-Term Services and Supports,” <https://www.medicaid.gov/medicaid-chip-program-information/by-topics/long-term-services-and-supports/downloads>

¹² Kaiser Family Foundation, The Kaiser Commission on Medicaid and the Uninsured, “Streamlining Medicaid Home and Community-Based Services: Key Policy Questions” (Issue Brief), March 2016, pp. 5-8.

¹³ Centers for Medicare & Medicaid Services, Medicaid.gov, “Medicaid Managed Care Enrollment Report,” <https://www.medicaid.gov/medicaid-chip-program-information/by-topics/delivery-systems/managed-care/medicaid-managed-care-enrollment-report.html>

¹⁴ Federal Register, “Medicaid and Children's Health Insurance Program (CHIP) Programs; Medicaid Managed Care, CHIP Delivered in Managed Care, and Revisions Related to Third Party Liability,” <https://www.federalregister.gov/documents/2016/05/06/2016-09581/medicaid-and-childrens-health-insurance-program-chip-programs-medicaid-managed-care-chip-delivered>.

¹⁵ Kaiser Family Foundation, The Kaiser Commission on Medicaid and the Uninsured, “Streamlining Medicaid Home and Community-Based Services: Key Policy Questions” (Issue Brief), March 2016.

¹⁶ Audra Wenzlow, Steve Eiken, and Kate Sredi (Truven Health Analytics for CMS), “Improving the Balance: The Evolution of Medicaid Expenditures for Long-Term Service and Supports (LTSS), FY 1981-2014,” June 3, 2016, at Medicaid.gov Long-Term Services & Supports page.

21 Fraud, Waste, and Abuse (FWA)

The amount spent annually on healthcare in the United States reached three trillion dollars in 2014 and continues to climb.¹ With this much money at stake, it is not surprising that a few of those involved in delivering, financing, or consuming healthcare engage in dishonest or even criminal conduct in order to obtain payments or benefits they are not entitled to. Improper activities—healthcare **fraud, waste, and abuse (FWA)**—make healthcare more expensive, and they also diminish its quality.

In this module we discuss the different forms of fraud, waste, and abuse; the major federal laws against FWA and the penalties for violating these laws; and the efforts of the government and health plans to detect and prevent FWA. After completing this lesson, you should be able to:

- Define the terms “fraud,” “waste,” and “abuse.”
- Explain how different provider compensation arrangements give rise to different kinds of FWA.
- Identify and describe the main federal laws related to healthcare FWA, including the criminal and civil penalties they impose.
- Discuss how health plans work to combat FWA.

What Are Fraud, Waste, and Abuse?

The phrase “fraud, waste, and abuse (FWA)” is commonly used to refer to a range of improper activities, and that is how we will use it in this lesson. But each of these words has a specific meaning.

- **Fraud** occurs when someone knowingly and intentionally makes a misrepresentation in order to obtain something she is not entitled to. It is usually an illegal act.

***Example:** Dr. Phillips knowingly and intentionally claims to have performed a service he did not in fact perform, in order to receive a payment from a health plan.*

- **Abuse** is behavior that is improper or deficient in some way and usually results in someone receiving something they are not entitled to. But unlike fraud, abuse *does not* involve willful misrepresentation or the violation of laws, regulations, or provisions of a contract.

***Example:** Dr. Woods performs a service for Julio that he sincerely believes to be necessary, and bills Julio’s health plan for it. But in fact, the service is no longer considered appropriate for Julio’s condition. This is not fraud, because there is no*

*misrepresentation—Dr. Woods did provide the service. But it is **abuse** because the health plan makes an unwarranted payment.*

- **Waste** is mismanagement, inappropriate actions or failure to act, or inadequate oversight that results in those paying for healthcare (health plans, the government, consumers) not receiving reasonable value for their money. It does not generally involve any violation of law.

***Example:** A group medical practice is poorly organized and managed, so that staff frequently spend time reentering information when patients see different physicians. As a result, the practice must charge consumers and health plans more to cover higher operating costs.*

Note that, while we can give examples of typical cases of these three types, the distinction between them is not always clear-cut. Waste may shade into abuse, and sometimes only a court can decide if an act constituted fraud rather than abuse. Nonetheless, it is important to understand the differences, and especially to keep in mind that fraud is an illegal act and often has a very specific meaning under criminal laws.

How Provider Compensation Affects FWA

You may recall from *Healthcare Management: An Introduction* (AHM 250) that health plan provider compensation arrangements form a continuum, with fee-for-service (FFS) at one end and capitation at the other.

- Under **fee-for-service**, providers are paid an amount for each service they have performed. The more services a provider delivers and bills for, the greater her compensation.
- Under **capitation**, providers are paid a flat amount to provide a certain type of services to a certain number of plan members, as services are needed. (Typically, a monthly fee is paid per member.) Whether the provider delivers many or few services, or none at all, she receives the same compensation.

There is a variety of other compensation arrangements. Some are similar to capitation in that the provider receives a global fee for providing care, rather than being paid for each service delivered. Some combine FFS and capitation-like payments. And often a health plan uses different arrangements for different types of services. Understanding these arrangements is important, because different arrangements generally lead to different kinds of FWA. In this section we will look at FWA under pure FFS and under pure capitation, to give a sense of the difference.

FWA in Fee-for-Service

As noted, under fee-for-service, the more services a provider delivers and bills for, the more he is compensated. This gives providers an incentive to bill for services they have not actually performed, or bill improperly in order to receive more than they are entitled to, or perform services that are unnecessary (**overutilization**).

- **False claims** involve billing for services that were not actually delivered.

***Example:** Dr. Chiu bills Melissa's health plan for tests he performed, but also for several tests that were not performed.*

- **Billing schemes** involve billing for services that were delivered, but in a way that inappropriately increases the amount received.

***Example:** Isabel is seen by a nurse in Dr. Miller's office, but the office bills her health plan for a visit with a physician, which costs more. This is called **upcoding**. (Letter-number codes are used to bill for healthcare services, and the wrong code is used.)*

- **Overutilization** occurs when a provider actually performs the services she bills for and bills for them correctly, but the services are not medically necessary, and she knows this.

***Examples:** Dr. Nicoletti sees Bill more often and provides more treatments than are required by his condition, with the aim of obtaining extra fees. Dr. Bates has tests performed on Jeremy that are not really warranted by his symptoms, so that he will receive greater compensation.*

FWA in Capitation

Under capitation and similar arrangements, providers are not paid more for providing more services, so there is no incentive for them to bill for services not performed or to perform unneeded services. Consequently, FWA is less of a concern than under FFS. But it does occur, in different forms.

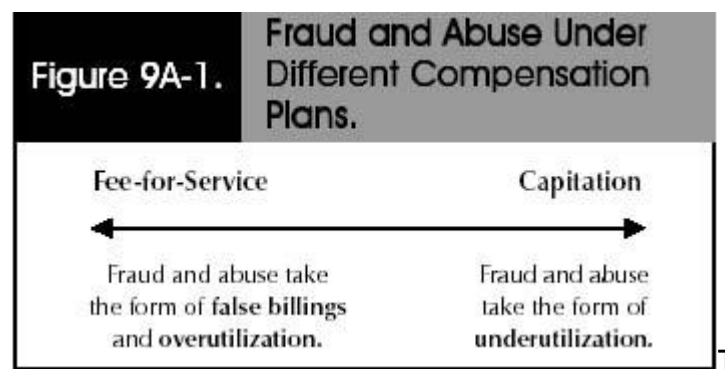
The most common type of fraud and abuse in capitation-type arrangements is **underutilization**—providers do not perform services that are medically necessary, or they provide substandard care. This is advantageous to the providers, since they receive the same compensation whether they deliver more or fewer service, and if they provide fewer their costs are lower, and they may be able to spend more time with other patients not covered by the capitation arrangement. When underutilization happens plan members do not receive the medical treatment they need, and the plan does not receive the services it has contracted and compensated the provider for.

***Examples:** Dr. Thomas doesn't see Jason as often as he should, given Jason's condition. Dr. Chatterjee's office delays the scheduling of requested appointments, so that the total*

number of appointments is reduced. Dr. Lowell often has his nurses or assistants see patients when he really should see them himself (substandard care).

Other forms of FWA also occur in capitation environments. One is **over-reporting of patient encounters**—providers overstate the number of times they serve health plan members in reports submitted to a plan. Plans rely on these reports to determine how much care physicians are providing and therefor how large future capitation payments should be. So while a provider who over-reports his encounters does not receive any additional income at the time, in the future he may receive capitation payments that are larger than justified.

Figure 9A-1 provides a quick overview of what types of FWA are associated with each type of compensation.



The Impact of FWA on the Cost and Quality of Healthcare

An enormous amount of money is lost to FWA. As an example, just in federal health benefit programs, it is estimated that in 2014 nearly \$124.7 billion went to fraudulent or improper payments.² The increased costs are clear under FFS, when providers bill for services not performed or not needed. But underutilization under capitation also has a financial impact, as health plans, plan members, and employers pay for services that are not delivered. The cost of FWA is borne directly by health plans and government programs (which pay more in benefits and provider compensation than they should) and indirectly by consumers, employers, and taxpayers (who pay more for coverage).

FWA affects not just the cost but the quality of healthcare. This is obvious with underutilization, when patients do not receive the care they need. But overutilization can also be harmful—when people receive treatments that are not necessary or appropriate for their condition, they may be exposed to unnecessary health risks. And when providers falsify a person’s diagnosis or misreport the treatments she has received, her future healthcare will be based on an erroneous medical history, which can lead to all sorts of problems.

Example: *Dr. Larson bills Jeanette’s health plan for several tests that he did not in fact perform on her. Her medical record now shows that she had these tests and the results were*

negative. Other doctors may refer to these negative results when treating Jeanette, which could result in a misdiagnosis.

The Laws Against Fraud

At the state level, health plans and other victims of FWA can sue perpetrators based on the common law definition of fraud, and most states have also enacted statutes making healthcare fraud a crime, often a felony. In addition, there are important federal laws in effect nationwide. Some of these focus on FWA in federally funded programs such as Medicare and Medicaid, but others can be deployed against fraud in the private sector as well. For instance, HIPAA (discussed below) makes it a federal crime to obtain money from a health plan (public or private) by fraud or false pretenses. And even those laws aimed at FWA in government programs are important for private-sector health plans, as many plans partner with the government to provide coverage through Medicare Advantage plans, Medicare Part D prescription drug plans, some Medicaid programs, and others. In this section we will discuss the most important federal laws.

FWA in Federal Health Benefit Programs

The federal government pays for healthcare for a large number of Americans, through Medicare, Medicaid, and other programs. These programs still largely compensate providers on a fee-for-service basis, so fraud is largely in the form of false billings, billing schemes (such as upcoding), and overutilization. However, there is a movement toward capitation and similar arrangements, so underutilization and other types of FWA associated with managed care are becoming more common.

HIPAA

The Health Insurance Portability and Accountability Act of 1996 (HIPAA) is broad legislation that regulates health insurance in many ways. In the area of FWA, HIPAA established **federal healthcare offenses**—it is a federal crime to engage in a scheme to defraud a health plan or obtain money from a health plan under false pretenses. “Health plans” include both government programs and private-sector health insurance plans. Some of the practices prohibited by HIPAA include requesting payment for services that were not provided, misrepresenting the nature of services that were provided, and falsely certifying that provided services were medically necessary. Penalties include fines of up to \$250,000 and prison terms of up to 10 years. HIPAA also provides funding for anti-FWA activities, as some of the money from fines is turned over to enforcement agencies to conduct additional investigations.

The Affordable Care Act (ACA)

The Affordable Care Act, another healthcare law with very broad scope, includes powerful measures to combat FWA. In the first three years of the law, the government recovered a

record-breaking \$10.7 billion from fraud. Some of the new tools the ACA has put in the hands of fraud fighters include:

- **Tougher sentencing.** Federal sentencing guidelines for healthcare fraud crimes resulting in over \$1 million in losses have been increased by 20 to 50 percent.
- **Enhanced screening.** Providers and suppliers who may pose a higher risk of FWA are now required to undergo more scrutiny, including license checks and site visits.
- **State-of-the-art technology.** To target resources to highly suspect behaviors, the Centers for Medicare & Medicaid Services (CMS) now uses advanced predictive modeling technology.
- **New resources.** The law provides an additional \$350 million over 10 years to boost anti-fraud efforts.
- **New initiatives.** The ACA created a partnership to bring together federal and state governments and the private sector and established the HEAT task force to combine and coordinate resources across the federal government. These anti-FWA initiatives are discussed below.

The Anti-Kickback Statute

The federal **anti-kickback statute** prohibits kickbacks in dealings involving any federal healthcare program. The definition of “kickback” under the statute is complex, but in general terms, a kickback occurs when one party directly or indirectly gives another party money or some other benefit in exchange for receiving increased business. This can encourage healthcare providers and health plans to take actions that are not in the best interests of patients and may increase healthcare costs.

Examples:

- *A physical therapy service makes payments to physicians and health plans when they refer patients.*
- *A drug manufacturer makes payments to physicians in exchange for prescribing its medications.*
- *A durable medical equipment company gives rebates and discounts to health plans and providers when they buy their products.*

However, there are some relationships and financial arrangements in the healthcare field that might seem to violate the prohibition against kickbacks but do not in fact create problems, and

regulations have deemed a number of such situations permissible (“safe harbors”). But if there is a violation of the statute, penalties include fines of up to \$25,000 and prison terms.

The Stark Laws

The **Stark laws** (the Ethics in Patient Referrals Act) prohibit a physician from referring Medicare or Medicaid patients for certain services or supplies (lab tests, therapy, medical equipment, etc.) to an entity in which the physician has a financial interest (such as ownership, investment, or a compensation arrangement). This is intended to prevent situations in which a physician makes a referral based not on the patient’s needs but on the physician’s financial gain. (Note, however, that there are numerous exceptions to the Stark laws.) If a violation occurs, the physician’s claims are not payable, she may have to pay penalties of up to \$15,000 for each service referred and up to \$100,000 for entering into an unlawful arrangement, and she can be excluded from Medicare and Medicaid.

Other Federal Laws

There are other federal laws that do not focus on healthcare but can be used to fight healthcare FWA.

- The **False Claims Act (FCA)** makes it illegal to present a false, fictitious, or fraudulent claim against the United States. This law has been used to prosecute fraudulent billings under the Federal Employees Health Benefits Program (FEHBP), Medicare, and Medicaid, as well as other federal contracts. In addition to fines and prison terms, additional penalties can be imposed equal to up to three times the amount of the false claim.
- The **mail fraud statute** makes it illegal to use the mail to further a scheme to defraud. This statute is useful in fighting healthcare fraud because the mail is often used to submit false bills. Penalties include fines of up to \$250,000 and prison terms.
- The **Racketeer Influenced and Corrupt Organizations (RICO) Act** makes it illegal to associate with a criminal enterprise and participate in its activities. It was originally enacted to fight organized crime, but it has also been used against healthcare fraud. Penalties include fines of up to \$25,000 and prison terms of up to 20 years.

Enforcement and Prevention

The Enforcers

Who enforces these federal laws against FWA?

- The Office of the Inspector General (OIG) of HHS has responsibility to identify and eliminate FWA in programs administered by HHS (such as Medicare and Medicaid) and

to enforce the federal FWA laws for those programs; the OIG does so through a nationwide program of audits, investigations, and inspections.

- **Medicaid fraud control units (MFCUs)**, run by the states with joint state and federal funding, investigate and prosecute fraud in the Medicaid program and have enforcement authority for Medicaid similar to the OIG's authority for Medicare.
- The Department of Justice and the FBI also investigate violations of federal laws that may result in criminal penalties.

Penalties

As we have seen, the federal FWA laws provide for a range of criminal and civil penalties.

- **Criminal penalties** include fines and prison sentences and can be severe. For example, under HIPAA someone who defrauds a health plan can be sentenced for up to 10 years.³ If the violation causes a serious bodily injury, it can be up to 20 years, and if it causes death, life imprisonment.
- **Civil monetary penalties (CMPs)**—fines of various amounts as well as damages—are imposed for civil violations of these laws. Under the False Claims Act a perpetrator may have to pay damages of up to three times his fraudulent gains.

***Example:** In one case, a doctor running a physical therapy company was convicted of healthcare fraud and ordered to pay \$16 million in restitution. But the government also brought a civil suit under the FCA, and damages could go up to three times the government's losses of \$45 million.⁴*

- **Exclusion** from federal health benefit programs may be imposed. A provider or health plan that has violated federal law may be prohibited from participating in Medicare, Medicaid, and other federal programs for up to five years. This can have a serious financial impact on many plans and providers.

The Affordable Care Act and Anti-FWA Initiatives

A Public-Private Partnership

Under the ACA, a ground-breaking partnership uniting public and private organizations in the fight against FWA was initiated. This voluntary collaboration includes the federal government, state officials, several leading private-sector health insurance organizations, and anti-fraud groups. The participants work together to share information and best practices to improve fraud detection, prevent payment of fraudulent billings, and detect and stop scams that affect both public and private payers. In addition, the partnership:

- helps those on the front lines of industry anti-fraud efforts share their insights with investigators, prosecutors, policymakers, and others;
- helps law enforcement officials identify and prevent suspicious activities;
- protects patients' confidential information; and
- uses the full range of tools and authorities provided by the Affordable Care Act and other laws to combat and prosecute illegal actions.

HEAT

Under the ACA, the Department of Health and Human Services and the Department of Justice joined together to create the **Health Care Fraud Prevention and Enforcement Action Team (HEAT)**. With the creation of HEAT, the fight against Medicare fraud became a cabinet-level priority. HEAT's mission:

- To gather resources across the federal government to help prevent FWA in the Medicare and Medicaid programs.
- To crack down on the people and organizations that abuse the system and cost Americans billions of dollars each year.
- To reduce healthcare costs and improve the quality of care by preventing fraudsters from preying on people covered by Medicare and Medicaid.
- To highlight best practices by providers and organizations dedicated to ending FWA in Medicare.
- To build on the existing partnerships between HHS and DOJ to reduce fraud and recover taxpayer dollars.

Health Plans' Anti-Fraud Efforts

SIUs

FWA costs private-sector health insurance plans billions of dollars a year, and plans work hard to combat it. Most plans have **special investigative units (SIUs)** that oversee and carry out the various activities of the plan to detect, deter, and prevent FWA. SIUs are made up of specially trained and experienced individuals and include claims examiners, statisticians, attorneys, certified medical coders, nurses and medical professionals, and people with a law enforcement background. SIUs apply technology (such as data mining and algorithms) to the plan's claims, receive tips from consumers and others, and investigate and pursue cases.

Detecting Fraud

Health plans take two main approaches to detecting incorrect or fraudulent claims, pre-payment and post-payment. In **pre-payment audit and review**, while claims are still in the adjudication process, those claims that have certain characteristics are identified (usually by electronic programs). These characteristics (called “edits”) are indications that the services billed may not have been performed or may not be billed correctly, or that some other form of FWA might be occurring.

***Example:** Dr. Rodriguez bills much more frequently for a certain test than other general practitioners. This may be a sign that he is having unnecessary tests done or billing for tests that were not performed. Or there may be a good reason why his patients are more likely to need the test than others. But it is probably worth investigating.*

Claims flagged in this way are referred to the SIU. The SIU reviews the claim and may ask the provider submitting it for more information. It may turn out that the bill is in fact valid, and it is paid; or it may be incorrect or even fraudulent, and it is not paid.

In **post-payment audit and recovery**, data on claims that have already been paid are analyzed, claims reviewed, incorrect claims identified, and efforts to recover the money paid made. Both the pre- and post-payment approaches require considerable resources, but they have different advantages and disadvantages:

- The pre-payment approach can delay payment to the provider, and under state laws, if a plan does not pay a claim within a certain time, it may have to pay a penalty. But if the claim is in fact incorrect and the plan catches it before it pays, it can avoid any financial loss.
- The post-payment approach does not delay payment, but it can be hard to recover money that has already been paid. Considerable documentation is required, and often cooperation from the provider is necessary. Sometimes recovery must be negotiated with the provider, so that the full amount is not repaid or repayment is deferred.

The efficient detection of invalid claims requires highly sophisticated technology and specialized personnel (statisticians, data analysts, and information technologists). It also requires a sufficiently large pool of claims data. If the software is not properly targeted or if the database is too small, the system will flag many claims that are in fact legitimate (false positives), and valuable time and resources will be wasted reviewing these claims instead of investigating truly invalid ones. This technology and personnel is costly, and small health plans may find it hard to maintain a sufficient database, so several companies offer anti-fraud services to health plans.

Corporate Compliance Programs

Health plans must take steps to ensure that they themselves to do not engage in any behavior that could result in payers (employers, the government, or plan members) spending more than they should, or that could even constitute fraud, waste, or abuse. To do so, many health plans have corporate compliance programs. If such a program is in place, showing that a plan is making a good faith effort to prevent FWA, the penalties of federal laws may be reduced if a violation does occur. The Office of the Inspector General (OIG) of HHS assists plans in their efforts to comply with the federal FWA laws in these ways:

- issuing Special Fraud Alerts that identify segments of the healthcare industry that are particularly vulnerable to abuse,
- developing guidance for model compliance programs for hospitals and other providers that health plans can customize to meet their own needs, and
- issuing advisory opinions to those seeking guidance on questions arising under the federal FWA statutes. For instance, a health plan might ask whether an activity it intends to engage in will violate the law.

Summary

Fraud, waste, and abuse make healthcare more expensive for everyone and can diminish its quality. When providers are compensated on a fee-for-service basis, false claims, billing schemes (such as upcoding), and overutilization occur. Under capitation and similar compensation arrangements, underutilization is more common. A number of federal laws can be deployed against healthcare fraud and provide for penalties against perpetrators—these include HIPAA, the False Claims Act, the Anti-Kickback Statute, and others. The Affordable Care Act brings new tools, resources, and initiatives to the fight. Health insurance plans work to combat FWA through SIUs, technology, and compliance programs.

Notes

¹ Centers for Medicare & Medicaid Services. 2015. National Health Expenditure Accounts. <https://www.cms.gov/research-statistics-data-and-systems/statistics-trends-and-reports/nationalhealthexpenddata/nationalhealthaccounts/historical.html>.

² According to the U.S. House of Representatives Ways and Means Committee, as reported in: Paul McCloskey. 2015. "The Fight Against Healthcare Fraud." *GCN*. <http://gcn.com/articles/2015/08/18/health-care-fraud.aspx>.

³ U.S. Code, Title 18, Chapter 63, Section 1347.

⁴ Beyond Health Care Reform. February 2, 2016. "Criminal Conviction Leaves Defendant Defenseless in FCA Civil Action." <http://beyondhealthcarereform.com/criminal-conviction-leaves-defendant-defenseless-in-fca-civil-action/>

22 Health Plan Governance

After completing this lesson, you should be able to:

- Explain what it means to say that the board of directors of a health plan governs the organization.
- Distinguish boards of not-for-profit plans from those of for-profit companies.
- Discuss issues in the composition of a board.
- Identify the main board committees and their functions.
- Discuss the relationship of the board and senior management.
- Identify the main duties and responsibilities of a board, including duty of care, of loyalty, and of oversight.
- Discuss the legal liability of a board and its members and the protections against it.

Governance

Governance may be defined as the efforts by the board of directors of an organization (such as a health plan), working with senior management, to:

- create a corporate vision and mission,
- develop corporate policy, and
- develop strategies to achieve the organization's mission and goals.

As part of governance, a health plan's board of directors:

- hires the CEO and may be involved in hiring other top officers and managers;
- provides guidance and oversight of the business of the plan; and
- exercises control over the plan by approving or not approving actions performed and decisions made or proposed by the officers.

However, the board is not usually involved in the day-to-day decisions of operating the business. As a result, board members have more objectivity and a broader perspective, and this enables them to oversee the organization's efforts to meet strategic goals. If the board becomes too involved in implementing policies, it may lose sight of these broader goals.

The terms "governance" and "management" are sometimes used interchangeably, but they are not the same thing. Recognizing the difference makes it easier to understand the different roles and responsibilities of the board and of the CEO and senior management. In simple terms, *governance is*

policy development and management is policy implementation. The board decides *if* an organization will do something, and management decides *how* it will be done.

Does this mean that management does not participate in policy development or that the board does not monitor the implementation of a policy or occasionally fine-tune it? No. There is rarely an absolute division between these two intertwined functions. The practical distinctions between governance and management must be tailored to fit each individual organization, and it must be revisited and updated as new circumstances and issues arise.

In a stock company, the shareholders (the owners of the company) participate in and influence governance. Specifically, common shareholders have voting rights.

- They elect members of the board. Shareholders may also call for a vote to terminate the current board if they are dissatisfied with the company's performance.
- They approve board actions at annual or special meetings of the shareholders. In most cases, voting can be done in person at these meetings or by proxy.
- They have the right to approve or disapprove mergers and acquisitions by vote.

To improve governance, many health plans are implementing **enterprise risk management (ERM)** programs. The NAIC has adopted the Corporate Governance Annual Disclosure (CGAD) model act¹ and the Own Risk Solvency Assessment (ORSA) model act,² which require ERM. These models are being enacted by many states, and they apply to state-regulated insurance companies. Since most health plan organizations include at least one state-regulated insurance company, they are affected by the ERM requirement.

What is ERM? We will explore this in more depth in the next lesson, but for now we offer a broad overview. **Enterprise risk management** is a framework of risk identification, controls, and reporting designed to provide the knowledge to make wise risk-based decisions while enabling the board to oversee the organization effectively.³

An ERM program usually includes three steps:

- determining existing and emerging risks,
- assessing the company's risk tolerance in each area, and
- implementing actions to minimize risk.

An ERM framework includes these five key elements:

- Identifying potential risks related to the organization's objectives and strategic plan
- Assessing the likelihood that a risk (event) will occur and the effect it will have
- Monitoring the occurrences of risks and their effects
- Managing the occurrences of risks and their effects

- Reporting the occurrences of risks and their effects (to the board and, as required, to regulators)

Under the ERM framework, each entity in an organization has an **enterprise risk governance team (ERGT)**, which embeds ERM oversight throughout the organization. Ultimately, the ERGT at the highest level of the plan's organizational chart will provide a leadership prototype for ERM across the organization.⁴ The appointment of a **Chief Risk Officer** or **ERM Program Manager** is also critical to the implementation and ongoing success of ERM.⁵ ERM requires that the board provide an effective risk governance structure that clearly defines and articulates roles, responsibilities, and accountabilities. The Board has ultimate accountability for risk decision-making and senior management has day-to-day responsibility of risk management.⁶

Under CGAD and ORSA, health insurer boards are held accountable by regulators for the effective oversight of all risk areas of the company.⁷ These new laws require an insurer to submit an annual report including information about its governance structure, risks, mitigations and controls, and oversight framework.

All organizations must address governance, but health plans face a number of special challenges, as we will see in this lesson and the next. For instance, health plans typically combine the financing and delivery of healthcare, which means combining a wide variety of functions and relationships into one organization.

***Example:** A health plan contracts with an independent practice association (IPA) to provide care for its members. The plan must ensure that the IPA adheres to practices and procedures set by the plan. But the IPA has its own corporate vision, culture, and goals that may differ from those of the health plan, and its own board and governance structure.*

***Example:** A health plan's board sets the goal of increasing membership. To do so, it needs to provide potential purchasers with detailed data on medical outcomes for its members, and it must obtain this data from its affiliated providers. The plan must have in place a system that fosters trust and cooperation among providers and plan managers.*

For-Profit vs. Not-for-Profit Organizations

Boards are similar for for-profit and not-for-profit health plans. But there is one major difference:

- For-profit boards are accountable to shareholders, and at least part of their corporate mission is to earn a return on shareholder investments.
- Not-for-profit boards do not answer to shareholders, nor is it their goal to earn a return on investment. Not-for-profit boards may be responsible to stakeholders, including plan members, regulators, and/or the community, if the organization's goals are not met. However, their primary duty is to the organization itself, not members or other interested parties.

Differences Between the Boards of For-Profit and Not-for-Profit Organizations

Board of a For-Profit Company	Board of a Not-for-Profit Organization
Elected by shareholders	Self-perpetuating, or elected or approved by members of other organizations
Directly accountable to shareholders	Duty is to organization, but may be accountable to members, regulators, and/or the community
Generally small in number	Generally large in number
Highly structured	Loosely structured
Directors' terms of service often limited by age only	Directors' terms often a set number of years and service limited by age
Directors more highly compensated	Directors receive lower compensation
Directors do not participate directly in raising capital	Directors may participate in raising capital

Some for-profit health plans are subsidiaries of not-for-profit organizations. Some are joint ventures of for-profit and not-for-profit entities. This can create special challenges:

- Although for-profit and not-for-profit boards operate similarly and have similar goals (a well-functioning health plan), their cultures are often very different and their perspective on organizational goals and strategic plans may clash.
- Such organizations may struggle to implement an ERM governance sensibility among board members.
- State regulations may govern the conversion of charitable assets.

Board Composition

An organization's articles of incorporation usually set a minimum number of directors, and state laws may also set a minimum. Some state laws require representation of health plan members, consumers, or providers on boards; some limit the number of providers (discussed later in the lesson). Federally qualified HMOs must meet requirements for board composition in the HMO Act of 1973 and related regulations.

Under CGAD, state insurance regulators will now review the makeup of a health plan board for appropriate background and experience. Regulators will seek to ensure that a board as a whole possesses the core competencies needed for oversight of a health plan:

- Financial literacy

- Accounting
- Business judgment
- Industry knowledge
- Management
- Leadership
- Vision
- Strategy

Because of fears of hacking, many state regulators are now adding cybersecurity to this list.

Boards sometimes include both inside and outside directors. An **inside director** is an employee, officer, or stakeholder of the organization; an **outside director** is not. Inside directors have first-hand knowledge and experience of the operations of the organization; outside directors bring other experience, knowledge, and perspectives. The proportion of inside to outside directors varies from organization to organization.

Some health plan boards include plan members to represent the interests of all members. Most health plans have consumer representatives on their grievance and appeals committee. Some state laws require board representation of members, consumers, or providers. For example, Arkansas requires that certain categories of providers and consumers be represented on HMO boards. Some state laws limit the percentage of providers on boards.

A chairperson is often elected by the board. Her duties typically include:

- scheduling board meetings,
- developing agendas,
- presiding at meetings,
- overseeing the board evaluation process, and
- planning for leadership succession.

Sometimes the chairperson is also the CEO of the organization, but often a separation of these two roles is desired or even mandated by regulation.

Some integrated delivery systems (IDSs) have a **representational board**. That is, IDS directors usually include physicians or hospital administrators who represent a specific constituency or organization that is part of the IDS. This may lead to **representational governance**—board members focus too much on the interests of the components they represent and not enough on those of the whole enterprise. This can violate directors' fiduciary duty to the enterprise. A balance must be struck. Constituent interests can and should be represented by directors, but a board must require a focus on the entire entity,

rather than allowing one component's representatives to drive its decisions. Enterprise risk management (ERM) can help a board achieve this.

The boards of for-profit provider-sponsored organizations (PSOs) often have several provider directors as well as outside directors who are not providers. Sometimes, the provider directors tend to continue operating the health plan like a medical institution, while the directors with a financial or business background understand that it must be run as a business if it is to succeed.

Board Committees

To help a board fulfill its responsibilities, there are usually several standing (permanent) committees, often including an executive committee. Board members may participate in one or more committees. A board can delegate some powers to its executive committee or other committees. State laws, the organization's articles of incorporation, or its bylaws typically provide for committees that have the power to exercise the authority of the board. However, committees may not act on certain substantial matters. They may not generally authorize distributions, approve proposed shareholders, fill board vacancies, or amend the articles or bylaws.

The number and types of committees vary, but they may include the following:

- Executive Committee
- Strategic Planning Committee
- Compensation Committee
- Nominating Committee
- Audit (or Audit and Compliance) Committee
- Governance, Risk, and Compliance (GRC) Committee
- Finance Committee
- Quality Management Committee
- Grievance and Appeals Committee

The Executive Committee handles issues related to overall policy for the organization, including lines of business the organization participates in, risk tolerance oversight, general employment policies, locations in which the organization operates, etc. The Executive Committee often acts on behalf of the full board between board meetings or when emergencies arise. This committee may also be responsible for strategic planning for the organization if a separate strategic planning committee does not exist.

The Strategic Planning Committee is responsible for developing the strategic direction for the health plan. This committee receives recommendations from management and potentially other board committees on determining and updating strategic goals of the company. This committee is also responsible for at least the first review and approval of the business plan. In some health plans, the final

approval or adoption comes from the full board. This committee considers proposed transactions such as mergers and acquisitions and makes recommendations to the board on them.

The Compensation Committee handles compensation of the CEO and sets general compensation and benefits policies for the organization. This committee may also function as the nominating committee (for the board and executive officers) if a separate nominating committee does not exist. It may also have responsibility for overseeing succession plans for the health plan and the board (or there may be a separate committee for this).

The Nominating Committee annually recommends to the full board nominations for new board members and possibly company officers, as required in the organization's bylaws. Some states require that the CEO of a health plan be hired through an application and qualification process; if so, the nominating committee performs the vetting process and makes recommendations to the board.

The Audit Committee arranges for external audits of the organization's operations and receives regular reports from the external auditors. It may also review regular reports by internal audit personnel to assess the organization's various risks. The audit committee is responsible for reporting significant financial risks to the full board.

In some companies, an Audit and Compliance Committee is responsible for compliance; in others, there is a separate Compliance Committee. Compliance responsibilities include ensuring that a compliance program is instituted and followed throughout the organization and that an annual review of the effectiveness of the compliance program is conducted.

To comply with CGAD and ORSA, many health plans have formed a Governance, Risk, and Compliance (GRC) Committee separate from the audit committee, to be responsible for oversight of enterprise risk management (ERM). Under CGAD, this committee also has oversight of actuarial function, broad investment policy, reinsurance decision-making, business strategy/finance decision-making, internal audit/financial reporting, compliance function, and market conduct decision-making. Many health plans also establish a risk management committee made up of plan staff representing different components of the plan. This committee works closely with the plan's chief risk officer and makes regular reports to the board.

There may be a Finance Committee separate from the GRC committee. It is responsible for approval of budgets, review of financial results, review of the annual audit, and approval of outside funding sources, as well as determination of the broad investment policy and types of investments, in compliance with states restrictions.

Health plan boards usually have a Quality Management Committee to put in place and oversee quality improvement programs and receive reports on quality initiatives from staff-level committees. This committee also directly oversees fraud, waste, and abuse (FWA) risks and controls. Working with the GRC committee, the QM committee has oversight of the quality reviews for CMS's star ratings. Many health plans also have a QM committee made up of staff members, outside physicians, and sometimes board members. It works closely with the plan's medical director and makes regular reports to the board.

The Grievance and Appeals Committee oversees the procedures for plan members' appeals and grievances and ensures that they are addressed efficiently and effectively according to the plan's standards and state laws and regulations. This committee often includes consumer representatives.

In addition to standing committees, the board may be authorized by the bylaws to create one or more special or ad hoc committees to address a specific issue or need of the board. Unlike standing committees, which are normally made up only of board members, special committees may include both board members and others. Members of a special committee usually have expertise in the particular area that will help them address the issue best.

***Example:** A special litigation committee may be temporarily established to oversee a legal challenge regarding breach of fiduciary duty. That committee would be made up of at least one board member but may also include one or more attorneys or other litigation experts who are not directors.*

The Board and Management

The board hires the **chief executive officer (CEO)** and entrusts her with broad administrative powers. The CEO is responsible for:

- leading the organization in the pursuit of the goals established by the board in the strategic plan,
- managing the organization day to day, and
- recruiting and developing a management team that can successfully implement the policies decided on by the board.

The CEO is also the primary liaison between the board and management.

In choosing a CEO, a board must consider the type of organization and its structure, risk profile, and maturity. For instance, a start-up IDS may need a leader who can make decisions with no precedents to rely on, build the business, and develop coalitions with groups who are resistant to change and new approaches. An established health plan will have very different requirements. In addition to hiring the CEO, the board may appoint some other officers, such as the treasurer and the secretary. The CEO hires top executives, sometimes subject to board approval. These can include:

- Chief Medical Officer (CMO)
- Chief Financial Officer (CFO)
- Chief Marketing Officer (CMO)
- Chief Operating Officer (COO)
- Chief Risk Officer (CRO)

The medical director usually has responsibility for provider relations, provider recruiting, quality management, utilization management, and medical policy.

In free-standing plans or large operations, it is common to have a finance director or chief financial officer. The CFO is generally responsible for oversight of all financial and accounting operations. In some plans this may include billing, management information services, enrollment, and underwriting as well as accounting, fiscal reporting, and budget preparation.

The marketing director may or may not be an officer of the company. He is responsible for establishing a cohesive marketing strategy for the health plan as well as for oversight of marketing representatives, advertising creation, client relations, and enrollment forecasting.

In larger plans, it is not uncommon to have an operations officer or director. He usually oversees claims, enrollment, underwriting (unless finance is doing so), member services, office management, and other functions.

The risk manager may or may not be an officer. He is responsible for establishing a risk culture that supports accountability in risk-based decision-making, including implementation and oversight of an enterprise risk management (ERM) framework.

Board Duties and Responsibilities

We have discussed in general terms the role of a health plan's board of directors. For a list of more specific duties and responsibilities, see below.

Duties and Responsibilities of a Health Plan Board of Directors

Overall governance:

- Participate in developing the overall corporate strategic plan.
- Set major policies for the company (such as goals for customer satisfaction, employee morale, and quality performance).
- Authorize major transactions, such as mergers and acquisitions.
- Identify and fulfill sound strategic and financial objectives, giving adequate attention and devoting adequate resources to risks.
- Support and oversee relevant business planning and proactive resource allocation.
- Support and oversee reliable risk management processes across business, operations, and control functions.

Evaluation:

- Evaluate the results of the company's operations.
- Assess programs and determine their reliability, independently from senior executives.
- Report findings objectively and independently to the board or appropriate board committees.
- Approve hiring of and oversee external auditors.

Oversight of executives:

- Hire the CEO and possibly other top executives.
- Oversee or conduct effective performance evaluations and approve executive compensation.

The board:

- Recruit and orient new board members and assess board performance.
- Ensure adequate competency (industry experience, knowledge, skills) of the board as a whole.
- Enforce adequate independence and active participation of members of the board.
- Make available and frequently use multiple informal channels of communication among board, management, and internal and external auditors, in order to create and enable a culture of openness.

Ethics:

- Regularly review and formally approve the company code of conduct, established in cooperation between the board and management.
- Reinforce corporate adherence to sound principles of conduct and segregation of authorities.
- Adopt and oversee Sarbanes-Oxley provisions, whether or not mandated, including but not limited to auditor independence and whistle-blower provisions.

Corporate directors are bound by several specific legal duties and obligations as defined in the corporate codes of the corporation's domiciliary state, in the corporation's articles of incorporation and bylaws, and in other governing rules. The directors must act only within the particular power and authority of their positions as directors. Individual directors generally do not have independent legal authority to govern the corporation. This is the responsibility of the board as a whole.

In their decisions, directors of both not-for-profit and for-profit organizations are required by law to exercise "good faith business judgment." This means they must fulfill three duties:

- Duty of care
- Duty of loyalty
- Duty of oversight (duty to supervise)

Doing so will help them avoid liability for their decisions (discussed below).

The Duty of Care

Duty of care means that directors must exercise their duties in good faith and with the same degree of diligence, care, and skill that an ordinary, typical, reasonable person would be expected to display in the same situation. Some key criteria used by regulators to evaluate whether directors are fulfilling their duty of care:

- The director's exercise of **independent business judgment**. Did the director critically evaluate the recommendations of the corporation's officers and employees and make an independent decision, or did she simply follow the recommendations without independent thought?
- The director's **decision-making based on knowledge** of the facts and circumstances. Was the director's decision based on a consideration of the facts relevant to a particular issue, or was it based on other factors such as the opinions of other directors?
- The director's **preparation for and participation** in meetings. Did the director attend meetings at which relevant information was presented and discussions held, or was she chronically absent?

The Duty of Loyalty

Corporation directors act as **fiduciaries**—that is, they hold positions of special trust and confidence and therefore have certain special obligations. **Duty of loyalty** means directors must act in the best interests of the organization, putting this before their own personal interests. For example, a person may not use his position as director to further his personal business interests.

To fulfill their duty of loyalty, directors must notify the organization of actual or potential conflicts of interest. They may have to recuse themselves from participation (voluntarily not take part) in board decisions on matters where they have a conflict. Many boards develop policies that address conflicts of interest. Some state laws allow boards to excuse themselves from obligations made by a director who had a conflict of interest and did not disclose it to the board, unless the action is shown to be fair despite the conflict of interest.

The Duty of Oversight (Duty to Supervise)

A board has a **duty of oversight**—that is, a **duty to supervise** the organization's officers and others who act on its behalf, including outside advisers such as accountants and attorneys. If a board is notified or becomes aware that these people may not be adequately performing their duties, the board must use reasonable care to investigate and correct their conduct.

In exercising their duty to supervise, the board and its committees can use and rely on the advice of lawyers, accountants, and others. But doing so does not relieve the board or any of its members from their ultimate responsibility as the corporation's accountable governing body.

Tone at the Top

The phrase "tone at the top" refers to the ethical climate established and maintained by an organization's board, board committees, and senior management. Factors include:

- The experience and business reputation of board members;
- The board's and committees' independence from management;
- The extent of the board's involvement and oversight of activities;
- The degree to which the board raises and pursues with management difficult questions regarding strategic plans and goals, financial decisions, or company performance; and
- The appropriateness of the board's actions.

In evaluating whether a board has adequately established and maintained an appropriate “tone at the top,” we can ask questions like these:

- Does the board take steps to ensure an appropriate tone?
- Are the board and audit committee involved sufficiently in evaluating the effectiveness of the “tone at the top?”
- Does the board specifically address management's adherence to the code of conduct?
- Are the code and other company standards applied even-handedly at all levels of the company?

Liability

An organization's board, its directors, and its officers may be legally liable for some actions.⁸

- The board of directors can be held liable as a group for its decisions.
- In unusual circumstances, directors can be held individually liable for board decisions; however, state laws and corporate charters limit directors' individual liability.
- The organization may be liable for the actions of its governing board or officers.
- Individual directors and officers may be personally liable for some actions.

There are four common ways to address liability:

- Enterprise risk management (ERM)
- State statutory limitations on liability
- Indemnification provisions in corporate bylaws
- Liability insurance

Of course, the best way to avoid liability is to ensure that actions for which an organization or individuals may be liable do not occur. One way to do this is through enterprise risk management—as mentioned

above, a framework for identification, controls, and reporting of risk, including risk of liability. (We will study ERM in greater detail in the next lesson).

Many states have laws limiting the liability of directors; some grant immunity from liability to directors of nonprofit organizations. However, state statutes include limitations and exclusions that leave openings for lawsuits.

An organization's bylaws can include an indemnification provision providing that, if a board member is legally obligated to make payments due to her liability for actions performed as a director, the organization will compensate her for her loss, including legal defense costs.

State laws and indemnification provisions in bylaws can protect individuals but not an organization. Liability insurance can reimburse both individuals and the organization.⁹ Most organizations purchase liability insurance for their directors and key officers. If directors and officers act in good faith and exercise duty of care but they or the organization are found liable, the insurer will reimburse them for losses. However, liability policies include limitations and exclusions and so do not provide total protection.

Keys to Effective Governance

Key elements of effective corporate governance at the board level include:

- Clearly defined roles and responsibilities
- Independent and actively participating board members
- Individuals who are suitable for their roles
- Directors who act in good faith and exercise a duty of care, loyalty, and oversight
- A board that provides sufficient oversight for all significant organization activities and key risk areas

Characteristics of effective boards include:

- A commitment to appropriate and continuing education of board members, including information about the health plan, the health care and insurance industries, and the board's duties and responsibilities.
- A regularly scheduled self-evaluation process for improvement of individual board members, committee functions, and the success of the board as a whole in its oversight responsibilities.
- An effort to develop itself and act as a single governing body, focused on a holistic risk position, without undue emphasis on any one component of the organization.
- The consistent application of analysis to complex problems and development of appropriate responses.

- An understanding of the economic, cultural, and regulatory environment in which the board and the organization operate.
- A vision of where the organization stands and where it is headed in the industry.
- Concrete, measurable goals for the board, management, and the corporate entity.
- The capacity to keep the organization on course in meeting its goals, using the corporate vision and mission as guides.

Summary

A health plan's board of directors is responsible for the governance of the plan. The board sets a mission and goals, develops a strategic plan, hires a CEO and other officers, and provides guidance and oversight. But the board does not usually get involved in the day-to-day management of the organization. Many health plans are adopting enterprise risk management in their governance, and new NAIC model laws require this for state-regulated insurers.

To be effective, board members must together have a variety of competencies. Board members must also exercise a duty of care (acting with good faith and diligence), a duty of loyalty (putting the interests of the organization first), and a duty of oversight (supervising the activities of the officers and others acting for the organization).

Sources

¹ National Association of Insurance Commissioners, "Corporate Governance Annual Disclosure Model Act," Model 305, 2015.

² National Association of Insurance Commissioners, "Own Risk and Solvency Assessment Model Act," Model 505, 2013.

³ Mark L. Frigo and Richard J. Anderson, "Embracing Enterprise Risk Management," COSO Committee of Sponsoring Organizations of the Treadway Committee, January 2011.

⁴ National Association of Insurance Commissioners, "NAIC Corporate Governance Annual Filing Guidance Manual," Section 1, October, 2013.

⁵ Ibid.

⁶ Carol S. Stern and C. J. Rathbun, "The Intersection of Risk Management and Corporate Governance," First Consulting & Administration, Inc., September 17, 2015.

⁷ National Association of Insurance Commissioners, "NAIC Own Risk and Solvency Assessment (ORSA) Guidance Manual," July, 2014.

⁸ Willis Group Holdings Limited, "Directors & Officers (D&O) Liability and Insurance: Frequently Asked Questions, 2014.

⁹ Ibid.

23 Accountability, Leadership, and Compliance

Lesson Objectives

After completing this lesson, you should be able to:

- Discuss in general terms the issue of accountability in health plans.
- Compare and contrast models of accountability, including enterprise risk management (ERM).
- Discuss how accountability is shaped by leadership and systems of organization control.
- Describe health plan compliance programs and compliance risk management (CRM).
- Explain how key elements of a compliance program—such as ongoing monitoring and audits, investigations, anonymous reporting, disciplinary action, and training—work.
- Discuss accountability for medical necessity decisions.

A number of terms that may be unfamiliar to you are used throughout this lesson. For a glossary, go to the end of the lesson.

Accountability

A primary driver of leadership and decision-making in health plans today is the demand for accountability and enterprise risk management.

- **Accountability** is acceptance of responsibility by a party. Specifically, a party willingly submits to a process by which one party is required to justify its actions and policies to another party. Accountability that is acknowledged and accepted will likely modify and motivate that person's actions to ensure certain minimal standards of quality.
- **Enterprise risk management (ERM)** is a process that places a greater emphasis on cooperation among departments to manage the organization's full range of risks as a whole.

In today's complex healthcare systems, a merger of accountability and risk management can create the most workable governance framework: an enterprise risk management program. An ERM program will support an effective level of risk management so that the health plan can identify, evaluate, prioritize, establish controls over, and report on its material risks, using a framework that is suited to the nature, scale, and complexity of those risks. This framework must be adequate to support reasonable and transparent risk and capital decisions throughout the organization.

Stakeholders

In the context of health plans, stakeholders include any individuals or organizations with whom a health plan interacts. Many of these stakeholders are held accountable and may also hold others accountable. The accountable stakeholders in health plans typically include:

- Health plan senior leaders
- Health plan boards of directors
- Hospitals and other institutional providers (such as skilled nursing facilities)
- Physicians and non-physician healthcare providers
- Professional medical associations
- Purchasers of services
- Third party vendors, suppliers, and consultants
- Private investors (in for-profit health plans)
- Individual plan members
- Lawyers and courts
- Some government agencies

Functions

Each stakeholder may have accountability in one or more of the following measurable functions:

- Professional competence (how capably the stakeholder does his or her job)
- Legal and ethical conduct (the stakeholder's adherence to legal and ethical standards)
- Financial performance (obtaining a return for other stakeholders)
- Equity of access (giving members and potential members equal opportunity to obtain healthcare)
- Public health promotion (making a contribution to help prevent illnesses and accidents in the general populace)
- Community benefit (being a good "corporate citizen" and, in the case of certain not-for-profit health plans, adhering to a charitable or community benefit mission)

Standards

For each area of accountability, there are sets of behavioral or quality standards against which performance is measured. For a particular person or entity there may be multiple sets of standards for accountability, each tied to a different stakeholder. For example, with regard to professional competence for health plans, different sets of performance standards may be established:

- through case law;

- by employers (directly through their agents or through the accreditation organizations); and
- by government agencies (in the form of state and federal regulatory oversight of health plan quality.)

Health plans working with government programs such as Medicare and Medicaid must meet additional requirements for accountability. The Centers for Medicare and Medicaid Services (CMS) not only issues requirements, it conducts a regular system of audits or examinations that result in “star” ratings for health plans. To a less prescriptive degree to date, the Affordable Care Act has set up parameters for insurance products that address the healthcare needs of the U.S.

The Challenge: Striking a Balance

A challenge for health plans is to establish organizational control in a way that enables, rather than erodes, personal responsibility.

- When accountability emphasizes external control, standards are exacting and individuals are held accountable to their superiors for meeting—or failing to meet—those standards.
- But when workflow and reporting channels of an organization are too structured or too formal, people in the organization may take less personal accountability.

In addition, people who work in an organization they feel is unsupportive, or work for leaders they feel to be unfair, are less likely to engage in helpful behaviors outside of their assigned job descriptions. Health plans need their employees and others who have a stake in the plan to define their responsibilities broadly and operate flexibly across departments, functions, and roles.

Example. Luis works in Smile Health Plan’s marketing department. He defines his job narrowly (“Sell, sell, sell!”) and ignores the need to work with the legal department to obtain regulatory approval for a change in benefit design. “Compliance is the legal department’s job, not mine,” he says. This could result in fines, penalties, or even a cease-and-desist order from a state insurance department or attorney general.

Example. Marie is an attorney for Smile Health Plan. She is charged with ensuring that customer service representatives give the proper description of how the plan is complying with a newly enacted benefit mandate. She sends an electronic copy of the law to the director of the customer service department, saying, “It’s their job to train staff, not mine.” But the director doesn’t understand the requirements of the law and does not implement it, making the plan noncompliant and subject to disciplinary actions.

Example. Caitlyn is physician in Smile’s network. She sees her job solely as treating patients and neglects “paperwork.” For instance, she does not respond to the plan’s Practice Patterns Survey or provide requested data feeds. This behavior impairs the plan’s oversight responsibilities, quality improvement efforts, and perhaps its legal reporting requirements.

Leaders of a health plan need to balance the way they structure expectations of accountability for employees and other stakeholders. Too much or too little structure can reduce individual levels of willing responsibility, while a balanced structure will tend to increase those levels. Employees need a sense of control over their ability to meet expectations, their resources, and their methods of task

production. Finally, health plan leaders need to identify, define, and reward the range of stakeholder activities so that stakeholders see the importance of what they are doing.

THREE REVIEW QUESTIONS

Models of Accountability

Outside healthcare, organizations typically establish accountability through selection, training, job design, goal setting, performance development, evaluations in terms of goals and performance targets, and corporate culture. But in the healthcare environment the challenge is more complex. Several models of accountability are at play in a health system, because of the wide variety and different functions of participants in the organization.

Several historical and emerging models of accountability can be seen in the modern health plan:

- Professional model
- Economic model
- Social model
- Concurrent/hybrid models
- Enterprise risk management (ERM)

The **professional model of accountability** relies extensively on personal relationships and trust and is focused on the relationship between a physician and a patient. It is designed for individuals and does not work well with large institutions, yet most participants in the healthcare system still rely on it for their own guidance as well as the assessment of others. These participants often negatively perceive the network of external controls imposed by health plans, state and federal government regulators, private accreditation organizations, risk management policies of the hospital or physician practice, and claims and payment structures. They see them as interfering with professional accountability and eroding the clinical and ethical quality of care.

Some health plans and some plan participants are turning to the **economic model of accountability**, which suggests that the delivery of healthcare should be more like a business. Patients are viewed as knowledgeable and specialized consumers, shopping for some optimal combination of price and quality. Under this model, accountability impacts the mechanisms of the marketplace, and failure to meet standards of practice will result in a loss of customers.

Other plans turn to a **social model of accountability**, which sees the health plan as more of a community of stakeholders. In this view, accountability is not achieved through marketplace supply and demand for services, but by voicing concerns. In other words, instead of withdrawing business from a stakeholder who does not meet standards, the other participants complain, protest, and offer an alternative. Under the social model, stakeholders can express themselves through a variety of mechanisms. Community boards, physician committees, citizen advisory groups, and governmental conduits are the standard mechanisms of expression and means of organizational control. Social media is becoming increasingly important for consumers and is yet another, very visible facet for organizational response and means of control.

All three of these models of accountability are in operation in the current healthcare environment. Various stakeholders, whether organizations or individuals, have some level of influence over the models and systems of accountability under which they operate.

***Example:** Well You Health Plan can choose a primarily economic model of governance and accountability through a shareholder-dominant board of directors. Or it can elect a social model through member and provider participation on the board of directors and advisory committees. The requirements for representation and participation of stakeholders in the domiciliary state of a health plan may influence or even dictate which model it can or will adopt.*

It is unlikely that an entire health plan system—including all its stakeholders—operates under a single model of accountability. Most healthcare systems have always featured some combination of two or even all three of these models of accountability (**hybrid models**) or have different models operating side by side (**concurrent models**). For example, a health plan may have more success with quality improvement efforts if physicians participate in governance teams (the social model), but the economic model may still govern the relationship between investors and health plans.

Because of international and national solvency concerns and laws, a model of accountability based on corporate governance and risk management is emerging and is also now part of the mix. **Risk governance**, in essence, is an appropriate and effective organizational framework of rules and practices by which senior leadership, along with the board of directors, establish an accountability model based on greater risk transparency in an entity's relationship with all its stakeholders. The governance of the health plan itself—or at a minimum, the insurers in the health plan—will need to move more to the **enterprise risk management (ERM)** model because of state insurance law changes.

In the ERM model, the board of directors has oversight responsibility for the key risk areas: the actuarial function, investment decision-making processes, reinsurance decision-making processes, business strategy/finance decision-making processes, the compliance function, financial reporting/internal audit processes, market conduct decision-making process, and cyber security. The leadership of the company—the CEO or president and the vice presidents or other senior leaders who are responsible for the day-to-day running of the company—must have the experience, knowledge, and understanding befitting their positions, so that their responsibilities are fulfilled effectively. Health plans and other participants, led by effective oversight of the leaders and the board in an ERM framework, use a more informed application of organizational control systems by employing rational methods to assess the appropriateness of different accountability models or combination of models and assigning the most effective model or combination to each particular relationship.

Leadership in Health Plans

All leaders, regardless of organizational level, engage in direction-setting (initiatives, planning, and measurements) and risk management.

- The qualitative nature of direction-setting does not vary substantially with organizational level, although the cost accountabilities (quantitative) do.
- Risk management activities differ from one level to the next but remain an essential component of leadership.

While a director of information services will be much involved in negotiating the network systems and physical environment in which her department is embedded, she must also work across all internal departments to negotiate for resources and accountability relationships. Other senior leaders are more likely to engage in activities that relate to the management of the health plan's external risks, such as negotiations with business partners, regulators, and external stakeholders. Where there is accountability, there is a leader charged with designing or maintaining the organizational control structures by which that line of accountability—risk identification, evaluation, prioritization, monitoring, and reporting—is managed.

What kinds of skills are important for a health plan executive?

- Vision
- Knowledge-based expertise
- A problem-solving approach that addresses increasing organizational complexity

Factors that demand these leadership skills are very much present in the health plan industry:

- **Environmental resources.** Executive mistakes are less critical in a resource-abundant environment than in a resource-scarce environment. Mistakes in an environment with highly conflicting demands for resources can be catastrophic, as profit margins shrink or even are replaced by losses.
- **Organizational complexity** is the degree of intricacy in a health plan's internal and external environment in terms of entity structure, stakeholders, markets, regulations, and physical configuration. Few modern economic environments are as complex as the health plan industry today.
- **Environmental change** in an industry can place great demands on leaders' abilities in effective strategic planning. The managed care environment is in a constant state of flux, including government requirements, competition, regulation, purchasing, providers, and consumer involvement.

Many entities in the healthcare delivery system go through cycles of downsizing or "right-sizing." Flexibility is becoming the watchword in the structure of organizations, whether they are health plans or companies that provide services in the broader healthcare system. Some consequences of downsizing:

- Managers find themselves with fewer resources while the same level of performance is demanded.
- Employees may operate with reduced managerial supervision and less specific job descriptions but with more tasks to accomplish.
- All staff members are expected to exhibit greater flexibility in the face of unpredictable demands on their time and skills.

This change environment expands the need for approaches to accountability that promote diverse and creative responses, rather than "by the book" solutions.

The increased need for flexibility and broader skill sets makes cross-training and interdisciplinary teams a critical part of the leadership framework. Increasingly, health plans are creating interdisciplinary teams from closely-related departments, such as customer service, utilization review, and claims processing. Not only do these highly interdependent teams possess an expanded ability to address multidimensional issues, but the individual members of the teams see a broader set of solutions that they can apply to particular issues. Over time, cross-functional learning occurs, providing the health plan with invested employees who are more capable and willing to take flexible approaches to organizational problem-solving and greater accountability.

For an example of some of the challenges that health plan leaders face, go to the end of the lesson.

Systems of Organizational Control

Because health plans function within a complex web of accountabilities to a wide array of stakeholders, their leaders must rely on systems of organizational and risk control. Such systems allow health plans to:

- Maximize individual initiative and creative risk-solving, yet develop an understanding of and a climate for cooperation toward common goals.
- Ensure that all participants in the organization follow a coordinated agenda in the face of disparate and often conflicting demands and goals of multiple stakeholders.
- Enhance organizational risk-monitoring and reporting so that the organization can meet its social, legal, and fiduciary responsibilities and requirements.

The president/CEO and the senior leadership team continue to have day-to-day responsibility for all activities in a health plan. But the ultimate responsibility and accountability for oversight and governance of a plan lies with the board of directors and board committees. With implementation of enterprise risk management (ERM), the board is accountable for oversight of sound risk decision-making.

The creation of a corporate governance framework includes the right committee structure at the board level. This allows for sufficient oversight by board members who have the experience and competency for the risks they oversee. As noted above, the specific critical risk areas that the board oversees include: ERM, the actuarial function, the investment decision-making process, the reinsurance decision-making process, business strategy, financial decision-making process, internal audit, financial reporting, compliance function, market conduct decision-making processes, and cyber security.

Suggested board committees and their delegated oversight roles:

- Audit: internal and external auditors, segregation of authorities
- Risk: enterprise risk management
- Compliance: compliance, code of conduct, and market conduct
- Finance: financial, investment, and budget

- Corporate governance: director independence and competency, as well as evaluation, executive compensation, selection criteria, succession planning, and training of the board and senior leadership.
- Strategic planning: business planning and resource allocation

ERM requires effective governance, which means it requires a governance structure that clearly defines and articulates roles, responsibilities, and accountabilities. The risk management framework creates a risk culture that supports accountability in risk-based decision-making and requires that high-level company personnel take responsibility for creating and maintaining an effective ERM program.

The accountability for ERM includes creating a risk management framework that includes an **enterprise risk governance team (ERGT)**. ERGT's purpose is to provide oversight and advice with respect to risk management and applicable policies and procedures for the day-to-day operations of the organization. ERGT members are typically an existing senior management team or senior officers with ultimate responsibility and decision-making in their roles as head of various departments, ensuring that all stakeholders are fairly and transparently represented in ERGT meetings.

REVIEW QUESTIONS

Compliance

Health plans need a strong enterprise risk management program operating within a culture of ethics and compliance.

- When a plan implements an ERM framework, it builds controls to manage risk, which operate at many levels within the organization.
- Different models of accountability are used within the organization to achieve the most effective operation, but there must also be high-level oversight that uses the corporate governance and risk management model of accountability.
- This includes monitoring for compliance risks and risk of criminal conduct, as well as implementing mitigation steps to modify the program based on risks found.

All of these requirements build the framework for a compliance and ethics program.

Compliance Programs

As we have seen, health plan structures are quite complex, and plans must comply with a large number of complicated state and federal laws and regulations, meet stakeholder expectations, and conform to economic realities. Not surprisingly, compliance presents a substantial management challenge. In response, many health plans today have established **compliance and governance programs** to prevent, detect, and address conduct by the health plan and its agents that violates legal and ethical requirements. Corporate governance can be defined as a compliance framework of rules and practices by which a board of directors helps ensure accountability, fairness, and transparency in a health plan's relationship with all its stakeholders.

An effective compliance program uses employee education, training, and accountability to ensure compliance with laws and regulations. A compliance program can be all-encompassing—covering all lines of business in which an organization is involved—or it can be less centralized, with a compliance structure scaled to each one or several specific lines of business. High-level oversight of such centralized or decentralized compliance programs are the responsibility of the enterprise risk governance team (ERGT) and the board of directors.

PART B

A compliance program is not just good corporate practice, it is a necessity. Healthcare misconduct is the focus of increased attention by the government and the media. A daunting assortment of federal and state agencies engage in the investigation and prosecution of healthcare wrongdoing. Obviously, it is better to prevent adverse events than to have to deal with them.

Beginning in 1998, the Office of the Inspector General (OIG) of the Department of Health and Human Services (HHS) began issuing guidance documents for voluntary compliance programs directed at various segments of the healthcare industry. Specific guidance has been published for hospitals, nursing facilities, third-party billers, and others, to encourage the development of internal controls. However, prior to these forms of guidance, prosecutors and practitioners alike turn to the **Federal Sentencing Guidelines for Organizations** (Guidelines), most recently updated in 2013. This framework (along with the requirements of state insurance laws) allows flexibility for a health plan to create a structure that best fits the scale and complexity of its organization, and it avoids a “one size fits all” standard for compliance.

A perfunctory, “check the box” approach to either an ERM or a Guidelines program will fail to promote the culture of risk, compliance, and ethics required by both programs. To create a culture that is both financially strong and committed to the highest requirements of integrity and ethics requires a risk culture and a culture of ethics and compliance, where risk management and controls are an ongoing activity, operating at all levels within the organization. This includes ongoing assessment of existing and emerging risks, as well as appropriate mitigation steps to design, implement, and modify a framework (controls, policies, and procedures) based on risks found. The periodic risk assessment process must include the risk of criminal conduct, to comply with the Guidelines

Obviously, any health plan would prefer to discover a problem before regulators and prosecutors do. A corporate compliance program creates a culture that heightens employee awareness, promoting detection and reporting of improper activities at an early stage and enabling the health plan to undertake remedial action as soon as possible.

If a health plan is charged with wrongdoing, it will not be enough for the organization to plead in its defense that it did not sanction the illegal activities of its employees. There is no requirement in the law that there be a finding of corporate willfulness or fault. Rather, the law-breaking employee’s intent can be attributed to the corporation as long as it involves conduct that is within the ordinary duties of employment and will inure to the benefit of the organization. However, if a health plan can document or demonstrate that the violator acted by circumventing the plan’s controls and accountabilities, this will go a long way toward eliminating or lessening penalties and lawsuits.

Compliance Risk Management

Compliance risk management (CRM) combines risk management and effective compliance. It combines ERM and federal Guidelines requirements and establishes a holistic, efficient control framework in an organization. Federal prosecutors routinely insist that a company seeking to settle a fraud case set up a corporate compliance program modeled on the Guidelines. Further, entities that have a compliance risk management (CRM) plan (whether patterned on the Guidelines or on state requirements) can sometimes lessen potential penalties by voluntarily reporting a criminal offense or regulatory violation to the appropriate government agency. The CRM framework enables effective governance by means of the same structure described earlier for ERM, including clearly defined and assigned accountabilities. The culture must support accountability in risk-based decision-making, and high-level personnel must:

- Be knowledgeable about the program.
- Exercise oversight to help ensure its effectiveness.
- Ensure that specific individuals have day-to-day responsibilities for each portion of the framework.
- Report to the health plan's governing authority (typically the Board of Directors).

This governance requires regular, transparent reports on the effectiveness of the overall program, as well as assessment of and reporting on whether adequate resources are being devoted to the program.

Developing a CRM Program

A step-by-step approach to implementing a CRM program includes:

1. **Gap analysis:** Use what is already in place and identify what needs to be changed to fully address the two framework requirements.
2. **Culture and structure review:** Ensure that the framework fits your organization's culture and management structure, so that your Chief Ethics and Compliance Officer (CECO) or Chief Compliance Officer (CCO) and your Chief Risk Officer (CRO) are empowered to establish a collaborative relationship and effectively merge the two programs.
3. **Governance configuration:** Form a company-level Enterprise Risk Governance Team (ERGT) comprised of senior management, which owns the program. This team leads the organization in establishing transparent risk-based decision-making, risk prioritization, and the new governance structure. The health plan must put the board of directors in charge of corporate risk strategy and policy setting, including receipt and adoption of the new, transparent risk reports.
4. **Risk assessment:** Perform a risk assessment to identify existing and emerging risks, in preparation for risk ranking, evaluation, and risk decision-making. The ERGT should draft the risk appetite statement for the board's adoption, establish risk tolerances and triggers, and implement a well-designed risk incident reporting protocol.
5. **Risk monitoring and reporting:** Key risk indicators (KRIs) applicable to each identified risk will need to be identified and potentially expanded for every area of the organization. Regular KRI reports, along with risk incident reporting, should be provided to the ERGT and ultimately to the board.

6. **Strategic planning:** CRM must be imbedded into the business and strategic plan, the capital management strategy, and the financial tactics of the organization, so that risk mitigation is allotted sufficient resources, including budget, system, and staff resources.

Enlisting the support of key employees is important to a successful compliance effort.

- A committee should be formed of individuals different areas: operations, finance, human resources, marketing, medical management, claims, internal audit, and legal.
- This group could include directors, executive officers, managers of important operating units or divisions, and legal counsel.
- By participating in such a work group, employees not only feel a part of the process, they help create companywide support for the compliance program.

Effective standards and procedures that govern a health plan's compliance program are specific to the health plan industry. Therefore, the team or work group formulating such standards and procedures typically begins by taking an inventory of all laws and regulations that govern the business, as well as compliance standards that the business has established, and then compares them to actual practice. A health plan will examine regulations of the Centers for Medicare and Medicaid Services, the federal Department of Health and Human Services, the state health department, and the state insurance department. The organization usually incorporates all of these into the organization's standards so that one integrated document governs all corporate conduct. Finally, besides these industry-specific standards, the organization's compliance standards might also include workplace standards that are generally applicable to all businesses. These might include standards governing the use of company property and proprietary information, accuracy in recordkeeping, and company policies regarding harassment and discrimination.

The organization can use this inventory for two purposes:

- To identify those departments and departmental activities likely to fall within the reach of each particular law or regulation.
- To conduct a comprehensive internal gap analysis of each key department.

This gap analysis, or risk assessment, forms the basis for the design of internal monitoring and control.

Members of the working group can use the inventory of applicable laws and regulations as a resource to review all of the functions and procedures in their own departments and to identify those risk areas where misconduct could occur. For example, such risks could relate to receiving claims for items and services not provided, improper diversion of payment intended for compensating a provider, and fraudulent misrepresentation of diagnoses and medical necessity of treatment. By identifying risk areas, each department can develop monitoring tools that specifically address the risk areas of that department, as well as design specific procedures to detect misconduct unique to that department.

To satisfy regulatory requirements, a health plan generally does more than establish compliance standards and procedures; it takes steps to achieve actual compliance with those standards and procedures. A foundational element is the creation of a system through which employees and other agents can report suspected fraud, waste, and abuse (FWA), other misconduct, or other potential risks

or gaps, without fear of retribution. The procedures for detecting and reporting should be straightforward, easily accessible, and simple enough for individuals to follow consistently.

An internal identification and reporting system benefits the company in at least two ways:

- It provides constant evaluation of the company by its own employees, who are the closest to all activities and decisions.
- It encourages employees and other stakeholders who might have previously felt forced to go directly to governmental authorities to report suspected misconduct through the designated company channels instead.

In an effective monitoring and reporting system, the health plan conveys to all personnel its ongoing and genuine interest in operating within a legal and ethical business culture. The company also designs a system that fosters a sense of confidence in the system and a culture of compliance. The support for such a comprehensive compliance and ethics framework is communicated through tone at the top and in the middle of the organization, and should resonate with each employee.

If a culture of ethics is lacking and employees are unable to internally report fraud, waste, abuse (FWA) or other significant violations of the plan's standards, they are allowed to report serious concerns to government agencies as "whistleblowers." Under federal law, a whistleblower has legal protections against retaliation by the organization, including protection of her job. (However, employers may be required to disclose the whistleblower's identity to law enforcement or government agencies.) If a government or legal action is pursued and penalties paid, the whistleblower may receive a portion, as well as having her costs, expenses, and attorney fees paid by the health plan.

Under effective CRM programs, a Chief Risk Officer (CRO) or risk program director is responsible for overseeing the company's risk management program, working with and through the ERGT. So that the CRO has the ability to carry out the duties of the position and ensure a commitment to this effort, she reports periodically to the board of directors.

It should be clarified that the Chief Risk Officer is not the same thing as the Chief Compliance Officer. Compliance and risk are seen by the NAIC and state laws as separate functions, although connected. However, it is possible in a smaller company that one person might serve in both roles.

The enterprise risk governance team is responsible for coordinating the identification, monitoring, and reporting of all risk assessments, mitigation, and progress of program changes. Among other things, the ERGT may review specific complaints or incident reports to decide whether they warrant a full investigation. The CRO and the ERGT maintain a reporting system that tracks risks, monitoring structures, incidents, and risk-based decisions. They would also determine whether mandatory reporting requirements apply in any specific case and, if so, to which regulatory or law enforcement agency a report should be provided.

Nothing in the world of health plan compliance is static. Laws, regulations, and court decisions (both state and federal) that affect health plans change regularly. Corporate compliance programs must be designed to remain viable by scheduling a reasonably frequent and comprehensive review of all relevant law and its applications to the health plan. This will typically involve both an ongoing monitoring of state and federal developments and an annual or biannual audit of the adequacy of the health plan's compliance standards.

Changes in law are not the only source of potentially hazardous change. The practices and products of health plans, if anything, change even more rapidly than laws and regulations. Health plans are creating new contractual relationships with provider organizations, designing new products that involve radically different operational approaches, and making a host of other operational changes that can have profound legal implications. Compliance officers must ensure that the business changes that the organization undertakes are handled in compliance with existing laws.

A health plan can follow every recommendation of every compliance expert, develop a plan that anticipates every regulator's and every prosecutor's objections, and create a structure that the authors of the Guidelines themselves could have written, and still fall short. The essential ingredient to any compliance plan is that the company's leadership—from the board of directors to the CEO and on down—must be committed to compliance and fully aware of the risks of noncompliance. Leaders and directors must take care not to be too single-minded in their oversight of the company's bottom line that they neglect to calculate compliance risks.

Compliance Mechanics

Ongoing Monitoring and Audits

A sound CRM program generally includes:

- A real-time system of control (monitoring), which may include daily activities (such as a second review of each vendor contract executed).
- A two-tiered auditing system, including annual and random audits.

Each department may conduct its own internal review at least once a year, or request that a review of its processes be done by a non-department staff member. The purpose of an annual controls audit is to:

- Identify whether gaps exist between laws and standards and actual practice.
- Assess whether the department's current monitoring procedures adequately detect potential risks.
- Assess the department's current functions and systems to decide if they adequately prevent or mitigate risks owned by that department.

Effective CRM programs also typically provide for random audits. The ERGT oversees such an audit program and establishes guidelines and procedures for it. An unannounced audit might be conducted if a department has previously identified high-risk gaps, has a surge in the number of incident reports, or carries high risk on a regular basis.

Anonymous Reporting

An anonymous reporting system makes it much more likely that employees will report risks and risk incidents. It also shows that the plan is more interested in learning about existing and emerging risks, including possible misconduct, than about the identity of the person reporting it. Such a system can include a standard reporting tool, a toll-free hotline number, and a mail-drop. A health plan may also extend anonymous reporting capability to enrollees, vendors, providers, and other stakeholders.

- This conveys the health plan's interest in operating a legal and ethical business.
- It also provides an additional avenue for identifying suspected risks, including misconduct, prior to any government agency investigation and intervention.

Typically, employers strive to ensure discretion, but they must also endeavor not to mislead individuals with promises of absolute anonymity. Plans generally cannot guarantee anonymity even to those who report misconduct through an apparently anonymous procedure. At some point, the company may need to identify the person who initiated the complaint, or their identity may become obvious. Some health plans address this issue before the onset of an investigation by sending a formal notice to the department from which a complaint has originated or, preferably, to the person who reported the concern. Among other things, this notice includes a statement that the law may require that the organization reveal the identity of the individual. It is a best practice to incorporate a similar statement into the company's standard reporting tool and in the taped message on the toll-free hotline.

Internal Investigations

An effective CRM program includes extensive fraud investigation procedures. Under one popular model, the ERGT refers those incidents that warrant a full investigation to a special investigative unit (SIU), which investigates the complaint and submits a written report of its findings. Several days before an investigation begins, the SIU will typically send a memorandum to the appropriate department head, which includes the following:

- Notification that the SIU will conduct a full investigation of a complaint
- The anticipated duration of the investigation
- The nature of the incident under investigation
- A statement that the investigators may interview and ask departmental personnel to provide information (which will be held confidential unless identification is required by government agencies)
- A statement that the SIU may refer the matter to federal, state, or local regulatory or law enforcement officials, and that cooperation with such officials may be required, potentially including court room testimony

An exception might be made from this prior notice if the department head is suspected of involvement in the fraud under investigation, in which case the investigation is made without notice. Finally, if the SIU determines that fraud has been or is being committed, the ERGT or its designee reports the incident to the applicable federal and/or state governmental agency within the required timeframe.

Through an internal investigation, a health plan gains an understanding of the facts, which it can then use to prevent further similar offenses. A plan can also obtain mitigation credit, in addition to the credit available for maintaining a CRM or ERM program, if it voluntarily reports a breach, suspected FWA, a criminal offense, or other reportable incident to the appropriate government agency and subsequently reveals the results of the internal investigation.

Disciplinary Action

The federal Guidelines discussed above require organizations to discipline individuals who violate compliance standards, but they do not mandate specific sanctions. A health plan, therefore, has discretion to decide the nature and severity of the discipline depending on facts and circumstances, as long as it applies the sanctions consistently.

- For an **unintentional and minor infraction**, an informal verbal reprimand often is an appropriate sanction. Such a reprimand puts the employee on notice that the conduct is a violation of the company's compliance standards and that further misconduct may lead to more severe sanctions.
- For **serious, intentional violations** of the company's compliance standards, firing and referral to the relevant governmental agency or law enforcement unit are appropriate.
- For **violations that fall between these two extremes**, the company may discipline an employee by demoting or transferring him, increasing monitoring or supervision, or suspending him without pay. Effective but less severe sanctions might include financial penalties, such as a temporary salary reduction, a denial of a salary increase, or denial of a bonus or incentive compensation.

The federal Guidelines also specifically state that management must be accountable for the misconduct of subordinates. Managers are subject to discipline for failure to detect compliance violations that occur within their departments. If a manager's negligence, carelessness, or inattention facilitates or prolongs misconduct within an operating unit, the health plan disciplines that manager in a way that is commensurate with the seriousness of the violation.

The compliance committee notifies the human resources (HR) department of every instance of employee misconduct. The committee's notice includes a description of the misconduct and a recommendation of a sanction. Although most HR departments have broad discretion over the imposition of sanctions against employees, the compliance program usually provides that HR confer with the employee's line manager and/or department head before imposing a sanction.

Employees are not the only individuals against whom health plans can take disciplinary actions. Others include enrollees, agents, vendors, and participating providers, under the contractual arrangements between the health plan and such individuals or entities. In such a case of disciplinary action, the compliance committee sends a notice to the accountable senior leader, including a recommended sanction. The senior leader would then have discretion over the type of sanction imposed against the enrollee, agent, vendor, or participating provider.

A health plan must handle any disciplinary action in a way that affords the individual some form of due process. This should include advance notice and an opportunity to respond to the charges. There should be appeal procedures for terminating or suspending employees, agents, vendors, enrollees, and providers.

Incentive Programs

Some organizations maintain a program that gives recognition to employees whose reports lead to the successful detection, investigation, and referral of misconduct. A compliance program might also include an employee reward program that disburses a cash award for each well-founded report of fraud or serious misconduct.

Training

A health plan typically distributes written compliance standards and procedures to all employees and other stakeholders. A plan generally gives all employees and stakeholders updated and new material whenever a change in law or in compliance standards or procedures makes it appropriate. The plan can deliver this information to employees in a separate compliance manual, as part of the plan's code of conduct, or as an additional section in the company's employee handbook (in hardcopy or electronically or both). Many plans require all employees to sign a written certification that they have received, read, and understood the standards and procedures.

However, distribution of written standards and procedures is not enough. An effective compliance program uses a variety of training and other interactive activities to communicate the company's policies and procedures. New employees receive a copy of the company's compliance standards and procedures immediately on hiring. They must attend an introductory training session that:

- Presents an overview of the applicable state and federal laws that regulate the health plan.
- Addresses the specific aspects of fraud associated with the health plan's activities.
- Thoroughly educates all personnel about the company's compliance program, including a review of all standards and procedures.

More specifically, the training sessions will ensure that employees and agents know how to report misconduct (anonymously or otherwise), know what happens to complaints after the initial report, and know what protections apply to people who use the reporting system.

The sound compliance program includes an annual "refresher course" for all current employees and other stakeholders. Such a course

- Reviews the company's standards and procedures.
- Highlights any particular problems or concerns in the company or in the industry generally.
- Explains any changes to the applicable laws or in the health plan's policies, procedures, and protocols that employees need to know to help ensure compliance.

Trainers for the introductory training sessions and the refresher courses can be internal or external. Large, well-developed HR department training programs can usually fulfill this responsibility. The training program must provide for documentation and maintenance of records of attendance. Maintaining records of attendance by topic and date ensures that all personnel receive sufficient compliance training and creates a record of the company's efforts to carry out an effective compliance program. Such documentation can be an important factor in a regulatory exam or investigation of misconduct.

Accountability and Medical Necessity

One of the most sensitive issues in health plans is **medical necessity** as a condition for covering a medical service or supply. Most health plans exclude from coverage any service or supply that is not medically necessary. Therefore, medical necessity comes up in the context *not of clinical decisions* about

treatment alternatives (which are made by the healthcare practitioner), but of *coverage decisions* (which are made by the health plan). In other words, medical necessity is a contractual issue; the health plan must decide whether the service or supply is covered under the terms of the contract's medical necessity requirement.

While medical necessity has been defined in a number of ways, most definitions state that a proposed treatment must be:

- Medical—provided or ordered by a licensed healthcare practitioner.
- Effective—proven or supported by scientific literature that suggests the treatment will result in the right benefits to the patient.
- Individualized—likely to be effective for this particular patient, given her clinical indications.
- Economical—as compared to other available, equally effective treatment alternatives.

The mechanisms and structures employed by health plans to make determinations of medical necessity vary. Accountability for medical necessity decisions is owned by several stakeholders.

- A health plan's medical director has direct responsibility for medical management. State law often requires that each plan have a medical director, and that he or she be a physician licensed in the plan's state. This means that the director is accountable for professional competence standards to the state medical board or other state regulator.
- The CEO of the plan typically has oversight of the medical director.
- Sometimes the medical director is assisted by associate medical directors, or by local medical directors who report to a national medical director.
- Other licensed healthcare professionals, notably nurses, are often initially involved in reviewing claims to help determine the medical necessity of the treatment.
- State and federal governments increasingly require documentation of external review of medical necessity decisions.
- Health plans are accountable to their members—the patients—for their determinations of medical necessity.

Most health plans recognize that utilization reviews (URs) protect the health plan as much as it does its members. The accountability for URs may be taken by a committee, to whom the medical director brings questions such as those of medical necessity. Increasingly, as health plans grow, and as public demands for accountability increase, health plans use specialized UR panels. For instance, pediatricians are assisting the medical director in difficult pediatric cases, cardiologists assist cardiology, and so on.

With regard to medical necessity, different stakeholders may make different claims:

- Professional organizations may attempt to assert jurisdiction over medical directors' professional competence.

- Executives may establish economic targets.
- Consumer advocates may file complaints or seek regulatory changes to increase stakeholder accountability for consumers overall.

Related to and overlapping with medical necessity and utilization review is clinical quality management. Essentially, the same stakeholders and areas of accountability apply in clinical quality management as in medical necessity. But the greater breadth of the subject matter brings into play additional mechanisms of organizational control. Quality committees, which oversee clinical quality (and perhaps deal with issues of peer review and provider credentialing) are a common mechanism for the exercise of health plans' organizational control. Such committees typically set standards, provide feedback, administer sanctions to providers, and advise the medical director and the management team on clinical quality improvement initiatives.

A **clinical practice guideline** is a utilization management and quality management mechanism designed to aid providers in making decisions about the most appropriate course of treatment for a specific case. Clinical practice guidelines are a particularly powerful tool in establishing accountability for relatively specific behaviors of providers. Indeed, the specificity of the standards make it particularly important that providers who are accountable to those standards feel that they have been arrived at by competent, unbiased parties. Clinical practice guidelines that are so developed, preferably by or in consultation with providers themselves, are more likely to be accepted by physicians than those perceived to be designed and imposed by the health plan. In any event, clinical practice guidelines are just that—guidelines, which may be modified by the provider to meet the special circumstances of a patient.

Ombudsman and Ethics Programs

An ombudsman program is designed to investigate and help resolve complaints against a health plan or any of its stakeholders. It can be viewed as another type of quality oversight. The ombudsman is generally a government appointee, who is intended to provide a neutral third-party perspective to help ensure healthcare stakeholder accountability to healthcare consumers. For instance, Congress funds ombudsman programs to protect the rights of vulnerable elderly users of long-term care. States have also established ombudsman programs for consumers of healthcare services. Ombudsman programs are, therefore, another tool for creating organizational control, emphasizing the organization's accountability, whether to health plan members, employees, providers, or other stakeholders.

Biomedical ethics, rather than remaining the exclusive domain of academics and researchers, has taken up residence in health plan ethics committees and programs. Ethics programs are designed to ensure that health plan policies and procedures are consistent with generally accepted principles of ethics. Such programs can provide valuable assistance to health plan executives by providing a forum for the discussion of difficult issues, reviewing or generating statements of plan policy, and assisting in the education of plan employees and other stakeholders.

Ethics programs represent an across-the-board effort to:

- Continuously communicate the health plan's mission, core values, and culture.

- Help executives and staff bring ethical considerations into daily decision-making as well as organizational activities and meetings.
- Develop measures for ethical considerations in selection processes for hiring, development, and promotion.
- Recognize health plans' ethical responsibilities in addition to their business obligations.
- Codify standards of ethical behavior.

There is a potential for conflict between the financial objectives of a health plan and the health interests of patients, which could take the form of a collision between medical ethics and business objectives.

There is a distinction between the objectives of health plans and those of other businesses:

- The product—healthcare—demands high standards of ethical behavior.
- The providers of care—physicians and other medical professionals—operate under their own recognized ethical standards.

The challenge is for health plans to consistently employ an ethical framework throughout all business functions, yet prosper in a highly competitive market environment.

Another ethical concern has to do with the privacy of the traditional doctor-patient relationship, and the potential impact of managed care programs (such as UR and member appeals and grievances) that rely on the sharing of patient-specific medical information. This can be perceived as conflicting with federally mandated protection of personally identifiable information, and it must be dealt with in a fully transparent manner among providers, members, and other stakeholders of the health plan.

Summary

Accountability occurs when parties willingly submit to a process by which they are required to justify their actions and policies to other parties. Enterprise risk management (ERM) is a process that combines accountability and risk and emphasizes cooperation among departments to manage an organization's full range of risks. ERM deploys different models of accountability as appropriate, in a rational way. The demand for accountability and ERM is driving leadership and decision-making in health plans today.

To prevent, detect, and address conduct by a health plan and its agents that violates legal and ethical requirements, many plans have established compliance programs. Compliance risk management (CRM) combines risk management and effective compliance. It uses both ERM and federal Guidelines requirements and establishes a holistic, efficient control framework in an organization.

A compliance program includes a variety of activities, such as ongoing monitoring and audits, investigations, anonymous reporting, disciplinary action, and training. Ombudsman and ethics programs can also play important roles in health plans.

Glossary

Accountability—acceptance of responsibility by a party. Specifically, a party willingly submits to a process by which one party is required to justify its actions and policies to another party.

Compliance and governance program—in health plans, a program to prevent, detect, and address conduct by the plan and its agents that violates legal and ethical requirements.

Compliance risk management (CRM)—an approach that combines risk management and effective compliance. It uses both ERM and federal Guidelines requirements and establishes a holistic, efficient control framework in an organization.

Enterprise risk governance team (ERGT)—a team made up of senior management whose purpose is to provide oversight and advice with respect to risk management and applicable policies and procedures for the day-to-day operations of the organization.

Enterprise risk management (ERM)—an approach to risk that places a greater emphasis on cooperation among departments to manage the organization's full range of risks as a whole.

Risk governance—in essence, an appropriate and effective organizational framework of rules and practices by which senior leadership, along with the board of directors, establish an accountability model based on greater risk transparency in an entity's relationship with all its stakeholders.

A Compliance Challenge

A new health insurance product has been designed by a health plan. If the plan unveils this product by the first of the next month it will have the first such product in the market and be just in time to offer the product to a large employer coalition that is putting its healthcare benefit business out to bid. The competitive advantage could be worth millions of dollars, but only if the product “hits the streets” next month.

However, the state requires insurance department approval of policy forms before selling any new product. Regulators have approved all but a single, minor aspect of the policy form, but will not grant approval in time for the product to be available for marketing. In short, to comply with the law is to hand millions of dollars of business over to competitors, miss corporate profit targets, and potentially forfeit the company’s year-end bonuses.

Obviously, the executive’s question to the compliance officer will be, “What are the costs of noncompliance?” If the issue were one of public safety, the commission of a fraudulent act, or some act of similar severity, it is likely to be an easier issue for the executive and the compliance officer than the above example. Few people will risk imprisonment, a corporate death sentence, or injury to life or limb for profits. Yet, this example, with neither life nor limb nor criminal statutes at stake, illustrates the type of issues a health plan’s leaders must sometimes address. They must consider the risk of fines or a cease-and-desist order from regulators.

Compliance officers who are asked to assess the risk of noncompliance can shrug their shoulders and say, “This is too subjective to predict.” Alternatively, they can hazard a guess. Or they can perform some sort of analysis of recent penalties to formulate an educated estimate of the potential costs – if the health plan is “caught.”

Calculations based on even recent regulatory experience may not predict regulatory penalties in the future as well as they would have. Regulatory standards of punishment appear to be on the rise as public scrutiny of health plans and their stakeholders has grown in intensity. Furthermore, if a regulator suspected that corporate executives calculated the cost of noncompliance and then decided the possible penalties were worth the profit, those penalties and fines could be much higher than forecast. Also, a health plan’s leaders must consider the risk of damage to its client relationships and its reputation – possibly an even higher cost than regulatory penalties could ever be.

General Sources

American Hospice Management Holdings, LLC (AHM), Compliance Program,
<http://www.americanhospice.com/>.

National Association of Insurance Commissioners, *Corporate Governance Annual Disclosure Model Act 305*, November 2014,
http://www.naic.org/documents/committees_e_isftf_corp_governance_related_cgad_model_act_140728.pdf.

National Association of Insurance Commissioners, *Corporate Governance Annual Disclosure Model Regulation 306*, November 2014,
http://www.naic.org/documents/committees_e_isftf_corp_governance_related_cgad_model_act_140728.pdf.

United States Sentencing Commission, *2013 USSC Guidelines Manual*,
<http://www.ussc.gov/guidelines/archive/2013-ussc-guidelines-manual>.

United States Sentencing Commission, *Federal Sentencing Guidelines for Organizations*, Section 8, Effective November 1, 2016, <http://www.ussc.gov/guidelines/archive/2013-ussc-guidelines-manual>.

24A Strategic Planning, Part I

Lesson Objectives

After completing this lesson, you should be able to:

- Define and describe strategic planning and explain how it can be advantageous for health plans.
- Define and distinguish a mission statement and a vision statement and state what purposes they serve.
- Describe situation analysis in strategic planning, including environmental analysis, environmental forecast, and internal assessment.
- Describe some of the tools used in situation analysis, including SWOT analysis, business portfolio analysis, and community analysis.
- Identify the characteristics of a good corporate objective.
- Discuss corporate strategy, including the types of strategies available to health plans and the difference between strategy and tactics.

Strategic planning plays a major role in the effective governance and leadership of health plans. As Howard R. Berry and Lou Pavia describe it, “Strategic planning is an important tool for management to set objectives, assemble facts, gain commitment, marshal resources, guide action, and understand risks. This is not to say that a managed care organization cannot be a winner without [a strategic plan], but the chances of winning are greatly enhanced by using the techniques of strategic planning that have been developed and tested in the corporate world and applying them to the managed healthcare environment.”¹

Part I of this lesson is an overview of strategic planning in health plans. Part II discusses some strategic planning issues.

Strategic Planning: An Overview

“If you don’t know where you are going any road can take you there.” —Lewis Carroll, *Alice in Wonderland*

Strategic planning is the process of identifying an organization’s long-term objectives and the broad, overall courses of action that the organization will take to achieve those objectives. Strategic planning can be described in simple language as seeking the answers to these three questions:²

- Where are you now?
- Where do you want to be?
- How do you get there?

It might seem, at first glance, that long-term planning makes little sense in the rapidly changing environment of health plans. Some may feel that the results of strategic planning simply do not justify the time and effort. Others may think that it is pointless to prepare for the next five years when marketplace and regulatory uncertainties make it difficult to predict what will happen in the next five months. But strategic planning pays off. Studies have shown that health plans that implement a strategic planning process typically have better business results than those that do not make or implement a plan. What are the advantages of strategic planning?

- A plan can be used as a “compass” to maintain the right direction in an environment of constant change. Without it, an organization may slip into a reactive rather than a proactive approach to the challenges it faces. Or it may simply follow the competition or the latest industry trends.
- A strategic plan can help a health plan recruit key personnel or obtain funding. Potential board members and senior management, as well as investors or donors, sometimes examine a health plan’s strategic plan before deciding to become involved with the organization.
- Strategic planning can translate the goals of the organization into specific day-to-day actions and objectives for each employee.
- If done in a participatory way, strategic planning can enhance teamwork and morale. Some organizations use strategic planning not only to focus on challenges, objectives, and actions, but to shape organizational culture.

Most companies develop a long-term strategic plan for at least the next three to five years. They may also develop a short-term tactical or business plan (with measurable objectives) for the first one or two years of the strategic plan.

Strategic planning may be driven by senior management, in which case control of the process resides with the CEO and senior staff. They may work with a few members of the board of directors or with a board planning committee. This group develops a strategic plan that it presents to the full board for review and approval. Some CEOs work alone on a preliminary draft of the strategic plan and then obtain input from key board members and executives. Others schedule a few days or a week-long retreat with senior management and/or board members to develop or revise a plan for presentation to the board. Sometimes the board establishes a long-term planning committee as a way to maintain board involvement in the planning process and to keep the board apprised of the organization's progress in attaining its goals.

In some health plans, the strategic planning process involves most, if not all, of the board members as well as key senior management staff. This group conducts an in-depth analysis of market conditions, organizational strengths and weaknesses, and resource requirements before coming together as a complete or representative group to develop an overall plan. Often the task of developing the written plan is assigned to a work group and the draft plan is presented at board and senior management meetings for review and revision.

The time required to conduct the planning process and create a written strategic plan varies, depending on the type of plan and how it is developed.

- An initial plan requires more time than an update to an existing plan.

- A plan developed by the CEO and revised based on input from key senior management and board members takes less time than a plan developed in a participatory manner.

Some health plans take several months or as long as a year each time they update their overall strategic plan. Others allocate intensive time to the planning process and can update a plan in a few days or weeks.

A strategic plan must accommodate change and be revised as circumstances warrant. The further an organization plans into the future, the less exact its planning can be because of unanticipated changes that may occur both within the organization and in its external environment. On a regular basis a health plan must re-evaluate both its goals (to determine whether they remain valid) and its strategies (to determine whether they can still achieve its goals). Each year when a health plan develops its business plan, it usually conducts a strategic review of current long-term objectives and strategies. Many organizations update their strategic plans by first evaluating the past year (comparing the actual results to the business objectives) and then adding a new final year to the long-term plan. This annual review process keeps the strategic plan current on a rolling basis.

There are many ways for health plans to approach strategic planning. Some experts suggest a detailed, step-by-step approach. *For more on one such approach, see below.*

The Strategy Change Cycle – 10 Step Process³

1. Initiating and agreeing on undertaking a strategic planning process.
2. Identifying organizational mandates.
3. Clarifying organizational mission and values.
4. Assessing the organization's external and internal environments.
5. Identifying the strategic issues facing an organization.
6. Formulating strategies and plans to manage the issues.
7. Reviewing and adopting the strategies and plan.
8. Establishing an effective organizational vision.
9. Developing an effective implementation process.
10. Reassessing strategies and the strategic planning process.

In this lesson we will focus on a strategic planning process that includes four primary activities:

- Defining or redefining the organization's mission and vision
- Conducting a situation analysis

- Establishing or revising corporate objectives
- Developing or revising corporate strategies

Mission and Vision

The Mission Statement

A **mission statement** succinctly sums up an organization's reason for existing and fundamental purpose. It is intended remain stable over time but can evolve and be redefined. A mission statement:

- Defines the scope of an organization's activities, answering the question, "What business are we in?"
- Separates what is important from what is not.
- States which markets will be served and how.
- Gives a sense of purpose and direction to employees.⁴

In some companies, each functional area develops its own mission statement based on the organization's overall mission.

For examples of mission statements, see the appendix at the end of this lesson.

According to Peter Drucker, the late marketing pioneer, a mission statement should answer these five questions:⁵

- What is our business?
- Who is our customer?
- What constitutes value for our customer?
- What will our business be?
- What should our business be?

Mission statements may provide these types of information:⁶

- The overriding purpose of the organization
- Defining the organization's customers
- The organization's scope and boundaries in terms of products, markets, and geography
- The organization's identity, distinctive capabilities, and basis for competitive advantage

- How the organization aims to create value for stakeholders (including shareholders, employees, and customers).

An effective mission statement is broad enough to encompass the types of business an organization is engaged in, but narrow enough to provide meaningful focus. A mission statement may include not only what an organization does today but what it may do tomorrow. (Sometimes this is expressed in a separate vision statement, as discussed below.) A mission statement must be realistic about what the organization can achieve, now and in the future.

Many health plans develop a mission focused on the customer needs it plans to meet rather than the products it plans to sell. An organization *could* define its mission as providing a certain type of health plan product. But what if new products are developed and become popular? By instead setting a mission of meeting certain needs of certain people or certain employers, an organization can keep up with changes in the marketplace. For instance, an organization might have the mission of “striving to be a leading health plan by meeting the needs of midsize employers and their employees and families with products and delivery systems that provide healthcare quality and value.” This statement gives it the flexibility to refocus its mission as products and needs change.

A mission should focus on customers who value the abilities and strengths of the organization. A health plan must seek to attract customers it can satisfy and be willing to turn away those whose business will not be cost-effective. A health plan that tries to be all things to all people will spend too much money trying to attract and keep consumers that it will not be able to satisfy in the long run.

Example: *Blue River Health Plan is successful with small employers, which typically select health plans based largely on cost. Blue River would likely find it difficult to attract and retain large national employers, which expect customized reports and similar services. A mission statement should focus on the market segment where Blue River has the most potential.*

A health plan does not generally change its mission frequently. But it needs to consider a change each time it conducts strategic planning, especially if circumstances have changed. For example, a not-for-profit plan might convert to for-profit status, or a new business unit might be created in response to legislation.

The Vision Statement

A vision is an ideal that an organization would like to achieve. How does a vision statement differ from a mission statement?

- A mission statement answers the questions: “What business are we in? and “What is our business for?”
- A vision statement answers the question: “What do we want the business to achieve in the future?”⁷

A vision statement is often created first, and a mission statement is then developed to reflect it.

For an example of a vision statement, see the appendix at the end of this lesson.

A vision statement goes beyond defining the scope of a company's operations. It is intended to motivate employees, to inspire enthusiasm and commitment in them. A vision statement is meant to evoke powerful and compelling mental images of the desired future state of the organization.⁸ The characteristics of a good vision statement include:⁹

- **Conciseness** –short and easy to communicate (often no more than 22 words)
- **Clarity** –easily understood
- **Future orientation** – expressing what the organization strives to achieve in the long term.
- **Stability** – flexible enough for changing conditions but not shifting constantly with short-term trends. For instance, “striving to expand healthcare coverage” is a stable goal because it will remain relevant and meaningful despite regulatory and technological changes.
- **Challenge** –setting a high but achievable goal.
- **Abstractness** –not stipulating a one-time, short-term objective (such as a specific sales or enrollment figure), but rather suggesting a wide range of activities over a long time.
- **Desirability/ability to inspire** – stating a desirable and inspiring goal that can motivate staff and facilitate leadership's achievement of results.

Mission and vision statements are not meant to be drafted and then locked away. If they are to be effective, leadership must share them with employees throughout the organization.¹⁰

Situation Analysis

After a health plan has defined its mission, it usually conducts a situation analysis. This involves:

- conducting an environmental analysis,
- developing an environmental forecast, and
- conducting an internal assessment.

Environmental Analysis

Environmental analysis is the ongoing compilation and examination of information about events and relationships in the environment an organization operates in, and about how they are likely to affect the organization. Most health plans concentrate on certain areas, such as:

- the plan's current target market,
- competitors' activities,
- legislative and regulatory developments, and

- other factors that affect purchaser behavior. These can include demographic changes, attitudes and beliefs about healthcare and health plans, economic conditions, and technological advances.

Environmental Forecast

Information obtained in the environmental analysis is used to develop an **environmental forecast**—a prediction of the major environmental changes that will affect a health plan’s activities. By estimating the character, magnitude, and timing of changes and their effects, the plan can reduce uncertainty and risk.

***Example:** Because of ongoing merger and acquisition activity in the industry, Green Valley Health Plan may be faced with a new competitor in its service area, with different strengths and weaknesses than existing competitors. In anticipation of this, Green Valley may want to develop new products or seek a strategic alliance of its own.*

***Example:** Green Valley operates a Medicare Advantage plan and a Medicare Part D prescription drug plan. Green Valley must, as much as possible, anticipate and be ready for legislation or regulatory changes that would change the rules these plans operate under.*

Internal Assessment

An **internal assessment** is an examination of a health plan’s current activities, its strengths and weaknesses, and its ability to respond to potential threats and opportunities. This allows a plan to take advantage of its strengths and to avoid or remedy its weaknesses. Among the internal factors that have a significant impact on a health plan’s strategic planning are its mission and vision, governance and leadership capabilities, financial condition, product portfolio, sales and marketing capabilities, provider networks, medical management techniques, claims and member services capabilities, and technological resources.

***Example:** Purple Hills Health Plan has several strengths: efficient billing processes, a productive sales force, and knowledgeable and responsive member services staff. On the other hand, it has insufficient financial resources and outdated information technology systems.*

Tools of Analysis

The information gathered in a situation analysis must be organized in a way that will help a health plan decide where and how to focus efforts and resources. We will look at three methods of doing this:

- SWOT analysis
- Business portfolio analysis
- Community analysis

SWOT Analysis

SWOT stands for Strengths, Weaknesses, Opportunities, and Threats. SWOT analysis enables planners to match a health plan’s strengths with marketing opportunities.

- A marketing opportunity arises when a combination of circumstances occurs that enable a plan to use its strengths or capabilities to take advantage of an environmental opportunity.
- A “strategic window” is the time period during which an optimum fit exists between a plan’s capabilities and the key requirements of an opportunity.
- SWOT analysis helps a plan identify the best possible places to devote resources so that strategic windows are exploited before they begin to close.¹¹

Example of a SWOT Analysis: GoodCare Health Plan

	Helpful	Harmful
Internal Origin	Strengths Strong Internet and social media presence Superior technology for internal analytics Superior technology to reach individual consumers Strong customer relationships, translating into strong customer-member retention Steady profit for past five years	Weaknesses Although customized health plans are important for most, GoodCare offers only standardized plans Multi-channel advertising does not necessarily result in increased customer base Only 14% of national market share Legislative environment creates uncertainty about future business models
External Origin	Opportunities Improved technology allows for greater customization of plan offerings Aging of population and increasing privatization of government-sponsored coverage offers opportunity to increase customer-member base	Threats Innovative competitors may lure members away, primarily through online outreach to individual consumers Company officers have substantial stock holdings, creating a danger of charges of insider trading and investigation by SEC

Business Portfolio Analysis

Companies that engage in more than one business or offer more than one product line can use business portfolio analysis as part of an internal assessment. In business portfolio analysis, a company evaluates its individual business units in terms of their potential contribution to the firm, given their strengths and weaknesses in relation to trends in their markets. This helps the board and senior management determine each unit’s potential for generating and requiring financial resources. Generally, companies will want to invest in units that are already profitable or have the potential to become so and to withdraw from units that lack such potential.

To conduct business portfolio analysis, a company must identify its **strategic business units (SBUs)**. An SBU is a major area of business distinct from other areas within a company. Specifically, an SBU:

- is operated as a separate profit center,
- has its own set or share of customers and competitors,
- generally has its own management, and
- is capable of having its own marketing strategy.

However, an SBU is usually dependent on the larger company for allocation of financial resources.

An SBU may be an entire company, a subsidiary, a division, a product line, a single product department, or a major market.

***Example:** Red Hills Healthcare Corporation identifies SBUs based on geographic markets (west, central, and east), demographic markets (employer groups, Medicare beneficiaries, and the Medicaid population), and products and services (HMO/PPO, third-party administration, pharmacy management, and behavioral healthcare).*

A business portfolio analysis considers factors related to:

- A SBU's market—market growth rate, market size, market stability, competitive intensity, government regulation, and technological requirements.
- The strength of the SBU in the market—relative market share, price competitiveness, product quality, customer loyalty, technological and financial resources, and management effectiveness.

A company can adopt one of four basic strategies with an SBU:

- **Build strategy**—seeking to increase an SBU's market share. This usually requires sacrificing immediate earnings to invest in growth.
- **Hold strategy**—trying to maintain an SBU's market position. This usually means investing only enough to hold promotional activities and customer services constant, although occasionally a company will increase short-term investment in the SBU in an attempt to enhance profits.
- **Harvest strategy**—seeking to maximize an SBU's short-term earnings and cash flow. This usually entails reducing the resources expended on the SBU and allowing its market share to decline. For example, a company may stop allocating money to an SBU for promotional activities, or it may reduce staffing.
- **Withdrawal strategy**—selling or discontinuing an SBU. This is done because the resources required to support the SBU can be more profitably employed elsewhere.

It may be helpful to view SBUs (including product lines) through the Product Portfolio Matrix originally developed by the Boston Consulting Group.¹² This tool shows where SBUs currently stand in terms of two factors, growth and market share. It divides SBUs into four types, with a certain cash flow generally associated with each type.¹³

Product Portfolio Matrix

	High Market Share	Low Market Share
High Growth Market	Star Cash Flow Modest, Positive or Negative	Question Mark Cash Flow Large, Negative
Low Growth Market	Cash Cow Cash Flow Large, Positive	Dog Cash Flow Modest, Positive or Negative

As shown, the Product Portfolio Matrix divides SBUs or products into four classes:

- **Stars**—high market share and high growth. These are market-leading businesses/products, but they are expected to need substantial investment to retain their position. So while such SBUs generate substantial income, a goodly portion of it must be reinvested in the SBU. Question: Will the market continue to grow?
- **Cash Cows**—high market share in a slow-growth marketplace. These businesses/products generate profits in well excess of the funds needed to maintain their position. They are often viewed as boring but valuable, as revenue from them can be reinvested elsewhere in the company.
- **Question Marks**—low market share in a high-growth marketplace. These SBUs are sometimes referred to as problem children. Questions: Is the market share low because the SBU is newly established or the product newly introduced? Is the organization approaching the marketplace incorrectly? What is the competition doing that we are not? Does the SBU/product simply require more investment to become a star?
- **Dogs**—low market share in a low-growth marketplace. They typically generate little profit or even operate at a loss, but are a considerable drain on human capital, managerial talent, and financial resources. Question: Should resources be reallocated elsewhere and the SBU closed or the product withdrawn? The answer is, more often than not, yes.

Community Analysis

For many not-for-profit organizations, an additional element in strategic planning is community analysis. This is a process in which the organization gathers information and evaluates trends related to its charitable mission or its benefit to the community. Community analysis helps the board and senior management determine demand for services and programs, environmental forces that may have an impact on demand, and the emergence of additional opportunities for the organization to fulfill its mission.

Objectives

After deciding on a mission (and perhaps also a separate vision) and conducting a situation analysis, the next step in strategic planning is establishing objectives. **Corporate objectives** (also called organizational objectives or corporate goals) are statements that describe the long-term results that an organization plans to achieve in carrying out its mission. Corporate objectives give activities a sense of purpose and motivate employees. They also make decisions more consistent and so increase management control over operations.

Corporate objectives are normally made for three to five years. They often deal with profit, growth, use of technology, quality of services, and sales and market share. For not-for profit health plans, they may also focus on benefits to the community and fulfilling the organization's charitable purpose. In light of increased attention in this area, most health plans today include objectives related to market conduct and compliance.

Effective corporate objectives are:

- clearly stated
- specific and measurable (if possible, quantifiable)
- realistic

When possible, an objective should specify in quantifiable terms what is to be accomplished and in what timeframe.

Examples: *Nonquantifiable objective: "To increase sales." Quantifiable objectives: "To increase total corporate sales by 10 percent within four years" or "To achieve a market share of 20 percent within the next four years."*

Examples of Health Plan Objectives

- Improve health and satisfaction outcomes during each of the next three calendar years for asthma, cardiovascular disease, and diabetes.
- Obtain a five star rating for a Medicare Advantage plan within two years.
- Reduce voluntary turnover to less than 9 percent of all employees each calendar year.

Corporate objectives are established for a whole organization. These overall objectives form the basis for the objectives of each functional area (marketing, claims, underwriting, legal, etc.). As overall objectives are translated into functional area objectives, they become more specific and more short-term. This makes them easier to plan and control.

Strategy

Corporate strategies are the long-term methods organizations use to achieve corporate objectives. There is a variety of strategies, and different organizations may use different strategies to meet the

same objectives. A single organization may use different strategies for different strategic business units (SBUs). Corporate strategies must be regularly re-evaluated to make sure they are still appropriate given the current environment. Organizations often establish contingency plans to be used if conditions change radically.

In developing corporate strategies, a health plan should focus on stakeholders. It should:

- Identify the stakeholders the plan depends on for success, such as members, employees, and shareholders.
- Recognize what the plan wants from each group of stakeholders (for instance, increased productivity from employees).
- Recognize what the stakeholders want from the plan. This brings an external perspective to strategies. For instance, members may want easier access to their records or products not currently offered.¹⁴

Growth Strategies

Most health plans develop strategies to promote growth. There are different approaches:

- Intensive growth strategies
- Integrated growth strategies
- Diversified growth strategies

Intensive growth strategies include:

- **Market penetration**—increasing sales of current products to current markets. This usually involves intensifying promotional efforts (such as increasing advertising, the size of the sales force, or sales incentives).
- **Market development**—increasing sales of current products to new markets. This often involves expansion to a new geographical area. (If proposals to allow sales of health insurance products across state lines are approved, this could intensify.)
- **Product development**—increasing sales by modifying current products or developing new (but related) products for current markets. For instance, a company might offer a dental plan to its current health plan customers.

Integrated growth strategies involve taking over another area or level of a company's industry. This may include:

- Horizontal integration—such as a merger of two health plans.
- Vertical integration—such as the formation of an integrated delivery system (IDS).

In **diversified growth strategies**, a company produces products or services unrelated to its existing business. This can be risky because a company will need substantially different knowledge, skills, and processes. But diversification can soften the blow of a dramatic downturn in a market. For instance, if a health plan is experiencing a drop in revenue in medical coverage, it might offer supplemental insurance products.

Strategy and Tactics

Along with a long-term strategy, a health plan needs a shorter-term tactical plan. While strategy consists of long-term methods to achieve corporate objectives, tactics involve using and managing specific limited resources to achieve sub-goals of overall objectives. A set of tactics work together to achieve overall goals.

Health Plan Strategy and Tactics: Key Differences¹⁵

	Strategy	Tactics
Focus	Overall corporate objectives	Sub-goals of overall objectives
Timeframe	Long-term	Short-term
Change?	Infrequently	Flexibility, change with conditions
Resources and methods	Experience, research, analysis, and communication	Experience, best practices, processes, and teams
Criteria for success	Clear organizational goals, plans, guideposts, and key performance measurements	Clear deliverables and outputs based on use of human resources, tools, and time
Accountability	Based on overall viability of the organization	Based on specific resources

While strategic planning and tactical planning can be thought of as distinct and separate processes, in practice they are closely linked and often overlap. Strategic planning must be integrated and coordinated with tactical planning in the functional areas of a health plan. Each area determines what its role is in the overall strategic plan and then sets its own objectives, strategies, and tactics accordingly.¹⁶

To illustrate how strategy differs from tactics, how they are linked, and how they are applied in functional areas, we offer an example.¹⁷ A provider group is building a preferred provider organization. Major tasks in the corporate strategy include:

- Developing the provider network and negotiating provider contracts
- Establishing a sales and marketing organization

- Developing a utilization management and quality assurance capability
- Developing a management information system
- Establishing an organizational infrastructure

Each of these major strategic tasks is broken down into tactical subtasks. For instance, the first strategic task above—developing a provider network—would include these subtasks:

- Analyze provider admission and cost information
- Identify desired providers
- Develop model provider contract
- Negotiate provider contracts
- Establish ongoing provider relations function

Issues in Strategic Planning

Senior management must try to anticipate the likelihood that a strategy will create problems and the consequences of such problems.

- Sometimes negative consequences can be quantified (such as the loss of funds invested in developing and launching a new product).
- Other times the risk may be difficult to quantify (such as damage to a plan's relationship with current providers when it seeks to contract with a new physician group).

Such risks must be identified and assessed, and the proposed strategy reevaluated to determine whether the potential rewards justify the risk.

In developing a strategic and tactical plan, the board and senior management must make sure that adequate resources are allocated to successfully implement it. These can include capital investment, areas of expertise, staffing allocations, and management oversight. Resources will be needed for both start-up and ongoing activities, and in multiple functional areas. Strategic planners must resist the tendency to underestimate needed resources. The implementation of strategic and tactical plans can be labor-intensive. Some organizations develop a formal human resource strategy to determine the number of employees and the types of skills needed and to ensure that they are provided. This may include retraining existing staff or hiring new staff.

Corporate strategies should be accompanied by action plans specifying who will do what and when. Strategic planners typically use project management tools to plan and monitor progress and to report results to the board and senior management. These tools help coordinate efforts and identify unanticipated problems that arise.

Setting too many goals or the wrong goals for employees as part of a strategic and tactical plan can be counterproductive.

- Goals can be so specific and narrow that they blind people to other important developments happening around them.
- Goals can have inappropriate timeframes. For instance, if goals emphasize immediate performance, employees may engage in short-term behavior that has negative long-term consequences.
- Goals can be too challenging. Setting a target unrealistically high can make people give up or ignore the goal, or even engage in unethical behavior to reach it (such as creating bogus accounts).

Goals should be seen as “...prescription-strength medication that requires careful dosing, consideration of harmful side effects, and close supervision.”¹⁸

For a strategic plan to be successful, employees must understand the reasons behind the strategy and their individual roles in implementing it. Effective communication is key. Communication is particularly important when a health plan pursues a strategy that potentially can have a major impact on the jobs of employees, such as a change to for-profit status, a merger, joint ventures, vertical integration, and targeting of new markets.

Example: *As part of its strategic plan, a not-for-profit health plan seeks to obtain more capital for expansion by converting to for-profit status. Some employees see this as a mistake. Others understand the need for it but have concerns that their departments will have to reassign employees from critical day-to-day operations to help implement the strategy. Others view the strategy and the associated changes as a threat to their current jobs, or are worried that their salary and benefits will be affected.*

Employees must understand the reasons for change and have an opportunity to provide input and ask questions about their new or changing responsibilities. An effective strategic plan provides for frequent communication in as many ways as possible, such as newsletters, emails, town hall meetings, work group meetings, and individual objective-setting sessions that link each employee’s compensation and performance review to the strategic plan. These efforts to communicate and encourage participation help build consensus and increase the likelihood of obtaining buy-in from the many employees responsible for implementing the strategy on a daily basis.

Summary

Strategic planning identifies an organization’s long-term objectives and the broad, overall courses of action that it will take to achieve those objectives. A mission statement sums up an organization’s reason for existing and fundamental purpose. A vision statement is an ideal that the organization would like to achieve and is intended to inspire and motivate employees.

After a health plan has defined its mission, it usually conducts a situation analysis. This includes an environmental analysis (gathering and examining information on the plan’s business environment), an environmental forecast (predicting environmental changes), and an internal assessment (examining the plan’s activities, strengths, and weaknesses). Various methods can be used to examine the information gathered in a situation analysis, including a SWOT analysis, a business portfolio analysis, and a community analysis.

After deciding on a mission and conducting a situation analysis, a health plan establishes corporate objectives, statements of the long-term results that the organization plans to achieve in carrying out its mission. Next, corporate strategies are developed—these are long-term methods that will be used to achieve corporate objectives. Finally, a strategic plan must be implemented by means of a tactical plan, involving shorter-term sub-goals of corporate objectives in each of the functional areas of the health plan.



Appendix: Mission Statements

Health Plan A's Mission Statement

Create a sustainable financing vehicle that will deliver easily accessible, high-quality, and affordable healthcare services across all commercial and governmental markets in the state of XYZ. The delivery of these services will be based on exceptional support capabilities for all customer constituencies in a caring environment, and with effective technology solutions.

Health Plan A's Vision Statement

Expand the access to quality healthcare and the availability of affordable healthcare coverage across the state of XYZ.

Health Plan B's Mission Statement

The mission of Health Plan B is to provide health benefit services of value to customers across the region comprised of State XYZ and the ABC Metropolitan Area. To fulfill this mission, Health Plan B commits to:

- Offer a broad array of quality, innovative insurance plans and administrative services that are affordable and accessible to our customers;
- Fairly address the needs of customers in each of the jurisdictions in which we operate;
- Conduct business responsibly as a non-profit health service plan, to ensure the plan's long-term financial viability and growth;
- Collaborate with the community to advance healthcare effectiveness and quality;
- Support public and private efforts to meet needs of persons lacking health insurance;
- Foster health systems integration and healthcare cost containment to benefit people in areas we serve; and
- Promote respect, fairness, and opportunity for our associates.

More Mission Statement Examples¹⁹

Cleveland Clinic: To provide better care of the sick, investigation into their problems, and further education of those who serve.

Mayo Clinic: To inspire hope and contribute to health and well-being by providing the best care to every patient through integrated clinical practice, education, and research.

AARP: To enhance quality of life for all as we age. We lead positive social change and deliver value to members through information, advocacy, and service.

Sources

- ¹ Howard R. Berry and Lou Pavia, "Strategic Planning and Implementation," in Peter Boland (ed.), *Making Managed Healthcare Work*, 1993.
- ² John M. Bryson, *Strategic Planning for Public and Nonprofit Organizations (4th Ed.)*, 2011, p.8.
- ³ Bryson, p. 47.
- ⁴ BusinessDictionary.com, "Mission Statement," <http://www.businessdictionary.com/definition/mission-statement.html>.
- ⁵ Peter F. Drucker, *The Five Most Important Questions You Will Ever Ask About Your Organization*, 2008.
- ⁶ William Phanuel Kofi Darbi, "Of Mission and Vision Statements and Their Potential Impact on Employee Behaviour and Attitudes: The Case of a Public But Profit-Oriented Tertiary Institution," *International Journal of Business and Social Science*, 3:14 (July 2012), p. 98.
- ⁷ Darbi, pp. 96-97.
- ⁸ Ibid.
- ⁹ Sooksan Kantabutra and Gayle C. Avery, "The Power of Vision: Statements That Resonate," *Journal of Business Strategy*, 31:1 (2010), pp.37-45.
- ¹⁰ Sooksan Kantabutra and Gayle C. Avery, "Essence of Shared Vision: Empirical Investigation," *New Zealand Journal of Human Resource Management*, Vol. 5 (2005), pp.1-28.
- ¹¹ Albert Humphrey, "SWOT Analysis for Management Consulting," *SRI Alumni Newsletter*, 2005; Nicole Fallon Taylor, "SWOT Analysis: What It Is and When to Use It," *Business News Daily*, April 1, 2016, <http://www.businessnewsdaily.com/4245-swot-analysis.html>; Dileep Baragde and Neeta Baporikar, "SWOT Analysis as a Tool to Enhance Competitiveness of Indian Software industries," *Marketing Management: Chronicle of the Neville Wadia Institute of Management Studies and Research*, 2015, <http://www.nevillewadia.com/images/Cronicle2015/Dileep-Baragde15.pdf>; B. Prabhakar, "SWOT Analysis of An Organization and Focus on Shareholders," *International Journal of Advances in Arts, Sciences, and Engineering*, 2:4 (January 2014), <http://www.ijoaase.com/Swot%20Analysis%20Of%20An%20Organisation%20And%20Focus%20On%20Shareholders%20pg%2057%20to%2059.pdf>.
- ¹² Bruce D. Henderson, "The Product Portfolio," *BCG Perspectives*, The Boston Consulting Group, 1970, https://www.bcgperspectives.com/content/classics/strategy_the_product_portfolio/.
- ¹³ Annmarie Hanlon, "How to Use the BCG Matrix Model," *Smart Insights Digital Marketing*, October 21, 2013, <http://www.smartinsights.com/marketing-planning/marketing-models/use-bog-matrix/>; Professional Academy, "Marketing Theories – Boston Consulting Group Matrix," <http://www.professionalacademy.com/blogs-and-advice/marketing-theories-boston-consulting-group-matrix>; Katherine Arline, "What Is a BCG Matrix?" *Business New Daily*, February 5, 2015, <http://www.businessnewsdaily.com/5693-bcg-matrix.html>.
- ¹⁴ Graham Kenny, "A List of Goals Is Not a Strategy," *Harvard Business Review*, November 19, 2014, <https://hbr.org/2014/11/a-list-of-goals-is-not-a-strategy>.
- ¹⁵ Jeremiah Owyang, "The Difference Between Strategy and Tactics," *Web Strategist.com*, January 14, 2013, <http://www.web-strategist.com/blog/2013/01/14/the-difference-between-strategy-and-tactics/>.
- ¹⁶ Suleiman Yahaya Sado, "The Role of Tactical Plans in Achieving Organizational Growth and Objectives: A Case Study of Zenith Bank PLC Damaturu Branch," *Mediterranean Journal of Social Science*, 5:17 (July 2014).
- ¹⁷ Peter Boland (ed.), *Making Managed Healthcare Work*, 1993, p. 219.

¹⁸ Lisa D. Ordonez, Maurice E. Schweitzer, Adam D. Galinsky, Max. H. Bazerman, “Goals Gone Wild: The Systemic Side Effects of Over-Prescribing Goal Setting,” *Harvard Business School*, Working Paper 09-083, 2009.
<http://amp.aom.org/content/23/1/6.short>.

¹⁹ Top Nonprofits, “50 Example Mission Statements,” <https://topnonprofits.com/examples/nonprofit-mission-statements/>.

24 B Strategic Planning, Part II

Lesson Objectives

After completing this lesson, you should be able to:

- Identify the key issues that must be considered in developing a strategic plan for a health plan.
- Discuss how those issues affect strategy and how they interact with one another.

Overview

In Part I of this lesson, we saw how a health plan's board of directors and senior management team

- define the organization's mission,
- assess the external environment and the organization's own capabilities and limitations,
- establish corporate objectives, and
- develop strategies and tactics to meet those objectives.

In Part II, we'll explore how, in developing a strategic plan, a health plan must generally consider these key issues:

- Markets and product portfolio
- Pricing
- Financial management
- Provider networks
- Medical management
- Customer service
- Information technology
- Legislation and regulation
- Corporate transactions

These issues must be considered in the context of trends and challenges facing the health plan industry, including:¹

- Coverage affordability
- Quest for value

- Increased market focus on the individual consumer
- Cybersecurity
- Globalization²

Markets and Product Portfolio

Target Markets

As part of a strategic plan, a health plan must assess and select target markets. These can include:

- Nongovernment groups
- Government employee groups
- Government-sponsored programs
- Individual purchasers

Within each of these larger markets are market segments:

- Nongovernment groups include single employer groups and multiple employer associations; employers are large, midsize, and small.
- Government employee groups include state and local employee plans, the Federal Employees Health Benefits Program (FEHBP), and TRICARE (for those associated with the military).
- Government-sponsored programs include Medicare, Medicaid, some state workers' compensation programs.
- Individuals can (as of this writing) be approached through or outside of ACA Marketplaces.

In choosing a market, a health plan must look at both the external and the internal environment. External issues include the needs and buying behavior of purchasers, the medical needs of particular populations, competition, and regulation. An example of the needs and buying behavior of purchasers: Many large employers expect customized products and services, but small employers are willing to forego customization to get a good price. In considering competition, a health plan looks at its current position, closely monitors its competitors' behavior, and try to predict their future actions. In some cases, a plan must be prepared to take fast competitive actions and marketplace movements.³

A health plan must determine what the market expects and then realistically assess its ability to deliver on those expectations. Key areas are customer service, pricing, and information systems. For instance, if a health plan does not have the personnel or information systems needed to provide the customized products and services that large employers demand, its strategic plan would likely focus on the small and midsize employer markets.

For a way to reach the individual market, see below.

Reaching the Individual Consumer Through Private Exchanges

Many employers seek to outsource health coverage administration and hold down their costs. Many employees want customized coverage. One way these dual goals can be met is through private exchanges.

The employer contributes a fixed amount per employee. Employees can pick and choose among the products available (sometimes from various companies) on the exchange and use the employer's contribution to help pay for them.

Health plans engaged in strategic planning might consider whether to participate in private exchanges established by third parties or create their own (or both).⁴

Product Portfolio

A health plan must also assess and develop its product portfolio—the number and variety of product designs or plans it offers. Some of the issues:

- The number and types of healthcare services and supplies covered
- Benefit and cost-sharing features, such as deductibles, copayments, coinsurance, limitations, and maximums
- Protocols for delivery of healthcare services and supplies (for example, which require preauthorization)
- Out-of-network coverage, if any, including services covered, benefits, and cost-sharing
- The breadth of the product portfolio (the number of “plans” that will be standardly offered to customers, based on variations of the above features)
- Willingness to customize products at the request of customers

As in choosing a market, in developing a portfolio a health plan's market position and the strengths and weaknesses of competitors should be considered. A portfolio must meet customers' needs while differentiating the organization from the competition. A plan must also evaluate its own strengths and weaknesses to determine the number and variety of products it is capable of delivering effectively and competitively. An organization may develop different products or product features for different geographical markets.

Example: *GoodLife Health concludes that purchasers and consumers value easy access to specialty providers. Good Life determines the number of competitors that offer this feature (external environment). It also considers what it must do to provide a competitively priced product with this feature, based in part its relationship with its network specialists and its ability to monitor the frequency and cost of specialty care (internal environment).*

The development of product portfolios can be influenced by state or federal regulations requiring health plans to cover certain healthcare services or provide certain benefits. Under the ACA, plans must offer a

package of essential health benefits (EHB) (although this may be affected by efforts to repeal and replace this law). And as we have learned in earlier lessons, states also have benefit mandates.

Pricing

In setting premiums, a health plan must ask two questions:

- How much are customers willing to pay for proposed products and services? This will depend in part on competitive pricing pressures.
- How much must the organization spend to provide these products and services?

Health plans typically distinguish between medical and administrative expenses.

- **Medical expenses**—payments to providers for the delivery of healthcare. (This usually includes the cost of quality improvement efforts, although this may be separate.)
- **Administrative expenses**—all other costs, such as office leases, equipment costs, sales and marketing, and salaries and benefits.

Often, the premium is based on the estimated cost of providing the product and services plus a targeted profit margin (or for nonprofits, a surplus). But if this produces a price customers are unwilling or unable to pay, product design options may be offered to lower the premium. For some Medicare and Medicaid products, the health plan does not set premium rates, the government does. In this case, the plan must determine if it can provide the products and services at the price set.

Financial Management

A health plan must:

- determine past and current expenses and revenues,
- project future expenses and revenues, and
- evaluate and use this information in developing and revising its strategic plan.

If a health plan's financial management systems are inadequate, the board and senior management will not have the information they need to make strategic decisions and changes.

Example: *Good Life Health Plan's revenues have recently begun to outpace expenses, but management puts off the development of a new product in the belief that there are insufficient resources.*

Example: *The management of Long Life Health Plan invests in an expensive technology without knowing that the plan has begun operating at a loss.*

Example: *Deep Valley Health Plan is beginning to lose money on dental coverage, but senior management does not learn this in time to make adjustments to turn things around and cut losses.*

Example: *High Hills Health Plan experiences rapid growth, but because it lacks accurate information it does not build the infrastructure it needs to serve its new customers.*

Strategic planning requires financial information from a plan's various functional areas, particularly medical management, claims, and provider contracting. The medical management and claims areas must communicate to the financial management/accounting area the actual and estimated medical costs based on claims paid, claims reported but not paid, and claims incurred but not reported. The financial management/accounting area must be aware of potential expenses resulting from provider reimbursement arrangements (such as physician and hospital withholds).

To make strategic decisions, a health plan must be able to analyze financial requirements and results for each of its market segments and product lines.

Example: *Based on information from its financial management system, Big Rock Health Plan decides to withdraw some plans from certain ACA Marketplaces and redirect resources to a supplement product with excellent long-term prospects.*

Provider Networks

Strategic planners must consider a health plan's provider network, including geographic coverage and number and types of providers. The objective is usually a full continuum of healthcare services, including hospitals, physicians, mental health care providers, and many specialists and services of various sorts. Planners often decide on first choices and back-up choices for each type of provider or service.

Many health plans make a concerted effort to recruit and retain quality providers who will reflect favorably on the plan, both in how they deliver care and their attitude toward the plan. Many plans approach providers as they do customers. They market the plan to providers by stressing the increase in patient volume and prompt reimbursements it offers. They seek to support providers in various ways, such as sharing data and helping with quality improvement efforts. Some health plans involve providers in strategic planning, as they would an important strategic partner. At a minimum, they communicate promptly and effectively with them.

Affordability is an important consideration. Some plans encourage network providers to participate in value-based care (VBC) clinical and financing models. HHS is moving in the direction of promoting VBC, so this is particularly important for plans serving Medicare and Medicaid populations.⁵⁶

Medical Management

A strategic plan must include strategies for medical management that address the need for:

- Clinical quality measures and performance
- Quality medical personnel to oversee medical management programs and processes
- State-of-the-art information systems to facilitate processes such as authorizations, large case management, and discharge planning

- Quick and effective response to the rapidly changing healthcare environment (including treatment of new diseases and new drugs, medical technologies, and medical procedures)

Some health plans contract medical management to a third-party vendor, either because they do not have the needed resources or for some other reason. Utilization and quality management organizations have experienced personnel, tested medical protocols, and large databases, enabling them to perform this function more effectively and at a lower cost than many health plans.

Customer Service

Health plans win and retain business in part by providing excellent customer service. This is obviously true for plans marketed to individual consumers or to employees with a choice of plans. But even if a health plan is the only option offered to a group of employees, the employer will not stay with the plan if customer service is poor. Therefore, a strategic plan must include the staffing, training, and equipment necessary to ensure that phone calls are answered promptly, hold times are kept to a minimum, and concerns are resolved to members' satisfaction. It must ensure that its complaint and appeals process handles member grievances in a timely and impartial manner.

A health plan's ability to provide effective member services in an increasingly consumer-centric environment is closely related to the effectiveness of its information technology systems. In addition, studies show that health plan members appreciate technological tools, such as those that make it easier to schedule doctor appointments or allow them to receive follow-up notifications on mobile phones. This is likely to become even more true as more digitally-savvy members of the Millennial generation enter the workforce, and health plans will ignore this trend at their peril. *For more on consumer preferences, see below.*

Consumer Preferences for Online and Mobile Capabilities in a Health Plan⁷

Most Preferred (Basic) Features

1. Online appointment scheduling with in-network providers
2. Out-of-pocket cost estimator
3. Mobile post-care instructions and follow-up notifications
4. Simple access to health records (online and mobile)
5. Central payment portal to both health plan and to providers

Nice to Have

1. Telehealth and online/mobile consultation
2. Personalized health and wellness predictor
3. Quality transparency tools (including ratings and reviews)

Information Technology

A strategic plan must include information technology. A plan that does not adequately take IT into account can result in catastrophic operational problems, while one that takes full advantage of advances in IT can deliver world-class service in all aspects of its operations.

IT can also help contain costs, a key consideration as plans compete in the marketplace and are required to keep administrative costs within ACA limits.

Just a few of the things IT can do for a health plan:

- Member services can quickly access the specific plan information needed to provide telephone support to members.
- Member enrollments and claims, provider reimbursements, and purchaser billing can all be handled more quickly and accurately.
- Medical outcomes can be tracked and used by medical management and providers to improve the quality of healthcare.
- The types of reports that purchasing groups (particularly large employers and government) often require as a condition for doing business can be generated.

Digital technology can positively impact health plans through four levers:⁸

- **Stronger connectivity.** Technology facilitates the transactions of individual consumers, making it easier for them to select the right product and engage more effectively with providers. IT also can link providers to research and data, so that they can better manage the health of plan members.
- **Greater operational efficiency.** Technology can be employed to change workflows, allowing not only improvements to current processes but the creation of new, more efficient processes.
- **Better decision-making through “big data.”** Technology makes access to and analysis of large amounts of data possible. In provider network management, this can support value-based pricing. It can also facilitate population health management, allowing for improved quality of care while reducing high-cost catastrophic incidents.
- **Increased innovation.** Technology is making new approaches to the delivery of healthcare possible, such as telemedicine, including virtual visits to the chronically ill.

While technology can spur innovation and facilitate competitiveness, it also presents a risk to a health plan and its members. Medical identities can be stolen through malware, leading to fraudulent payments, inappropriate treatment for members (based on inaccurate medical records), and substantial legal liability. Health plans must consider not only how to avoid cybersecurity incidents but also how to respond to them should they occur.

Legislation and Regulation

In strategic planning, a health plan must take legislation and regulation into account, both existing laws and rules and those that may be coming. New legislation can create opportunities. The Medicare Modernization Act opened up Medicare to greater participation by private-sector health plans, both in Part D prescription drug coverage and Medicare Advantage.⁹ The ACA made it possible for private plans to sell to individual consumers and small businesses through government-sponsored Marketplaces (exchanges).

But in deciding whether to pursue such opportunities, a health plan must carefully consider regulations. A plan seeking to enter the Medicare Advantage market must consider CMS's strict sales and marketing guidelines. To participate in the ACA Marketplaces, a plan must offer products that meet the ACA's strict standards. Strategic planners must judge whether an opportunity created by legislation is a good one for the plan—that is, whether the product and market is a good match for the plan's expertise and capabilities. For instance, participation in the ACA Marketplaces has been profitable for some health plans, especially those with previous experience serving needy populations. But other plans have seen losses and have withdrawn from this market.¹⁰

As of this writing, the legislative and regulatory environment is again in flux, as many are calling for repeal and replacement of the ACA. Health plans undertaking strategic planning will have to consider any changes that are made and how to navigate the transition to a new system. The ACA Marketplaces may no longer exist, or their rules may be substantially different. Some product offerings may no longer be appropriate, but new opportunities for private-sector plans to participate in government programs may be created.

Corporate Transactions

Many health plans use strategic partnerships, joint ventures, mergers, or acquisitions to achieve their strategic objectives. For instance, a health plan organization that sets a strategic goal to enter a new market might consider an alliance with another plan. This can be particularly advantageous if the new market differs significantly from the organization's existing markets in areas such as buying behavior, medical needs, administrative demands, and regulatory requirements.

Example: *Healthy Life Health Plan wants to enter the Medicare Advantage market. But strategic planners conclude that Healthy Life's provider networks and medical management functions are not adequate for a large elderly population, member services staff would require considerable training to effectively handle the types of issues and questions typical of a Medicare population, and the compliance area does not have enough familiarity with Medicare filing procedures to obtain the required regulatory approvals within the proposed timeframe.*

But Healthy Life does not abandon its plan. It decides to merge with or acquire another health plan already established in the Medicare Advantage market. In this way Healthy Life both gains the expertise it lacks and immediately acquires a Medicare Advantage book of business—"instant expertise" and "instant market share."

Another example involves a new product:

Example: *Blue Mountain Health Plan, a traditional HMO, wants to add an open access HMO to its product portfolio. Blue Mountain could develop this product internally, utilizing its existing resources. Or it could form a strategic partnership with a provider network, or merge with an organization that already has an open access HMO in its portfolio.*

The right alliance can enhance a health plan's ability to address other key strategic issues.

- A merger or acquisition can produce economies of scale that can make an organization more competitive.
- A strategic partnership can enable a health plan to offer more providers and more types of providers in its network
- A plan can improve certain areas (for instance, medical management or member services) by forming an alliance with a plan strong in these areas.

For some opportunities a health plan may be able to pursue through alliances, see below.

Potential Opportunities for Growth

Health plans seeking to move into new markets have many options. There are new geographic regions, large or small employer groups, and various government-sponsored programs. Some areas of potential:

- **Medicare.** Just as the huge Baby Boom generation is becoming eligible for Medicare, there is increasing legislative sentiment to further expand opportunities for private-sector health plans to offer Medicare coverage.
- **Medicaid.** States may be given further leeway in the administration of their Medicaid programs through Section 1332 waivers, and this may create more opportunities for participation by private plans.
- **Global markets.** The last several decades have brought a tremendous expansion of the middle class outside the United States. Health plans with international capabilities may wish to consider exploring opportunities in these global markets.

Putting It All Together

In this lesson we have discussed a number of key issues. The overall objective of the strategic planning process is to assemble these separate but interrelated issues into a cohesive and effective corporate strategy. Also, when the board and senior management consider one key issue, they often must consider other related issues. Some examples:

- A health plan's market strategy can be significantly influenced by its capabilities in certain functions such as medical management and member services.
- There are few operational issues that are not in some way impacted by information technology.

- Legislative and regulatory activities in recent years have affected many other aspects of a health plan's strategic planning, including targeted markets, product offerings, pricing, and provider networks.

Summary

As a health plan develops a strategic plan, it must take account of a wide variety of key issues: markets and product portfolio, pricing, financial management, provider networks, medical management, customer service, information technology, legislation and regulation, and corporate transactions (such as partnerships, joint ventures, mergers, and acquisitions). These issues must be considered both separately and in relation to one another. And they all must be put together to create a comprehensive and coherent strategic plan.

Sources

¹ Deloitte, "2016 Health Plan Industry Outlook: Interview with Greg Scott," *2016 Life Sciences and Health Care US Outlook*, <https://www2.deloitte.com/us/en/pages/life-sciences-and-health-care/articles/health-plans-outlook.html>.

² Lisa Smith, "Top Five Industry Challenges of 2015: What to Watch and What's Being Done about Them," *Managed Healthcare Executive*, December 8, 2014, <http://managedhealthcareexecutive.modernmedicine.com/managed-healthcare-executive/news/top-five-industry-challenges-2015>.

³ Maria Auxiliadores do Nascimento Melo and Denise Dumke de Medeiros, "A Model for Analyzing the Competitive Strategy of Health Plan Insurers Using a System of Competitive Intelligence," *The TOM Magazine* 19:3 (2007), pp. 206-216.

⁴ Akshay Kapur, Ashish Kaura, and Minoo Javanmardian, "Private Health Insurance Exchanges: Fueling the 'Consumerization' of Employer-Sponsored Health Insurance," *Strategy& (Formerly Booz & Company)*, March 26, 2012, <http://www.strategyand.pwc.com/reports/emergence-private-health-insurance-exchanges>.

⁵ Paul Keckley, Bill Copeland, and Greg Scott, "The Future of Health Care Insurance: What's Ahead," *Deloitte Review*, July 24, 2013, pp. 128-129, https://dupress.deloitte.com/content/dam/dup-us-en/articles/the-future-of-health-care-insurance-whats-ahead/DR13_the_future_of_health_care_insurance_whats_ahead3.pdf.

⁶ Michael E. Porter, "A Strategy for Health Care Reform: Toward a Value-Based System," *The New England Journal of Medicine*, July 9, 2009, pp. 109-112, <http://www.nejm.org/doi/full/10.1056/NEJMp0904131#t=article>.

⁷ Adapted from Keith Fengler, Jaime Estupinan, and Kenny Chan, "What Consumers Most Want from Health Insurers' Technology," *Strategy + Business*, June 29, 2016, <http://www.strategy-business.com/article/What-Consumers-Most-Want-from-Health-Insurers-Technology?gko=54657>.

⁸ Basel Kayyali, Steve Kelly, and Madhu Pawar, "Why Digital Transformation Should Be a Strategic Priority for Health Insurers," *McKinsey & Company*, May 2016, <http://www.mckinsey.com/industries/healthcare-systems-and-services/our-insights/why-digital-transformation-should-be-a-strategic-priority-for-health-insurers>.

⁹ Thomas R. Oliver, Philip R. Lee, and Helene L. Lipton, "A Political History of Medicare and Prescription Drug Coverage," *The Milbank Quarterly*, June 2004, pp. 283-354. <https://www.ncbi.nlm.nih.gov/pmc/articles/PMC2690175/>

¹⁰ Catherine Murphy-Barron, Craig Kelzur, Jill Van Den Bos, Margaret Chance, and Paul Houchens, "Ten Critical Considerations for Health Insurance Plans Evaluating Participation in Public Exchange Markets," *Milliman Healthcare Reform Briefing Paper*, December 2012, <http://us.milliman.com/uploadedFiles/insight/healthreform/ten-critical-considerations.pdf>

25 Course Summary

Lesson Objective

The objective of this final lesson is to review the content of the 24 lessons of the course. After completing this lesson, you should be better able to describe, explain, and discuss the many aspects of health plans. In this lesson we briefly summarize the main points of each of the lessons. We also offer a practice exam testing your knowledge of the material in the course. To prepare for the final exam, we encourage you to review each lesson separately and try to answer all the review questions in each lesson.

The Health Plan Environment (Lesson 1)

Health insurance plans operate and compete in an environment of external forces. This environment is shaped and changed by a variety of factors, including legislation and regulation (influenced by politics and the courts), the overall economy, the health plan marketplace, demographics, consumer expectations, and innovation of various sorts. The major players in this environment are consumers, purchasers (employers and government programs that buy health coverage), providers of healthcare services, and payors (health plans, government, and others who pay providers). Other participants and stakeholders include local communities, uninsured or underserved populations, vendors that provide services to plans, academic medical centers, patient advocacy groups, and the federal and state governments.

Health Plan Organization (Lessons 2, 3, and 4)

Health plans are usually legally organized as corporations. These include stock companies, which are owned by shareholders and operated for their benefit, and mutual insurance companies, which are owned by their policyholders. A health plan may be a not-for-profit organization, which must be operated for the benefit of the organization, the community, or a charity, but not private individuals. Not-for-profit health plans may enjoy tax advantages, but they may also find it more difficult than for-profit plans to raise money for expansion and improvements. The entities that health plans work with (providers and others) may be legally organized as corporations, sole proprietorships, or partnerships of different kinds. These include limited liability companies (LLCs), which combine features of a partnership and a corporation, and professional corporations, the owners, directors, and officers of which must be licensed professionals.

Health plans can improve their performance and meet strategic goals by restructuring. They can create subsidiaries, spin off units into subsidiaries or independent companies, set up holding companies to own and control several related companies, change from a mutual insurer to a stock companies or vice versa, convert from a not-for-profit organization to for-profit corporation, and reengineer the structure and processes of the organization. Health plans can also collaborate or combine with other health plans or organizations. They join in strategic partnerships in particular areas or form joint ventures that they both own and control. They can acquire or merge with other organizations.

There are many types of health plans, as well as different healthcare organizations working with health plans. Health plans can be structured in different ways and combined into an integrated delivery system (IDS). Third-party administrators (TPAs) and utilization review organizations (UROs) provide services to plans. Healthcare providers form organizations of various sorts to offer care to consumers and contract with health plans. Some companies provide support services to providers and provider organizations.

Health plans are commonly sponsored by insurance companies, investor groups, other health plans, providers, and employers; government, medical schools and universities, consumer groups, and labor unions may also play a role. Providers can be compensated by health plans and other payors a variety of ways. The tradition system is fee-for-service, and this remains predominant. Pay-for-performance, care coordination, shared saving, and bundled payments can be added to and supplement an FFS system. Global payments actually replace fee-for-service.

Health plans and other organizations may integrate. Business entities in the same market segment may join together in horizontal integration, those in different segments of the same industry in vertical integration, and those in unrelated industries in conglomerate integration. Structural integration involves mergers, acquisitions, and common ownership and control; virtual integration does not, simply two entities working closely together. An employer may sponsor a self-funded health plan for financial reasons and to gain more flexibility in plan design. An employer may contract directly with providers to serve its employees. Multiple employers may join forces in a purchasing alliance to negotiate with health plans. An employer may provide coverage to employees through a private exchange.

Laws and Regulators (Lessons 5 and 6)

A wide variety of laws affect health plans. These include case law (common law), statutes, and administrative rules and regulations, at both the state and the federal level. They include model laws developed by the National Association of Insurance Commissioners (NAIC) and adopted, in whole or in part, by many states. A few of the most important federal laws affecting health plans:

- The **Health Maintenance Organization Act of 1973** sets requirements and grants advantages to HMOs that choose to become federally qualified.
- The **Employee Retirement Income Security Act of 1974 (ERISA)** sets standards for most employment-based health benefit plans, including self-insured plans.
- The **Americans with Disabilities Act (ADA)** seeks to ensure the rights of people with disabilities.
- The **Health Insurance Portability and Accountability Act of 1996 (HIPAA)** addresses many aspects of health coverage, including access to coverage and the privacy and security of health information.
- The **Medicare Prescription Drug, Improvement, and Modernization Act of 2003 (MMA)** further expanded health plan participation in Medicare and established Medicare Part D (prescription drug benefits).
- The **Deficit Reduction Act of 2005 (DRA)** made a number of changes in the rules for health insurance plans participating in Medicaid and Medicare.
- The **Genetic Information Nondiscrimination Act of 2008 (GINA)** prohibits insurers and employers from discriminating against individuals based on genetic information.
- The **Patient Protection and Affordable Care Act of 2010 (Affordable Care Act, or ACA)** contains a great many provisions that seek to ensure that everyone can access and afford health coverage and that coverage meets people's needs.

Regulators of health plans are concerned with quality of care, access to care, financial stability and solvency, licensure, benefits, enrollment, sales and marketing, provider contracting, premiums and rating practices, management and organizational structure, reporting and filing, and confidentiality. At the state level, the main agency governing health plans is the insurance department, but other entities (such as the health department and the attorney general's office) regulate some aspects of plans. At the federal level, health plans that provide coverage to Medicare and/or Medicaid beneficiaries are supervised by the Centers for Medicare and Medicaid Services and its divisions. Plans that offer coverage under the Federal Employees Health Benefits Program are regulated by the Office of Personnel Management. Many other federal agencies have an impact on various aspects and operations of health plans.

State Regulation (Lessons 7, 8, and 9)

The extent and focus of state regulation vary according to the type of organization:

- Health maintenance organizations (HMOs) must meet various requirements in financial matters, healthcare delivery, filing and reporting, and disclosure.
- Preferred provider organizations bearing financial risk must meet financial standards; non-risk PPOs are often regulated as downstream contractors of insurers or employer-sponsored health plans. Exclusive provider organizations (EPOs) are regulated much like PPOs.
- The regulation of point-of-service (POS) plans differs for standalone and wrap-around products.
- The activities of TPAs and entities performing utilization review are also subject to regulation.

States have laws requiring health plans to provide coverage for certain healthcare services or medical conditions. Self-funded employer plans and government plans are exempt from state mandates but subject to those of federal laws; most plans are subject to both state and federal mandates. States have laws regulating the relationship between a health plan and providers. Any willing provider (AWP) laws give any provider who meets a health plan's terms and conditions the right to serve its members. Direct access laws give plan members the right to see certain specialists without a referral. Various laws require or prohibit certain provisions in the contract between a health plan and a provider.

Other state laws affect health plans. These include general business laws, healthcare laws, and insurance laws, as well as laws that specifically target health plans or even certain types of health plans. These laws regulate many aspect of health plan operations:

- Sales and marketing, including licensing of sales representatives, advertising, and trade practices.
- Small group and individual health coverage, including benefits, rating, and underwriting.
- Antitrust issues, readability, reporting of data, privacy, claims, coordination of benefits, antifraud efforts, continuation of coverage, conversion, and state governments as purchasers of health care.

Workers' Compensation (Lesson 10)

States generally require employers to provide workers' compensation benefits for their employees. These benefits cover work-related injuries and illnesses, and they pay for medical expenses, lost earnings, and rehabilitation. An employer may provide workers' compensation by purchasing private insurance, participating in a state program (which may be optional or mandatory), or by self-insuring (self-funding) its own program. Workers' compensation programs (unlike employer-sponsored health plans) do not charge employees premiums or cost-sharing.

Pharmacy Benefits (Lessons 11 and 12)

Pharmacy benefit managers (PBMs) administer prescription drug benefits for health plans and employers, and today they handle the large majority of such benefits. Various state laws regulate PBMs:

- Under any willing provider (AWP) laws, PBMs must accept into their networks any pharmacy willing to accept their terms and conditions. Under freedom of choice (FOC) laws, a PBM must reimburse a pharmacy for services rendered, even if the pharmacy is not in the PBM's network.
- The substitution of generics for brand-name drugs and biosimilars for biologic drugs are subject to laws that vary considerably from state to state.
- Some state laws impose requirements on PBMs regarding the prices they pay pharmacies for generic drugs.

The manufacture of drugs is regulated by the federal Food and Drug Administration (FDA). The dispensing of drugs by pharmacies is largely regulated by the states, which have standards and requirements in several areas. Internet pharmacies are regulated by the states, but state regulation has not been effective, and in response the federal government has taken a number of actions. Compounding pharmacies must be licensed by the state, and they have the option of being registered with the FDA.

Market Conduct Exams (Lesson 13)

A market conduct examination (MCE) is an investigation or audit by state insurance regulators of the practices and operations of an insurer in the consumer marketplace. The purpose is to establish that the insurer is conducting its business fairly and responsibly and in compliance with the state's laws and regulations. An MCE may look at various business functions, and regulators may take a range of actions in response to a variety of triggers. The findings in an MCE final report may be the basis of disciplinary action and/or a compliance plan.

Federal Regulation (Lessons 14, 15, and 16)

The federal Employee Retirement Income Security Act (ERISA) regulates employee benefit plans, including health plans. Both fully insured and self-funded plans are covered by ERISA, but they are treated somewhat differently. ERISA preempts state laws that at least somewhat directly affect employee benefit plans. Therefore, ERISA determines what state laws can be applied to employer-sponsored health plans and what legal challenges can be made to their administration. In much of the litigation involving ERISA plans, the key issue is whether the state law or laws on which a civil action is based are preempted by ERISA. If the law addresses what is considered an integral part of the administration of ERISA plans, it will generally be preempted.

Three federal laws affect wellness programs:

- HIPAA nondiscrimination rules, as modified by the ACA, apply to wellness programs that are part of group health plans or considered group health plans themselves. Many of these programs offer incentives to employees to participate in certain activities or achieve specified health standards (such as cholesterol levels below a threshold). The rules seek primarily to ensure that these incentives do not unfairly disadvantage individuals with health problems.
- The Genetic Information Nondiscrimination Act (GINA) includes rules on requests by wellness programs for genetic information of employees or their spouses, usually as part of a health risk assessment.
- The Americans with Disabilities Act (ADA) requires wellness programs to make reasonable accommodation if a disabled person is unable to meet a standard or engage in an activity to earn an incentive. It also establishes rules for programs making disability-related inquiries about employees or requesting medical examinations.

Federal Programs (Lessons 17, 18, 19, and 20)

Medicare is a federal program that provides health benefits to individuals 65 and older; those with severe, long-term disabilities; and a few with certain diseases. Medicare consists of four parts:

- Medicare Part A—coverage of hospital care, hospice, and nursing facility and home care for those being treated for an acute illness.
- Medicare Part B—coverage of physician services, outpatient care, and many other medical services.
- Medicare Part C (Medicare Advantage)—a program under which private-sector health plans provide Medicare coverage.
- Medicare Part D—prescription drug coverage.

Beneficiaries must pay significant deductibles, coinsurance, and copayments, and many buy private Medicare supplement (Medigap) insurance to cover some of these costs for Parts A and B. Those eligible for Part A and Part B generally have the option to enroll in a Medicare Advantage plan. MA plans must provide at least the coverage of Parts A and B, and some offer additional benefits. There are several types of MA plans, including HMOs, PPOs, private fee-for-service plans, and others.

Medicaid is a federal-state program that provides health coverage to some poor people. For those who qualify, coverage is extensive, and cost-sharing is generally minimal. Those eligible include:

- people in certain categories, such as children, pregnant women, some very low-income parents, and elderly and disabled persons;
- in states that have adopted the ACA's Medicaid expansion, all working-age adults with income below 133 percent of the federal poverty level; and

- in some states, the medically needy—people who do not meet the requirements for a Medicaid category but have high medical expenses.

Medicaid provides coverage of long-term care (nursing home or home health care for those with chronic physical or cognitive impairments), but this differs from Medicaid medical coverage in important ways. Middle-income people can qualify, but they must spend their assets on their care until they have little left, and while on Medicaid they must spend most of their income on care. Medicaid has traditionally focused on facility care, but there has been a major shift to home and community-based services (HCBS).

The Children's Health Insurance Program (CHIP) provides health coverage to children in families with income too high to qualify for Medicaid but often too low to afford private health insurance. Basic Health Programs (BHPs) are intended for those with income somewhat above Medicaid eligibility levels and are intended to give low-wage earners continuity of coverage and of relationships with their healthcare providers.

Fraud, Waste, and Abuse (Lesson 21)

Fraud, waste, and abuse make healthcare more expensive for everyone and can diminish its quality. When providers are compensated on a fee-for-service basis, false claims, billing schemes (such as upcoding), and overutilization occur. Under capitation and similar compensation arrangements, underutilization is more common. A number of federal laws can be deployed against healthcare fraud and provide for penalties against perpetrators—these include HIPAA, the False Claims Act, the Anti-Kickback Statute, and others. Health insurance plans work to combat FWA through special investigative units, technology, and compliance programs.

Governance, Accountability, Compliance, and Strategic Planning (Lessons 22, 23, and 24)

The governance of a health plan is the responsibility of its board of directors. The board establishes a mission, sets goals, develops a strategic plan, hires a CEO and other officers, and provides guidance and oversight. Many health plans are adopting enterprise risk management (ERM) in their governance, and new NAIC model laws require this for state-regulated insurers.

Accountability occurs when parties willingly submit to a process by which they are required to justify their actions and policies to other parties. Enterprise risk management combines accountability and risk and emphasizes cooperation among departments to manage an organization's full range of risks. ERM deploys different models of accountability as appropriate, in a rational way. The demand for accountability and ERM is driving leadership and decision-making in health plans today.

To prevent, detect, and address conduct by a health plan and its agents that violates legal and ethical requirements, many plans have established compliance programs. A compliance program includes a variety of activities, such as ongoing monitoring and audits, investigations, anonymous reporting, disciplinary action, and training. Compliance risk management (CRM) combines risk management and effective compliance.

Strategic planning identifies an organization's long-term objectives and the broad, overall courses of action that it will take to achieve those objectives.

- A mission statement sums up an organization's reason for existing and fundamental purpose.

- A vision statement is an ideal that the organization would like to achieve and is intended to inspire and motivate employees.
- A situation analysis includes an environmental analysis, an environmental forecast, and an internal assessment.
- Corporate objectives state the long-term results that the organization plans to achieve in carrying out its mission.
- Corporate strategies are long-term methods that will be used to achieve corporate objectives.
- A tactical plan involves shorter-term sub-goals of corporate objectives in each of the functional areas of the health plan.

A health plan's strategic plan must take account of a wide variety of key issues: markets and product portfolio, pricing, financial management, provider networks, medical management, customer service, information technology, legislation and regulation, and corporate transactions (such as partnerships, joint ventures, mergers, and acquisitions).